TRANSFORMING AMERICAN HEALTHCARE

A Six-Pillar Framework for System Transformation

Health Transformation Review

April 2026

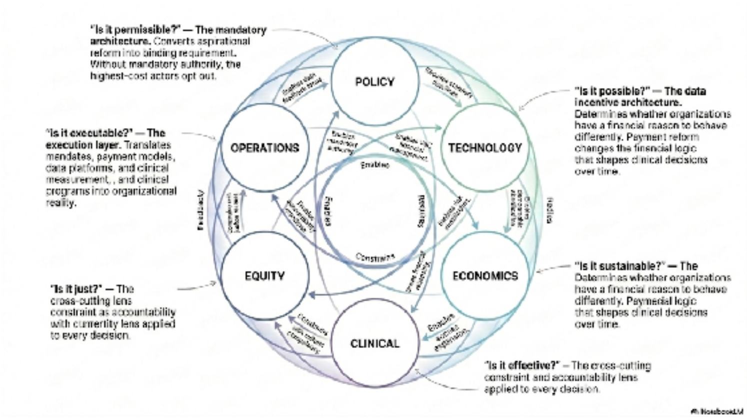


Table of Contents
Page numbers update automatically. To refresh in Google Docs: click the table of contents and click the refresh icon.

Page Left Intentionally Empty

PREFACE

Why This Book Exists

American healthcare is the most expensive, most administratively complex, and most inequitably distributed healthcare system in the developed world. This is not a new observation. It has been documented, analyzed, debated, and lamented for decades. What is new — or more precisely, what is newly urgent — is that the system is now visibly breaking under pressures that cannot be managed through incremental adjustment. The financial logic of fee-for-service payment, the demographic reality of an aging population, the exhaustion of a clinical workforce buried in administrative burden, and the structural inadequacy of a hospital network designed for a different century are converging into a crisis that requires systematic response, not marginal reform.

This book is a response to that crisis. It is designed for the healthcare leaders, policymakers, analysts, and students who must understand not just what is wrong with American healthcare but what a transformed system would look like, how to get there, and what the organizations executing that transformation need to know to succeed. It is simultaneously an academic text — grounded in evidence, rigorous in its analytical framework, honest about what is uncertain — and a practitioner's guide — organized around decisions, investments, contracts, and programs that real organizations must make in real time.

The book is organized around the six-pillar framework: Policy, Economics, Operations, Clinical, Equity, and Technology. This framework emerged from the conviction that healthcare transformation fails when it addresses any single pillar in isolation. Payment reform without clinical redesign produces financial savings that evaporate when care models do not change. Clinical quality improvement without payment reform produces better care that the financial system immediately discourages. Technology investment without operational readiness produces platforms that no one uses. Equity goals without structural SDOH investment produce aspirations without outcomes. The six pillars must move together — which means the people responsible for each pillar must understand all the others.

Vermont runs through this book as its primary teaching case. This requires explanation, because Vermont is a small state — 647,000 people, 14 hospitals, roughly the population of a mid-sized American city — and its direct policy relevance might seem limited. Vermont is in this book not because Vermont is large or economically powerful, but because Vermont is early. Early in its demographic aging. Early in its hospital financial fragility. Early in its recognition that voluntary payment reform cannot produce the structural change a system in crisis requires. And early in its willingness to legislate the structural responses — Acts 167 and 68, the AHEAD Model participation, the Rural Health Transformation Program — that other states will reach for when their own systems arrive at Vermont's current condition.

Vermont is also unusually transparent. The combination of a small state's intimacy, a strong public-sector analytical tradition, and an active regulatory body in the Green Mountain Care Board has produced a public record of healthcare system analysis — the Oliver Wyman Act 167 report, the GMCB's monthly hospital data, the AHS transformation reports, the AHEAD State Agreement — that gives this book a documentary foundation most states cannot match. Vermont's healthcare transformation is happening in public, with detailed data, and the lessons are directly transferable.

A word about what this book is not. It is not a policy brief for or against any particular political agenda. Healthcare transformation is contested terrain, and this book presents the evidence and the analytical frameworks that inform decisions without pretending those decisions are purely technical. The five goals of Vermont's Act 167 — reduce inefficiencies, lower costs, improve health outcomes, reduce health inequities, and increase access to essential services — are goals that virtually every person across the political spectrum would endorse. The means for achieving them are legitimately contested, and this book engages that contestation honestly.

It is also not a prediction. The transformations described in this book are underway but unfinished. The outcome of Vermont's experiment — whether the hospital system stabilizes or contracts, whether global budgets reduce costs or constrain access, whether the Agency of Human Services can execute the restructuring its mandate requires — is genuinely uncertain. This book provides the analytical framework for understanding what is happening and why, and for contributing to better decisions as the story unfolds. The decisions, and their consequences, belong to the people who make them.

April 2026

INTRODUCTION

The Transformation Imperative

A System at the Breaking Point

On September 18, 2024, the Green Mountain Care Board of Vermont convened a public meeting at which Oliver Wyman Healthcare and Life Sciences Group presented the findings of a year-long, $1 million statewide hospital systems analysis commissioned under Act 167 of 2022\. The room was full. The findings were not surprising to anyone who had been paying attention — but they were stated with a precision and a comprehensiveness that made them impossible to dismiss.

Nine of Vermont's fourteen hospitals were reporting operating losses in 2023, with the worst reaching negative 8.9%. If current trends continued, thirteen of the fourteen would be reporting losses by 2028\. To simply break even, the hospital system must overcome a five-year deficit ranging from a best-case estimate of 700 million to a more realistic projection of 2.4 billion, driven by skyrocketing labor and operating costs. Average premiums for silver exchange plans had increased 108% in six years, while median household income had grown 22%. Out-of-pocket maximums had more than doubled in five years. The working-age population was declining; the over-65 population was growing toward 30% of the state's total by 2040\.

The Oliver Wyman consultants offered a three-imperative summary of what Vermont needed to do: build housing and fix transportation; pay hospitals with reference-based pricing and move to global budgets; and move all care possible out of hospitals. These were not incremental recommendations. They were structural prescriptions for a system that, absent structural change, was heading toward collapse.

Vermont is not an outlier. It is a preview.

The structural forces producing Vermont's crisis — demographic aging, hospital financial fragility, the unsustainability of commercial cross-subsidy, the failure of voluntary payment reform, the exhaustion of a workforce buried in administrative complexity — are present to varying degrees in every American state. Vermont faces them earlier and more acutely because of its demographics and its economic structure. But the states of the Northeast, the rural Midwest, and eventually the national system will arrive at Vermont's current condition on a lag of ten to twenty years. What Vermont is forced to solve now, the rest of the country will be forced to solve later.

This book is written in that context. It is not a retrospective analysis of a completed transformation. It is a real-time account of a transformation in progress, with all the uncertainty, difficulty, and instructiveness that implies.

What Transformation Actually Means

The word 'transformation' is used so frequently in healthcare that it has nearly lost its meaning. Press releases describe routine process improvements as transformational. Strategic plans invoke transformation as aspiration rather than commitment. This book uses the word in its precise sense: transformation is structural change that alters the fundamental incentives, relationships, and capabilities of a system, producing different behaviors and different outcomes rather than incremental improvements within the existing structure.

By this definition, most of what has been called healthcare transformation over the past fifteen years has not been transformational. Voluntary accountable care organization programs that high-cost providers can decline to join are not transformational. Shared savings models that reward hospitals for reductions in utilization they would have achieved anyway are not transformation. Electronic health record adoption that digitizes paper processes without changing care delivery is not transformation. Quality measurement initiatives that produce reports without affecting payment are not transformation.

Transformation, in the sense this book uses the term, looks like what Vermont is attempting: mandatory reference-based pricing that eliminates the ability of dominant hospitals to charge commercial payers three to four times Medicare rates; statutory global hospital budgets that change every hospital's financial incentive from maximizing volume to managing population health; a Statewide Health Care Delivery Strategic Plan that commits Vermont's Agency of Human Services to specific, measurable outcomes by specific dates; and capital investment through the Rural Health Transformation Program that builds the community infrastructure that the payment reform requires to produce population health improvement rather than just cost reduction.

Whether this constitutes successful transformation will be determined by what happens over the next five to ten years. What is already clear is that Vermont has moved further toward genuine structural change than any comparably sized system in recent American health policy history.

The Six-Pillar Framework: Why All Six Must Move Together

The central analytical contribution of this book is the six-pillar framework for healthcare transformation: Policy, Economics, Operations, Clinical, Equity, and Technology. The framework is not a checklist of things that should be done. It is an integrated analytical architecture for understanding why transformation attempts fail when they address any pillar in isolation.

The most common failure mode in healthcare transformation is single-pillar thinking. A payer that focuses exclusively on the Economics pillar — redesigning payment models — while leaving the Technology, Clinical, and Operations pillars unchanged produces payment reform that providers cannot operationalize. Hospitals without population health analytics infrastructure cannot manage global budgets. Clinicians without team-based care models cannot deliver the primary care access that makes reduced hospitalization possible. Administrators without administrative simplification cannot absorb the management complexity of value-based contracts on top of fee-for-service billing requirements.

Conversely, a technology vendor that focuses exclusively on the Technology pillar — building sophisticated health information exchange and population health platforms — while leaving the Economics and Operations pillars unchanged builds tools that no one has the financial incentive or organizational capacity to use. Vermont's Oliver Wyman analysis found this pattern explicitly: VITL (Vermont's health information exchange) exists but is not widely used, OneCare (Vermont's ACO) exists but participation is voluntary and incomplete, and the 'all-payer' system does not actually include all payers. The infrastructure exists; the incentives and operations to use it effectively do not.

The six pillars interact through a logic that this book develops in detail in Chapter 1\. The short version is this: payment reform (Economics pillar) changes what behaviors the financial system rewards; clinical redesign (Clinical pillar) changes how care is actually delivered; technology investment (Technology pillar) provides the data infrastructure that makes new care models manageable at scale; policy and regulatory change (Policy pillar) creates the mandatory architecture that ensures voluntary actors cannot opt out of structural reform; equity investment (Equity pillar) ensures that transformation benefits reach the populations that need them most and does not concentrate gains among those already well-served; and operational execution (Operations pillar) converts strategy into implemented programs, trained staff, and measured outcomes.

Remove any one pillar and the transformation fails. Vermont's reform agenda is instructive precisely because Acts 167 and 68, the AHEAD Model, and the Rural Health Transformation Program together address all six pillars simultaneously — imperfectly and incompletely, as any real-world policy effort does, but with a comprehensiveness that distinguishes Vermont's approach from single-pillar interventions.

A Note to the Reader — How to Use This Book

The Four Reader Profiles

This book has four primary reader profiles. Each has a different relationship to the material and a different way of getting maximum value from the book.

The Healthcare Policy Professional

You work in a government agency, a legislative office, a think tank, or a foundation focused on healthcare policy. You have substantive policy knowledge but may not have deep familiarity with the operational mechanics of healthcare transformation — what it actually takes to implement a global budget, build a community health team, or manage a 14-hospital transformation planning process.

Recommended reading path: Read the Preface and Introduction in full, then Chapter 1 (six-pillar framework) and Chapter 2 (The Execution Sequence), with Appendix A for the full Vermont system portrait. For each pillar you care most about, read the corresponding chapter in depth. Read Chapter 16 (AHS Restructuring) carefully — it is the most direct bridge between policy mandate and organizational design. Read Chapter 15 (future forecast) for the national context.

The Healthcare Executive or Administrator

You lead a hospital, health system, health plan, ACO, or healthcare services organization. You are making decisions — about contracts, capital investments, workforce, technology — that the transformation environment is complicating. You need the analytical framework but you especially need the operational implications.

Recommended reading path: Read the Introduction for the overarching argument, then go directly to the chapters most relevant to your immediate decisions. If you are navigating a VBC contract, Chapter 6 (Economics) is your entry point. If you are building care management programs, Chapter 10 (Clinical). If you are evaluating technology investments, Chapters 12 and 13 (Technology). Read Chapters 7 and 8 (Operations) for the hospital transformation planning framework. Return to Chapters 1-3 for the integrating logic.

The Vermont Practitioner

You work for AHS, GMCB, DVHA, the Vermont legislature, a Vermont hospital, a Blueprint practice, or a Vermont community organization. You are living inside the transformation this book describes. You need the national analytical context but you especially need the Vermont-specific operational guidance.

Recommended reading path: Read Appendix A (Vermont System Portrait) for system context, then Chapter 2 (The Execution Sequence) and Chapter 3 (Policy — Acts 167 and 68) first, since these are the chapters most directly grounded in Vermont's current situation. Then read Chapter 16 (AHS Restructuring) as the operational framework for the work you are doing. Use the implementation timeline tables throughout Part II as a reference guide to where Vermont is in each pillar's trajectory. The Vermont in Practice sidebars throughout are written for you specifically.

The Healthcare Student or Researcher

You are building foundational knowledge of healthcare transformation, either in a graduate program or as an independent researcher. You need the analytical architecture, the evidence base, and the theoretical frameworks, not just the Vermont case study.

Recommended reading path: Read the entire book in chapter order. Chapter 1 establishes the framework; Chapters 3-13 develop each pillar with full academic grounding before the Vermont application; Chapters 14-16 provide the advisory infrastructure, future context, and AHS restructuring roadmap. The Key Concepts sections at the end of each chapter are the glossary; the source notes are the bibliography. Use Vermont as the case study while developing transferable analytical skills.

The Vermont Thread: How It Runs Through the Book

Vermont appears in this book in four distinct registers, and recognizing which register you are reading helps calibrate how to use the material.

- Vermont as diagnostic case: The Oliver Wyman analysis of Vermont's hospital financial crisis, the GMCB price data, and the AHS demographic projections establish Vermont as an instance of a generalizable pattern of rural healthcare system stress. When these appear, they are teaching the pattern, not just Vermont's version of it.
- Vermont as legislative laboratory: Acts 167, 51, 68, and 62 are specific legislative interventions in a specific state, but their design logic — the reform cascade structure, the mandatory versus voluntary distinction, the statutory accountability metrics — is generalizable to any state undertaking comparable reform.
- Vermont as operational field report: The AHS transformation reports, the RHRC engagement methodology, the 14-hospital planning process, and the Health Care Delivery Advisory Committee's work are real-time accounts of a transformation in progress. They convey the texture of execution — the difficulties, the adaptations, the gap between strategy and implementation — that no policy analysis can substitute for.
- Vermont as national template: Chapter 16 (AHS Restructuring) and Chapter 15 (The Future) explicitly use Vermont as a template for the national condition. Here the argument is prospective: if Vermont succeeds, this is what success looks like; if Vermont faces these organizational design challenges, every state will face them; if Vermont's approach to the AHS restructuring mandate works, other states can adapt it.

The HTR Platform and This Book

This book is part of the transformation knowledge ecosystem. The technical infrastructure framework — its intelligence feed, Research Lab tools, Academy curriculum, and Advisory services — provides the operational complement to the analytical framework the book establishes. Understanding the relationship between the book and the platform helps readers get maximum value from both.

The book provides the analytical framework: the six-pillar architecture, the evidence base for each pillar's mechanisms, the Vermont case study, and the conceptual vocabulary for understanding transformation. The HTR platform provides the operational tools: the APM Shared Savings Calculator that applies the Economics pillar's financial modeling to specific contracts; the VBC Transformation Readiness Assessment that diagnoses an organization's readiness across 30 dimensions in all six pillar domains; the Health Equity Studio that operationalizes the Equity pillar's disparity analysis and HEROI scoring for specific patient populations; the Hospital Financial Stress Test that models an organization's position under RBP and global budget scenarios; and the AI Analyst that retrieves and synthesizes the framework intelligence on any topic in real time, available at /chat or as the right-sidebar widget throughout the platform.

The Advisory services described in Chapter 14 are the institutional bridge between the book's frameworks and specific organizational decisions. Every concept in this book exists at the intersection of understanding and doing — grasping the framework clearly enough to explain it, and translating the framework into a specific decision, program, contract, or investment in a specific organizational context. The book handles the first challenge; the Advisory services handle the second.

A Note on the AHS Restructuring Chapter

Chapter 16 — The AHS Restructuring Roadmap — occupies a unique position in this book. It addresses the Statutory Mandate embedded in Act 68, conducts an Organizational Diagnosis of why AHS must restructure rather than merely plan, and provides a six-pillar framework for the 2028 Statewide Health Care Delivery Strategic Plan. It is written for the AHS staff, Health Care Delivery Advisory Committee members, Vermont legislators, and hospital leaders who must execute that mandate.

If you are a Vermont practitioner, Chapter 16 is written for you directly. It is designed to be usable as a working document — a framework you can bring to a planning meeting, a governance structure you can adapt for your organization, a metrics framework you can reference when building the Strategic Plan's accountability architecture.

If you are not a Vermont practitioner, Chapter 16 is still important reading, because the organizational design questions it addresses — how does a state agency restructure from program administration to population health system operation, how do you align a multi-program organization around geographic service areas, how do you build the analytics capacity for strategic decision-making when that capacity does not currently exist — are questions every state will face. Vermont's answers may not be directly transferable, but the analytical framework for thinking through the answers is.

What This Book Cannot Do

Intellectual honesty requires stating what this book cannot do, alongside what it can.

This book cannot predict whether Vermont's transformation will succeed. The statutory mandates are in place. The capital is flowing. The organizational machinery is in motion. But the outcome depends on thousands of decisions that will be made by hospital executives, primary care physicians, community health workers, legislators, regulators, and patients over the next five to ten years. This book provides the analytical framework for those decisions; it cannot make them.

This book cannot substitute for the operational expertise required to execute transformation. The frameworks in this book are accurate and the evidence base is solid, but implementing a global budget requires actuarial expertise, regulatory knowledge, and operational experience that a book cannot provide. That is what Advisory services, technical assistance organizations, and experienced practitioners are for.

This book cannot resolve the political contests that surround healthcare transformation. Reference-based pricing is opposed by hospital systems that benefit from high prices. Global budgets are feared by hospitals that worry about access restrictions. AHS restructuring is resisted by programs that do not want to be reorganized. These are legitimate conflicts of interest, and they will not be resolved by analytical frameworks. They will be resolved by political processes — legislative deliberation, regulatory proceedings, stakeholder negotiations — that this book can inform but cannot replace.

What this book can do: provide the analytical architecture for understanding what is happening and why; supply the evidence base for evaluating what is working and what is not; develop the conceptual vocabulary for communicating clearly across the disciplines — clinical, financial, policy, technological, operational — that transformation requires; and give Vermont practitioners the framework to see their specific work in the context of a coherent transformation agenda. That is enough to be worth writing, and worth reading.

Sources for Chapters 1-3: Oliver Wyman Healthcare and Life Sciences Group, Act 167 Community Engagement: Recommendations (August 2024); Vermont Agency of Human Services, Vermont Rural Health Transformation Program Application (November 2025); Vermont RHT Program Activity Details (December 2025); GMCB VHCURES Data and Analytics documentation; KFF State Health Facts (Vermont); Vermont Department of Health; Act 68 of 2025; Act 167 of 2022; AHEAD State Agreement (January 2025); all sources cited in subsequent chapters.

Chapter 1: The Six-Pillar Framework

The Six-Pillar Framework

Why Transformation Fails When Any Pillar Is Neglected, and How All Six Must Move Together

Chapter 1 establishes the six-pillar framework that organizes this entire book. It answers three questions: what each pillar is, how the pillars interact with each other, and why transformation fails when any pillar is neglected. It also introduces the Vermont thread that runs through every subsequent chapter — establishing Vermont's system in broad strokes before Chapters 2 and 3 develop the detail.

```
                  THE SIX-PILLAR FRAMEWORK FOR HEALTHCARE TRANSFORMATION

        1. POLICY                                  2. ECONOMICS
       "Is it                                     "Is it
        permissible?"                              sustainable?"

       Acts 167 & 68                              RBP + Global
       AHEAD Agreement                            Budgets + AHEAD

                                 SYSTEM
                                OUTCOMES
      6. OPERATIONS  ◄─────────              ─────────►  3. TECHNOLOGY
      "Is it                    All Six                  "Is it
       executable?"             Required                  possible?"

      RHRC · CIN                                         VHCURES·FHIR
      AHS HSA Model                                      Analytics

       5. EQUITY                                  4. CLINICAL
      "Is it just?"                              "Is it
                                                  effective?"
      SDOH · HEROI                               Blueprint · CoCM
      Rural Access                               CCBHC · PCMH

  Dependency types:  ──── enables   ──── drives   ---- constrains   ···requires
```

Figure 1.1 — The Six-Pillar Framework: six interdependent domains, each answering a critical diagnostic question, all required for system transformation. Interactive version available at htrintelligence.com/framework. Source: six-pillar framework documentation; Vermont Health Transformation / HTR (2026).

The Problem with Single-Pillar Thinking

In 2010, the Affordable Care Act created the Center for Medicare and Medicaid Innovation with a mandate to test new payment and delivery models and scale what worked. Over the following fifteen years, CMMI launched more than fifty models, investing billions of dollars and the attention of thousands of healthcare organizations. The results were modest. A 2023 evaluation found that the vast majority of CMMI models had not achieved statistically significant reductions in spending or improvements in quality. Most were discontinued. A handful showed promise and were expanded.

The pattern of failure across CMMI models is instructive. Models that addressed only the Economics pillar — changing what Medicare paid without addressing how care was delivered or whether providers had the infrastructure to respond — generated financial pressure without behavioral change. Models that addressed only the Clinical pillar — redesigning care coordination or care management programs — produced clinical improvements that the payment system then penalized by reducing volume-based revenue. Models that required Technology pillar investment — electronic health records, population health platforms, data exchange — found that providers lacked the operational capacity to use the tools effectively.

The lesson is not that payment reform, clinical redesign, or technology investment is wrong. It is that each intervention, pursued in isolation, runs into the constraints created by the pillars it ignores. This is the core analytical argument of the six-pillar framework: healthcare transformation is a system problem, and system problems require system solutions.

Every major healthcare transformation framework in the past thirty years has identified the same list of necessary conditions. Pay differently. Redesign care delivery. Build data infrastructure. Address social determinants. Ensure equity. Execute operationally. The list is right. The reasoning behind it is right. And yet the transformation that the list is supposed to produce has not arrived — not in most states, not at national scale, not with the speed and completeness the evidence has long indicated is possible.

The problem is not the list. The problem is treating it as a list.

A checklist is a set of independent items. You can check them in any order. Checking one does not affect the others. Failing to check one does not prevent you from checking the rest. A checklist of healthcare transformation requirements implies that you can make progress on any one of these independently, and that partial completion is partial progress. This is wrong. The six pillars of the six-pillar framework are not independent items on a checklist. They are a system of interdependent components in which each pillar's effectiveness depends on the others, in which the failure of any single pillar cascades through the system, and in which the order and timing of interventions matters as much as the content of those interventions. Treating the six pillars as a checklist is precisely how decades of technically correct healthcare reform recommendations have produced inadequate results.

"A policy intervention that passes question one but fails questions three, four, five, or six is not a solution — it is a well-intentioned mistake waiting to be made."

— HTR Framework Documentation

The Six Pillars Defined

The six pillars are not equal in their causal priority — payment reform (Economics) and enabling legislation (Policy) are preconditions that the other pillars depend on — but they are equal in their necessity. A transformed healthcare system requires all six to be functional.

Policy — **The legislative and regulatory environment that creates the mandatory architecture for transformation. Policy establishes what actors must do, what they are prohibited from doing, and what governance structures ensure accountability. Voluntary reform operates within the Policy pillar's constraints; structural transformation requires it to change. Vermont application: *Acts 167 and 68, the AHEAD Model State Agreement, and the Statewide Strategic Plan mandate are Vermont's Policy pillar interventions. Each converts voluntary reform into statutory obligation.***

Economics — **The payment and financial architecture that determines what behaviors the system rewards. The Economics pillar encompasses payment reform (from fee-for-service to value-based and global payment), financial modeling, total cost of care management, and the hospital financial sustainability that transformation requires. Vermont application: *Reference-based pricing (FY2027), global hospital budgets (FY2028-2030), and the AHEAD Model's total cost of care accountability are Vermont's Economics pillar interventions.***

Operations	**The organizational capacity, management infrastructure, administrative simplification, and implementation discipline that converts strategy into executed programs. The Operations pillar is where transformation plans become real or remain documents — the graveyard of well-designed reforms that could not be implemented. Vermont application:** ***The 14-hospital transformation planning process, RHRC technical assistance, the AHS HSA Coordinator model, the Division of Planning and Effectiveness, and EMS regionalization are Vermont's Operations pillar investments.***
Clinical	**The care delivery models, quality measurement frameworks, clinical workforce design, and patient care programs that determine how care is actually delivered. The Clinical pillar is where payment reform becomes patient outcomes — or fails to. Vermont application:** ***The Blueprint for Health (all-payer PCMH program), Collaborative Care Model behavioral health integration, CCBHC expansion, and the primary care investment strategy are Vermont's Clinical pillar interventions.***
Equity	**The programs, investments, and accountability structures that ensure transformation benefits reach all populations — and specifically those most harmed by the current system's failures. Equity is not a soft afterthought; it is an analytical requirement. A transformation that reduces average costs while widening disparities has failed. Vermont application:** ***Geographic equity across Vermont's 14 HSAs, SDOH investment through the EAST Fund and Blueprint HRSN screening, rural access preservation in the RBP and global budget design, and health equity metrics in Act 167/68 reporting are Vermont's Equity pillar interventions.***
Technology	**The data infrastructure, health information exchange, analytical platforms, and clinical technologies that make population health management possible at scale. The Technology pillar is the information substrate on which all other pillar functions depend — you cannot manage what you cannot measure. Vermont application:** ***VHCURES (all-payer claims database), VITL/VHIE (health information exchange), the AHS-GMCB analytics vendor, AI scribe grants, and Vermont's Clinically Integrated Network are Technology pillar investments.***

The table below characterizes each pillar's diagnostic function and structural role in the system — not as a repetition of the definitions above, but as a specification of what each pillar produces when functioning and what question it answers in the transformation logic.

Pillar	Diagnostic question	Structural role in the system	What it produces when functioning
Policy	Is it permissible?	The mandatory architecture. Converts aspirational reform into binding requirements. Without mandatory authority, the highest-cost actors opt out and voluntary reform achieves marginal results.	Statutory mandates that eliminate voluntary opt-out; accountability timelines that cannot be deferred; federal-state agreements that commit both parties to specific outcomes.
Economics	Is it sustainable?	The incentive architecture. Determines whether organizations have a financial reason to behave differently. Payment reform does not change clinical behavior directly — it changes the financial logic that shapes clinical decisions over time.	Payment models in which population health management is financially rational; price structures that break the commercial cross-subsidy chain; accountability for total cost of care rather than encounter volume.
Technology	Is it possible?	The data substrate. Makes population health management, VBC contract execution, equity measurement, and strategic planning analytically feasible. Without data infrastructure, the other pillars are managing blind.	Attribution lists, TCOC measurement, HEDIS stratification, risk stratification, care gap identification, AI-assisted clinical workflows — all the analytical functions that VBC and population health management require.
Operations	Is it executable?	The execution layer. Translates statutory mandates, payment models, data platforms, and clinical programs into organizational reality. The most analytically sound framework fails if the implementing organization lacks capacity, infrastructure, and management discipline.	Hospital transformation plans that are implemented, not filed; AHS organizational capacity sufficient to manage 14 simultaneous transformation processes; project management infrastructure that tracks progress against statutory deadlines.
Clinical	Is it effective?	The mechanism of change. Payment reform changes incentives; clinical redesign changes behavior. The financial case for preventive investment only materializes when clinical programs actually deliver the utilization reduction that produces savings.	Primary care transformation that prevents hospitalizations; behavioral health integration that reduces crisis presentations; care management that closes gaps before they become acute episodes.
Equity	Is it just?	The cross-cutting constraint and accountability lens. Not a separate program — a dimension applied to every decision in every other pillar. Transformation that improves averages while widening disparities has failed by the framework's own standards.	Stratified measurement that reveals disparities hidden in aggregates; payment designs that do not penalize providers serving high-SDOH populations; clinical programs that reach disadvantaged populations; technology infrastructure that produces disaggregated data.

Figure 1.2 — The six pillars: diagnostic questions and structural roles in the six-pillar framework. Source: six-pillar framework documentation; Acts 167 and 68 of 2025\.

The Architecture of Interdependency: The Fifteen Dependency Relationships

```
THE FIFTEEN DEPENDENCY RELATIONSHIPS — COMPLETE MAP

  POLICY (1) ——enables——► ECONOMICS (2)     "Act 68 forces mandatory RBP + global budgets"
  POLICY (1) ——requires——► OPERATIONS (6)   "Statutory deadlines force AHS execution capacity"
  POLICY (1) ——enables——► EQUITY (5)        "Acts 167/68 embed equity accountability into law"

  ECONOMICS (2) ——drives——► CLINICAL (4)    "Global budgets make population health financially rational"
  ECONOMICS (2) ——requires——► TECHNOLOGY (3)"Attribution, TCOC, shared savings require VHCURES data"
  ECONOMICS (2) ——requires——► EQUITY (5)    "VBC contracts ignoring social risk penalize safety-net providers"

  TECHNOLOGY (3) ——enables——► ECONOMICS (2) "VHCURES analytics make APM financial management possible"
  TECHNOLOGY (3) ——enables——► EQUITY (5)    "Stratified VHCURES data reveals disparities hidden in averages"
  TECHNOLOGY (3) ——enables——► CLINICAL (4)  "Risk stratification, care gaps, SDOH flags — tech enables PHM"

  CLINICAL (4) ——requires——► OPERATIONS (6) "Blueprint PCMHs, CoCM, CCBHC all need workforce + admin infra"
  CLINICAL (4) ——enables——► EQUITY (5)      "Blueprint + CCBHC expansion is primary geographic access gap mechanism"

  OPERATIONS (6) ——enables——► POLICY (1)    "AHS monthly transformation reports feed back into policy design"
  OPERATIONS (6) ——requires——► TECHNOLOGY (3)"CIN analytics, VHCURES, FHIR compliance require IT/admin staff"

  EQUITY (5) ——constrains——► POLICY (1)     "Every policy decision must be evaluated for equity impact first"
  EQUITY (5) ——constrains——► CLINICAL (4)   "Implicit bias, cultural competence, language access constrain care deliver

  Legend:
  ——enables——►   Upstream presence expands downstream effectiveness (green)
  ——drives——►    Upstream change directly produces downstream behavioral change (blue)
  ——requires——►  Downstream cannot function without upstream in place (amber)
  - -constrains- Downstream imposes requirements on upstream design (dashed gray)
```

Figure 1.3 — The Architecture of Interdependency: all fifteen directed dependency relationships between the six pillars, with mechanisms and Vermont evidence. Interactive clickable version at htrintelligence.com/framework. Source: six-pillar framework documentation; Vermont Health Transformation / HTR (2026).

The six pillars do not operate independently. They interact through a set of dependencies and feedback loops that determine whether transformation produces the outcomes it promises. Understanding these interactions is the core analytical skill this book develops.

The foundational dependency is between Policy and Economics. Payment reform (Economics) requires policy authority — either statutory mandate or regulatory power — to be effective at scale. Vermont learned this lesson through a decade of voluntary payment reform efforts: the Vermont All-Payer ACO Model, OneCare's voluntary hospital participation, VITL's voluntary HIE connectivity. Each produced partial results because voluntary participation allowed the highest-cost actors to opt out. Act 68's mandatory RBP and global budget requirements are a Policy pillar response to the failure of Economics pillar interventions without Policy pillar backing.

The second critical dependency is between Economics and Clinical. Payment reform changes the financial incentive structure; clinical redesign changes the care delivery model to respond to the new incentives. A hospital operating under a global budget has a financial incentive to prevent hospitalizations — but it can only act on that incentive if it has the primary care network, care coordination infrastructure, and behavioral health integration to actually keep patients out of the hospital. Without the Clinical pillar investment, the Economics pillar incentive produces financial pressure without clinical improvement.

The third dependency is between Technology and all other pillars. You cannot attribute patients to hospitals for global budget purposes without accurate patient matching across providers. You cannot identify care gaps for quality improvement without population-level claims and clinical data. You cannot model the financial impact of service line closures without analytics infrastructure. You cannot monitor equity metrics without data stratified by race, income, and geography. Technology is the information substrate on which every other pillar's management functions depend.

The Equity pillar has a different kind of dependency: it is simultaneously a constraint on every other pillar and a beneficiary of all of them. Every payment reform intervention, clinical redesign, and technology investment must be evaluated for its equity implications — does it reduce disparities or exacerbate them? Does it reach the populations that need it most or concentrate benefits among those already well-served? Vermont's Act 168 goals explicitly include reducing health inequities alongside reducing costs and improving outcomes, recognizing that transformation that achieves the first two without the third is incomplete.

The Operations pillar is the translation layer — the organizational capacity that converts policy intent, payment incentives, technology tools, clinical models, and equity goals into implemented programs with trained staff and measured outcomes. A transformation strategy that is analytically sound, financially compelling, technically feasible, and clinically validated will still fail if the organization responsible for executing it lacks the project management infrastructure, the change management capability, and the sustained leadership attention that execution requires.

These five relationships are the framework's structural core. What follows develops the complete architecture: all fifteen directed dependency relationships between and among the six pillars, the specific mechanisms by which each dependency operates, the failure modes that emerge when dependencies are ignored, and the Vermont evidence that confirms the theoretical structure with empirical specificity. The six-pillar framework map — available as an interactive visualization on the technical infrastructure framework — renders these fifteen relationships as a navigable diagram.

Policy Dependencies: Three Relationships That Establish the Mandatory Architecture

Policy is the pillar from which mandatory authority flows. Its three outbound dependencies — to Economics, Operations, and Equity — are among the most critical in the framework, because they are the relationships that convert every other pillar from optional to required.

Policy ➝ Economics: Mandatory authority enables structural payment reform

Dependency type: ENABLES (critical path)

The most important dependency in the entire framework. Payment reform that is voluntary produces voluntary results. Vermont spent a decade under OneCare Vermont — a voluntary ACO model — and produced modest results among participating hospitals while non-participating hospitals continued operating under fee-for-service without consequence. Act 68 of 2025 changed the architecture: reference-based pricing is mandatory for all Vermont hospitals beginning FY2027, and global budgets are mandatory beginning FY2028. This is not an incremental improvement on the voluntary model. It is a structural change that eliminates the opt-out that made the voluntary model ineffective.

Vermont evidence: Oliver Wyman's Act 167 diagnosis explicitly found that voluntary reform was insufficient given the financial trajectory. The $700 million to $2.4 billion five-year deficit projection assumes the continuation of the commercial pricing structure that mandatory RBP is designed to dismantle. Without the Policy pillar's mandatory authority, the Economics pillar remains aspirational.

Policy ➝ Operations: Statutory deadlines force execution capacity

Dependency type: REQUIRES (critical path)

One of the most underappreciated dependencies in healthcare transformation. Organizations develop the capacity to execute complex initiatives when they have non-negotiable deadlines, not when they have good intentions. Act 68's December 2028 Statewide Strategic Plan deadline, the FY2027 RBP mandate, and the FY2028 global budget mandate are not just policy milestones — they are organizational forcing functions that require AHS to build the project management infrastructure, analytics capability, and staffing depth that the transformation requires.

Without the Policy pillar imposing external accountability, the Operations pillar faces the chronic problem of healthcare administration: transformation planning that is perpetually in progress but never complete, capacity building that is always on the agenda but never funded, and organizational development that yields to operational demands every time there is a conflict. Vermont's statutory deadlines are the mechanism that prevents this. They are deliberately irreversible.

Policy ➝ Equity: Statutory mandates embed equity accountability

Dependency type: ENABLES

Acts 167 and 68 both contain explicit equity requirements — they are not aspirational statements about desired outcomes but operational mandates: equity metrics must be tracked, disparities must be reported, and the Statewide Strategic Plan must address geographic and demographic equity gaps. This is the Policy pillar enabling the Equity pillar's accountability function. Without statutory embedding, equity monitoring is a discretionary activity that competes with operational priorities. With it, equity measurement is a compliance requirement.

Vermont evidence: the Health Care Delivery Advisory Committee established under Act 68 is required to consult with the Health Equity Advisory Commission and to incorporate equity considerations into the Statewide Strategic Plan. This institutional design is the Policy pillar building the Equity pillar's accountability structure into the statutory framework.

Economics Dependencies: Three Relationships That Transmit Incentive Change

Economics is the pillar through which policy mandates are converted into behavioral change at the organizational and clinical level. Payment models do not change behavior directly — they change the financial logic that shapes decision-making. The Economics pillar's three outbound dependencies — to Clinical, Technology, and Equity — describe the mechanisms through which this conversion happens.

Economics ➝ Clinical: Payment incentives drive care delivery redesign

Dependency type: DRIVES (critical path)

The most important economics dependency. Under fee-for-service, preventing a hospitalization costs the hospital revenue. A primary care practice that successfully manages a diabetic patient's condition and prevents a $15,000 hospitalization has generated no billable encounter for the prevented admission. The clinical logic and the financial logic point in opposite directions. Under global budgets, the calculus reverses: every avoidable hospitalization is a cost against a fixed revenue envelope, and every prevented admission is a margin improvement. The payment model makes population health management financially rational for the first time.

Vermont evidence: Vermont's Blueprint for Health demonstrates this dependency in reverse. The Blueprint's 5.8:1 return on investment — $5.8 million in reduced medical expenditure per $1 million invested in primary care — is achievable because the Blueprint's all-payer PMPM payments to PCMHs create a financial structure that rewards the clinical investment. Remove the Economics pillar and the Blueprint's clinical model produces its results without capturing its financial returns. Sustain the Economics pillar through AHEAD's global budgets and the Clinical pillar's population health logic becomes fully incentive-aligned.

Economics → Technology: VBC contracts require data infrastructure

Dependency type: REQUIRES

Value-based care contracts are agreements to share financial accountability for a population's total cost of care. They cannot be executed without the data infrastructure to measure that total cost. Attribution lists, TCOC benchmarks, shared savings calculations, risk adjustment factors, quality measure reporting — all of these require the all-payer claims data, the population health analytics platform, and the FHIR-based data exchange that the Technology pillar provides. An organization that signs an APM contract without adequate data infrastructure is making a financial commitment it cannot monitor, manage, or dispute.

Vermont evidence: Vermont's AHEAD global budgets for hospitals, beginning January 2027, require GMCB to set prospective annual budgets based on historical Medicare spending adjusted for population risk. This calculation requires accurate VHCURES data, validated risk adjustment methodology, and an analytics platform capable of producing hospital-specific TCOC projections. The AHS-GMCB analytics vendor procurement underway in 2025-2026 is the Technology pillar investment that makes the Economics pillar's AHEAD implementation analytically executable.

Economics → Equity: VBC design must include social risk adjustment

Dependency type: REQUIRES

Value-based care contracts that do not adjust for social risk systematically penalize providers serving high-SDOH populations. A hospital in Essex County, Vermont — serving a population with higher poverty rates, housing instability, food insecurity, and transportation barriers — faces higher care management costs and lower quality measure performance potential than a hospital in Burlington serving a more economically stable population. If their global budgets are set at the same percentage of Medicare rates without social risk adjustment, Essex County's hospital is being held to a standard that does not reflect its population's actual cost burden.

Vermont evidence: Vermont's AHEAD global budget design must address this explicitly. The methodology for social risk adjustment in Vermont's hospital global budgets is an active design question as of early 2026\. Getting it wrong — designing an Economics pillar instrument that ignores the Equity pillar's requirement — would produce a payment system that penalizes the hospitals serving the most vulnerable Vermonters.

Technology Dependencies: Three Relationships That Enable Analytical Management

Technology is the data substrate on which every other pillar's management functions depend. It has the broadest set of outbound dependencies in the framework — three enabling relationships to Economics, Equity, and Clinical that describe what becomes possible when data infrastructure is adequate, and what remains impossible when it is not.

Technology → Economics: Analytics makes VBC financial management possible

Dependency type: ENABLES

This dependency runs in both directions between Technology and Economics: the Economics pillar requires data infrastructure (documented above), and the Technology pillar enables the financial management capability that VBC contracts require. The distinction is between existence and effectiveness. An organization can enter an APM contract without adequate analytics — the contract still exists, the financial obligations are still real. But the organization cannot manage its financial performance under that contract, cannot identify where its costs are above benchmark, cannot target care management investment toward the highest-return opportunities, and cannot dispute errors in the payer's shared savings calculations.

Vermont evidence: VHCURES population health analytics make the APM financial modeling described in Chapter 7 possible. The APM Shared Savings Calculator, the Hospital Financial Stress Test Model, and the VBC Readiness Assessment — all tools in the HTR Research Lab — are built on VHCURES data logic. Vermont hospitals that invest in VHCURES analytics capability before AHEAD launches in January 2027 will be able to manage their global budgets proactively rather than reactively.

Technology → Equity: Disaggregated data reveals disparities hidden in averages

Dependency type: ENABLES (critical)

The Equity pillar's entire analytical function depends on the Technology pillar's ability to produce demographic stratification. You cannot measure a disparity you cannot see. Vermont's 91% primary care access rate — four points above the national benchmark — is a genuine achievement. It also hides an 11-point gap between white Vermont adults and BIPOC Vermont adults. That gap is invisible in aggregate data. It becomes visible only when VHCURES data is stratified by race/ethnicity, and that stratification is only possible when the data contains adequate demographic information.

Vermont evidence: Vermont's race/ethnicity data in VHCURES is incomplete — commercial claims frequently lack demographic fields. This is not a minor data quality issue; it is a Technology pillar gap that directly limits the Equity pillar's analytical capability. The Vermont Department of Health Health Equity Data Report (January 2025\) documents this limitation explicitly. Closing the Technology gap is prerequisite to the Equity pillar's disparity measurement function.

Technology → Clinical: Data infrastructure enables population health management

Dependency type: ENABLES

The clinical programs described in Chapter 10 — Blueprint PCMH, CoCM, CCBHC, CHT deployment — can all be designed and implemented without data infrastructure. What they cannot do without data infrastructure is function as population health programs rather than individual encounter programs. The difference is analytically fundamental. Risk stratification identifies which patients are most likely to deteriorate; without it, care management deploys reactively, after hospitalization rather than before. Care gap identification triggers outreach for overdue preventive services; without it, gaps close only when patients happen to schedule appointments. SDOH screening tools flag patients for CHT navigation; without the data platform to connect the flag to the navigation workflow, the flag is generated and ignored.

Vermont evidence: Blueprint's clinical registry is the Technology pillar product that enables Blueprint's Clinical pillar outcomes. The 5.8:1 ROI documented for Blueprint is achievable precisely because Blueprint combines the Clinical pillar's care team model with the Technology pillar's population health analytics. The AI scribe grants in the RHT Program are the Technology pillar's contribution to the Clinical pillar's workforce capacity problem.

Clinical Dependencies: Two Relationships That Connect Care Models to System Requirements

Clinical is the pillar that converts payment incentives into actual health outcomes. Its two outbound dependencies — to Operations and Equity — describe the operational requirements of clinical programs and their equity implications.

Clinical → Operations: Care models require execution infrastructure

Dependency type: REQUIRES

The most consistently underestimated dependency in healthcare transformation. A care model is a diagram until it has workforce, credentialing, administrative infrastructure, and operational management. The Collaborative Care Model requires a behavioral health care manager employed by a primary care practice. The CCBHC model requires a certified entity with the organizational capacity to serve all patients regardless of insurance status. Blueprint PCMHs require care coordinators, health information exchange connectivity, and NCQA recognition processes. None of these are available without Operations pillar investment.

Vermont evidence: Oliver Wyman's Act 167 analysis found that Vermont's administrative cost per adjusted discharge is 91% above the national benchmark. This is not merely an inefficiency problem — it is a signal that Vermont's Operations pillar infrastructure is absorbing cost without producing the execution capability that clinical transformation requires. The RHRC engagement with all 14 Vermont hospitals is specifically designed to build the Operations pillar capacity that Clinical pillar programs need.

Clinical → Equity: Care delivery expansion is the primary access gap mechanism

Dependency type: ENABLES

Geographic and demographic disparities in healthcare access are primarily clinical access problems — not enough primary care providers, not enough behavioral health capacity, not enough community health workers in underserved communities. The Equity pillar's access gap goals cannot be achieved without the Clinical pillar's care model expansion. Blueprint's PCMH expansion into Northeast Kingdom communities, the CCBHC network's geographic reach, and the CHT deployment in high-SDOH communities are Clinical pillar investments that directly serve Equity pillar objectives.

Vermont evidence: Essex County's 8% uninsurance rate and limited primary care access are equity problems with a clinical solution — expanding Blueprint-participating practices in underserved HSAs, deploying telehealth through the RHT Program to reduce geographic access barriers, and certifying additional CCBHCs in Northeast Kingdom communities. Remove the Clinical pillar investment and the Equity pillar's access gap remains unmeasured but unaddressed.

Operations Dependencies: Two Feedback Relationships That Close the Loop

Operations is the pillar that executes all other pillars' intentions. Its two outbound dependencies run back to Policy and Technology — feedback relationships that make the system self-correcting rather than purely top-down.

Operations → Policy: Implementation data feeds back into policy design

Dependency type: ENABLES

This is the feedback loop that allows the policy architecture to be self-correcting. AHS's monthly transformation reports to the Vermont Legislature — required under Act 68 — document whether transformation is proceeding on the statutory timeline, where gaps have emerged, and what adjustments are required. The GMCB's RBP methodology for FY2027 is being refined based on hospital financial data from the operations layer. The December 2028 Statewide Strategic Plan will be drafted based on what the Operations pillar has learned from implementing the preceding three years of transformation. This is not passive reporting — it is active policy learning.

Vermont evidence: AHS's November 2025 transformation report changed the policy understanding of Vermont's administrative cost problem by documenting the UVMMC $3,826 per adjusted discharge figure against the $1,427 benchmark. That Operations pillar data point is now embedded in the policy case for mandatory transformation. Without the operations feedback loop, policy would operate on assumptions rather than evidence.

Operations → Technology: Workforce runs the data infrastructure

Dependency type: REQUIRES

The Technology pillar's infrastructure — VHCURES analytics, the CIN data platform, FHIR compliance, AI governance programs — does not operate itself. It requires IT staff to maintain systems, data analysts to generate the reports that clinical and financial management teams use, privacy officers to manage HIPAA compliance, and cybersecurity staff to protect the infrastructure from the ransomware threats that have disabled rural health systems nationally. The Operations pillar's workforce is the human substrate on which the Technology pillar's data infrastructure runs.

Vermont evidence: Vermont's rural CAHs — Grace Cottage, Gifford Medical Center, North Country Hospital — have limited IT staff. This Operations pillar constraint directly limits their Technology pillar capability. The Vermont CIN's shared services model addresses this: a shared analytics and IT function within the CIN provides all 14 hospitals access to Technology pillar capability they cannot individually afford. The CIN is an Operations pillar solution to a Technology pillar gap.

Equity Constraints: Two Relationships That Apply the Justice Lens to the Whole System

Equity's two outbound dependencies are constraining rather than enabling — they impose requirements on Policy and Clinical that must be designed in from the beginning rather than retrofitted. Equity is not the last pillar in the sequence; it is the lens applied to every other pillar's decisions throughout the design and implementation process.

Equity → Policy: Equity accountability constrains every policy decision

Dependency type: CONSTRAINS

Every policy decision in Vermont's transformation — the RBP rate methodology, the global budget design, the COE designation framework, the Statewide Strategic Plan structure — must be evaluated for equity impact before it is finalized. This is not post-hoc equity review; it is an equity constraint embedded in the policy design process. A RBP methodology that sets rates uniformly without adjusting for the market conditions faced by hospitals in underserved communities is technically correct and practically inequitable. An equity constraint would require that rate methodology to account for geographic access implications before adoption.

Vermont evidence: Act 68's requirement that the Health Care Delivery Advisory Committee consult with the Health Equity Advisory Commission is the statutory mechanism for this constraint. The interaction is not advisory — it is structural. Equity considerations are required inputs into the policy process, not optional enhancements.

Equity → Clinical: Equity constrains how care is delivered, not just to whom

Dependency type: CONSTRAINS

Implicit bias in clinical settings, cultural incompetence, language access barriers, and differential treatment quality by race/ethnicity are clinical practice problems with equity implications. They are not solved by expanding access — expanding access to biased care widens some disparities while narrowing others. The Equity pillar constrains the Clinical pillar's design by requiring that care models address cultural competence, language access, and practice variation as design requirements, not implementation afterthoughts.

Vermont evidence: Vermont's BIPOC adults have a 79-81% primary care access rate against 91% statewide — but that gap reflects not only access barriers but differential engagement rates that are partly driven by trust and cultural competence factors in clinical settings. Blueprint's HRSN screening program is a Clinical pillar program designed with the Equity pillar's requirement that screening be linguistically and culturally appropriate and that the navigation it generates connects to resources that actually serve the communities being screened.

The Failure Cascade: What Happens When a Pillar Is Missing

The most rigorous test of a dependency framework is not whether it describes how things work when all components are present — it is whether it accurately predicts what fails when a component is removed. The fifteen dependencies described above generate specific, testable failure cascade predictions. The following analysis applies this test to each pillar.

Pillar removed	Immediate failure	Second-order failure	System-level outcome	Historical example
Policy	Payment reform remains voluntary. Highest-cost actors opt out. Rate benchmarks have no enforcement mechanism.	Clinical programs continue but without the financial incentive alignment that makes population health investment rational. Technology infrastructure is built but not used for financial management because APM contracts are not mandatory.	Incremental improvement among willing participants; no system-level change; ongoing premium inflation; hospital financial crisis continues on Oliver Wyman's trajectory.	Vermont 2010-2022: OneCare Vermont voluntary ACO. Modest results. 9/14 hospitals in losses by 2023\.
Economics	Clinical programs continue under fee-for-service incentives. Hospitals remain financially rewarded for volume, not value.	Primary care investment produces outcomes but cannot capture the financial return — the 5.8:1 ROI accrues to payers, not providers. Technology infrastructure is built but used for billing optimization rather than population health.	Clinical quality improves at the margin; costs continue rising; the financial case for transformation investment cannot be made; hospital financial fragility deepens.	U.S. national experience: 20+ years of clinical quality improvement without payment reform. Quality improved significantly; cost growth continued.
Technology	VBC contracts signed but not managed. Attribution errors go undetected. TCOC measurement is unreliable. Quality measures cannot be stratified.	Economics pillar contracts produce unexpected financial losses because the organization cannot identify where costs exceed benchmark. Equity pillar cannot measure disparities it cannot see. Clinical pillar cannot identify high-risk patients before they hospitalize.	VBC contracts produce financial losses for providers who cannot manage them; organizations exit VBC; payment reform stalls; equity gaps widen invisibly.	Most ACO failures in the early CMMI model period: organizations entered models without analytics infrastructure and produced unexpected losses.
Clinical	Payment reform changes incentives but has nothing to incentivize. Global budgets create financial pressure without the clinical programs that reduce costs sustainably.	Operations pillar manages financial pressure through service reductions rather than care model redesign. Equity pillar's access gaps widen as clinical capacity is cut to meet budget constraints.	Hospital financial crisis accelerates under global budget pressure without the clinical programs that make global budgets sustainable. Rural hospitals close.	Maryland HSCRC early period: global budgets without adequate primary care investment produced hospital financial pressure without the community health improvement that makes global budgets sustainable long-term.
Equity	Transformation proceeds but disparities widen. Average outcomes improve while geographic and demographic gaps grow.	Policy decisions that are technically optimal produce inequitable geographic distributions of access and services. Technology infrastructure is built but not used for stratified measurement. Clinical programs improve average quality without closing disparity gaps.	System transformation that increases overall efficiency while reducing equity — a technically successful and ethically failed transformation.	National VBC experience: most APM models improved average quality; most widened racial and socioeconomic disparities in quality. The efficient provider disadvantage penalized safety-net organizations.
Operations	Statutory mandates exist on paper. Clinical programs are designed. Data platforms are procured. Nothing is implemented.	Policy accountability fails because there is no organizational capacity to meet statutory deadlines. Economics pillar contracts are signed but not managed. Technology platforms are deployed but not used.	Transformation documents accumulate; statutory deadlines are extended; the December 2028 Strategic Plan is descriptive rather than committal; Vermont's Oliver Wyman deficit trajectory continues.	Most state health reform efforts of the 2010s: correct policy, adequate financing, sophisticated data — organizational capacity insufficient to execute at statutory speed.

Figure 1.4 — Failure cascade analysis: what breaks when each pillar is absent. Source: the framework analysis; CMMI evaluation literature; Oliver Wyman Act 167 Report; Vermont experience.

Using the Dependency Map as an Analytical Tool

The dependency map is not merely a theoretical framework — it is a practical analytical tool for three specific management functions: transformation sequencing, risk identification, and investment prioritization.

Transformation Sequencing: What Must Come First

The dependency structure generates a partial ordering of interventions. Some investments cannot produce their expected results until enabling investments are in place. Getting the sequence wrong is not merely inefficient — it produces failed interventions that generate political resistance to subsequent correct interventions.

The correct sequencing logic from the dependency map:

- **Policy before Economics:** Mandatory authority must precede payment reform deployment. Vermont's Acts 167 through 68 establish mandatory authority before RBP and global budgets take effect. This sequencing is not coincidental — it is the precondition for payment reform's effectiveness.
- **Technology before Economics (analytics function):** The analytics infrastructure for VBC contract management should be in place before full-risk contracts are signed. Vermont hospitals entering AHEAD in January 2027 should complete HCC gap analyses and VHCURES attribution modeling before the performance year begins.
- **Operations before Clinical (capacity building):** The organizational infrastructure for care model execution should be built before care models are deployed at scale. The RHRC engagement with 14 Vermont hospitals is building Operations pillar capacity before AHEAD's clinical accountability requirements take full effect.
- **Equity throughout, not at the end:** Equity is a constraint, not a final review. Designing the RBP methodology, global budget structure, COE designations, and analytics vendor requirements without equity review is cheaper and faster — and produces inequitable

instruments that require expensive redesign or produce inequitable outcomes.

Risk Identification: Reading the Dependency Map for Vulnerabilities

The dependency map identifies Vermont’s transformation vulnerabilities with precision. The critical path runs through the Technology pillar’s analytics capability: the AHEAD global budgets require VHCURES analytics, the equity measurement requires demographic stratification, and the clinical risk stratification requires the population health platform. The AHS-GMCB analytics vendor procurement — underway in 2025-2026 but not yet complete — is the single highest-risk gap in Vermont’s current transformation architecture.

A delay in analytics vendor selection and deployment does not merely delay one program. It delays the financial management capability for AHEAD, the equity measurement capability for the Statewide Strategic Plan, and the risk stratification capability for Blueprint’s CCBHC expansion. Three dependencies converge on this single Technology pillar investment.

Investment Prioritization: Where Marginal Investment Has the Highest Return

Not all pillar investments have equal leverage in the dependency structure. An investment in a pillar with many inbound dependencies — one that is required by multiple other pillars — has higher leverage than an investment in a pillar that is only required by one.

The Technology pillar has the highest inbound dependency count in Vermont’s current architecture: Economics requires it, Clinical requires it, and Operations requires it. The Operations pillar has the second highest: Clinical requires it and Policy requires it. This dependency structure implies that the highest-leverage investments in Vermont’s current transformation phase are:

- **Analytics vendor deployment (Technology pillar — Economics \+ Equity \+ Clinical all depend on it)**
- **AHS organizational capacity building (Operations pillar — Policy accountability \+ Clinical execution both require it)**
- **HCC gap closure at AHEAD-participating hospitals (Technology \+ Economics intersection — pre-launch analytics investment with direct financial impact at AHEAD start)**

The Vermont Thread: Six Pillars in One System

Vermont's healthcare transformation is the closest thing to a controlled experiment in simultaneous six-pillar intervention that American health policy has produced. The following table maps Vermont's primary reform vehicles onto the six-pillar framework, establishing the connective tissue that runs through every subsequent chapter.

Pillar	Primary Vermont intervention	Secondary interventions	Chapter
Policy	Acts 167 (2022), 51 (2023), 68 (2025): the legislative reform cascade; AHEAD Model State Agreement (January 2025\)	Act 55 (drug pricing); Act 49 (insurer solvency); Act 62 (HIE governance); federal RHT Program award	Chapter 4: The Policy Pillar in Practice — CMMI Models, Waiver Strategy, and the Federal-State Policy Interface
Economics	Reference-based pricing (mandatory, FY2027); global hospital budgets (FY2028-2030); AHEAD total cost of care accountability	Act 68 hospital spending reduction (2.5%, FY2026); GMCB FY27 budget guidance (-1% commercial rate); Act 55 drug cap ($135M savings)	Chapter 6: The Economics Pillar in Practice — VBC Financial Modeling, APM Readiness, and the ROI of Transformation
Technology	VHCURES all-payer claims database; Vermont CIN (RHT Program); AHS-GMCB analytics vendor; AI scribe and RPM grants	VITL/VHIE HIE; DVHA HIT Plan (Act 62); statewide EHR feasibility assessment; FHIR compliance investments	Chapters 12 and 13
Clinical	Blueprint for Health (all-payer PCMH \+ CHT); CoCM and MHI behavioral health integration; CCBHC expansion (7 entities by 2026\)	Hub and Spoke SUD model; PACE program development; COE regionalization; dementia care infrastructure	Chapter 10: The Equity Pillar — Closing Gaps, Not Just Averaging Them
Equity	Geographic equity monitoring (HSA-level); SDOH integration through AHS HSA Coordinators; rural access preservation in RBP/global budget design	HRSN universal screening (Blueprint); CCBHC access for all populations; Northeast Kingdom focus; GLP-1 access advocacy	Chapter 12: The Technology Pillar — Data Infrastructure for a Transformed Health System
Operations	14-hospital transformation planning (RHRC \+ AHS); Vermont CIN shared services; Act 68 transformation grants ($2M); PMO establishment	EMS regionalization; administrative simplification (CON, prior auth); AHS restructuring; Division of Planning and Effectiveness	Chapters 8, 11

Figure 1.5 — Vermont's six-pillar transformation map. Sources: Acts 167, 51, 55, 62, 68 of 2022-2025; AHEAD State Agreement; Vermont RHT Program Application; Oliver Wyman Act 167 Report.

This table is not a comprehensive inventory of Vermont's healthcare reform. It is a navigation map — a way of locating Vermont's specific interventions within the analytical framework that the subsequent chapters develop. A reader working through Chapter 4 (Policy pillar) will recognize Acts 167 and 68 as the Policy pillar's legislative expression. A reader in Chapter 6 (Economics pillar) will see reference-based pricing and global budgets as the Economics pillar's operative mechanisms. The framework gives every Vermont policy intervention a structural home and every analytical concept a Vermont application.

The Framework Map as a Communication Tool

Beyond its analytical function, the six-pillar interdependency map serves a communication purpose that is equally important: it gives practitioners a shared vocabulary and a shared mental model for discussing transformation across professional boundaries.

Healthcare transformation involves clinicians, economists, data scientists, policy professionals, equity advocates, and operational managers — professionals whose training gives them deep expertise in their own domain and limited fluency in the others. The economist who designs the global budget methodology may not see the clinical care management program whose financial returns the budget depends on. The clinical informaticist building the population health platform may not see the equity requirement that stratification must be demographic, not just clinical. The policy professional drafting the Statewide Strategic Plan may not see the operations capacity constraints that determine whether the plan is executable on its statutory timeline.

The framework map makes these cross-domain dependencies visible to people whose professional training did not equip them to see them. When a clinician clicks on the Clinical pillar and sees the dependency arrow to Operations, the map communicates — without requiring an economics lecture — that their care model's execution depends on organizational infrastructure decisions being made in a different part of the agency. When a policy professional sees the Equity pillar's constraining arrows to Policy and Clinical, the map communicates that equity review belongs in the design process, not the approval process.

This communication function is not secondary to the analytical function. Healthcare transformation's most persistent failure mode is not analytical error — it is organizational silo failure, in which technically correct decisions made in isolation produce collectively incorrect outcomes. The framework map is designed to make the silos visible.

"The map does not tell you what to do. It tells you what to pay attention to — which decisions affect which other decisions, and in which direction. That is the beginning of coordination."

Key Concepts in This Chapter

Term	Definition
Six-pillar framework	The analytical architecture organizing healthcare transformation across six interdependent domains: Policy, Economics, Operations, Clinical, Equity, and Technology. Transformation requires all six pillars to be functional; failure in any one pillar undermines the others.
Single-pillar thinking	The failure mode in which a transformation initiative addresses one pillar — typically payment reform or clinical redesign — without attending to the constraints and dependencies created by the other five pillars.
Reform cascade	A legislatively structured sequence in which each reform intervention builds on the institutional capacity and political legitimacy created by its predecessor. Vermont's Act 167 → Act 51 → Act 68 sequence is the prototype.
Structural transformation	Change that alters the fundamental incentives, relationships, and capabilities of a system — as distinguished from incremental improvement within the existing structure. This book uses 'transformation' in this precise sense throughout.
Vermont as preview	The analytical premise that Vermont's healthcare challenges — demographic aging, hospital financial fragility, commercial cross-subsidy unsustainability, voluntary reform failure — are present nationally on a 10-20 year lag, making Vermont's responses a template for the nation's future policy toolkit.
Critical path dependency	A dependency relationship in which the failure of the upstream pillar immediately disables the downstream pillar's core function. The Policy → Economics dependency (mandatory authority enables payment reform) and the Economics → Clinical dependency (payment incentives drive care redesign) are critical path dependencies.
Enabling dependency	A dependency relationship in which the upstream pillar's presence expands the downstream pillar's effectiveness, but the downstream pillar can function at reduced capability without it. Technology → Clinical (data infrastructure enables population health management) is an enabling dependency: Blueprint can function without full VHCURES analytics, but its population health management capability is significantly constrained.
Constraining dependency	A dependency relationship in which the downstream pillar imposes requirements on the upstream pillar's design. Equity → Policy (equity accountability constrains policy decisions) and Equity → Clinical (equity constrains care delivery design) are constraining dependencies. Unlike enabling dependencies, these run against the primary direction of authority in the framework.
Failure cascade	The sequence of second- and third-order failures produced when a single pillar is absent or underfunded. Understanding failure cascades is essential for investment prioritization — the pillar whose absence produces the longest cascade is the highest-priority investment.
Dependency leverage	The number of other pillars that depend on a given pillar, weighted by the type of dependency. High leverage pillars — those with multiple inbound critical path dependencies — are the highest-priority investments because their failure or weakness cascades to multiple other pillars simultaneously. In Vermont's current architecture, Technology has the highest dependency leverage.
Sequencing error	The error of deploying a downstream pillar investment before the upstream enabling investment is in place. Sequencing errors produce failed interventions and political resistance to correct subsequent interventions. Vermont's AHEAD global budget launch (January 2027\) before the AHS-GMCB analytics vendor is fully deployed is a potential sequencing error in the Technology-Economics dependency.
six-pillar framework map	The interactive visualization available on the technical infrastructure framework (relevant policy monitoring resources/framework) showing all fifteen dependency relationships between the six pillars. Clicking any pillar shows its inbound and outbound dependencies with relationship type, mechanism, and Vermont evidence. Source code available as a standalone React component for integration into institutional platforms.

Sources: six-pillar framework documentation; Act 167 of 2022; Act 68 of 2025; Oliver Wyman Act 167 Community Engagement: Recommendations (August 2024); Vermont AHEAD State Agreement (January 2025); CMMI model evaluation literature; Maryland HSCRC performance data; Vermont Blueprint for Health ROI study (Population Health Management); AHS Health Care System Transformation Reports (2025); Vermont Department of Health Health Equity Data Report (January 2025).

#

#

Chapter 2: The Execution Sequence: Why Order Is Not Optional

The six pillars of healthcare transformation — Policy, Technology, Economics, Clinical, Equity, and Operations — are often presented as a framework of simultaneous imperatives. Each pillar is necessary. Each depends on the others. Remove any one and the system fails. This is true. But it obscures a critical practical question that every health system leader, state official, and reform architect must answer before a single dollar is spent or a single care model is redesigned: in what order do you actually build this?

The answer is not arbitrary, and it is not the order in which the pillars are typically defined or discussed. The execution sequence is determined by dependency logic — the hard reality that some things cannot function until other things are already in place. Getting the order wrong does not merely slow progress; it produces structural failures that are expensive to reverse and politically difficult to explain.

Vermont provides both the positive and negative proof cases for this argument. The Blueprint for Health and the AHEAD Model demonstrate what is achievable when the foundational stages are built in the right order. OneCare Vermont, the state's Accountable Care Organization, demonstrates with equal clarity what happens when foundational stages are skipped or assumed. Most instructively, OneCare's failure validates the specific sequencing argument this chapter makes: that Technology must precede Economics-as-management, not follow it. Understanding why leads directly to the correct execution order.

Vermont: The Crisis That Made Sequencing Non-Negotiable ***National readers unfamiliar with Vermont's system need the following facts before the sequencing argument becomes fully legible. These numbers explain not just why Vermont needed reform — but why the sequencing of that reform could not be improvised. 9 of 14 Vermont hospitals reported operating losses in FY2023, with deficits reaching −8.9%. Oliver Wyman's trajectory analysis projects 13 of 14 in losses by 2028 absent structural intervention. The cumulative five-year system deficit is projected between $700 million and $2.4 billion. Vermont's average silver exchange plan premium rose from $456 per month in 2018 to $948 in 2024 — a 108% increase over six years. Median household income grew 22% over the same period. Out-of-pocket maximums more than doubled. The University of Vermont Medical Center, with approximately 50% of the state's hospital market share, charged approximately 417% of Medicare rates for outpatient services in 2022 — a pricing structure that flows directly into premiums with no regulatory brake. Vermont's working-age population is projected to decline 13% by 2040 — from 367,000 to approximately 318,000 — while residents aged 65+ rise from 21.7% to over 30%. This demographic shift progressively transfers patients from commercial insurance to Medicare and Medicaid, both of which reimburse below cost, collapsing the cross-subsidy that has sustained Vermont's rural hospitals. This is the fiscal environment in which every reform decision this chapter analyzes was made. Vermont did not choose careful sequencing because it was disciplined. It was forced to sequence correctly because it had run out of time to get it wrong. See Appendix A for the full Vermont System Portrait.***

```
THE SIX-PILLAR EXECUTION SEQUENCE  (Policy → Technology → Economics → Clinical → Equity → Operations)

  STAGE 1 —— POLICY ——————————————————————————————————————————————————————————— [FOUNDATION]
  │   Establish enforceable authority · mandate participation · secure transformation capital
  │   Vermont: Acts 167 → 51 → 68 · AHEAD Federal Agreement · $195M RHT Program award
  │   WHY FIRST: Nothing can be authorized, built, or funded without statutory permission.
  │              Mandatory architecture prevents high-cost actors from opting out (OneCare lesson).
  ▼
```

```
STAGE 2 —— TECHNOLOGY ——————————————————————————————————————————————————————— [SUBSTRATE]
│  Build integrated clinical + claims data infrastructure before assuming financial risk
│  Vermont: VHCURES · VITL/VHIE · AHS-GMCB Analytics Vendor (McKinsey) · Vermont CIN
│  WHY SECOND: Technology is the operating system. Managing global budgets without integrated
│              data = "Managing Blind" failure mode that destroyed OneCare Vermont.
▼
STAGE 3 —— ECONOMICS ———————————————————————————————————————————————————————— [INCENTIVES]
│  Deploy global budgets + reference-based pricing onto a system that can already see itself
│  Vermont: Global hospital budgets FY2028 · RBP ≤200% Medicare FY2027 · AHEAD TCOC activation
│  WHY THIRD: Economics-as-management requires the technology substrate already operational.
│             Payment design (Economics-as-design) proceeds in parallel during Stage 2.
▼
STAGE 4 —— CLINICAL ————————————————————————————————————————————————————————— [MECHANISM]
│  Redesign care delivery models on aligned financial incentives
│  Vermont: PCMH transformation · CoCM behavioral health integration · CCBHC expansion · Blueprint 5.8:1 ROI
│  WHY FOURTH: Clinical transformation reverts when incentives revert. Must follow payment alignment.
│              Without Technology, clinical programs cannot identify which patients need intervention.
▼
STAGE 5 —— EQUITY ——————————————————————————————————————————————————————————— [CALIBRATION]
│  Audit, calibrate, and correct — social risk adjustment embedded in payment design
│  Vermont: HRSN screening · EAST Fund SDOH · social risk adjustment in global budgets · REH model
│  WHY FIFTH: Equity is a constraining dependency running through all prior stages. Stage 5 measures
│             whether equity commitments in Stages 3–4 have actually produced equitable outcomes.
▼
STAGE 6 —— OPERATIONS ——————————————————————————————————————————————————————— [EXECUTION]
   Close the administrative cost gap · translate strategy into implemented programs
   Vermont: RHRC methodology · shared administrative services · 14-hospital transformation planning
   WHY SIXTH: Vermont's $1,303/discharge administrative gap vs. benchmark = primary direct savings source.
              Operational redesign requires the payment model it is optimizing for to already be live.
```

Figure 2.1 — The Six-Pillar Execution Sequence: revised order (Policy → Technology → Economics → Clinical → Equity → Operations), key actions, and sequencing rationale. The critical insight is Technology before Economics — the inverse of OneCare Vermont's fatal sequencing error. *Source: HTR Analysis (2026).*

The OneCare Failure: A Sequencing Autopsy

Before presenting the execution sequence, it is worth examining OneCare Vermont's failure in structural terms, because the failure was not random and it was not primarily the result of bad intentions or poor execution. OneCare failed because of sequencing errors baked into its design from the beginning. Identifying those errors precisely is more useful than simply noting that the ACO wound down after a decade of operation.

To understand the failure, readers new to this topic need a brief orientation. An Accountable Care Organization, or ACO, is a group of hospitals, physicians, and other providers who coordinate care for a defined patient population with the goal of reducing costs. When the ACO succeeds in keeping patients healthier and reducing unnecessary utilization, it shares in the savings generated for the payer. Vermont created OneCare as its primary ACO vehicle under the Total Cost of Care model — a payment structure that held OneCare financially responsible for the total healthcare spending of an attributed Vermont population. On paper, this was sound design. In practice, it encountered three sequencing failures that compounded into a cascade the organization could not survive.

Sequencing Failure 1: Economics Without Policy Readiness

The most fundamental problem with OneCare was that Vermont attempted to operate a sophisticated payment reform model — one requiring system-wide accountability — without the policy foundation to enforce participation. Provider participation in OneCare was voluntary. Hospitals, physician practices, and other providers could choose whether or not to join.

This matters structurally, not just administratively. The Total Cost of Care model holds the ACO financially accountable for the total spending of an attributed population. But when high-cost, high-revenue providers remain outside the ACO — because participation is optional — the ACO is held responsible for costs it cannot influence. A hospital system generating significant revenue from inpatient admissions has a rational financial incentive to stay outside a model that rewards reducing those admissions. When it does, the ACO's ability to manage total cost is structurally undermined from day one.

The policy foundation — the legislative and regulatory authority to require participation, or to make opting out economically irrational — was never established for OneCare. Vermont's more recent legislative progression, from Act 167 through Act 51 to Act 68, reflects a hard-won recognition of this failure: mandatory architecture converts voluntary reform into binding structural change. The AHEAD Model's federal agreement builds on this by embedding participation requirements that OneCare never had.

What optional participation actually means for a hospital CFO ***Imagine you are the chief financial officer of a Vermont hospital. Your hospital generates $40 million annually from inpatient admissions. OneCare Vermont is asking you to join an ACO that will hold you financially accountable for reducing those admissions — reducing your revenue. Participation is voluntary. You have three choices: join and accept the risk; join and manage costs without affecting your revenue base; or don't join. For many hospital systems, the rational choice was the third. This is not a failure of values — it is a predictable response to an incentive structure that the policy framework failed to close. Vermont's hospital fiscal crisis makes this more urgent: nine of fourteen hospitals reported operating losses in 2023, with projections indicating thirteen of fourteen will be in deficit by 2028. Average silver exchange plan premiums rose from $456 in 2018 to $948 in 2024 — a 108% increase — while median household income grew only 22%. This structural crisis is precisely what Acts 167, 51, and 68 were designed to address by converting voluntary participation into a mandatory statutory requirement.***

Sequencing Failure 2: Economics Without Technology — The 'Managing Blind' Failure Mode

The second and most instructive sequencing failure compounds the first. OneCare attempted to manage a complex, population-level payment model — one involving real financial risk — without the technology infrastructure required to do so. Specifically, OneCare did not build or acquire an integrated data system combining timely clinical data with claims data in a way that made population health management operationally feasible.

For readers new to this topic: claims data is the financial record of healthcare — every bill submitted to an insurer. It tells you what was done, where, when, and at what cost. Clinical data is the medical record — diagnoses, lab results, medications, care plans. It tells you why something was done and what the patient's health status actually is. Neither dataset is sufficient alone for managing a value-based payment model. You need both, integrated, and available in near-real time.

OneCare operated without this integrated infrastructure at meaningful scale. The consequences were severe and entirely predictable. Care managers could not identify high-risk patients proactively — they were working from claims data arriving 60 to 90 days after care was delivered, meaning they were identifying patients who had already been hospitalized rather than preventing hospitalization in the first place. This is the 'Managing Blind' failure mode: assuming downside financial risk under a Total Cost of Care model without the analytics to track total cost of care in real time. The result is unexpected and unexplainable financial losses — not because the model is wrong, but because the organization cannot see what it is supposed to be managing.

This failure directly informs the execution sequence this chapter recommends. OneCare followed what sounds like a logical order: design the payment model, then figure out the technology. But OneCare's experience reveals the flaw in that logic: the gap between 'design the payment model' and 'build the technology to manage it' is precisely where organizations assume risk before they have visibility. The correct sequence closes that gap by requiring the technology substrate to be operational before the payment model goes live.

***Why timely data matters as much as integrated data** Consider a care manager responsible for 500 high-risk patients under a Total Cost of Care model. Her job is to identify patients who are deteriorating and intervene before they are hospitalized. If her data arrives 60 days after the fact, she learns in March that a patient was hospitalized in January. The intervention opportunity is gone. The cost has already been incurred. She is reviewing history, not managing risk. If her data arrives within 24 to 48 hours — integrated clinical and claims feeds updated daily — she can see that a patient filled a new prescription last week, had an abnormal lab result three days ago, and has not seen their primary care provider in six months. She can call today, before the hospitalization happens. Vermont's technology infrastructure for the AHEAD Model addresses this directly: VHCURES provides longitudinal population data; VITL/VHIE provides the clinical data exchange; and the AHS-GMCB Analytics Vendor (awarded to McKinsey in early 2026) provides the modeling layer for global budget management and equity monitoring. This is the substrate OneCare never built.*

The Failure Cascade: How Sequencing Errors Compound

The two sequencing failures above did not operate independently. They compounded into a cascade that progressively undermined OneCare's ability to function. Because participation was optional, OneCare could not achieve the provider network coverage required to manage total cost of care. Because it could not manage total cost of care, it could not demonstrate savings. Because it could not demonstrate savings, it could not attract the capital required to build the technology infrastructure that might have allowed it to manage costs more effectively. Because it lacked that technology, its care management remained limited, its financial reporting remained inadequate, and its clinical value remained undemonstrated. And because it could not demonstrate clinical value, skeptical providers had no reason to join — returning the cascade to its starting point.

This is what a sequencing failure looks like at the system level: not a single catastrophic event, but a slow structural unraveling in which each missing foundation makes the next stage harder to execute. OneCare Vermont wound down its ACO operations after roughly a decade. The failure was architectural, not operational.

```
ONECAREVERMONT FAILURE CASCADE — A SEQUENCING AUTOPSY

  SEQUENCING GAP 1: ECONOMICS WITHOUT POLICY
  ┌──────────────────────────────────────────────────────────────────┐
  │ ERROR: Voluntary participation — no mandate for high-cost providers │
  │ DIRECT: High-revenue hospitals opt out; ACO held accountable for   │
  │         costs it cannot influence                                  │
  │ CASCADE: Total cost of care cannot be managed at system level      │
  │          → Savings targets missed repeatedly                       │
  └──────────────────────────────────────────────────────────────────┘
                           ▼ compounds with ↓

  SEQUENCING GAP 2: ECONOMICS WITHOUT TECHNOLOGY
  ┌──────────────────────────────────────────────────────────────────┐
  │ ERROR: No integrated real-time clinical + claims data; 60–90 day   │
  │        reporting lag — "Managing Blind" failure mode               │
  │ DIRECT: Care managers identify hospitalized patients after the fact,│
  │         not before. Proactive intervention is structurally impossible│
  │ CASCADE: High-cost events not prevented → financial exposure grows │
  │          → unexpected and unexplainable losses accumulate          │
  └──────────────────────────────────────────────────────────────────┘
                           ▼ compounds with ↓

  SEQUENCING GAP 3: PAYMENT RISK BEFORE INFRASTRUCTURE
  ┌──────────────────────────────────────────────────────────────────┐
  │ ERROR: Full financial risk assumed before technology was ready     │
  │ DIRECT: Cannot demonstrate savings → funders withdraw capital      │
  │ CASCADE: No capital → cannot build the technology that would close │
  │          the gap → performance remains poor → more providers opt out│
  └──────────────────────────────────────────────────────────────────┘
                           ▼

  SYSTEM-LEVEL OUTCOME: ORGANIZATIONAL WIND-DOWN (~10 YEARS)
  ┌──────────────────────────────────────────────────────────────────┐
  │ Three gaps mutually reinforcing. No operational decision could     │
  │ reverse the cascade once the architecture was locked in.           │
  │ FAILURE WAS ARCHITECTURAL, NOT OPERATIONAL.                        │
  │                                                                    │
  │ Vermont's Response: Acts 167 → 51 → 68 (mandatory participation)   │
  │ + AHS-GMCB Analytics Vendor (Technology before Economics goes live) │
  └──────────────────────────────────────────────────────────────────┘
```

Figure 2.2 — Vermont anchors and OneCare sequencing failures by execution stage. Source: sequencing_timeline_v5.pptx, Slide 2. HTR Analysis (2026)

The OneCare Failure Cascade: A Structural Diagnosis

The table below traces how OneCare's three sequencing errors compounded into an organizational failure from which no amount of subsequent operational excellence could recover.

Sequencing Gap	Structural Error	Direct Consequence	Cascade Effect
Policy (Stage 1)	Voluntary participation — no mandate to require high-cost provider enrollment	High-revenue hospitals opt out; ACO held accountable for costs it cannot influence	Total cost of care cannot be managed at system level → savings targets missed
Technology (Stage 2)	No integrated real-time clinical + claims data; 60–90 day reporting lag	'Managing Blind' failure mode — care managers identify hospitalized patients after the fact, not before	Proactive intervention impossible → high-cost events not prevented → financial exposure accelerates
Economics (Stage 3)	Payment model assumed financial risk before technology was ready to support it	Unexpected losses → inability to demonstrate savings to payers or funders	Capital unavailable to build technology → technology gap persists → cycle repeats
System-level cascade	Three gaps mutually reinforcing	Organizational wind-down after ~10 years of operation	Failure was architectural, not operational — no management decision could reverse it once the sequence was locked in

Figure 2.3 — The OneCare Failure Cascade: how three sequencing errors compounded into organizational wind-down. *Source: HTR Analysis (2026). Based on GMCB records, AHS November 2025 Transformation Report, and OneCare Vermont public filings.*

Resolving the Chicken-and-Egg: Why Technology Precedes Economics

The OneCare failure resolves a tension that has confused reform architects since the beginning of value-based care: the apparent chicken-and-egg relationship between Technology and Economics. The confusion runs in both directions. On one hand, you cannot manage economics without technology. A global budget model without integrated, timely data is a financial accountability structure with no operational mechanism for accountability. On the other hand, you cannot fund technology without economics. The infrastructure required for population health management requires significant sustained investment. Someone must decide to fund it.

The resolution requires distinguishing three things that reform frameworks typically conflate:

Economics-as-design — the analytical work of defining the payment architecture: units of accountability, metrics, risk allocation, performance periods. This is planning work that can and should happen during the technology build. It does not require the technology to be running. A technology vendor building a care management platform needs to know what the payment model is measuring — the design specifies the build requirements.

Policy-as-capital — the funding of the technology build itself. This is a critical insight that most frameworks miss entirely: the capital for Stage 2 technology infrastructure is a Policy-pillar function, not an Economics-pillar function. Vermont's $195 million Rural Health Transformation award is the clearest example — federal grant capital secured through the Policy pillar that funds the technology substrate. This is fundamentally different from the incentive architecture the Economics pillar creates.

Economics-as-management — the ongoing operational work of running global budgets: monitoring performance, calculating shared savings, adjusting payments, holding providers accountable. This work absolutely requires the technology infrastructure to already be functional. It cannot begin until the data substrate is in place.

The sequencing logic that follows is unambiguous: Policy secures both the mandate and the capital. Technology gets built, funded by Policy capital, while payment architecture is designed in parallel. Economics goes live — as management, not just design — only when the technology can support it. OneCare inverted this by going live with financial risk before the technology was ready. The result was the 'Managing Blind' failure mode described above.

There is also a second important distinction embedded in the Economics pillar that the correct sequence makes clearer: the difference between Incentive Architecture and Transformation Capital. The Economics pillar's structural role is to create the financial environment that makes population health management rational — global budgets, reference-based pricing targeted at 200% of Medicare or below, the 'Balloon Squeeze' logic ensuring that price controls do not simply shift costs into volume expansion. This is permanent structural change to the payment environment. Transformation Capital — the RHT award, the AHEAD EAST Fund providing up to $150 million annually — is time-limited bridge funding for infrastructure. Confusing the two leads to treating one-time grants as permanent subsidies, which is as structurally unsound as treating incentive architecture as capital investment.

The Six Stages of Execution

With OneCare's failure as the empirical anchor and the Technology-Economics dependency resolved, the correct execution sequence can be stated with both logical clarity and practical grounding.

Stage 1: Policy — Mandatory Architecture and Transformation Capital

Policy is the absolute precondition for everything that follows, but its role is larger than simply establishing regulatory permission. The Policy pillar has two functions that no other pillar can perform: it creates the mandatory architecture that prevents high-cost actors from opting out of reform, and it secures the transformation capital that funds the technology build.

Vermont's legislative progression illustrates both functions. Act 167 established the diagnostic foundation — the evidence base demonstrating Vermont's healthcare system had reached a structural breaking point, with nine of fourteen hospitals in operating losses and a cumulative five-year system deficit projected at between $700 million and $2.4 billion. Act 51 moved to the planning phase, defining the transformation architecture. Act 68 operationalized the mandate — converting voluntary reform into binding statutory requirements and establishing the regulatory framework for global budgets and reference-based pricing. The AHEAD Model federal agreement layered in the federal partnership and funding commitments that made the technology build financially feasible.

The $195 million Rural Health Transformation award is a Policy-pillar outcome, not an Economics-pillar outcome. This distinction matters because it clarifies where transformation capital comes from: not from the payment model itself, but from the policy and regulatory relationships that secure federal investment. Reform architects who expect the payment model to self-fund the technology build will consistently find themselves in the same position as OneCare — financially exposed before the infrastructure is ready.

Policy at Stage 1 must address the participation question directly. A reform architecture that allows high-cost, high-revenue providers to opt out of accountability creates a structural gap that no subsequent stage can fill. Vermont's hard experience with OneCare is the proof case. The mandatory architecture of Acts 51 and 68 is the corrective response.

Stage 2: Technology — Building the Substrate Before Assuming Risk

Technology is the operating system on which every subsequent stage runs. This is the sequencing insight that OneCare's failure makes most vividly clear. Technology is not a layer added on top of a functioning reform — it is the foundation that makes reform functional. The entire logic of placing Technology before Economics rests on this point: you cannot assume financial risk under a global budget model before you can see what you are managing.

The technology infrastructure required for serious value-based transformation has several non-negotiable components. First, it must integrate clinical data with claims data. Vermont's infrastructure addresses this through VHCURES — the all-payer claims database providing foundational longitudinal data for population health analytics and total cost of care modeling — and VITL/VHIE, the Vermont Health Information Exchange providing the clinical data substrate for real-time information sharing across the provider network. Neither alone is sufficient. VHCURES without clinical data tells you what things cost but not how to change them. Clinical data without VHCURES tells you about individual patients but not about population-level cost patterns.

Second, the data must be timely. Data arriving 60 to 90 days after care supports retrospective reporting but not proactive care management. The ability to identify patients at risk before they become expensive requires data updated within days. Vermont's AHS-GMCB Analytics Vendor addresses this by providing the modeling needed for hospital global budget design, total cost of care tracking, and equity monitoring — the analytics gap identified in the November 2025 AHS report as a critical missing piece.

Third, the Vermont Clinically Integrated Network functions as a technology-led institution providing shared analytical capabilities to rural hospitals that individually lack the scale to build these functions. Rural transformation cannot depend on every small hospital independently developing sophisticated population health analytics. The shared infrastructure model is both economically rational and operationally necessary.

The payment architecture design happens in parallel during Stage 2 — this is the Economics-as-design work that specifies what the technology needs to do. But the Economics pillar does not go live, does not assume risk, and does not begin performance management until the technology substrate is functional. That is the sequencing discipline that OneCare lacked and that the AHEAD Model is designed to enforce.

Stage 3: Economics — Incentive Architecture on a Visible System

Economics is the third stage, not the second, because its management function requires the technology substrate already to be operational. When the data infrastructure is in place, the Economics pillar deploys the payment architecture that converts population health management from a clinical aspiration into a financially rational imperative.

Vermont's economic architecture has two structural components that work together. Reference-based pricing, targeted at 200% of Medicare rates or below, addresses the unit price problem: Vermont's commercial insurance rates have run far above Medicare, creating a cross-subsidy that is neither sustainable nor equitable. But price controls alone create a 'Balloon Squeeze' — if unit prices are capped without controlling volume, providers simply generate more volume to maintain revenue. Global hospital budgets address the volume dimension by setting a fixed total revenue envelope for each hospital, making volume expansion financially irrational. Together, reference-based pricing and global budgets break the fee-for-service trap that has made population health management economically punishing for providers.

The AHEAD EAST Fund — providing up to $150 million annually in Medicare funds — operates alongside the incentive architecture as transformation capital for community health investment. This is the distinction between Incentive Architecture and Transformation Capital in practice: global budgets and reference-based pricing permanently restructure the payment environment; the EAST Fund provides time-limited bridge capital for the workforce development, telehealth infrastructure, and community health worker programs the transformed system requires.

The equity constraint on payment design also becomes operationally urgent at Stage 3. Global budgets and reference-based pricing, if designed without social risk adjustment, can inadvertently penalize safety-net providers serving high-SDOH populations — providers who appear less efficient by standard metrics because their patient populations have more complex social needs. Vermont's Northeast Kingdom is the clearest example: a global budget benchmark calibrated without accounting for the region's underlying social risk profile will generate financial pressure on the region's hospitals that reflects social disadvantage, not operational inefficiency. Social risk adjustment must be embedded in the payment design, not retrofitted afterward.

Stage 4: Clinical — Redesigning Care on Aligned Incentives

Clinical redesign is where the economic return on the entire investment is realized. It is the stage at which payment incentives, technology infrastructure, and operational capacity converge to change what actually happens between a clinician and a patient. The Patient-Centered Medical Home transforms primary care from acute episodic treatment into proactive, team-based population management. The Collaborative Care Model integrates behavioral health into primary care. The Certified Community Behavioral Health Clinic expands access for the highest-need populations. Vermont's Blueprint for Health generated a 5.8 to 1 return on investment from its primary care investment — among the most robust ROI findings in the value-based care literature.

Clinical redesign comes fourth because clinical transformation without economic alignment reverts when the incentives revert. This is among the most empirically robust findings from a decade of payment reform research. Clinicians redesign their practice in response to the incentives they face and the data available to them. Without a payment model that rewards the redesigned care — already live at Stage 3 — and without the technology to identify which patients need which interventions — already built at Stage 2 — clinical transformation is isolated, unsustainable, and unable to demonstrate the population-level outcomes that justify its cost.

Stage 5: Equity — Constraining Dependency and System Calibration

Equity is fifth in execution sequence but must be understood as a constraining dependency that runs through every preceding stage — not a final add-on applied to a finished system. The distinction between equity as a design constraint and equity as an audit function is critical for understanding what Stage 5 actually does.

At Stage 3, equity constrains the payment design through social risk adjustment. At Stage 4, equity constrains the clinical redesign through cultural competence requirements, language access mandates, and targeted deployment in underserved areas. Vermont's 11-point primary care access gap between white and BIPOC residents is not closed by a Stage 5 audit — it is closed by Stages 3, 4, and 5 working in sequence: payment design that does not penalize safety-net providers, clinical redesign that deploys Community Health Teams in underserved areas, and Stage 5 audit that measures whether those interventions are actually closing the gap.

What Stage 5 adds is the systematic, population-level measurement of whether the equity commitments embedded in earlier stages have produced equitable outcomes — and the corrective action, informed by real data from a functioning system, when they have not. Vermont's HRSN screening at scale, the EAST Fund for social determinants, and the Rural Emergency Hospital model provide the operational mechanisms for this stage. The REH model — preserving 24/7 emergency access in geographically isolated communities — is an equity intervention that could not be deployed without the operational and clinical infrastructure of Stages 2 through 4 already in place.

Stage 6: Operations — Closing the Administrative Cost Gap

Operations is the final stage and the execution layer that converts all prior-stage investments into organizational reality. Vermont's administrative cost data makes the operational opportunity concrete: Vermont's critical-access and rural hospitals spend $2,730 per discharge on administration, compared to a national benchmark of $1,427. This $1,303 per-discharge gap — across Vermont's hospital system — represents the primary source of the $100 million or more in projected direct savings from shared services and administrative simplification.

Operations is last in execution sequence not because it is least important — administrative cost reduction is one of the most direct levers for financial sustainability — but because operational redesign requires the technology infrastructure, payment incentives, and clinical models of the preceding stages to already be in place. A hospital system that redesigns its administrative operations before its payment model is live will optimize for a fee-for-service world that the reform is designed to replace. The Rural Health Redesign Center methodology — performing financial and operational baselines for each of Vermont's fourteen hospitals, identifying the specific administrative gaps, and designing the shared services architecture — is an operational function that produces durable results only when the payment environment it is optimizing for is already operational.

The Sequence in Practice: Overlap, Iteration, and Political Reality

The six-stage sequence described in this chapter is a logical structure grounded in dependency relationships, not a rigid waterfall that prohibits parallel work. In practice, payment architecture design proceeds in parallel with the technology build. Operational planning begins before the technology is complete. Clinical pilots run before the payment model fully covers the redesigned care. This is normal and expected.

What the sequence provides is a framework for understanding what happens when dependencies are violated — specifically, when a later stage assumes operational risk before an earlier stage has delivered its foundational output. When a technology build is incomplete, the payment model that depends on it should not go live with full financial risk. When a payment model has not yet aligned incentives, clinical transformation initiatives will face reversion pressure. When workforce is insufficient to deliver a clinical model, the model must be scoped to what is actually deliverable.

The most important practical insight from Vermont's experience — positive and negative — is that the sequence failures that produce the most damage are the ones that appear to be minor timing decisions. OneCare did not make a dramatic choice to ignore technology. It made the reasonable-sounding decision to launch the payment model while technology was still being developed. That decision, compounded by the absence of mandatory participation, produced a failure cascade that no amount of subsequent operational excellence could reverse.

The framework presented in this chapter gives reform architects the analytical basis to resist that pressure — to hold the sequence with discipline while managing the political reality that demands visible progress at every stage, and to distinguish between genuine parallel progress and premature sequencing that builds failure into the system from the start.

Sequencing Summary

Stage	Pillar	Key action	Why this sequence	Vermont anchor
1	Policy	Establish enforceable authority, mandate participation, and secure transformation capital	Nothing can be authorized, built, or funded without statutory permission. Policy also secures the capital (RHT award) that funds the technology build. Mandatory architecture prevents high-cost actors from opting out.	AHEAD Model federal agreement; Acts 167, 51, and 68; $195M Rural Health Transformation award; mandatory provider reporting requirements
2	Technology	Build integrated clinical + claims data infrastructure — real-time — before financial risk is assumed	Technology is the operating system. It must be operational before the Economics pillar goes live. Managing global budgets without integrated data is the 'Managing Blind' failure mode that destroyed OneCare.	VHCURES all-payer claims database; VITL/VHIE clinical data exchange; AHS-GMCB Analytics Vendor (McKinsey); care management platforms; AI-assisted documentation
3	Economics	Deploy global budgets and reference-based pricing onto	Economics-as-management requires the technology substrate to already be operational.	Global hospital budgets; Reference-Based Pricing at ≤200% Medicare; AHEAD

Stage	Pillar	Key action	Why this sequence	Vermont anchor
		a system that can already see itself	Payment design (Economics-as-design) happens in parallel during Stage 2, but the Economics pillar goes live only when data infrastructure is ready.	EAST Fund up to $150M annually; Total Cost of Care model activation
4	Clinical	Redesign care delivery models and transform clinical practice on aligned incentives	Clinical transformation is where ROI is realized — but only when payment incentives are live and technology can identify which patients need which interventions.	PCMH transformation; Collaborative Care Model (CoCM); CCBHC expansion; Blueprint for Health 5.8:1 ROI from primary care investment
5	Equity	Audit, calibrate, and correct — including social risk adjustment embedded in payment design	Equity is a constraining dependency that must be embedded in Stages 3–4 and audited here against real population data. Without social risk adjustment, global budgets penalize safety-net providers.	HRSN screening at scale; EAST Fund for social determinants; social risk adjustment in global budgets; Rural Emergency Hospital model
6	Operations	Close the administrative cost gap and translate strategy into implemented programs	Operations is the execution layer. Vermont's $1,303 per-discharge administrative gap vs. national benchmark is the primary source of projected direct savings from shared services and simplification.	Rural Health Redesign Center (RHRC) methodology; shared administrative services; 14-hospital transformation planning; Community Health Teams

Figure 2.4 — The Six-Stage Execution Sequence: pillar, key action, sequencing rationale, and Vermont anchor evidence. *Source: HTR Analysis (2026).*

The table above summarizes the revised execution sequence: Policy → Technology → Economics → Clinical → Equity → Operations. The key change from earlier sequencing frameworks is the placement of Technology before Economics. This change is not cosmetic. It reflects the central lesson of OneCare's failure: that assuming financial risk before the data infrastructure is operational is not a sequencing preference but a structural error with predictable consequences.

Vermont's Blueprint for Health succeeded because it built the technology substrate incrementally over years before the full payment accountability of the AHEAD Model was assumed. OneCare failed because it assumed that accountability first. The execution sequence this chapter recommends ensures that every state and health system learning from Vermont's experience avoids that error by design rather than discovering it through failure.

Chapter 3: The Policy Pillar — Legislative Architecture for Structural Reform

The Policy Pillar — Legislative Architecture for Structural Reform

How Vermont's Acts 167 and 68, the Oliver Wyman Analysis, and the AHEAD Agreement Created the Mandatory Architecture for Structural Reform

Source integration: Act 167 (2022), Act 68 (2025), Oliver Wyman Act 167 Community Engagement: Recommendations (September 2024, revised October 2024), GMCB February 2026 RBP/Global Budget Report, Vermont Rural Health Transformation Program Application (November 2025), AHS January 2025 Legislative Presentation.

The Anatomy of a System-Level Reform Mandate

Most healthcare reform unfolds incrementally — a new demonstration model here, a revised billing rule there — accumulating over years into changes that are difficult to attribute to any single decision. Vermont's reform trajectory between 2022 and 2025 is different in kind, not degree. Within three years, the Vermont General Assembly enacted a sequence of legislation that collectively mandated the most comprehensive statewide healthcare restructuring attempted by any U.S. state since Maryland's all-payer model in the 1970s. Understanding this sequence — its logic, its mechanisms, and its unresolved tensions — is essential preparation for any healthcare leader operating in an environment of payment and delivery system transformation.

The sequence has a clear internal logic. Act 167 of 2022 was a diagnostic mandate: commission a rigorous, public, community-grounded diagnosis of what is actually wrong. Act 51 of 2023 was a planning mandate: begin doing something about it. Act 68 of 2025 was an operational mandate: specific tools — reference-based pricing, global hospital budgets, a Statewide Health Care Delivery Strategic Plan — were authorized and given statutory deadlines with real consequences for non-compliance. Each act built on the evidence and institutional capacity developed by its predecessor. The whole sequence constitutes what policy scholars call a reform cascade: a legislatively structured process in which each intervention creates the conditions for the next.

This chapter section examines each act, the analytical infrastructure it created, and the six-pillar implications for organizations navigating comparable reform environments. It draws heavily on the Oliver Wyman Act 167 Community Engagement: Recommendations report — the most comprehensive independent public diagnosis of a state hospital system in recent American health policy history — which must be read as the intellectual foundation for everything Vermont's legislature did in 2025 and everything AHS is mandated to deliver by 2028\.

The Three Imperatives: Oliver Wyman's Central Finding

Before examining the legislative sequence, it is worth stating plainly what the Oliver Wyman analysis concluded at the highest level of abstraction. After 230 meetings across Vermont's 14 Hospital Service Areas, engagement with more than 3,100 participants representing over 100 organizations, and systematic analysis of financial, demographic, and quality data spanning every hospital in the state, Oliver Wyman's executive summary distilled Vermont's challenge to three imperatives:

Imperative 1: Build housing and other facilities and fix transportation.

Imperative 2: Pay PPS hospitals with reference-based pricing and move to global budgets/capitation for all when conditions for success are met.

Imperative 3: Move all care possible out of hospitals.

These three imperatives are not a wish list. They are the analytically derived conclusion of a year-long process that examined every dimension of Vermont's system. Each imperative maps directly onto the six-pillar framework: the housing and transportation imperative is an Equity and Operations pillar challenge; the pricing and global budget imperative is an Economics and Policy pillar challenge; and the shift of care out of hospitals is a Clinical pillar challenge that requires Technology and Operations pillar execution. The entire Vermont reform agenda can be read as a simultaneous, coordinated intervention across all six pillars — which is precisely why it is analytically instructive for any organization facing comparable pressures.

Oliver Wyman was equally direct about what will not work: more funding without structural change. Their analysis showed that simply increasing funding to attract more providers would drive up prices without improving access given Vermont's labor market constraints. Raising taxes or commercial premiums further risks driving residents and businesses out of the state. And the rural nature and shrinking population of most Vermont communities means that many areas simply cannot sustain a hospital with a full-time, full-spectrum of specialties. The implication is unambiguous: easy solutions — pour in money, add providers, maintain the status quo — will not resolve Vermont's healthcare crisis. The system must be structurally redesigned.

The Platform for Change: Vermont's System in Crisis

The Oliver Wyman report documented five interconnected challenge domains that constitute what it called the Platform for Change — the evidence base that makes transformation not just desirable but unavoidable. Each domain has precise, quantified data that the book's previous draft understated. The full picture is considerably more alarming than most public discussion of Vermont healthcare acknowledges.

Decreasing Affordability

Vermont's healthcare cost trajectory has been disconnecting from household income for at least six years, with no natural correction mechanism in sight. Oliver Wyman documented that average premiums for silver exchange plans reached $985 in 2024 — a 108% price increase in six years. Individual and small group plan premiums increased 60-80% in the same period. Out-of-pocket maximums more than doubled in five years. Meanwhile, Vermont median household income grew only 22% between 2018 and 2022\. Hospital approved charge increases grew 38% over the same period, compared to income growth of 22%. The gap is structural: hospital price growth has been running at two to three times income growth, and there is no market mechanism that closes this gap without regulatory intervention.

The premium data from Vermont Blue Cross Blue Shield confirms and extends this picture. Average annual per-member-per-month medical and pharmacy costs rose from $481 in 2020 to $812 in 2024 for local claims — a 69% increase in four years. Out-of-pocket maximums doubled in five years. A 2025 state survey found that many Vermonters, even those with insurance, delay care due to fears of medical debt. The affordability crisis is no longer a risk on the horizon; it is a current condition affecting clinical behavior.

Deteriorating Hospital Financial Sustainability

The financial projections in the Oliver Wyman report are the most important data in the entire Vermont reform story, and they need to be stated with the precision Oliver Wyman provided, not rounded down to reassuring generalities.

In FY2023, nine of Vermont's fourteen hospitals reported operating losses, with the worst reaching \-8.9%. Oliver Wyman modeled two scenarios for the trajectory through 2028\. Under the conservative scenario — 3.5% annual non-340B revenue growth and 5% annual expense growth — 13 of 14 hospitals are projected to report losses by 2028, with a cumulative 5-year system-wide deficit of $700 million against break-even ($1.4 billion against a 3% target operating margin). This is before accounting for the fact that Vermont hospitals collectively requested $285 million in budget increases for FY25 versus FY24 alone, suggesting the conservative scenario may already be understated.

Under the more realistic scenario — accounting for 10% annual expense growth on non-physician labor, 7-8% growth on other operating expenses, and constrained revenue — the 5-year cumulative deficit reaches $2.4 billion against break-even, or $3.1 billion against a 3% operating margin. This is approximately $3,700 to $4,800 per Vermont resident over five years. The UVM Health Network, which accounts for more than half the state's hospital capacity, faces $1.5 billion in cumulative deficits over five years in the high-expense scenario.

These projections assume no structural change. They are not predictions of what will happen — they are the analytical case for why structural change is necessary. Without the interventions mandated by Acts 167 and 68, Vermont's hospital system does not gradually decline; it collapses, with hospitals closing unexpectedly rather than through orderly transformation.

Vermont in Practice: The Hospital Financial Crisis by the Numbers

Oliver Wyman's financial modeling established the quantitative case for urgent intervention. The key data points every healthcare leader and policymaker should know:

- 9 of 14 Vermont hospitals reported operating losses in FY2023, with losses reaching \-8.9%
- Under conservative assumptions (3.5% revenue growth, 5% expense growth): 13 of 14 hospitals report losses by 2028; $700M cumulative 5-year system deficit
- Under realistic assumptions (7-8% expense growth): $2.4B cumulative 5-year system deficit against break-even; $3.1B against 3% operating margin
- Vermont hospitals requested $285M in budget increases for FY25 vs. FY24 — suggesting the conservative scenario is already understated
- UVM Health Network alone faces $1.5B cumulative deficit over 5 years under the high-expense scenario
- Average silver exchange plan premium reached $985 in 2024 — a 108% increase in 6 years
- Out-of-pocket maximums more than doubled in 5 years
- Hospital charges grew 38% since 2018; median household income grew only 22%

These are not projections of modest stress. They are projections of systemic collapse without intervention. The entire Vermont legislative reform agenda from 2022 to 2025 is a response to this specific data.

The Demographic Trap

Vermont's population dynamics create what Oliver Wyman called a demographic trap — a self-reinforcing cycle from which conventional market responses provide no escape. Vermont's population is aging and shrinking simultaneously: the 65+ population is projected to increase 57% by 2040, rising from 21.7% in 2020 to exceed 30% of total population. The working-age population (20-64) will decline 13% by 2040, from 367,000 to 318,000. By the mid-2030s, Vermont will have nearly as many residents over 65 as under 20\.

The consequences cascade in multiple directions at once. Aging drives higher utilization of complex, expensive services: cancer, heart disease, stroke, dementia, and frailty-related hospitalizations all increase with age. A rising 65+ population means a rising proportion of Medicare and Medicaid patients — payers that reimburse below cost — at the expense of the commercially insured working-age population that cross-subsidizes hospital operations. The shrinking commercial base makes the cross-subsidy less viable precisely as the demand for it grows. Oliver Wyman was direct: the cross-subsidy model that has sustained Vermont's rural hospital system is becoming structurally impossible to maintain as demographics shift.

This dynamic operates at the hospital service area level, not just statewide. Oliver Wyman's HSA-level projections showed that every service area except Burlington will experience population decline and aging — with the Northeast Kingdom (Caledonia, Essex, Orleans counties) facing the most severe trajectory. Newport's 65+ population, already at higher proportions than statewide averages, will represent a majority of the HSA's healthcare demand by 2040\.

The Interconnected Failure System

Perhaps the most analytically powerful contribution of the Oliver Wyman report is its systems map of how Vermont's healthcare failures interconnect and amplify each other. The map traces how a housing shortage creates a staffing crisis (workers cannot afford to live near the hospitals that need them), how staffing shortages create capacity constraints, how capacity constraints produce ED crowding and delayed discharges, how delayed discharges require expensive travel nursing that drives up costs, how cost increases drive premium increases, how premium increases reduce affordability, how reduced affordability produces avoidable ED visits from patients who delayed primary care, completing the cycle.

External forces compound these internal dynamics: inflation drives labor and supply costs beyond what hospital budgets can absorb; an aging population shifts the payer mix toward lower-reimbursing Medicare and Medicaid; the cumbersome Certificate of Need process slows capital investment; licensure complexity impedes provider recruitment; and out-of-state care migration reduces procedure volumes, further undermining the clinical volume needed to sustain specialist expertise.

The systems map has a direct implication for policy design: interventions that address only one node of this system will be absorbed by the others. A workforce recruitment program without affordable housing produces recruits who cannot stay. A primary care access expansion without transportation infrastructure does not reach rural patients. Reference-based pricing without global budgets controls unit prices but allows volume to expand. Oliver Wyman's three imperatives are formulated precisely to address the highest-leverage nodes simultaneously, not sequentially.

Healthcare Equity Gaps

Oliver Wyman documented equity challenges across three categories that map directly onto the equity pillar's disparity root cause taxonomy. Gaps in culturally competent care were the most frequently cited equity challenge in community meetings — particularly for patients who do not speak English as a first language, LGBTQ+ patients, patients with disabilities, and patients in rural communities with cultural mistrust of mainstream healthcare institutions. Vermont's foreign-born population, while lower than the national average at 4.5%, is concentrated in Burlington and surrounding communities where language access remains inadequate relative to need.

Gaps in workforce culture and psychological safety were the second category — not just about patients but about providers. Community meeting participants described weight bias, stigma, and a lack of diversity in clinical leadership that affects both care quality and staff retention. Keeping healthcare staff is a function of cost of living, inflation, and affordable housing, multiple stakeholders noted — connecting the workforce equity problem directly to the SDOH infrastructure problem.

Lack of coordination between healthcare and community services was the third category — the failure to close referral loops between clinical care and housing, transportation, food security, and behavioral health. Multiple meeting participants noted that EMR systems do not talk to each other, that patients must navigate 31 EMS agencies for transfers, and that no centralized system guides patients through the coverage and financial assistance they are entitled to.

The Perception vs. Reality Problem

One of Oliver Wyman's most valuable contributions is a systematic correction of widely held misconceptions about Vermont's healthcare infrastructure — misconceptions that, if left uncorrected, produce policy responses calibrated to a system that does not exist. The following table reproduces Oliver Wyman's analysis:

Common perception	What the data actually shows
Vermont has an 'All Payer' model	The 'All Payer' system does not include FFS Medicare or approximately 40% of commercially insured Vermonters. It covers a minority of total healthcare spending.
Vermont has a statewide ACO (OneCare)	OneCare participation is voluntary and does not include several hospitals or many community-based providers. OneCare has been unable to provide the information needed to guide patient care and anticipate needs.
Vermont has a functioning State HIE (VITL)	Participation in VITL is voluntary and many community-based providers are not included. VITL lacks pharmacy claims data and is not viewed as user-friendly by providers.
Vermont does not have enough primary care providers	If primary care providers were supported to see 3 patients per hour, there are enough for well into the future. HRSA recognizes no Health Profession Shortage Areas in Vermont. The problem is productivity and practice model, not pure headcount.

These corrections matter for the book's argument because they explain why Vermont's reform agenda is more complex than it appears from the outside. A state that appears to have all-payer payment reform, an ACO, and a health information exchange actually has voluntary, partial implementations of each — generating the appearance of a transformed system while the underlying fee-for-service, fragmented, data-poor reality persists. Acts 167 and 68 are, in part, a legislative response to this gap between perception and reality.

Act 167 (2022): The Diagnostic Mandate

Vermont passed Act 167 in the immediate aftermath of the COVID-19 pandemic, when the financial fragility of Vermont's hospital system had become impossible to ignore. The act had three core directives: commission a comprehensive statewide hospital systems analysis; direct AHS and GMCB to develop a federal multi-payer payment model proposal; and mandate design work on hospital global budgets. The three directives are analytically linked — you cannot design a credible global budget without understanding each hospital's actual cost structure and community role, and you cannot negotiate a federal model without a clear picture of total cost of care and where it diverges from benchmarks.

The Oliver Wyman Process: A Template for System-Level Reform

GMCB contracted with Oliver Wyman at a fixed price of $1,050,000 for work running from August 2023 through September 2024\. The scale of the engagement was unusual for a state of Vermont's size: over 230 meetings across all 14 Hospital Service Areas, involving more than 3,100 participants from over 100 organizations. Meeting types included 57 sessions with hospital leadership and boards, 45 sessions with state partners, 50 public HSA-level community meetings averaging 68 participants each, 35 provider meetings, and 15 sessions specifically designed to reach diverse populations whose voices are typically absent from healthcare planning processes.

The political economy of this process deserves study because it is a template for system-level reform in any context. Publishing recommendations to close or restructure named services at named community hospitals is not analytically difficult — it is politically treacherous. Communities organize around their local hospitals. Hospital closures generate intense local resistance. The Vermont process managed this by front-loading community engagement before recommendations, establishing the community meetings not as forums for defending the status quo but as inputs to a shared diagnosis. This sequencing — listen first, diagnose second, recommend third — produced findings with enough public legitimacy to survive the inevitable political backlash from hospitals whose specific services were named for restructuring or closure.

The methodology also reflects a design philosophy worth naming explicitly: the report moved from hospital-centered point solutions to systematic redesign; from individual stakeholder consultation to community-driven process; and from post-crisis remedy to pre-planned transformation. These are not rhetorical preferences — they reflect Oliver Wyman's learned lesson from failed hospital reform processes elsewhere, where solutions designed around individual institutional interests rather than community needs predictably failed to achieve systemic change.

The Five Act 167 Goals as an Accountability Framework

Act 167 established five statutory goals for Vermont's healthcare transformation that have since become the accountability framework for everything that followed: reduce inefficiencies; lower costs; improve health outcomes; reduce health inequities; and increase access to essential services.

These goals matter not as aspirational language but as operational accountability criteria embedded in subsequent reporting and regulatory requirements. AHS's monthly reports to the legislature track progress on these five dimensions. The GMCB's hospital budget review process references them. The Rural Health Transformation Program application explicitly maps each proposed initiative to them. When Act 68's Health Care Delivery Advisory Committee develops the Statewide Strategic Plan, it will be measured against them. This is how statutory goal-setting works when properly designed: not as a preamble, but as a persistent accountability structure that outlives the bill that created it.

Vermont in Practice: Oliver Wyman's Priority Action Lists for 2025

The Oliver Wyman report concluded with explicit priority action lists for each actor in Vermont's governance system — one of the most operationally specific outputs of any state healthcare consulting engagement on record. These lists were published in September 2024; their implementation status one year later is a direct measure of Vermont's reform momentum.

Priority legislative changes for the Vermont General Assembly:

- Remove barriers to building affordable housing for Vermont residents and newcomers
- Approve funding for EMS transformation
- Expand broadband coverage to rural areas
- Review existing AHS agency structure and program list to identify overlaps and efficiency opportunities
- Develop state regulations for Rural Emergency Hospital and free-standing birthing center designations
- Expand professional licensure and practice scope for nurses, EMTs, and pharmacists

Priority transformation programs for AHS to initiate in 2025:

- Regionalize specialty care services across hospitals
- Professionalize and regionalize EMS
- Improve care coordination for heavy utilizers — elderly, mental health, neuro-divergent, and foster care populations
- Develop dual-eligible targeting, care planning, and coordination
- Coordinate and optimize statewide electronic medical record systems

Priority regulatory changes for GMCB:

- Permit no further increases in commercial subsidization for hospital financial shortfalls
- Refrain from licensing any further hospital-based outpatient department units
- Simplify and shorten the Certificate of Need process
- Encourage free-standing diagnostic, ambulatory surgical, and birthing centers
- Begin movement to reference-based pricing, ideally at 200% of Medicare or less for PPS hospitals
- Require all hospitals to use the same accounting agency and method for budget submissions

The alignment between this list and what Act 68 subsequently mandated is not coincidental. The Oliver Wyman recommendations are the intellectual source material for Act 68's operative provisions.

The Oliver Wyman System Redesign Blueprint

The Oliver Wyman report's most detailed section — running from pages 32 through 114 of the full document — provides a three-tier system redesign framework that any healthcare system undertaking structural transformation can apply. The framework organizes recommendations at the state level, the system level, and the individual hospital level, recognizing that each tier requires different interventions executed by different actors on different timelines.

State-Level Recommendations: Infrastructure and Governance

At the state level, Oliver Wyman identified four domains of foundational infrastructure that hospital transformation cannot succeed without, regardless of payment model or clinical redesign. These recommendations are addressed primarily to AHS and the Vermont General Assembly, not to hospitals themselves.

The first domain is governance clarity. Oliver Wyman found that AHS and GMCB are under-resourced for the transformation work they are being asked to lead, that roles between the two agencies need clarification, and that the process requires a dedicated project management office and facilitation support. The specific recommendation to add a Division of Planning and Effectiveness at GMCB — to calculate the impact of service site changes on hospital budgets, monitor access and affordability of community providers, and track quality, access, and equity metrics — is a significant organizational design recommendation that has direct implications for AHS's own restructuring mandate under Act 68\.

The second domain is AHS structural alignment. Oliver Wyman recommended that AHS align its agency sub-units with Hospital Service Areas and meet regularly with hospitals and providers in each HSA. It also recommended that AHS fully computerize and integrate its sub-units — an implicit diagnosis that AHS's current organizational structure impedes the kind of coordinated population health management that a transformed system requires. This recommendation is the foundation of the AHS restructuring chapter developed later in this book.

The third domain is state-level investments in Social Determinants of Health: specifically housing, transportation, and broadband. Oliver Wyman was unusually direct about including these as healthcare system recommendations rather than separate social policy recommendations — reflecting the systems analysis that showed housing shortage driving staffing crisis driving cost escalation. The implication is that AHS's healthcare reform mandate cannot be separated from its housing, transportation, and economic development responsibilities. This is a governance challenge as much as a policy challenge.

The fourth domain is administrative simplification: streamlining prior authorization, shortening the CON process, simplifying Medicaid billing, and upgrading IT infrastructure and interoperability. Oliver Wyman documented the direct cost of administrative complexity — a cumbersome CON process prevents cost-effective contracts from being secured in time, and fragmented IT systems mean a provider may need to contact 31 separate EMS agencies to arrange a patient transfer.

System-Level Recommendations: Regionalization

At the system level, the Oliver Wyman blueprint calls for a fundamental redesign of how Vermont's 14 hospitals relate to each other and to the communities they serve. The core concept is regionalization: rather than each hospital attempting to provide a full spectrum of services to its local population — a model that is increasingly unsustainable given population density and physician shortage — Vermont should develop regional Centers of Excellence (COEs) in specific specialties, concentrating volume where it can sustain clinical expertise, while ensuring access through improved transportation and telehealth.

Oliver Wyman developed specific COE assignments for 10 of Vermont's 13 current Hospital Service Areas across 24 specialty categories. The University of Vermont Medical Center received 16 COE designations, covering all major specialties except acute general surgery, mental health emergency department, pediatric psychiatry, and skilled nursing. Southwestern Vermont Medical Center received 9 COE designations. Most community hospitals received between 1 and 6 designations in specialties where they have sufficient volume and expertise to sustain clinical excellence.

Hospital	COE Count	Key Specialties
UVM Medical Center (Burlington)	16	Cancer (complex), neurology, women's health, rehabilitation, most surgical specialties
Southwestern VT Medical Center (Bennington)	9	Cancer surgery (non-complex), geriatric care, orthopedics, psychiatry (adult/adolescent), radiation therapy
Central VT Medical Center (Barre)	5	Geriatric care, infusion therapy, neurology, psychiatry (adult), radiation therapy
Brattleboro Memorial Hospital	6	Acute general surgery, cancer surgery, geriatric care, orthopedics, robotic surgery
Northwestern Medical Center (St. Albans)	4	Cancer surgery (non-complex), infusion therapy, neurology, radiation therapy
Northeastern VT Regional Hospital (St. Johnsbury)	4	Cancer surgery (non-complex), infusion therapy, minimally invasive surgery, radiation therapy
Rutland Regional Medical Center	5	Acute general surgery, geriatric care, minimally invasive surgery, neurology, psychiatry (adult)
Springfield Hospital	2	Memory care, psychiatry (adult)
Copley Hospital (Morrisville)	2	Orthopedics, rheumatology
Mt. Ascutney Hospital (White River Junction)	1	Rehabilitation
Grace Cottage, Gifford, North Country, Porter	TBD	Further discussion required as part of regionalization plan

The COE framework serves multiple goals simultaneously. Concentrating surgical and specialty volumes creates the patient throughput needed to maintain clinical excellence — a surgeon performing 200 procedures a year maintains skills that one performing 20 cannot. Concentrating volumes in financially viable hospitals reduces the total number of facilities that need to sustain high-cost infrastructure. And freeing smaller hospitals from the expectation that they must provide full-spectrum care allows them to redesign their role — becoming hubs for primary care, mental health, geriatric services, and community health rather than trying to maintain services they cannot staff or afford.

The system redesign blueprint also calls for a fundamental shift in the geography of care: move all care possible out of hospitals. Community-based care, primary care, mental health care, and housing capacity should all increase to divert care to lower-cost settings. Home-based care and mobile care programs should expand. Ambulatory surgical units and free-standing diagnostic centers should replace hospital-based equivalents for low-risk procedures. The savings from this shift — estimated at over $300 million in indirect savings over five years — come from delivering the same clinical value at a fraction of the cost of hospital-based provision.

Hospital-Level Recommendations: Repurposing or Closure

At the individual hospital level, Oliver Wyman provided the most politically sensitive recommendations in the report: several hospitals are at risk of closing their inpatient beds and should consider repurposing their facilities and clinical staff through one of several designated models. The three primary repurposing options are the Rural Emergency Hospital designation (providing 24/7 emergency services and observation without inpatient beds), the Community Ambulatory Care Center (outpatient and primary care services without inpatient capacity), and the Care at Home support program (community-based services without a hospital facility).

Oliver Wyman was explicit that these recommendations are not about abandoning communities — they are about matching the level and configuration of facility to actual community health need and financial sustainability. A community that cannot sustain a full inpatient hospital but has reliable access to a Regional Specialty Center via improved EMS and transportation is better served than one that maintains an understaffed, financially distressed inpatient facility that cannot reliably staff its emergency department or maintain physician expertise across specialties.

The report also identified specific operational improvements for hospitals that will remain inpatient facilities: UVMMC was specifically called out to examine its overhead and administrative costs, particularly the proportion of providers supporting non-patient-care activities. Reducing exceedingly high administrative costs at some hospitals was identified as one of the three primary sources of direct savings — contributing to the more than $100 million in direct savings projected from administrative consolidation and shared services.

Vermont in Practice: The $400M Transformation Savings Estimate

Oliver Wyman projected that full implementation of the hospital transformation recommendations would yield more than $400 million in direct savings over five years — independent of savings from payment reform activities like AHEAD. The savings sources break down as follows:

Sources of direct savings (more than $100M):

- Close inpatient facilities with unsustainable financials, whose deficits would otherwise require escalating cash infusions
- Reduce hospital administrative costs, which are exceedingly high at some facilities and have been driving budget increases system-wide
- Reduce costs through synergies — shared services and staff across individual hospitals

Sources of indirect system savings (more than $300M):

- Deliver same procedures in ambulatory surgical units and outpatient settings, where low-risk procedures can be performed at substantially lower cost
- Expand access to primary care settings to manage patient needs, reducing preventable emergency department visits
- Address social determinants of health, reducing inpatient stays and ED visits that are avoidable if housing and transportation needs are met upstream

These savings can be reallocated to improve Vermont's healthcare system outside of hospital-based care: investing in community-based care (primary care, home health), investing in social needs programs (housing, mental health), and bending the trend of rising health insurance premiums.

The $400M figure is significant in context: Vermont's Rural Health Transformation Program award of $195M over five years represents less than half of the projected savings from structural transformation alone. The capital investment is smaller than the structural problem it is meant to help solve — which is why AHS has been clear that the RHT funding is a tool for executing reform, not a substitute for reform.

Act 68 (2025): The Operational Mandate

Signed by Governor Scott on June 12, 2025, Act 68 is the legislative operationalization of the Oliver Wyman diagnosis. The alignment between Oliver Wyman's priority recommendations and Act 68's operative provisions is direct and traceable: Oliver Wyman recommended beginning movement to reference-based pricing at 200% of Medicare or less; Act 68 mandated it by hospital fiscal year 2027\. Oliver Wyman recommended moving to global budgets when conditions for success are met; Act 68 required them for non-critical access hospitals by FY2028 and all hospitals by FY2030. Oliver Wyman recommended that GMCB add a Division of Planning and Effectiveness; Act 68 authorized three new permanent classified positions at GMCB. Oliver Wyman recommended reviewing AHS agency structure and program list; Act 68 directed AHS to develop a Statewide Health Care Delivery Strategic Plan by December 2028\.

The act's purpose statement is worth reading in full because it encodes the Oliver Wyman framework into law. The General Assembly declared the purpose of Act 68 to be transformation of and structural changes to Vermont's health care system, advancing five goals: improvements in health outcomes and reducing access disparities; an integrated care system with robust coordination and increased investment in primary care, home health, and long-term care; stabilizing providers and controlling costs beginning with reference-based pricing and continuing to global hospital budgets; evaluating progress through standardized accountability metrics; and establishing a system that attracts and retains high-quality health care professionals. Every one of these goals is a direct response to a finding in the Oliver Wyman report.

The Reference-Based Pricing Architecture

Act 68's RBP provisions represent a significant departure from the voluntary, incentive-based payment reform approach that dominated American health policy for the previous fifteen years. The act does not invite hospitals to experiment with alternative pricing — it directs GMCB to establish, by rule, maximum amounts that hospitals shall accept as payment in full, not later than hospital fiscal year 2027\. The language is mandatory and statewide.

The underlying price problem that RBP addresses is more severe than most national discussions of hospital pricing acknowledge. GMCB's February 2026 report documented that Vermont hospitals charge commercial payers approximately 250-300% of Medicare rates on average — but this average conceals extraordinary variation. Outpatient imaging at some Vermont hospitals reaches 715-944% of Medicare rates. Inpatient mental health services at the same hospitals range from 123% to 409% of Medicare. UVMMC outpatient charges specifically averaged 417% of Medicare rates in 2022 — among the highest in the nation. Hospitals are not clustered near a rational break-even point of 136% of Medicare; they are distributed across a range from roughly profitable to wildly extractive, with no relationship between price and quality.

The design principles embedded in Act 68's RBP statute reflect the cross-state evidence base. Prices must be based on a percentage of Medicare reimbursement — establishing a transparent, nationally comparable anchor. Consideration must be given to each hospital's specific community context: payer mix, labor costs, social risk factors, and role in Vermont's system. And hospitals are explicitly prohibited from charging patients or insurers more than the established reference-based price, closing billing workarounds that undermined earlier price transparency experiments.

State / Program	Key design features and outcomes
Vermont Act 68	Mandatory statewide maximum allowable payments through GMCB rate-setting authority. Methodology set by rule FY2027, effective FY2028. RBP then reviewed annually as part of hospital budget process.
Oregon (2019)	State employee plan only. 200% in-network / 185% out-of-network cap. Exemptions for rural and critical access hospitals. Savings exceeded $107.5M in first 27 months. All 24 affected hospitals remained in-network.
Montana (2016)	State employee plan. Direct contracting at percentage of Medicare. Initial $48M in savings but politically fragile without statutory mandate — illustrating Vermont's choice to legislate rather than demonstrate.
Washington (2021)	Public option ('Cascade Care'). Hospital reimbursement capped at 160% of Medicare for participating networks. Voluntary participation, expanding to state employee plans.
Maryland (2014–present)	All-payer system with fixed annual revenue for hospitals based on standardized prices and volume expectations. Includes Medicare waiver. Strong incentives for population health management. The closest model to Vermont's Act 68 trajectory.

The Global Budget Mandate

Act 68 requires GMCB to establish global hospital budgets for non-critical access hospitals by hospital fiscal year 2028, and for all Vermont hospitals by 2030\. This statutory obligation makes Vermont the first U.S. state outside Maryland on a legislatively mandated path to a comprehensive all-payer global budget system.

Understanding why Vermont needs global budgets in addition to reference-based pricing requires understanding the balloon squeeze problem that Oliver Wyman identified. Reference-based pricing controls unit prices but does not control volume. A hospital facing lower per-service revenue can compensate by delivering more services — scheduling more imaging, performing more procedures, admitting patients who could be managed in outpatient settings. Total hospital spending equals price times volume; controlling only price leaves volume as a release valve. Global budgets close this loophole by capping total hospital revenue regardless of service volume, fundamentally changing the hospital's financial incentive from maximizing throughput to maximizing population health per dollar of revenue.

Vermont has attempted global budget design four times in the past decade: the GMCB net patient revenue cap in 2012, the Rutland Regional pilot proposed in 2014 (never implemented), the Vermont All-Payer ACO Model from 2017 to 2025, and the current Act 167/Act 68 mandate. Each prior attempt foundered on the same obstacle: voluntary participation. When payers can opt out of a global budget arrangement, the highest-cost, most commercially valuable services migrate out of the capped system, leaving lower-reimbursing services behind and making the global budget financially unworkable. Act 68 eliminates the voluntary option. This is the critical design difference between Vermont's current trajectory and everything that preceded it.

The Statewide Health Care Delivery Strategic Plan and AHS Restructuring

The most consequential long-term provision of Act 68 is the requirement that AHS develop a Statewide Health Care Delivery Strategic Plan, to be delivered to the legislature by December 2028 and updated every three years thereafter. This plan is the statutory vehicle for answering the foundational question that underlies all of Vermont's reform work: what should Vermont's healthcare delivery system actually look like?

Act 68 established the Health Care Delivery Advisory Committee to support the plan's development, with responsibilities including setting affordability benchmarks, monitoring system performance, advising on the continuous evaluation process for the healthcare delivery system, and working with AHS to develop and maintain the strategic plan. The committee's mandate also requires incorporating recommendations from the Vermont Steering Committee for Comprehensive Primary Health Care — ensuring that primary care's central role in the redesigned system is structurally embedded in the planning process.

Oliver Wyman's state-level recommendations are the most detailed public blueprint available for what this strategic plan should address. The four domains Oliver Wyman identified — governance clarity, AHS structural alignment with HSAs, state-level SDOH investments, and administrative simplification — constitute the operational dimensions of the strategic plan. The AHS restructuring chapter later in this book develops a full six-pillar framework for how AHS should approach this planning mandate, drawing directly on both the Oliver Wyman recommendations and the Act 68 statutory requirements.

Vermont in Practice: The Reform Cascade Timeline — Statutory Deadlines

For healthcare leaders, policymakers, and organizations doing business with Vermont's system, the following statutory deadlines define the transformation calendar:

FY2026 (underway as of this writing):

- GMCB hospital budget guidance includes negative 1% commercial rate growth benchmark — the first negative benchmark in Vermont history
- AHS hospital transformation grants of $2M under Act 68; all 14 hospitals developing transformation plans with RHRC support
- Vermont Medicaid global budget for hospitals begins January 2026 under AHEAD Cohort 2 State Agreement
- Act 55 outpatient drug cap at 120% of ASP effective April 2025, generating estimated $135M in annual savings

FY2027 (effective October 2026):

- Reference-based pricing for hospital services implemented — maximum amounts hospitals can charge commercial payers, set by GMCB rule

- GMCB publishes hospital price transparency dashboard
- RBP vendor selection and methodology finalized; rulemaking completed

FY2028 (effective October 2027):

- Global hospital budgets established for non-critical access hospitals
- AHS Statewide Health Care Delivery Strategic Plan delivered to legislature by December 2028
- AHEAD Model Cohort 2 full operation; Medicare FFS hospital global budgets active
- CMMI transformation grants of $1.2M available through AHEAD Cooperative Agreement Year 3

FY2030 (effective October 2029):

- Global hospital budgets for all Vermont hospitals, including critical access hospitals

These deadlines are not aspirational. They are statutory obligations. Organizations planning capital investments, contract negotiations, workforce strategies, or technology deployments that interact with Vermont's hospital system must treat these dates as certainties, not contingencies.

The Rural Health Transformation Program: Capital for an Operational Strategy

In November 2025, AHS submitted Vermont's application to the federal Rural Health Transformation Program — a $50 billion national fund created by the One Big Beautiful Bill Act of 2025, with $10 billion available annually from 2026 through 2030\. Vermont was awarded $195 million in December 2025, nearly double what the state had expected to receive and among the highest per-capita awards nationally.

The RHT application is itself a teaching document for healthcare transformation practice, because it is structured entirely around the Act 167 and Act 68 goals. Vermont's vision, stated explicitly in the application, is to ensure that rural residents have access to the right care, at the right time, in the right place, at an affordable cost — a direct echo of the AHS health care reform vision statement. The application's three strategic goals map onto Oliver Wyman's three imperatives: strengthening hospital sustainability and primary care (Move all care possible out of hospitals); building workforce pipeline and retention capacity (Build housing and fix the conditions that drive workforce shortage); and leveraging innovative strategies to reduce costs (Pay with reference-based pricing and move to global budgets).

AHS Secretary Samuelson's framing of the award is the correct analytical frame: the $195 million is not Vermont's healthcare reform plan. It is capital to execute a plan that already exists. The funding is explicitly one-time money, and AHS has stated publicly that it must be treated as a supplement to ongoing reform work, not a lifeline. The distinction between investment capital and reform strategy is one of the most persistently confused in health policy. States that treat federal grants as reform strategies — spending the money on programs without addressing underlying structural problems — predictably return to crisis when the grant period ends. The Vermont approach, which explicitly aligns every RHT initiative to the statutory Act 167 and Act 68 goals, is a model of how capital investments should be positioned within a structural reform agenda.

The specific initiatives funded through Vermont's RHT award include telehealth expansion for specialty care access in rural hospitals, workforce recruitment and retention programs with five-year service obligations, hospital analytics infrastructure to support regional transformation planning, dialysis capacity development for nursing home residents, and CCBHC expansion to certify five additional community behavioral health clinic entities by July 2026\. Each of these investments addresses a specific node in Oliver Wyman's systems failure map — and each is designed to reduce ongoing structural costs rather than subsidize their continuation.

Cross-State Lessons: What Vermont's Model Generalizes

Vermont's reform architecture is instructive because it addresses structural problems present, in varying degrees of severity, across rural America. The analytical framework Oliver Wyman applied, and the legislative mechanisms Vermont enacted in response, generalize well to any state or regional system facing demographic aging, hospital financial fragility, workforce shortage, and the failure of voluntary payment reform to achieve systemic change.

Five generalizable lessons emerge from the Vermont case.

First, diagnosis precedes legislation. Vermont's Act 68 is as effective as it is because it was preceded by Act 167's comprehensive, public, community-grounded diagnostic process. Legislatures that skip the diagnostic phase and proceed directly to structural mandates produce policies that are technically sound but politically unsustainable. Vermont's sequence — diagnose publicly, build shared understanding, then mandate — is a process design that other states should study.

Second, voluntary reform achieves voluntary compliance. Vermont's decade-long experience with voluntary global budget arrangements — the All-Payer ACO Model, OneCare participation, VITL connectivity — produced partial, fragile outcomes precisely because voluntary participation allows the highest-cost actors to opt out. The Oliver Wyman report documented this plainly in its perception-versus-reality analysis. Act 68's mandatory architecture is a direct response to that experience.

Third, price control without volume control is insufficient. Reference-based pricing is necessary but not sufficient for controlling total hospital spending. The Act 68 sequence — RBP first, then global budgets — reflects the analytical understanding that price and volume must both be addressed. Oregon's RBP experience, Montana's experience, and Maryland's global budget model all confirm this.

Fourth, housing and transportation are healthcare infrastructure. Oliver Wyman's finding that housing shortage drives staffing crisis drives cost escalation is not a soft social observation — it is a hard systems analysis. Healthcare reform strategies that treat housing and transportation as separate social policy domains rather than healthcare infrastructure will fail to achieve the workforce stability and access improvements they require.

Fifth, the AHS restructuring is the strategic plan. Oliver Wyman's explicit recommendation that AHS align its sub-units with Hospital Service Areas, add a Division of Planning and Effectiveness, and restructure to better coordinate health and social service needs at the community level is the organizational design specification for what Act 68's Statewide Strategic Plan must produce. The book develops this specification fully in the AHS Restructuring chapter that follows.

Key Concepts Introduced or Expanded in This Chapter

Term	Definition
Reform cascade	A legislatively structured process in which each reform intervention creates the institutional capacity and political legitimacy for the next. Vermont's Act 167 → Act 51 → Act 68 sequence is the exemplar.
Platform for Change	Oliver Wyman's term for the five interconnected challenge domains — decreasing affordability, deteriorating sustainability, demographic trap, interconnected failure system, and equity gaps — that collectively make structural transformation unavoidable for Vermont.
The three imperatives	Oliver Wyman's highest-level summary of what Vermont must do: build housing and fix transportation; pay PPS hospitals with reference-based pricing and move to global budgets; move all care possible out of hospitals.
Center of Excellence (COE)	A hospital or facility designated to concentrate volume and expertise in a specific specialty, receiving referrals from other hospitals and ensuring that patient volumes are sufficient to maintain clinical quality. Vermont's regionalization

Term	Definition
	plan designates COEs across 24 specialty categories at 10 of 14 current Hospital Service Areas.
Balloon squeeze problem	The phenomenon in which controlling hospital prices without controlling volume allows hospitals to compensate for lower per-service revenue by delivering more services. Reference-based pricing controls price but not volume; global budgets are required to address both simultaneously.
Statewide Health Care Delivery Strategic Plan	The plan AHS is required to deliver to the Vermont legislature by December 2028 and update every three years, establishing the vision, goals, affordability benchmarks, and accountability metrics for Vermont's transformed health care delivery system. Established by Act 68\.
Hospital Service Area (HSA)	A geographic area defined around a hospital and the population it primarily serves. Vermont has 14 HSAs corresponding to its 14 hospitals. The HSA is the fundamental unit of analysis for Vermont's regionalization and transformation planning.
Rural Emergency Hospital (REH)	A federal designation allowing a hospital to provide 24/7 emergency services and observation care without maintaining inpatient beds — an option for Vermont hospitals that cannot sustain full inpatient capacity but need to maintain emergency access for their communities.

Sources: Act 167 of 2022 (Vermont); Act 51 of 2023 (Vermont); Act 68 of 2025 (Vermont); Oliver Wyman Healthcare and Life Sciences Group, Act 167 Community Engagement: Recommendations, September 2024 (revised October 2024 and January 2025); Green Mountain Care Board, Act 68 Update on Hospital Reference-Based Pricing and Global Budgets, February 17, 2026; Vermont Agency of Human Services, Vermont Rural Health Transformation Program Application, November 3, 2025; Vermont Agency of Human Services / Brendan Krause, Vermont's Health Care Reform Efforts, presented to Vermont House Health Care Committee, January 31, 2025; Green Mountain Care Board, 2024 Annual Report, January 15, 2025\.

Chapter 4: The Policy Pillar in Practice — CMMI Models, Waiver Strategy, and the Federal-State Policy Interface

The Policy Pillar in Practice — CMMI Models, Waiver Strategy, and the Federal-State Policy Interface

How Healthcare Leaders Navigate the Federal Policy Landscape: APM Mechanics, 1115 Waivers, CMMI Model Selection, and the Regulatory Strategy That Determines Whether Payment Reform Succeeds or Stalls

From Policy Architecture to Policy Practice

Chapter 4 established Vermont's legislative reform architecture — the Acts, the mandates, the timelines, and the analytical foundation of the Oliver Wyman diagnosis. This chapter turns from architecture to practice: how do healthcare organizations, state agencies, and policymakers actually navigate the federal policy landscape that shapes every transformation decision?

The federal policy environment is not a monolithic force that organizations respond to passively. It is a complex, evolving, and frequently internally contradictory set of programs, rules, incentives, and administrative interpretations that skilled policy practitioners can navigate strategically. Understanding the CMMI model landscape, the mechanics of Medicaid section 1115 waivers, the structure of APM contracts, and the federal-state relationship that determines whether state-level reform efforts are supported or blocked by federal requirements is not academic knowledge — it is operational necessity for any organization or state agency working on healthcare transformation in 2026\.

The CMMI Landscape — What Exists, What Works, and What Has Been Discontinued

The Center for Medicare and Medicaid Innovation has been the primary federal vehicle for payment reform experimentation since its creation under the ACA. Over fifteen years, CMMI has launched more than fifty models, invested billions of dollars, and engaged thousands of healthcare organizations across the country. The results have been mixed in ways that carry specific lessons for organizations designing their own value-based care strategies.

The CMMI Track Record — An Honest Assessment

A 2023 independent evaluation of CMMI models found that the vast majority had not achieved statistically significant reductions in Medicare spending or improvements in quality. Most models were discontinued. The models that showed the most promise — ACO REACH (formerly Direct Contracting), AHEAD, the Making Care Primary model — share a set of characteristics that distinguish successful from unsuccessful CMMI designs.

Characteristic	What it means	Models with this characteristic	Models without it
Mandatory or near-mandatory participation	When high-cost, high-volume providers cannot opt out, the model's financial logic applies to the full population. Voluntary models allow opt-out by the actors who benefit most from the status quo.	AHEAD (for participating states, all hospitals must participate). Maryland All-Payer Model (mandatory for all Maryland hospitals). Vermont Act 68 (mandatory for all Vermont hospitals).	Most CMMI ACO models. MSSP Track 1\. Bundled Payments for Care Improvement (voluntary). Most demonstration projects.
Two-sided risk that is real	Financial downside for underperformance is sufficient to change organizational behavior. One-sided shared savings with minimal upside and no downside does not change hospital or health system decision-making at scale.	ACO REACH (mandatory downside risk). Global budgets (hospitals bear population health risk directly). AHEAD (state-level accountability with meaningful consequences).	MSSP Track 1 (upside only for most participants). Most early CMMI bundled payment models with modest downside. Primary Care First (limited financial exposure).
Adequate implementation support	Organizations need technical assistance, analytical tools, and sufficient runway to redesign care delivery before being held financially accountable for outcomes they cannot yet control.	AHEAD (multi-year pre-performance period with substantial cooperative agreement funds). Vermont's RHRC engagement. Making Care Primary (practice transformation support).	Many early CMMI models that launched with insufficient operational support and were discontinued when providers could not implement effectively.
Alignment with state regulatory environment	Federal models that conflict with or duplicate state regulatory structures create compliance complexity without adding value. Models designed in partnership with state agencies produce better results.	AHEAD (designed explicitly as federal-state partnership; requires state agreement and state regulatory alignment). Vermont-specific APM (state-federal co-design). Maryland TCOC (built on Maryland's existing HSCRC regulatory authority).	Federal models imposed on states without coordination with state Medicaid agencies, insurance regulators, or hospital budget review processes.

Figure 4.1 — CMMI model success factors: what distinguishes models that produce results from those that don't. Source: the framework analysis of CMMI evaluation literature; CMS model documentation.

The Current Active CMMI Model Landscape (2026)

As of 2026, the CMMI model portfolio has been significantly rationalized from its peak breadth under prior administrations. The models with the highest relevance to Vermont-style transformation are:

Model	Key features and relevance
AHEAD (Achieving Healthcare Efficiency through Accountable Design)	Six-state voluntary total cost of care model. Vermont in Cohort 2 (begins January 2027). Hospital global budgets for Medicare FFS. Enhanced primary care payments. $150M/year additional Medicare funds for Vermont. The most relevant model for state-level transformation.
ACO REACH (Realizing Equity, Access, and Community Health)	Medicare FFS ACO model with mandatory two-sided risk. Full-risk capitation option. Equity focus including SDOH requirements. For organizations managing attributed Medicare populations outside of a state-level model, ACO REACH is the highest-accountability current federal ACO option.
Making Care Primary (MCP)	Multi-payer primary care transformation model in 8-state initial cohort. 10-year model. Three-stage progression from basic to advanced primary care capabilities. States not participating in AHEAD may find MCP a viable primary care transformation vehicle.
BALANCE Model	GLP-1 obesity drug coverage for Medicare and Medicaid enrollees at negotiated prices. Voluntary for manufacturers, states, and plans. Medicaid launch May 2026; Medicare Part D January 2027\. Critical equity and chronic disease management model.
Bundled Payments for Care Improvement Advanced (BPCI-A)	Episode-based payment for acute and post-acute care episodes. Voluntary. For health systems and post-acute providers, BPCI-A remains a viable two-sided risk VBC entry point for specific service lines (joint replacement, cardiac, etc.).

Section 1115 Waivers — The State's Primary Federal Policy Tool

Section 1115 of the Social Security Act gives CMS authority to waive certain Medicaid requirements and provide federal matching funds for experimental programs that promote program objectives. For states attempting comprehensive healthcare reform, the 1115 waiver is the most powerful federal policy tool available — enabling states to design Medicaid programs that would not be permitted under standard federal rules, to test new benefits and delivery models, and to demonstrate approaches that, if successful, can become national policy.

What 1115 Waivers Can and Cannot Do

Understanding the scope and limits of 1115 waiver authority is essential for any state or organization involved in Medicaid reform. The following framework organizes the most common 1115 waiver applications:

1115 Waiver Authorities: What States Can Accomplish

Coverage expansions beyond standard Medicaid categories — including populations not otherwise eligible, non-traditional benefit structures, and premium assistance programs that allow states to use Medicaid funds toward employer-sponsored insurance.

- Vermont's Global Commitment to Health demonstration extends coverage and benefits beyond standard Medicaid state plan requirements, enabling HRSN (health-related social needs) services, expanded developmental disability benefits, and substance use disorder treatment models not available under the standard state plan.

Delivery system and payment reform — enabling states to implement global budgets, ACO arrangements, and alternative payment models for Medicaid that are not permitted under standard Medicaid FFS or managed care rules.

- Vermont's alignment of Medicaid global budgets with AHEAD is executed through the Global Commitment to Health demonstration, which provides CMS authority to implement the Medicaid component of Vermont's hospital global budget design.

SDOH investments — since 2021, CMS has permitted states to use 1115 waiver funds for health-related social needs services, including housing supports, food assistance, and transportation, for Medicaid enrollees at risk of institutionalization or with complex social needs.

- Vermont's recent 1115 amendment approval authorizes HRSN services — enabling Blueprint's universal SDOH screening to connect to Medicaid-funded services, not just community resources.

Work requirement programs — subject to ongoing legal and policy uncertainty. H.R. 1 (2025) explicitly authorizes work requirements for Medicaid expansion enrollees, resolving the prior legal uncertainty in favor of state authority. Vermont has not implemented work requirements and has not signaled intent to do so.

The Vermont 1115 Waiver Portfolio

Vermont operates several 1115 demonstrations that together constitute its Medicaid transformation infrastructure. The Global Commitment to Health demonstration — Vermont's primary 1115 waiver — was originally approved in 2005 and has been renewed and amended repeatedly. The current authorization runs through December 2027, with the AHEAD alignment provisions approved in 2024\. Vermont's AHEAD Medicaid global budgets for hospitals, beginning January 2026, operate under this demonstration authority.

Vermont's 1115 portfolio illustrates a principle worth stating explicitly: 1115 waivers are negotiated instruments. They are not applications that CMS approves or denies based on a checklist; they are the product of sustained federal-state negotiation in which the state advocates for the policy design it wants and CMS negotiates conditions, monitoring requirements, and budget neutrality standards that protect the federal government's financial interests. Vermont's ability to implement its current AHEAD-aligned Medicaid policy is the direct product of years of negotiation that produced a federal-state agreement on methodology, accountability, and withdrawal conditions. States that engage with CMS as partners in designing innovative demonstrations get better waiver terms than states that submit standard applications.

Budget Neutrality — The Binding Constraint

Every 1115 waiver is subject to a budget neutrality requirement: the federal government will not spend more under the demonstration than it would have spent under the standard program. Budget neutrality is calculated against a 'without-waiver' baseline — what Medicaid spending would have been if the state had not implemented the demonstration — and must be maintained over the demonstration period.

Budget neutrality is simultaneously the most important financial discipline in 1115 waiver design and the most technically complex. The without-waiver baseline is inherently uncertain — it requires projecting what would have happened counterfactually. States and CMS frequently negotiate over baseline methodology. Vermont's experience with AHEAD budget neutrality negotiations — where the methodology for calculating the Medicare global budget baseline was a specific condition of the state's AHEAD participation — illustrates that these technical negotiations have major financial consequences. Getting the baseline right (or wrong) can determine whether a waiver is financially sustainable for the state or produces unexpected federal financial pressure.

Prior Authorization Reform — The Administrative Pillar of Policy Strategy

Prior authorization — the requirement that providers obtain health plan approval before delivering certain services — is one of the most significant drivers of administrative burden, care delay, and provider frustration in American healthcare. The average physician practice spends 14.9 hours per week on prior authorization tasks. Prior authorization denials delay care for 33% of patients and cause some to abandon treatment entirely. The administrative cost of prior authorization nationally is estimated in the tens of billions of dollars annually.

Prior authorization reform is a Policy pillar issue because it requires either federal rulemaking or state legislation to change meaningfully. Individual payers reforming their own prior authorization processes voluntarily have limited impact; the burden arises from the aggregate of PA requirements across dozens of payers, each with different criteria, forms, and approval timelines.

Federal Prior Authorization Reform: What H.R. 1 and CMS Rules Require

The Improving Seniors' Timely Access to Care Act, passed in 2022, required Medicare Advantage plans to implement electronic prior authorization, transparency reporting, and expedited review timelines. CMS implemented rules in 2024 requiring payers to respond to expedited PA requests within 72 hours and standard PA requests within 7 days — a significant improvement from previous norms where standard reviews could take weeks.

H.R. 1 (2025) included additional prior authorization reform provisions: public reporting of PA denial rates by service type, required disclosure of clinical criteria used in PA decisions, and stronger external appeal rights for denied requests. These are incremental improvements, not transformational changes. Full elimination of PA for evidence-based, guideline-concordant care — the policy that would most reduce administrative burden — remains aspirational.

Vermont's Prior Authorization Reform — Act 167's Provisions

Act 167 explicitly included prior authorization reform as one of its covered domains, directing AHS to develop simplified prior authorization processes and directing GMCB to report on prior authorization burden and denial rates. Vermont's Health Care Advocate has documented PA denial rates and challenged insurance rate filings that include excessive PA complexity costs. Act 68 builds on this by requiring insurers to demonstrate that administrative cost structures, including PA programs, are consistent with medical loss ratio requirements.

Oliver Wyman's community engagement findings are explicit about the real-world impact: providers described the CON process and prior authorization as major barriers to efficient care delivery, with cost-effective contracts unable to be secured in time due to administrative friction. Vermont's transformation agenda treats administrative simplification — including prior authorization reform — as an Operations pillar requirement that enables every other transformation initiative.

The State-Federal Policy Interface — Navigating the Relationship

For state health agencies and the organizations that work with them, the federal-state policy relationship is simultaneously a resource and a constraint. Federal funding through Medicaid, CMMI models, HRSA grants, and the RHT Program provides the capital that makes transformation possible. Federal requirements — budget neutrality, CMMI model conditions, Medicaid state plan requirements — constrain the policy designs states can pursue. Federal policy uncertainty — created by administrative transitions, legislative changes like H.R. 1, and rulemaking processes that can reverse course — creates planning challenges for states with multi-year transformation timelines.

Managing Federal Policy Risk — Vermont's Approach

Vermont's AHEAD State Agreement exemplifies sophisticated federal policy risk management. The agreement includes specific conditions under which Vermont can unilaterally withdraw from AHEAD — before launch, with 30-day notice; after launch, with 6-month notice. The agreement includes conditions that protect GMCB's regulatory independence. It includes explicit provisions about methodology acceptance and dispute resolution. Vermont negotiated these withdrawal rights precisely because a nine-year commitment to a federal model involves significant policy risk — the model could change under a future administration, the methodology could produce unexpected financial outcomes, or federal policy changes could make AHEAD participation financially unsustainable.

Vermont's approach — engage the federal government as a partner, negotiate specific protections against adverse policy changes, maintain clear withdrawal rights, and design state-level statutory authority (Act 68\) that does not depend on federal model continuation — is the right model for any state undertaking long-term federal-state policy commitments. States that design transformation strategies that are entirely dependent on continued federal program participation are strategically exposed; states that use federal models to accelerate and fund state-designed transformation that has independent statutory authority are more resilient.

The Policy Strategy Framework for Healthcare Organizations

For healthcare organizations navigating the policy environment — hospitals, health systems, payers, physician groups — the Policy pillar generates three distinct strategic functions: regulatory compliance (understanding and meeting mandatory requirements), opportunity identification (finding federal and state programs that can fund or accelerate transformation goals), and advocacy (shaping policy in directions favorable to the organization's mission and the populations it serves).

Function	Key activities	Vermont-specific examples	the framework advisory service
Regulatory compliance	Monitor rulemaking; assess financial and operational impact of new requirements; develop compliance roadmaps; manage comment letter processes	Act 68 RBP compliance planning for FY2027; AHEAD hospital global budget participation; H.R. 1 Medicaid work requirement implementation timeline	Policy Strategy service line: Phase 1 Landscape and Exposure Assessment
Opportunity identification	Track CMMI model launches and eligibility; identify state waiver opportunities; evaluate RHT Program grant eligibility; assess AHEAD primary care participation options	Vermont hospitals: Act 68 transformation grants ($2M); AHEAD EAST Fund; RHT Program subrecipient grants; BALANCE Model GLP-1 coverage	Policy Strategy service line: federal program opportunity assessment
Policy advocacy	Participate in public comment on proposed rules; engage state legislature on transformation legislation; provide testimony on hospital financial sustainability; participate in GMCB public meetings	Vermont hospital leaders: engagement with AHS transformation planning; community input on COE designations; participation in Health Care Delivery Advisory Committee public comment	Policy Strategy service line: Phase 3 Implementation Support — legislative testimony and comment letter drafting

Figure 4.2 — Policy strategy functions for healthcare organizations. Source: Transformation Advisory framework.

The Medicaid Policy Landscape — What H.R. 1 Changes and What It Does Not

The 2025 reconciliation law has changed the Medicaid landscape in ways that every healthcare organization and state agency must understand. The following framework distinguishes between provisions already in effect, provisions taking effect in late 2026 and 2027, and provisions whose implementation timeline remains uncertain.

H.R. 1 Medicaid provision	When it takes effect	Financial impact	Vermont-specific implications
Provider tax restrictions: ban on new or higher provider taxes in expansion states; phase-down of existing taxes	In effect for new taxes; phase-down beginning 2027	Reduces states' ability to finance their Medicaid matching share; intensifies pressure on state Medicaid budgets	Vermont's financing mechanisms will need review; state budget implications require modeling through 2031
State-directed payment caps at 100% of Medicare (expansion states)	2027	Reduces enhanced Medicaid payments that many states use to supplement provider rates above standard Medicaid levels; significant for safety-net hospitals	Vermont hospitals receiving state-directed supplemental payments face revenue reduction; GMCB global budget design must account for this
Work and community engagement requirements (80 hours/month)	Late 2026 (states must implement by December 31, 2026\)	10 million projected uninsured nationally by 2034; variable state impact depending on implementation; administrative burden on states and enrollees	Vermont has not expressed intent to implement work requirements; federal law now clearly authorizes them. Coverage implications for Vermont's expansion population require monitoring.

H.R. 1 Medicaid provision	When it takes effect	Financial impact	Vermont-specific implications
Eligibility redetermination every 6 months (vs. annual)	2026	Increases administrative burden; produces coverage churn as enrollees who remain eligible lose coverage due to administrative failures; disproportionate impact on working families	Vermont DVHA will face increased redetermination workload; outreach to ensure eligible Vermonters maintain coverage is an equity priority
Medicaid reimbursement reductions (DSH cuts, payment rate changes)	2026-2031 (phased)	$911B over 10 years nationally; $137B in rural areas; hospitals will experience revenue reductions	All 14 Vermont hospitals face reduced Medicaid revenue; most severe impact on CAHs with high Medicaid payer mix; integrated into hospital financial sustainability modeling

Figure 4.3 — H.R. 1 Medicaid provisions: timeline, financial impact, and Vermont implications. Sources: KFF analysis; CBO estimates; Pew Charitable Trusts (January 2026).

For Vermont specifically, the Medicaid revenue reductions create a financial stress that compounds the hospital financial crisis Oliver Wyman documented before H.R. 1 passed. Vermont's hospitals were already projecting 5-year cumulative deficits of $700 million to $2.4 billion under the pre-H.R. 1 baseline. The additional Medicaid revenue reductions accelerate the financial timeline for hospitals that were already financially distressed, and reduce the financial cushion for hospitals that were marginally sustainable. This is the fiscal environment in which Vermont's transformation must succeed — which makes the urgency of the transformation not just a policy preference but a fiscal survival requirement.

Policy Strategy for Healthcare Leaders — The Practitioner Framework

Policy strategy for healthcare leaders is not primarily about lobbying or advocacy — though those functions matter. It is primarily about anticipation: understanding what the policy environment will require of organizations 18-36 months from now, positioning the organization to comply at manageable cost, and identifying the policy opportunities that can be converted into strategic advantage.

The Policy Monitoring Framework

Every healthcare organization needs a policy monitoring infrastructure proportionate to its size and regulatory exposure. The minimum viable policy monitoring framework includes four functions:

- **Federal rulemaking surveillance: Tracking proposed rules from CMS, ONC, and other relevant agencies through the Federal Register notice and comment process. Final rules typically take effect 60 days after publication, but proposed rules give 60-90 days of advance notice that organizations can use for impact assessment and comment letter preparation.**
- **State legislative monitoring: Tracking legislation that affects the organization's operating environment through the state legislative session. Vermont's pattern of annual healthcare legislation — each session producing significant new requirements or opportunities — means that organizations not engaged in the legislative process are consistently surprised by its outputs.**
- **CMMI model pipeline monitoring: Tracking CMMI model announcements, application deadlines, and eligibility criteria. New CMMI models typically have narrow application windows; organizations that are not monitoring for them miss the opportunity to participate on favorable terms.**
- **Regulatory intelligence: Monitoring GMCB decisions, CMS guidance documents, and state agency program updates that affect payment and operating requirements. Vermont's GMCB publishes hospital budget guidance, RBP methodology updates, and program decisions that directly affect every Vermont hospital's financial planning.**

The Policy Impact Assessment Framework

When a new policy is identified — a proposed rule, a new CMMI model, a state legislative development — the organization needs a structured impact assessment process. The Policy Strategy service line uses a four-phase engagement structure (described in Chapter 16), but the core analytical logic can be applied internally:

- Exposure mapping: What specifically does this policy change require of our organization? What are the financial, operational, and compliance implications? What is the timeline?
- Scenario analysis: Under what range of outcomes (final rule differs from proposed rule, implementation delayed, model parameters change) does our strategic position change?
- Response strategy: What is the optimal response — compliance roadmap, comment letter participation, CMMI model application, advocacy engagement?
- Stakeholder communication: Who needs to know about this policy change, and what decisions does it require from the board, executive team, clinical leadership, and financial team?

Key Concepts

Term	Definition
Section 1115 waiver	Authority under the Social Security Act allowing CMS to waive standard Medicaid requirements and approve experimental programs that promote program objectives. Vermont's Global Commitment to Health demonstration is its primary 1115 vehicle.
Budget neutrality	The federal requirement that 1115 waiver spending not exceed what Medicaid spending would have been without the waiver, calculated against a negotiated without-waiver baseline.
CMMI (Center for Medicare and Medicaid Innovation)	Federal entity created by the ACA to test new payment and delivery models. Has launched 50+ models since 2010; AHEAD is the most relevant current model for state-level transformation.
Two-sided risk	A payment arrangement in which the organization shares in both savings (upside) and losses (downside) relative to a financial target. Mandatory downside risk changes organizational behavior more effectively than one-sided shared savings.
Prior authorization	Health plan requirement for pre-approval before delivering specified services. Oliver Wyman documented PA complexity as a major administrative burden for Vermont providers; Act 167 and Act 68 both address PA simplification.
Making Care Primary (MCP)	CMMI's 8-state multi-payer primary care transformation model. A potential vehicle for states not participating in AHEAD seeking federal support for primary care investment.
Federal Register	The official journal of the U.S. federal government, where CMS and other agencies publish proposed rules (open for public comment) and final rules (legally binding). Essential monitoring source for healthcare policy practitioners.

Sources: CMS CMMI model documentation; CMS 1115 waiver framework; KFF Medicaid policy analyses; H.R. 1 One Big Beautiful Bill Act (July 2025); Vermont Global Commitment to Health demonstration (CMS approval January 2025); Act 167 of 2022; Act 68 of 2025; AHEAD

State Agreement (January 2025); American Medical Association prior authorization burden survey; Georgetown Center for Children and Families.

Chapter 5: The Economics Pillar — Global Budgets, Reference-Based Pricing, and the Financial Architecture of Reform

The Economics Pillar — Global Budgets, Reference-Based Pricing, and the Financial Architecture of Reform

Global Budgets, Reference-Based Pricing, and the Financial Architecture of System Change

Sources: Oliver Wyman Act 167 Community Engagement: Recommendations (2024); Act 68 of 2025 (Vermont); GMCB Act 68 RBP/Global Budget Update, February 2026; AHEAD State Agreement (January 2025); Maryland All-Payer Model data (HSCRC, CMS, Commonwealth Fund, Milbank Fund); Vermont RHT Program Application (November 2025); AHS Legislative Presentation (January 2025).

The Economics Pillar: Why Payment Reform Is the Master Variable

Of the six pillars in the six-pillar framework, the Economics pillar is the one that determines whether the other five actually produce results. You can design perfect care protocols, build excellent data infrastructure, and train a superb clinical workforce — but if the payment system rewards volume over value, every other improvement will be absorbed by the financial logic of fee-for-service. Payment reform is not one element of healthcare transformation. It is the precondition on which all other transformation depends.

This chapter develops the economics of healthcare transformation through three lenses that are analytically inseparable: the mechanics of how hospitals are actually paid today and why that model is failing (with Vermont as the case study), the technical architecture of the alternative payment mechanisms designed to replace it (reference-based pricing and global budgets), and the financial modeling discipline that organizations need to navigate the transition. Vermont's reform journey — from the Act 167 diagnostic through the Act 68 mandate through the AHEAD Model state agreement — is the most complete real-world laboratory for these economic questions currently operating in the United States.

The Fee-for-Service Trap — Why the Current System Fails

Fee-for-service payment is the foundational economic structure of American healthcare: providers are paid for each service they deliver, with payment amounts set by negotiated contracts or government rate schedules. The system's economic logic is simple and, from a provider's perspective, entirely rational: more services delivered means more revenue. The problem is that this rationality at the individual transaction level produces irrational outcomes at the system level.

The Volume Incentive Problem

Under fee-for-service, a hospital that prevents a hospitalization loses revenue. A hospital that prevents a readmission loses revenue. A hospital that successfully manages a patient's chronic condition in primary care so they never need the emergency department loses revenue. Every improvement in population health, every success in care coordination, every dollar spent on preventing the expensive acute event — each of these reduces the hospital's income under the fee-for-service model.

This is not a criticism of individual hospital managers or clinicians, who are generally motivated by genuine commitment to patient care. It is an observation about the structural incentive embedded in the payment architecture. When the financial incentive and the clinical incentive point in opposite directions, the financial incentive wins — not always, not in every case, but systematically and at scale. This is why fifteen years of voluntary value-based care programs have moved the needle on payment reform without fundamentally changing the volume-driven behavior of the hospital sector.

"The concept of the global budget is the fundamental piece that flipped the incentives for hospitals in the right direction. It was such a monumental change, a sea change, in the way that hospitals thought about raising revenue."

— Maryland state regulator, quoted in JAMA Network Open study of the Maryland All-Payer Model

The Price Extraction Problem

Fee-for-service payment creates a second structural problem distinct from the volume incentive: it enables hospitals with market power to charge commercial payers prices far above their actual cost of care. In competitive markets, prices are disciplined by competition. In highly concentrated hospital markets — which describe the majority of U.S. hospital markets, and all of Vermont's — there is no effective competitive discipline on commercial prices.

Vermont's price data, documented in the GMCB's February 2026 report and confirmed by independent analyses, is among the most extreme in the country:

Average commercial-to-Medicare ratio, Vermont hospitals 250–300% vs. \~136% break-even point	**Outpatient imaging — worst case 944% of Medicare rate at one Vermont hospital**
UVMMC outpatient charges (2022) 417% *of Medicare — among highest nationally (RAND)*	**Potential savings at 200% Medicare (2018–2023) $400M** *VEHI \+ VSEA combined, per GMCB analysis*

Figure 5.1 — Vermont hospital commercial pricing vs. Medicare and break-even benchmarks. Sources: GMCB Act 68 RBP Update (February 2026); RAND Hospital Price Transparency Study.

The gap between the break-even point (\~136% of Medicare) and the actual commercial prices being charged (250–300% on average, with outliers reaching 944%) is not driven by higher quality, better outcomes, or greater complexity. Oliver Wyman's analysis found no relationship between price and quality across Vermont's hospital system. It is driven by market power — the ability of a dominant provider to demand above-cost prices from payers who have no viable alternative for their members.

The consequence for insurance premiums is direct and arithmetically straightforward. When a hospital charges 300% of Medicare for a service instead of 136%, and when commercial insurance covers the majority of that cost, premiums must rise to cover the difference. Vermont's premium trajectory — individual market average monthly premium from $456 in 2018 to $948 in 2024, a 108% increase in six years — tracks hospital charge growth with high fidelity. These are not coincidental trends. They are the mathematical output of a price structure that has no regulatory discipline.

Vermont average monthly silver marketplace premium (2018–2024)

2018	**456$**
2019	**474$**
2020	**468$**
2021	**580$**
2022	**730$**
2023	**875$**

2018	**456$**
2024	**948$**

Figure 5.2 — Vermont silver marketplace premium trajectory 2018–2024, showing 108% increase over six years. Source: Oliver Wyman Act 167 Report; GMCB analysis.

The Cross-Subsidy Collapse

The fee-for-service model sustains itself through cross-subsidy: commercial payers (who pay above cost) effectively subsidize Medicare and Medicaid patients (who pay below cost). This cross-subsidy is the financial foundation of every U.S. hospital that serves a mixed payer population — which is to say, every hospital.

Vermont's demographic trajectory, documented in detail by Oliver Wyman, is destroying the cross-subsidy model. As the 65+ population expands from 21.7% in 2020 toward 30%+ by 2040, the proportion of patients covered by below-cost Medicare and Medicaid grows. As the working-age population (20-64) declines from 58.8% toward 52%, the pool of commercially insured patients shrinks. The shrinking commercial base must bear a growing cross-subsidy burden — which forces premiums higher — which reduces commercial enrollment — which shrinks the commercial base further. This is not a gradual trend. It is a reinforcing spiral.

Oliver Wyman's financial projections capture where this spiral leads: under the conservative scenario, 13 of Vermont's 14 hospitals report operating losses by 2028, with a cumulative 5-year system deficit of $700 million to $2.4 billion depending on expense growth assumptions. The hospital system does not gradually lose efficiency under this trajectory. It collapses.

Reference-Based Pricing — Mechanics, Evidence, and Vermont's Design

Reference-based pricing (RBP) is the first operational tool in Vermont's economics reform sequence. It addresses the price extraction problem directly: rather than allowing hospitals and payers to negotiate prices in bilateral, opaque arrangements where hospital market power determines the outcome, RBP establishes a transparent external benchmark — typically a percentage of Medicare reimbursement — as the maximum a hospital may accept.

How RBP Works: The Mechanism

The core RBP mechanism is straightforward: the regulatory authority (in Vermont's case, GMCB) establishes, by rule, the maximum amounts that hospitals shall accept as payment in full for items and services delivered. These maximum amounts are calculated as a percentage of Medicare reimbursement for the same or similar service. The percentage is calibrated to allow hospitals to cover their actual costs (break-even at \~136% of Medicare) while eliminating the excessive markup that drives premium inflation.

Act 68's RBP architecture reflects four design principles that distinguish Vermont's approach from weaker price transparency experiments:

- **Mandatory, not voluntary — every hospital, every commercial payer, by FY2027**
- **Binding ceiling, not advisory guidance — hospitals cannot charge or collect more than the established rate**
- **Transparent benchmark — Medicare rates provide a nationally comparable, publicly available anchor**
- **Premium passthrough monitoring — GMCB and DFR must verify that price reductions translate to lower premiums**

GMCB's February 2026 report introduced one additional design refinement that reflects the Vermont-specific complexity of applying RBP to a highly varied 14-hospital system. Rather than applying a single statewide RBP multiplier, GMCB will consider each hospital's specific community context: payer mix, labor costs, social risk factors, and role in Vermont's system. A critical access hospital serving a predominantly Medicaid and Medicare rural population has different cost economics than a large PPS academic medical center. Vermont's RBP methodology will accommodate these differences while maintaining the essential principle that commercial prices may not exceed a defensible multiple of Medicare.

The Evidence Base: Cross-State Results

Vermont is not designing RBP in a vacuum. Three states have implemented meaningful price reference programs with documented results, providing the evidence base for Vermont's design choices.

State / Program	Design	Scope	Key Results
Oregon (2019)	200% of Medicare in-network; 185% out-of-network. CAH exemptions.	State employee plan (\~300,000 covered lives)	$107.5M saved in first 27 months (Health Affairs). All 24 affected hospitals remained in-network. No documented access harm.
Montana (2016)	Direct contracting at % of Medicare for state employee plan.	State employee plan	$48M initial savings. Politically fragile without statutory mandate — illustrates Vermont's correct choice to legislate rather than demonstrate.
Washington (2021)	'Cascade Care' public option. 160% of Medicare for participating networks.	Voluntary public option	Expanding to state employee plans. Growing commercial adoption.
Maryland (2014–present)	All-payer rate setting; hospitals receive fixed annual revenue (global budget). Medicare waiver.	All payers, all hospitals statewide	$1.4B in Medicare hospital savings to date. 7% reduction in hospital admissions. $800M reduction in hospital spending. Model extended and expanded.
Vermont Act 68 (FY2027)	Mandatory statewide RBP maximum set by GMCB rule. Initially commercial payers; excludes Medicare/Medicaid.	All Vermont hospitals, all commercial payers	Targeting implementation FY27; first negative commercial rate benchmark (-1%) in effect FY26 as transition step.

Figure 5.3 — Cross-state reference-based pricing and rate regulation: design and outcomes. Sources: GMCB Act 68 Update (February 2026); Health Affairs; Commonwealth Fund; CMS.

The Oregon results are the most directly applicable to Vermont's situation because Oregon implemented RBP for a state employee plan while Vermont is implementing it statewide. The critical finding — all 24 affected hospitals remained in-network — directly addresses the most frequently raised concern about RBP: that hospitals will walk away from the regulated rates and leave patients without in-network access. The Oregon evidence is unambiguous on this point: hospitals did not exit the market when rates were set at 200% of Medicare, because 200% of Medicare is still substantially above their actual costs.

What RBP Does — and What It Does Not Do

GMCB's February 2026 report was notably precise about the scope of RBP's impact. Understanding what RBP accomplishes, and what requires complementary mechanisms, is essential for policymakers and healthcare leaders designing transformation strategies.

RBP does:

- Reduce hospital leverage to demand excessive prices from commercial payers
- Narrow unjustified price variation across hospitals for the same service

- Protect patient affordability by establishing binding price ceilings
- Limit monopolistic pricing power in concentrated markets
- Incentivize operational efficiencies over price increases as the margin lever
- Serve as a necessary input for global budget design by establishing the price denominator

RBP does NOT:

- Control total hospital spending — only the price component, not volume
- Guarantee savings pass-through to consumers without explicit monitoring requirements
- Restructure incentives around prevention or population health
- Guarantee hospital efficiency, sustainability, or quality performance
- Solve the cross-subsidy collapse driven by demographic aging

This is why Act 68 sequences RBP first, global budgets second. RBP is a necessary but insufficient tool. Global budgets address what RBP leaves unresolved: total hospital spending, volume incentives, and population health management.

Vermont's Phased Implementation Timeline

GMCB is implementing RBP in two phases, reflecting both the regulatory complexity of establishing a statewide rate-setting methodology and the operational reality that hospitals need runway to adapt their financial models.

Phase	Timeline	Mechanism	Key Actions
Phase I — Differential Price Growth	FY26–FY27 (now underway)	Hospital Budget Guidance	FY27 guidance recommends \-1% commercial rate growth benchmark — first negative benchmark in Vermont history. Targets 'outlier services' with highest prices and volumes for initial reductions.
Phase II — Rate Setting (RBP)	FY27–FY28	GMCB Rule-Making	RFP for RBP vendor released December 2025\. Discovery and design August 2025–August 2026\. Policy and regulatory development January 2026–January 2027\. Final RBP methodology released March 2027 for FY28 hospital budget guidance.
Phase III — Global Budget Integration	FY28 and FY30	Statutory Mandate	Non-CAH global hospital budgets by FY28. All-hospital global budgets by FY30. RBP embedded as price component of global budget methodology.

Figure 5.4 — Vermont RBP and global budget implementation phases. Source: GMCB Act 68 Update, February 2026\.

The Phase I action — the negative 1% commercial rate benchmark for FY2027 — deserves particular attention because it is already producing results before formal RBP is in place. Combined with FY2026 measures including the Act 55 outpatient drug cap and hospital budget enforcement, GMCB reduced Vermont hospital commercial revenue by $230.65 million in FY2026: $104.3M from the Act 55 drug cap, $94.58M from hospital budget orders, and $31.76M from budget enforcement. This is the largest single-year price reduction in Vermont hospital history, accomplished through regulatory authority that already existed, signaling what becomes possible when RBP is formally implemented.

Hospital Global Budgets — Architecture, History, and Vermont's Mandate

A hospital global budget is a prospectively set annual revenue cap for a hospital, established before the year begins, based on population health needs and past performance rather than on the volume of services delivered during the year. Under a global budget, a hospital receives a fixed total payment regardless of how many patients it sees or how many services it provides. This single design choice — decoupling revenue from volume — fundamentally transforms the hospital's financial incentive structure.

The Economic Logic: Flipping the Incentive

Under fee-for-service, revenue \= price × volume. To increase revenue, a hospital must increase either the price per service or the number of services delivered. Under a global budget, revenue is fixed. The only way to improve financial performance is to reduce costs — which means managing population health better, preventing hospitalizations, reducing avoidable readmissions, improving efficiency, and delivering care in less expensive settings.

This is not a theoretical incentive. The Maryland experience, documented over a decade of all-payer global budget operation, demonstrates the behavioral change in concrete terms. As one Maryland state regulator described it: the global budget is the fundamental piece that flips the incentives for hospitals — hospitals had to transition from business strategies that chase volume to ones that focused on population health management. That transition does not happen overnight, but it does happen, and the documented savings are substantial.

The global budget also creates something fee-for-service never can: revenue predictability. Under FFS, a hospital's revenue depends on patient volume, acuity, service mix, and billing accuracy — all variable and partially outside the hospital's control. Under a global budget, the hospital knows its annual revenue before the year begins. This predictability enables the long-term capital planning, workforce investment, and care redesign that hospital transformation requires. For Vermont's financially distressed rural hospitals, revenue predictability is not a secondary benefit — it is a survival mechanism.

Five Design Dimensions Every Global Budget Must Resolve

Global budget design is technically complex, and the design choices are consequential. GMCB's February 2026 report identified five dimensions that any global budget methodology must address, along with the core tradeoff embedded in each. These dimensions apply equally to Vermont's GMCB-designed commercial budgets, the AHEAD Model Medicare budgets, and the Medicaid global budget already in operation.

Design dimension	The choice	Key tradeoff	Vermont's current direction
Scope	Which services are inside vs. outside the budget? (Capital, high-cost drugs, outpatient, physician services?)	Broader scope \= more comprehensive cost control. Narrower scope \= simpler to administer but leaves cost-shifting opportunities.	TBD through rulemaking. Maryland excludes physician office services and outpatient drugs. Vermont likely to follow similar structure initially.
Population adjustment	How does the budget account for patient demographics, complexity, and socioeconomic factors?	No adjustment rewards hospitals serving healthier populations. Over-adjustment reduces cost-control incentives.	GMCB considering payer mix, labor costs, social risk factors for each hospital. Critical for Vermont's CAHs vs. UVMMC.
Volume treatment	Does the budget adjust if patient volume changes significantly?	Fixed budget \= maximum cost-control incentive. Volume-adjusted budget \= lower risk for hospitals, less population health incentive.	Maryland preserved revenue initially when volume decreased to ease transition. Vermont likely to adopt similar glide path.

Design dimension	The choice	Key tradeoff	Vermont's current direction
Quality linkage	How are quality metrics integrated — penalty, incentive, or both?	Too small a linkage \= no behavioral change. Too large \= financial instability for hospitals still improving.	Act 68 requires standardized accountability metrics. GMCB quality programs (readmissions, HACs) will be embedded.
Flexibility mechanisms	Provisions for mid-year adjustments, carryover of savings, risk corridors?	More flexibility \= lower hospital risk, easier adoption. Less flexibility \= stronger cost-control signal.	GMCB authorized to identify conditions requiring modification or termination of RBP or budget parameters.

Figure 5.5 — Five dimensions of global budget design with Vermont's current direction. Source: GMCB Act 68 Update (February 2026); Act 68 of 2025\.

Vermont's Global Budget History: Four Attempts, One Lesson

Vermont has been attempting to implement hospital global budgets for over a decade. The history of those attempts is essential context for understanding why Act 68's mandatory approach represents a fundamental departure from everything that preceded it, and why the departure was necessary.

Approach	Year(s)	Structure	Outcome and lesson
GMCB Net Patient Revenue Cap	2012–present	Regulatory cap on net patient revenue each hospital may receive	Partial constraint. Controlled revenue growth within individual hospital budgets but did not align incentives across payers or address volume gaming.
Rutland Regional Medical Center Pilot	2014 (proposed)	Hospital payments based on historical revenue plus inflation, adjusted for demographics	Never implemented. Hospital opposition and implementation complexity prevented launch. Showed that hospital buy-in is necessary but not sufficient without statutory mandate.
Vermont All-Payer ACO Model	2017–2025	Voluntary commercial payer/provider participation in hospital global payments aligned across Medicare, Medicaid, and commercial based on historical spend	Voluntary participation meant highest-cost commercial payers opted out. OneCare Vermont could not compel participation. Model produced modest results but never achieved the all-payer alignment that makes global budgets effective. Expired end of 2025\.
Act 167 / Act 68 Mandate	2022–present	GMCB directed to design and implement mandatory global budgets: non-CAH hospitals by FY2028, all hospitals by FY2030	In progress. Removes the voluntary participation flaw that undermined all prior attempts.

Figure 5.6 — Vermont global budget history: four attempts and the central lesson. Sources: GMCB Act 68 Update (February 2026); Oliver Wyman Act 167 Report.

The central lesson of this history is unambiguous: voluntary models fail because the actors who benefit most from the status quo — the highest-priced hospitals and the commercial payers with the most leverage — can and do opt out. A global budget that is optional for the highest-price hospital in a market is not a global budget; it is a demonstration project. Act 68's statutory mandate eliminates the opt-out. This is the design difference that matters.

"*If the model is not adequately regulated and sufficiently resourced, it will fail dramatically and harm patients and harm our health system.*"

— Owen Foster, Chair of the Vermont Green Mountain Care Board, on signing the AHEAD State Agreement, January 2025

Maryland — A Decade of Global Budget Evidence

Maryland is the only U.S. state to have operated a mandatory all-payer hospital rate setting system with global budgets for more than a decade. Its experience is the closest available empirical evidence for what Vermont is attempting, and it deserves detailed examination — both for what it demonstrates is possible and for the limitations the evidence also reveals.

The Maryland Architecture

Maryland's Health Services Cost Review Commission (HSCRC) was established in 1972 with statutory authority to set hospital rates for all commercial payers — a unique regulatory authority that no other state possesses. The HSCRC received a Medicare demonstration waiver in 1977, enabling all-payer rate setting to include Medicare. The current Global Budget Revenue (GBR) methodology, implemented through a renegotiated CMS waiver in 2014, fixes each hospital's total annual revenue before the year begins, regardless of how many patients are served or services provided. Prices for individual services adjust dynamically during the year to meet the fixed revenue target.

The model's accountability structure has three components beyond the global budget itself: a Medicare Performance Adjustment (1% of Medicare revenues) tied to total cost of care outcomes; Quality Based Reimbursement (2% of all-payer revenue) linked to patient experience, safety, and outcomes; and incentives for reducing hospital-acquired conditions, potentially avoidable utilization, and readmissions. The budget constraint is non-negotiable; the quality components are calibrated to reward the population health management behavior the global budget is designed to incentivize.

The Results

The evidence from Maryland's global budget model is substantial and consistent across multiple independent evaluations:

Medicare hospital savings to date $1.4B vs. projected national trend (HSCRC)	**Reduction in hospital admissions 7% Medicare patients; avoidable admissions down \>6%**
Reduction in hospital spending $800M *Medicare Part A and B combined (CMS evaluation)*	**All-payer per capita growth cap 3.58%** *committed growth rate under Maryland model*

Figure 5.7 — Maryland All-Payer Global Budget Model results summary. Sources: HSCRC; CMS; Commonwealth Fund; Milbank Memorial Fund.

Maryland also reduced hospital spending for commercial beneficiaries, cut ED visits and inpatient admissions across all payer types, and significantly improved outcomes for dual-eligible (Medicare/Medicaid) patients — the highest-cost, highest-complexity population segment that Vermont also needs to serve better. The 14-day follow-up visit rate and 30-day unplanned readmission rate both improved for non-teaching hospitals.

The Limitations — What Maryland Teaches About What Global Budgets Cannot Do Alone

The Maryland evidence is not uniformly positive, and intellectual honesty requires presenting the limitations alongside the achievements. The Milbank Fund's evaluation found mixed results on quality metrics and no improvement in overall population health measures like smoking rates or obesity prevalence. The reason is analytically important: the global budget creates a powerful incentive to reduce hospital utilization, but it does not automatically create the community infrastructure needed to address the health determinants that drive that utilization.

When Maryland reduced avoidable hospital admissions, the reduction was concentrated among healthier patients — people who, with adequate primary care and social support, would not have needed hospitalization. The sickest patients continued arriving at emergency departments, and hospitals actually spent more time treating them. The lesson is that a global budget without commensurate investment in primary care, behavioral health, housing, and transportation does not improve population health — it just reduces hospital revenue while leaving the underlying drivers of high utilization intact.

Vermont has heard this lesson clearly. The AHEAD State Agreement explicitly links hospital global budgets with the Equity, Access, and Statewide Transformation (EAST) Fund — providing up to $150 million in additional Medicare funds annually starting in 2027 to invest in primary care, mental health, home health, and long-term care services. The Rural Health Transformation Program award of $195 million provides capital for workforce, telehealth, and community infrastructure. Vermont's global budget design is, from the beginning, paired with the community investment that Maryland added only after its first years of operation.

The AHEAD Model — Medicare's Entry into Vermont's Reform

The AHEAD (Achieving Healthcare Efficiency through Accountable Design) Model represents the federal government's partnership with states committed to genuine all-payer total cost of care transformation. Vermont signed its AHEAD State Agreement on January 17, 2025, committing to participate in Cohort 2 beginning January 1, 2027\. The agreement was a 3-1 vote by GMCB, with the dissenting member citing concerns about implementation complexity — a vote that reflects the genuine difficulty and ambition of what Vermont has committed to.

What AHEAD Actually Is

AHEAD is a voluntary state total cost of care model that aims to drive state-level healthcare transformation and multi-payer alignment. The model has three primary components that operate in concert:

- **Hospital Global Budgets — participating hospitals receive prospective, biweekly Medicare FFS payments instead of traditional claims-based payments, providing revenue predictability while creating population health incentives**
- **Primary Care AHEAD — voluntary program for primary care practices to receive enhanced prospective, risk-adjusted Medicare payments for coordinated care, with four payment pathway options including fully prospective models**
- **State accountability — Vermont commits to controlling total cost of care growth, increasing primary care investment as a percentage of total spending, and improving population health outcomes and equity metrics**

The AHEAD Financial Package for Vermont

The AHEAD State Agreement provides Vermont with a substantial financial resource package alongside its accountability commitments:

- Up to $150 million in additional Medicare funds annually starting in 2027, with inflation adjustments each year
- $1.2 million in federal transformation grants beginning Cooperative Agreement Year 3 (2027) for health system transformation
- Equity, Access, and Statewide Transformation (EAST) Fund for investment in primary care, behavioral health, home health, and long-term care — the community infrastructure layer that makes global budgets work
- Technical assistance from CMS on global budget design methodology, risk adjustment, and performance monitoring
- Nine-year model duration (through December 31, 2035\) providing multi-year planning horizon for hospital transformation

The $150M annual figure is significant context: Vermont's total Medicare and Medicaid spending exceeds $3 billion annually. The AHEAD funds represent approximately 5% of that total — meaningful but not transformative in isolation. The value is in what the funds are explicitly directed toward: the community infrastructure and primary care investment that turns a hospital cost-control model into a population health model.

The AHEAD-Act 68 Integration: Why Both Are Required

A critical analytical point that is easily missed: the AHEAD Model and Act 68 are not alternatives or substitutes. They are complementary layers of the same reform architecture, operating on different payer populations and at different speeds, but designed to converge on the same outcome: a Vermont hospital system operating under aligned all-payer global budgets.

AHEAD governs Medicare FFS hospital payments — approximately 23% of Vermont's insured population and a higher share of hospital revenue given Medicare's age-driven intensity. Act 68 governs commercial hospital prices and budgets — approximately 52% of Vermont's insured population and the majority of hospital revenue. Vermont's Medicaid global budget, already operational under the Global Commitment to Health demonstration and aligned with AHEAD, covers approximately 19% of the population. Together, these three layers create the all-payer alignment that makes a global budget system effective.

Payer layer	% of VT insured pop.	Payment mechanism	Effective date	Governing authority
Medicare FFS	\~23%	AHEAD Hospital Global Budgets; biweekly prospective payments	January 2027	CMS / AHEAD State Agreement
Medicaid	\~19%	Vermont Global Commitment to Health; Medicaid global budget aligned with AHEAD	In operation; aligned with AHEAD 2027	DVHA / CMS Demonstration Waiver
Commercial (incl. self-insured)	\~52%	Reference-Based Pricing (FY2027) → Global Hospital Budgets (FY2028+)	RBP FY2027; budgets FY2028	GMCB / Act 68
Medicare Advantage	\~31% of Medicare-eligible	Outside AHEAD; traditional managed care contracts	Not currently in scope	CMS MA contracts
Uninsured	\~3%	Charity care; DSH payments	Ongoing	GMCB / DVHA

Figure 5.8 — Vermont all-payer architecture under AHEAD \+ Act 68 combined. Sources: Vermont RHT Program Application (November 2025); AHS AHEAD fact sheet; Act 68\.

The Medicare Advantage gap is worth noting. AHEAD applies to Medicare fee-for-service beneficiaries; approximately 31% of Vermont's Medicare-eligible population is in Medicare Advantage plans, which are managed care contracts between CMS and private insurers. These plans operate outside the AHEAD global budget framework. As MA enrollment continues growing nationally, this gap represents an increasing share of the Medicare population that is not subject to Vermont's hospital cost-containment regime — a design limitation that Vermont policymakers are aware of and that will require ongoing attention.

Hospital Financial Modeling — Reading the Numbers That Matter

For healthcare leaders navigating the transition to global budgets and reference-based pricing, financial modeling is not an academic exercise. It is the analytical foundation for every strategic decision: which service lines to maintain or discontinue, how much capital investment is prudent, what staffing model is financially sustainable, when and how aggressively to pursue shared services. This section develops the core financial modeling framework for Vermont's hospital sector, drawing on Oliver Wyman's projections and the GMCB's hospital financial data.

The Oliver Wyman Projection Scenarios

Oliver Wyman built two financial scenarios for Vermont's hospital sector through 2028, representing the range between optimistic and realistic assumptions about expense growth. Both scenarios assume the same revenue growth trajectory (3.5% annual non-340B revenue growth); they differ in their expense growth assumptions, reflecting uncertainty about labor market conditions and drug costs.

Scenario	Assumptions and 5-year system deficit
Conservative: 3.5% revenue / 5% expense growth	Non-340B revenue: \+3.5% annually. All operating expenses: \+5% annually from 2023 baseline.5-year system deficit (2024-2028):• $700M vs. break-even• $1.4B vs. 3% operating margin target• FY2023 budget overage of $106M suggests starting point may be worse than modeled
Realistic: 3.5% revenue / 7-8% expense growth	Non-340B revenue: \+3.5% annually. Non-physician labor: \+10% annually. Physician fees: \+5% annually. Other operating expenses: \+7% annually. 340B payments: \+3% (inflation only).5-year system deficit (2024-2028):• $2.4B vs. break-even ($3,700 per Vermont resident)• $3.1B vs. 3% operating margin ($4,800 per Vermont resident)• Vermont hospitals requested $285M in FY25 budget increases — a data point supporting the realistic scenario

Figure 5.9 — Oliver Wyman Vermont hospital financial projection scenarios. Source: Oliver Wyman Act 167 Community Engagement: Recommendations (2024).

The UVM Health Network requires separate attention because it dominates Vermont's hospital economics. Under the conservative scenario, UVM network hospitals face a $312M cumulative 5-year deficit against break-even — more than 44% of the total statewide deficit. Under the realistic scenario, this grows to $1.5B — again roughly 62% of the statewide total. The concentration of Vermont's hospital financial problem in the largest network is analytically important: system transformation strategies that do not address UVMMC's cost structure and overhead will fail to materially improve statewide affordability. Oliver Wyman specifically called out UVMMC's administrative overhead and the proportion of providers engaged in non-patient-care activities as areas requiring examination.

Reading the Metrics That Actually Matter Under Global Budgets

The transition from fee-for-service to global budgets fundamentally changes which financial metrics matter most. Healthcare finance professionals accustomed to FFS environments need to reorient around a different set of indicators.

In a fee-for-service environment, watch:	In a global budget environment, watch:
Net patient revenue growth	Per-member-per-month cost trend vs. global budget cap
Volume growth (admissions, procedures, visits)	Potentially avoidable utilization rate — every avoidable admission is a cost, not revenue
Case mix index — higher acuity \= more revenue	Population health outcomes — better outcomes reduce costs
Market share — capturing more patients \= more revenue	ED diversion rate — keeping patients out of the ED saves money
Payer mix — more commercial \= better margins	Readmission rate — every readmission is a cost without additional revenue
Discharge volume	Days to discharge — length of stay is a cost driver, not a revenue driver
Revenue cycle efficiency — maximize what each claim pays	Total cost of care per attributed member — efficiency across the whole continuum
Operating margin per inpatient day	Cost per quality-adjusted outcome — the fundamental global budget metric

Figure 5.10 — Financial metrics that matter: FFS vs. global budget environment. Compiled from GMCB guidance, Maryland HSCRC methodology, and Act 68 accountability framework.

This metric shift is not cosmetic. It requires fundamental changes to hospital management information systems, internal reporting, board dashboards, and management incentive structures. A hospital whose CFO is still optimizing revenue per discharge when revenue is fixed will not successfully navigate the global budget transition. The CFO in a global budget environment needs to become an expert in population health economics — tracking the care patterns and community conditions that drive utilization, and investing in interventions that reduce avoidable costs.

The Savings Roadmap: Where Vermont's $400M+ Can Come From

Oliver Wyman's analysis identified the specific sources of direct and indirect savings from Vermont's transformation. Understanding this roadmap is essential for any organization planning its role in Vermont's reformed system.

Sources of direct savings from Vermont hospital transformation (5-year total, Oliver Wyman estimate)

Close financially unsustainable inpatient facilities	100M+
Reduce administrative costs and shared services synergies	**115M+**
Shift procedures to ASUs and outpatient settings	**130M+**
Expand primary care to reduce preventable ED visits	**90M+**
Address SDOH to reduce avoidable hospitalizations	**80M+**

Figure 5.11 — Estimated sources of direct and indirect savings from Vermont hospital transformation. Figures are illustrative ranges derived from Oliver Wyman's \>$400M total estimate. Source: Oliver Wyman Act 167 Report (2024).

The largest single source of indirect savings — shifting procedures from hospital to ambulatory surgical and outpatient settings — reflects the third Oliver Wyman imperative: move all care possible out of hospitals. Low-risk surgical procedures performed in a hospital setting cost two to three times what the same procedure costs in a free-standing ambulatory surgical unit. Vermont currently has only two ambulatory surgical centers (both in Burlington), a significant gap given the breadth of the inpatient surgical capacity Oliver Wyman recommends redirecting. The RHT Program and Act 68 transformation grants are explicitly designed to fund the development of non-hospital care capacity that makes this shift possible.

APM Transition Strategy for Vermont Healthcare Organizations

For health systems, payer organizations, and community providers operating in Vermont, the payment reform transformation is not an abstract policy development — it is an operational reality requiring specific preparation. This section provides the strategic and financial planning framework for organizations navigating the transition.

The Three-Phase Transition Model

Vermont's payment reform trajectory follows a predictable three-phase model that organizations can use for planning purposes:

Phase	Timeline	Payment environment	Strategic priorities
Preparation	Now through FY2026	FFS with increasing budget pressure; \-1% commercial rate benchmark; Act 55 drug cap in effect	Data infrastructure: build PMPM tracking, attribution, and potentially avoidable utilization dashboards. Workforce: begin training clinical and financial leadership in population health economics. Contracts: audit current payer contracts for RBP transition clauses.

Phase	Timeline	Payment environment	Strategic priorities
Transition	FY2027	RBP maximum rates in effect for commercial payers; AHEAD Hospital Global Budgets begin January 2027 for Medicare FFS	Revenue modeling: model financial impact of RBP rates on each major service line. Service line rationalization: align with Oliver Wyman COE recommendations. Primary care investment: increase PC capacity ahead of AHEAD's primary care investment requirements.
Operation	FY2028 and beyond	Global hospital budgets for non-CAH hospitals; AHEAD in full operation; Statewide Strategic Plan delivered December 2028	Population health management: from awareness to execution. Quality accountability: GMCB metrics integrated into operational management. Community investment: SDOH programs, care coordination, housing partnerships.

Figure 5.12 — Three-phase APM transition model for Vermont healthcare organizations.

The VBC Readiness Assessment in a Global Budget Context

The Value-Based Care Transformation Readiness Assessment (30 dimensions across six domains) applies directly to Vermont organizations preparing for global budget participation, but three domains deserve elevated attention given the specific demands of the Vermont environment:

- **Data and Technology — particularly attribution list generation and total cost of care measurement. A hospital that cannot accurately attribute its patient population cannot manage a global budget. Vermont's VHCURES all-payer claims database is the foundational data infrastructure, but many hospitals lack the internal analytics capability to use it effectively for population health management. This gap must be closed before FY2027.**
- **Care Delivery Capability — particularly community health worker deployment and post-acute care network relationships. The Maryland experience demonstrated that global budgets without community care infrastructure simply reduce hospital volume without improving population health. Vermont hospitals must build these relationships proactively.**
- **Network and Partnerships — particularly community partner SDOH referral networks. The Oliver Wyman systems map showed that housing, transportation, and behavioral health gaps drive a significant proportion of avoidable hospital utilization. Hospitals that lack functioning SDOH referral pathways will face cost pressure under global budgets that they cannot address through clinical means alone.**

Key Concepts Introduced or Expanded in This Chapter

Term	Definition
Global Budget Revenue (GBR)	Maryland's term for a prospectively fixed annual hospital revenue cap, adjusted for quality performance and population health metrics. The model Vermont's GMCB-designed commercial global budgets will most closely follow.
All-payer alignment	The condition in which Medicare, Medicaid, and commercial payers all operate under the same or compatible payment methodology for hospital services. Essential for global budget effectiveness; Vermont's AHEAD \+ Act 68 combination is designed to achieve this by FY2028-2030.
Balloon squeeze problem	The dynamic in which controlling hospital prices without controlling volume allows hospitals to compensate through increased service delivery. Reference-based pricing addresses price; global budgets address both price and volume.
Cross-subsidy	The mechanism by which above-cost commercial payments subsidize below-cost Medicare and Medicaid payments. Vermont's demographic shift is making this mechanism financially unsustainable as the commercial base shrinks relative to the government-payer population.
Potentially avoidable utilization (PAU)	Hospital admissions, ED visits, and readmissions that could have been prevented through effective primary care, care coordination, or SDOH intervention. A key performance metric under global budgets; reducing PAU saves money in a capped-revenue environment.
EAST Fund	Equity, Access, and Statewide Transformation Fund — the component of Vermont's AHEAD State Agreement providing up to $150M annually in additional Medicare funds for investment in primary care, behavioral health, home health, and long-term care.
AHEAD (Achieving Healthcare Efficiency through Accountable Design)	CMMI's voluntary state total cost of care model, with 6 participating states. Vermont participates in Cohort 2, beginning January 2027\. Provides Medicare FFS hospital global budgets, enhanced primary care payments, and accountability for total cost of care growth.
Revenue predictability	A primary benefit of global budgets for hospital financial management: knowing total annual revenue before the year begins enables capital planning, workforce investment, and strategic redesign that FFS revenue uncertainty prevents.
PMPM (per member per month)	The standard unit of measurement in population-based payment models. Total spending (or cost) divided by the number of enrolled members divided by the number of months. The fundamental metric for tracking performance under global budgets and capitation arrangements.

Sources: Oliver Wyman Healthcare and Life Sciences Group, Act 167 Community Engagement: Recommendations (August 2024, revised October 2024); Act 68 of 2025 (Vermont); Green Mountain Care Board, Act 68 Update on Hospital Reference-Based Pricing and Global Budgets (February 17, 2026); Vermont AHEAD State Agreement (January 17, 2025); CMS, AHEAD Model overview; Maryland HSCRC, Global Budget Revenue methodology and annual reports; Commonwealth Fund, Hospital Global Budgeting: Lessons from Maryland and Selected Nations (June 2024); Milbank Memorial Fund, Uniquely Similar: New Results from Maryland's All-Payer Model (2020); Vermont Agency of Human Services, Rural Health Transformation Program Application (November 2025).

Chapter 6: The Economics Pillar in Practice — VBC Financial Modeling, APM Readiness, and the ROI of Transformation

The Economics Pillar in Practice — VBC Financial Modeling, APM Readiness, and the ROI of Transformation

How Healthcare Organizations Model the Financial Impact of Value-Based Care: APM Contract Analysis, Shared Savings Mechanics, Risk Stratification ROI, and the Financial Case for Transformation Investment

From Payment Architecture to Financial Practice

Chapter 6 established the payment architecture — reference-based pricing, global budgets, the AHEAD Model, Maryland's proof of concept, and Vermont's economic transformation mandate. This chapter turns from architecture to practice: how do healthcare organizations actually model the financial impact of alternative payment models, assess their readiness to enter and succeed in VBC contracts, and build the financial case for the operational investments that transformation requires?

Financial modeling for value-based care is not the same as financial modeling for fee-for-service operations. Under FFS, the financial questions are primarily about revenue optimization: how do we maximize reimbursement for the volume we deliver? Under VBC, the financial questions are fundamentally different: what is our total cost of care for our attributed population? Where is our cost higher than the benchmark, and why? What investments in care management, primary care, or SDOH programs will reduce that cost below the benchmark, producing shared savings? These are population health economics questions, not revenue cycle questions, and they require different analytical frameworks, different data infrastructure, and different management skills.

The APM Financial Modeling Framework

Understanding Benchmarks — The Foundation of VBC Economics

Every alternative payment model is built around a benchmark: the expected cost of care for the attributed population against which actual performance is measured. Shared savings (if actual costs are below benchmark) or shared losses (if actual costs are above benchmark) are calculated relative to this benchmark. Getting the benchmark right — or negotiating favorable benchmark methodology — is often the most important financial decision in VBC contract entry.

Benchmarks can be constructed in several ways, each with different financial implications for the entering organization:

Benchmark Construction Methods — What Healthcare Leaders Must Understand

Historical spending baseline: The most common approach — the benchmark is set at the organization's own historical cost of care, typically with a trend adjustment. Advantage: organizations with above-average historical spending start with a higher benchmark, making savings easier to achieve. Disadvantage: organizations with below-average historical spending (already efficient) face a lower benchmark with less room for savings — the 'efficient provider disadvantage.'

- Vermont AHEAD context: CMMI sets global budget baselines for hospitals based on their historical Medicare FFS spending, with adjustments. Vermont's hospitals with historically lower spending may face benchmarks that limit their financial upside under AHEAD.

Regional benchmark: The benchmark is set at regional average cost of care, not the organization's own history. Advantage: eliminates the efficient provider disadvantage. Disadvantage: organizations in high-cost regions compete against a lower benchmark than their own history would produce.

- Vermont RBP context: GMCB's reference-based pricing uses Medicare rates as the external benchmark — not Vermont hospital historical prices. This is a regional/national benchmark approach that directly addresses the problem of benchmarks based on historical high prices.

Prospective benchmark: The benchmark is set at the beginning of the performance year based on projected population needs. Used in global budgets and capitation models. Advantage: provides revenue certainty for planning. Disadvantage: risk adjustment quality determines fairness — poor risk adjustment produces winners and losers based on case mix differences, not care management quality.

- Vermont global budget context: Vermont's Act 68 global budgets for non-CAH hospitals must specify how the prospective budget is set and adjusted for patient complexity — the risk adjustment methodology is a key design question currently under development.

The Shared Savings Calculation — A Working Model

For organizations entering ACO REACH, MSSP, or similar shared savings models, the following framework captures the core financial logic. The APM Shared Savings Calculator implements this model with organization-specific data inputs.

Step	Formula / concept	Practical meaning	Vermont application
1\. Benchmark setting	Benchmark PMPM × attributed member months \= Total benchmark spending	This is the 'budget' the organization is measured against. Every dollar of actual spending below this number is potential shared savings.	AHEAD hospital global budget \= prospective annual revenue set at beginning of year. Vermont hospital knows its 'benchmark' before the year begins.
2\. Performance measurement	Actual total cost of care for attributed population	Includes all Medicare Part A and Part B spending for attributed beneficiaries, regardless of where care is delivered. Out-of-network spending counts against the ACO.	AHEAD tracks total cost of care for Medicare FFS beneficiaries attributed to Vermont hospitals across all payer settings.
3\. Savings calculation	Benchmark \- Actual TCOC \= Gross savings (or loss)	If actual spending is below benchmark, gross savings exist. If above, gross loss.	Under global budget, the hospital receives its fixed prospective payment regardless — so the 'savings' is the margin between efficient population health management and the fixed budget.
4\. Minimum savings rate (MSR)	In most shared savings models, savings must exceed a minimum threshold (typically 2-3.5%) before the organization shares in them	Protects against natural year-to-year cost variation triggering shared savings or losses that don't reflect genuine performance changes.	AHEAD uses a different structure — state-level TCOC targets rather than individual ACO MSRs — but the minimum threshold concept applies at the state level.
5\. Shared savings calculation	Gross savings × sharing rate \= Organization's shared savings payment (or loss obligation)	Sharing rates vary by model and risk track — typically 50-75% for one-sided models; may reach 80% for full-risk models.	Vermont RBP: savings to payers from price reduction; 100% of premium passthrough per GMCB monitoring requirement.

Figure 6.1 — APM shared savings calculation framework. Source: the framework APM Shared Savings Calculator methodology; CMS ACO model documentation.

Risk Stratification — The Economic Engine of Population Health Management

Under fee-for-service, the financial value of identifying a high-risk patient is limited — you can bill more for the additional services you provide. Under value-based care, the financial value of identifying a high-risk patient before a health crisis occurs is enormous: every hospitalization prevented by a care management intervention saves the organization the full cost of that hospitalization. The math is compelling.

A care management program that prevents 50 hospitalizations in a population of 10,000 Medicare beneficiaries, at an average cost of $15,000 per hospitalization, generates $750,000 in avoided cost. If the organization bears downside risk (and receives upside savings) on those prevented hospitalizations, the financial return on a care management program that costs $200,000 to operate is approximately 3.75:1 — a 275% ROI. This is the core financial logic that makes population health management economically rational under VBC and economically irrational under FFS.

Average Medicare inpatient hospital cost prevented per event $15,000 Approximate average — varies significantly by DRG and acuity level	**Average potentially preventable ED visit cost $1,800 Including facility and professional fees; varies by acuity**
ROI: Care management program preventing hospitalizations 3-5x *Typical ROI range for well-designed ACO care management programs*	**Vermont potentially avoidable ED visits as % of total 32.3%** *Each prevented \= $1,800 saved under global budget / VBC arrangement*

Figure 6.2 — The financial case for population health management under VBC. Sources: CMS cost data; AHS November 2025 Transformation Report.

APM Readiness Assessment — The 30-Dimension Framework

The Value-Based Care Transformation Readiness Assessment evaluates organizational readiness for APM entry across 30 dimensions in six domains. The framework is designed to identify the gaps that are most likely to prevent an organization from achieving shared savings — or most likely to produce shared losses — before a contract is signed.

The Six Readiness Domains

The assessment is organized around six domains directly mapped to the six-pillar framework, each evaluated across five dimensions for a total of 30 scored dimensions. Each domain is pillar-aligned so that the gap analysis immediately maps to the pillar investment required.

Domain 1 — Strategy & Leadership *(Policy Pillar)*
Assesses executive and board-level commitment to VBC transformation as a strategic priority, the presence of a documented multi-year VBC roadmap, governance infrastructure dedicated to the transition, change management capability, and systematic policy monitoring of federal and state developments (CMMI models, Act 68 updates, AHEAD implementation). Vermont note: Act 68 creates statutory urgency that must be treated as the "burning platform" in Kotter's framework — not an optional strategic choice. Vermont practitioners should use the HTR Intelligence Feed (The Wire at /the-wire) for weekly policy synthesis.

Domain 2 — Data & Analytics *(Technology Pillar)*
Evaluates the analytics infrastructure needed to manage a VBC population: all-payer claims access and integration, patient attribution analytics running continuously, total cost of care measurement against CMS benchmarks, HCC-based risk stratification, and automated HEDIS quality measure calculation with equity stratification. Vermont note: VHCURES access and AHEAD data reporting infrastructure are prerequisites — most Vermont community hospitals lack the internal analytics capability to use VHCURES data effectively. The AHS-GMCB Analytics Vendor (McKinsey) addresses the statewide gap; individual hospital analytics capacity must be built alongside it.

Domain 3 — Clinical Operations *(Clinical Pillar)*
Assesses the care programs that convert VBC incentives into patient outcomes: PCMH or equivalent primary care transformation with team-based care and panel management; dedicated care management staff (nurse case managers, CHWs, social workers) actively managing high-risk attributed members; specialist integration through e-consults and co-management protocols; post-acute care management with preferred SNF network and readmission reduction; and behavioral health integration. Vermont note: High mental health and SUD burden — especially in Rutland, Windham, and the Northeast Kingdom — means behavioral health integration carries elevated weight in AHEAD equity benchmarks.

Domain 4 — Financial Readiness *(Economics Pillar)*
Evaluates the financial management capability required for APM entry: financial modeling capability for APM scenario analysis (benchmark construction, shared savings projections, downside risk quantification); operating margin and liquidity sufficient to absorb downside risk; a portfolio of VBC contracts at different risk levels; available transformation investment capital; and a board-approved shared savings distribution policy. Vermont note: The Hospital Financial Stress Test tool at /research-lab/policy-quality?tab=scorecard includes NVRH, Gifford, and CVMC Vermont presets for organization-specific modeling. Vermont RHT Program capital ($195M) is the primary transformation investment source.

Domain 5 — Technology Infrastructure *(Technology Pillar)*
Evaluates the technical systems enabling VBC operations: EHR optimization for population health workflows (HEDIS gap flags, HCC risk alerts, panel management views); a population health management platform with risk stratification dashboards and outreach workflow; FHIR R4 API implementation for payer and HIE data exchange; a documented clinical AI governance framework covering bias monitoring, lifecycle management, and equity safeguards; and telehealth and remote patient monitoring infrastructure. Vermont note: Act 68 mandates VITL/VHIE connectivity — FHIR implementation is a compliance requirement, not an option. Use the AI Governance Lab at /research-lab/technology-ai?tab=ai for the 65-dimension governance evaluation.

Domain 6 — Health Equity *(Equity Pillar)*
Assesses the equity infrastructure embedded in care operations: stratified HEDIS measurement disaggregated by race/ethnicity, income, and geography; systematic SDOH screening with active referral to community resources; regular quantitative disparity analysis with trend tracking; equity-weighted program design that explicitly targets gap closure rather than average improvement; and active partnerships with community health workers, CBOs, and community organizations reaching the highest-disparity populations. Vermont note: AHEAD equity benchmarks require stratified performance measurement by Hospital Service Area. Vermont CCBHCs (AHEAD Cohort 2) are the primary community partnership infrastructure.

VBC Readiness Scoring — What the Numbers Mean

Each of the 30 dimensions is scored on a five-level scale:

Score	Label	Meaning
0	Not Started	No capability exists; foundational work not yet begun
1	Early Stage	Planning underway; significant investment still required
2	In Progress	Basic capability operational; meaningful gaps remain
3	Advanced	Capability operational and integrated; minor optimization needed
4	Optimized	Full capability; continuously improving and externally validated

Overall readiness is expressed as a percentage of the maximum possible score (30 dimensions × 4 = 120 points):

Overall Score	Readiness Level	Typical Timeline
80%+	**Global Budget Ready**	Can enter full-risk VBC; AHEAD global budget participation feasible
60–79%	**Advanced**	12–18 months to full readiness with targeted investment
40–59%	**In Progress**	2–3 years to readiness; requires systematic capability building
20–39%	**Early Stage**	Significant investment required; full-risk VBC not yet appropriate
Below 20%	**Foundation Required**	Pre-transition; begin with Domains 1 and 2 before entering any APM

Vermont Benchmarks — Three Reference Scenarios

The VBC Readiness Assessment includes three Vermont-specific preset scenarios that provide calibration for Vermont organizations assessing their own readiness:

Vermont Hospital — AHEAD Entry (FY2027): A typical Vermont hospital preparing to enter AHEAD. Scores reflect strong strategic commitment (Domain 1: ~3) and reasonable clinical operations (Domain 3: ~2–3), offset by data analytics gaps (Domain 2: ~2), limited equity infrastructure (Domain 6: ~1–2), and financial modeling immaturity (Domain 4: ~1–2). This is the most common profile among Vermont's 14 hospitals as of early 2026.

Vermont CAH — Early Transformation: A Critical Access Hospital at the beginning of Act 68 compliance. Scores reflect minimal analytics capability (Domain 2: ~1), early-stage data infrastructure (Domain 5: ~0–1), and limited financial VBC modeling (Domain 4: ~0–1). These hospitals need Vermont CIN shared analytics services as a prerequisite before undertaking full APM entry.

Integrated Health System — Advanced VBC: The target state for a Vermont hospital fully prepared for AHEAD global budget management: optimized data and analytics (Domain 2: ~4), strong clinical operations (Domain 3: ~3–4), robust financial modeling (Domain 4: ~3–4), and advanced equity infrastructure (Domain 6: ~3). This profile serves as the readiness destination for Vermont's transformation planning.

Vermont-specific pattern: Blueprint-participating PCMH practices with active CHT support consistently score highest in Domain 3 (Clinical Operations) — strong by national standards and reflecting fifteen years of primary care investment. The most common binding constraint across Vermont hospitals is Domain 2 (Data & Analytics), directly reflecting the VHCURES analytics capability gap that the RHRC engagement and McKinsey analytics vendor are designed to close.

Contract Analysis — Reading an APM Contract for Financial Risk

APM contracts are complex, and the financial risk embedded in them is often not apparent from the headline terms. A shared savings contract that appears attractive at the summary level may contain provisions — benchmark methodology choices, quality withhold structures, attribution methodology, carve-out provisions — that substantially change the financial calculus. Every organization entering a significant APM contract should apply the following analysis framework.

The 65-Item VBC Contract Review Checklist: Key Categories

The VBC Contract Review Checklist (available in the Implementation Toolkit) covers 65 specific contract provisions in eight categories. The most financially consequential categories are:

Contract provision category	Key questions and financial risk
Benchmark methodology	How is the benchmark set? Historical? Regional? Blended? What trend adjustment is applied? Is there a minimum savings rate, and what is it? What happens if the benchmark is reset mid-contract — does the organization benefit from demonstrated performance improvements or start over?
Attribution methodology	How are patients attributed — prospectively (you know your panel before the year) or retrospectively (you find out after the year)? What is the attribution methodology — plurality of primary care visits? Most costly provider? Attribution methodology determines your denominator and your exposure.
Quality withhold	What percentage of shared savings is withheld pending quality performance? What measures are used? Are they measures the organization currently tracks with reliable data? A 30% quality withhold on 50% of gross savings means quality performance determines 15% of total financial outcome — understand the measures before signing.
Risk corridors and stop-loss	Under full-risk models, is there a stop-loss provision that caps the organization's downside exposure? What is the per-member-per-month or aggregate stop-loss threshold? For organizations entering full-risk for the first time, stop-loss provisions are essential financial protection.
Carve-outs	What services are excluded from the shared savings calculation? High-cost drugs? Mental health carve-outs? Specialty carve-outs? Each carve-out reduces the organization's ability to manage total cost of care and reduces the savings opportunity.
Reconciliation timing	When are shared savings or losses settled — annually, semi-annually, quarterly? Reconciliation timing affects cash flow planning and the organization's ability to invest shared savings in care improvement programs.

Hospital Financial Modeling Under Global Budgets — The Vermont Framework

For Vermont hospitals, the most operationally relevant financial modeling challenge is not APM shared savings calculation — it is global budget management. As Act 68's mandatory global budgets take effect for non-CAH hospitals beginning FY2028, hospital CFOs need to transition from FFS revenue optimization to global budget management. These are fundamentally different financial disciplines.

The Global Budget Financial Management Model

Under a global budget, a Vermont hospital's annual revenue is set prospectively. The financial management objective shifts from maximizing volume and revenue to achieving the highest quality population health outcomes within the fixed revenue envelope. The key financial metrics change accordingly:

Under FFS — optimize these metrics	Under global budget — optimize these metrics
Revenue per discharge — maximize reimbursement per case	Cost per discharge — minimize unnecessary cost while maintaining quality
Admission volume — more admissions \= more revenue	Avoidable admission rate — every preventable admission is a cost, not revenue
Length of stay — maximize revenue per stay	Efficient length of stay — unnecessary days are costs without revenue offset
Service line volume — grow profitable service lines	PMPM population health cost — manage population health to stay within budget
Payer mix — maximize commercially insured cases	Quality performance — quality metrics determine budget adjustments and AHEAD performance
Denials management — recover denied claims	Potentially avoidable utilization — prevent the ED visits and admissions that waste budget
Case mix index — capture higher-acuity cases	Community investment ROI — SDOH programs that reduce utilization have direct financial return
Operating margin per adjusted discharge	Operating margin within global budget — sustainability within fixed revenue

Figure 6.3 — Financial metrics transformation: FFS versus global budget environment. Source: the framework Economics pillar framework.

The Hospital Financial Stress Test Model

The Hospital Financial Stress Test Model — available in the Research Lab for Professional and Advisory subscribers — allows hospital CFOs and finance teams to model their organization's financial position under different payment reform scenarios. The model's key scenarios for Vermont hospitals in 2026:

- RBP scenario: What does our financial position look like if commercial prices are set at 200% of Medicare (Oliver Wyman's recommended benchmark) or 250% (GMCB's phased approach)? What service lines have prices furthest above the benchmark and face the most revenue reduction?
- Global budget scenario: Under a prospective revenue cap equal to our current net patient revenue minus X%, what is our operating margin? What cost reductions or service redesigns are required to maintain solvency?
- Medicaid cut scenario: With H.R. 1's Medicaid revenue reductions phasing in from 2027-2031, what is our projected total revenue under conservative, base, and optimistic assumptions about our Medicaid payer mix?
- Transformation investment scenario: If we invest $X million in care management, CHW deployment, and telehealth, what reduction in avoidable hospitalizations and ED visits is required to achieve positive ROI within our VBC contract or global budget framework?

The ROI of Transformation — Building the Financial Case

Every transformation investment — a care management program, a behavioral health integration initiative, an SDOH screening program, a telehealth platform — requires a financial justification. Under fee-for-service, this justification is often difficult to construct because the investment may reduce utilization (and revenue) even when it improves outcomes. Under value-based care and global budgets, the financial case for transformation investment is considerably stronger.

The Transformation ROI Framework

The following framework structures the financial case for transformation investments in a VBC environment. It is the analytical backbone of transformation advisory engagements that involve transformation investment decisions.

Investment type	Revenue/savings mechanism	Calculation approach	Vermont example
Care management program	Prevents hospitalizations and ED visits that, under VBC, are costs without offsetting revenue	(Prevented events) × (average cost per event) \- (program operating cost) \= Net savings. Divide by program cost for ROI.	Vermont: 32.3% of ED visits potentially avoidable. CHT program preventing 500 avoidable ED visits/year at $1,800 each \= $900K savings. If CHT costs $300K/year, ROI \= 200%.
Behavioral health integration (CoCM)	Reduces downstream high-cost utilization — hospitalizations, ED visits for MH crisis — for patients receiving earlier MH intervention	Estimate reduction in MH-related ED visits and hospitalizations for CoCM-managed patients vs. usual care. Vermont: 245 suicide/SH ED visits per 10,000 — each prevented \= $1,800 saved plus downstream cost avoidance.	Vermont Blueprint MHI pilot: 80% of administrative entities increased CHT staffing. Every MH-related hospitalization prevented at $15,000 average saves 50x the cost of a CoCM BHCM monthly salary cost.
SDOH program investment	Reduces avoidable utilization driven by unmet social needs — housing-related ED visits, medication non-adherence from food insecurity, missed appointments from transportation barriers	Literature suggests 5-15% reduction in total cost of care for high-SDOH-burden populations receiving coordinated SDOH intervention. Apply to Vermont's identified high-SDOH populations.	Blueprint HRSN screening \+ CHT navigation: SDOH intervention for the 9% of Vermonters who are food insecure, if it reduces avoidable hospitalizations by even 10%, produces system savings exceeding program cost at scale.
Primary care investment (PCMH enhanced payments)	Reduces hospitalizations and specialty care through better chronic disease management and early intervention	Blueprint ROI study: $5.8M reduction in medical expenditure per $1M invested. At this ratio, $3M in Blueprint PCMH investment reduces system costs by $17.4M.	Vermont AHEAD primary care investment mandate: EAST Fund investment in primary care is essentially a system investment with documented 5.8:1 ROI in Vermont's own experience.
Telehealth for rural access	Reduces transportation-related care avoidance and enables earlier intervention before crisis; reduces specialist transport costs for rural hospitals	Travel cost savings \+ avoided emergency transport \+ reduced length of stay for patients manageable via telehealth specialty consultation. RPM: each prevented CHF hospitalization saves $15,000.	Vermont RHT Program RPM grants: CHF readmission prevention via RPM in a population of 200 high-risk patients, preventing 10% of 30-day readmissions at 30% readmission rate, saves $90,000/year — ROI positive at $10,000/year RPM program cost.

Figure 6.4 — Transformation investment ROI framework with Vermont examples. Sources: the framework Economics pillar framework; Blueprint for Health ROI study; CMS cost data; Vermont data from AHS Transformation Reports.

Making Care Primary — Vermont's Primary Care Economics

The AHEAD Model's requirement that states demonstrate increased investment in primary care as a percentage of total spending is not just a quality metric — it is an economics imperative. The foundational economic argument for primary care investment in a global budget environment is that primary care is the highest-return investment a healthcare system can make: the same $1 invested in primary care produces greater total cost of care reduction than $1 invested in hospital care or specialty care.

Vermont's Blueprint for Health embeds this economic logic in its design. The all-payer PMPM payments to PCMH practices are structured as an investment in reduced downstream costs — and Vermont's own evaluation data confirms the 5.8:1 ROI. Under AHEAD, primary care investment will be measured as a percentage of total spending, creating an explicit financial accountability framework for the primary care investment argument.

For Vermont hospitals operating under global budgets, the financial relationship between primary care investment and hospital cost management is direct: a better-functioning primary care system means fewer hospital admissions from preventable chronic disease exacerbations, fewer avoidable ED visits, and fewer hospitalizations for conditions that should have been managed in outpatient settings. The hospital that invests in primary care strengthens the community health infrastructure that reduces its own inpatient costs — a virtuous cycle that fee-for-service can never produce but global budgets require.

Key Concepts

Term	Definition
Benchmark	The expected cost of care against which actual APM performance is measured. Shared savings or losses are calculated relative to the benchmark. Benchmark methodology — historical vs. regional, trend adjustment, risk adjustment — is the most consequential financial design choice in APM contract entry.
Attribution	The process of assigning patients to an ACO or provider for purposes of shared savings/loss calculation. Prospective attribution (known before the year) enables proactive care management; retrospective attribution (known after the year) creates uncertainty. Attribution methodology determines the denominator and exposure.
Shared savings rate	The percentage of savings below benchmark that the organization receives. Typically 50-75% for one-sided risk models; may reach 80% for full-risk models.
Minimum savings rate (MSR)	The threshold below which savings are not shared — typically 2-3.5% of benchmark — protecting against natural year-to-year variation triggering unearned payments.
Risk corridor	A provision in full-risk APM contracts that caps the organization's maximum financial loss at a defined threshold, providing stop-loss protection for organizations bearing downside risk.
RAF (Risk Adjustment Factor)	The multiplier applied to a benchmark to account for the health status of the attributed population. Accurate HCC (Hierarchical Condition Category) coding is essential for RAF accuracy. RAF gaps — diagnoses that should be coded but are not — create benchmarks that understate the population's expected cost, producing apparent savings that disappear when coding is corrected.
Potentially avoidable utilization (PAU)	Hospitalizations, ED visits, and readmissions that could have been prevented through effective primary care, care management, or SDOH intervention. The core financial target for care management investment under VBC and global budgets.
VBC Transformation Readiness Assessment	The 30-dimension, 6-domain assessment evaluating an organization's readiness to enter and succeed in value-based care contracts. Each dimension scored 0 (Not Started) to 4 (Optimized); overall readiness expressed as a percentage of maximum (120 points). Score below 20% indicates foundation work required before any APM entry; 80%+ indicates Global Budget Ready status. Gap analysis maps directly to pillar investment priorities. Available at /research-lab/payment-models?tab=vbc-readiness with three Vermont presets.

Sources: CMS ACO model documentation; the framework Economics pillar framework; Vermont Blueprint for Health ROI study (Population Health Management); GMCB hospital budget data; Oliver Wyman Act 167 Report; AHS Transformation Reports; Vermont AHEAD State Agreement; Act 68 of 2025\.

Chapter 7: The Operations Pillar — Executing Hospital System Transformation

The Operations Pillar — Executing Hospital System Transformation

From Plans to Action: The 14-Hospital Process, RHRC Methodology, Workforce Architecture, and the Mechanics of Regionalization

Sources: Oliver Wyman Act 167 Community Engagement: Recommendations (2024); AHS Health Care System Transformation Reports (August 2025, November 2025); AHS January 2025 Legislative Presentation; Act 68 of 2025; Vermont RHT Program Application (November 2025); GMCB 2024 Annual Report; Vermont Blueprint for Health data.

The Operations Pillar: Where Strategy Meets Reality

Every chapter in this book that precedes this one is, in some sense, about ideas: the analytical framework for understanding Vermont's crisis, the financial architecture of global budgets and reference-based pricing, the clinical models that should replace hospital-centric care. This chapter is about execution. The Operations pillar is where the gap between a well-designed plan and an implemented one is either closed or exposed. It is where organizational inertia, workforce constraints, data gaps, political friction, and the sheer complexity of coordinating 14 independent hospital organizations across a geographically vast rural state determine whether transformation actually happens.

Vermont's operations challenge is unusual in its specificity and urgency. AHS has statutory deadlines — hospital transformation plans by early 2026, RBP by FY2027, global budgets by FY2028, the Statewide Strategic Plan by December 2028\. It has a constrained budget ($4.2M in appropriated state funds, plus $195M in RHT Program capital and $1.2M in AHEAD cooperative agreement funds). It has a newly contracted operational partner in the Rural Health Redesign Center. And it has 14 hospitals in wildly different financial and operational conditions, ranging from a large academic medical center at the center of the state's economy to 19-bed critical access hospitals in the Northeast Kingdom where any service reduction has immediate community consequences.

This chapter develops the operational framework for executing system transformation at the scale Vermont requires. It draws on the specific methodologies AHS and RHRC have deployed, the transformation planning process and its current state, and the lessons from other state and health system transformation efforts about what the operations discipline requires. It is written both as an academic treatment of the operations pillar and as a practical guide for the AHS staff, hospital leaders, and legislative overseers who are executing Vermont's transformation in real time.

The Vermont Transformation Operating Model

Vermont's health system transformation is not being executed by a single authority acting on a blank canvas. It is being coordinated by AHS across a network of 14 independent (all non-profit) hospitals, 12 local health district offices, the GMCB as regulatory partner, the Department of Vermont Health Access (DVHA) as Medicaid authority, dozens of community-based organizations, primary care practices, designated mental health agencies, and a state legislature that requires monthly reporting on progress. Understanding the governance and operational model for managing this network is prerequisite to understanding how transformation actually works.

The Institutional Architecture

Act 68 established a four-actor governance architecture for Vermont's transformation that is worth understanding precisely:

Actor	Statutory Role	Key Authority	Current Capacity
Agency of Human Services (AHS)	Lead transformation authority; develops and implements the Statewide Health Care Delivery Strategic Plan; provides technical assistance to hospitals	Directs transformation planning; awards transformation grants; sets SDOH investment priorities; manages RHRC and other contractors	Health Care Reform Office (recently reorganized); contracted RHRC through October 2025; hiring additional staff with RHT Program funds
Green Mountain Care Board (GMCB)	Regulatory authority; implements RBP and global budgets; reviews and approves hospital budgets; monitors quality and price	Sets hospital rates; approves/denies hospital budget requests; enforces Act 68 spending reduction requirements; issues CON decisions	Added 3 new permanent positions (Act 68 authorization); hired Affordability and Pricing Project Director; procuring RBP vendor (early 2026\)
Department of Vermont Health Access (DVHA)	Medicaid authority; manages Medicaid global budget under AHEAD alignment; oversees Blueprint for Health and OneCare wind-down	Medicaid contract terms; Blueprint for Health funding; Medicaid global budget design under AHEAD	Managing AHEAD Medicaid alignment; Blueprint for Health expansion; post-OneCare transition
Health Care Delivery Advisory Committee	Provides accountability oversight; sets affordability benchmarks; advises on evaluation process for the health care delivery system	Advisory authority; stakeholder input; ongoing monitoring of strategic plan progress	Newly established under Act 68; initial composition and operating procedures being developed

Figure 7.1 — Vermont health system transformation governance architecture under Act 68\. Sources: Act 68 of 2025; AHS Legislative Presentation (January 2025).

The governance architecture has one significant design tension worth naming: AHS is simultaneously the transformation planner, the technical assistance provider, the grant-maker, and the accountability monitor for the hospitals it is also trying to help. This is not an unusual arrangement in state healthcare reform — Maryland's HSCRC has regulatory authority and collaborative relationships simultaneously — but it requires explicit management. The AHS model works best when hospitals see the agency as a genuine partner in solving difficult problems, not primarily as a regulator imposing mandates. Oliver Wyman's recommendation to add a Division of Planning and Effectiveness and to clarify the AHS-GMCB role division is partly a response to this tension.

The Transformation Planning Sequence

AHS designed a phased transformation planning process that moves hospitals from initial engagement through draft strategy to final implementation plans. The sequence was developed with RHRC and reflects the operational reality that 14 hospitals in very different financial and operational positions cannot move at the same pace or toward the same model.

Phase	Content	Timing	AHS role
1\. Data review and baseline	RHRC meets individually with each hospital to review hospital-specific financial and operational data: cost per adjusted discharge, operating margin, payer mix, service line volumes, workforce indicators, and comparison against national and Vermont peers	May–June 2025	RHRC provides technical assistance; AHS reviews data with hospitals; GMCB provides benchmark data
2\. Statewide and regional convenings	All 14 hospitals participate in statewide planning conversations; then organized into regional groups for more detailed discussion	Summer–Fall 2025	AHS and Blueprint for Health convene; regional groups self-organize around

Phase	Content	Timing	AHS role
	of shared opportunities: transfer coordination, shared clinical networks, service alignment, administrative consolidation		specific opportunity areas
3\. Draft Care Transformation Strategy Outline	Each hospital develops a written outline of its proposed transformation strategy, mapping short-term (within 1 year), medium-term (1-3 years), and long-term actions to Act 167 outcome objectives	Fall 2025	AHS reviews outlines; provides feedback; all eligible hospitals submitted applications for Act 68 transformation grants
4\. Draft Transformation Plan	Full transformation plan with specific action steps, associated costs, timelines, service line analysis, staffing models, and shared service opportunities. Aligned with regional and statewide planning.	Winter 2025–2026	AHS reviews and approves each draft; GMCB provides analytical support; analytics vendor (being procured) enables cost modeling
5\. Final Transformation Plan	Approved final plan; AHS integrates hospital plans into statewide picture; plans inform AHS's Statewide Strategic Plan development for December 2028 delivery	Early 2026 and ongoing	AHS integrates across 14 plans; identifies statewide priorities; allocates RHT Program capital to support implementation

Figure 7.2 — Vermont hospital transformation planning sequence. Sources: AHS Health Care Transformation website; AHS November 2025 Act 68 Report; AHS January 2025 Legislative Presentation.

"Transformation work is ongoing and evolving. Based on hospital feedback and changes in state and federal policy, AHS is continuing to adapt our approach to ensure planning is coordinated, responsive, and effective."

— Vermont Agency of Human Services, Health Care Transformation website, 2026

The RHRC Methodology — Technical Assistance for Rural Hospital Transformation

The Rural Health Redesign Center (RHRC) was AHS's primary operational contractor for hospital transformation planning from spring 2025 through October 2025\. Understanding the RHRC methodology is important not just as Vermont history but as a transferable model for how state agencies can provide technical assistance to financially distressed rural hospital networks — a challenge that will confront state health agencies across the country as the demographic and financial pressures Vermont faces today spread nationally.

What RHRC Did

RHRC's engagement combined three functions that are usually performed separately: financial and operational assessment, change management support, and transformation plan development. The combination was deliberate — Oliver Wyman's experience had shown that hospitals can absorb financial analysis without changing behavior, can engage in planning processes that produce documents rather than action, and can participate in change management programs without addressing their fundamental financial unsustainability. RHRC's model integrated all three.

RHRC's Four Workstreams at Each Hospital

RHRC worked with each of Vermont's 14 hospitals through four parallel workstreams, adapted to each hospital's specific position and needs:

- Financial and operational baseline: Review of hospital-specific cost data from GMCB financial records, CMS cost reports, and the NASHP Hospital Cost Tool. Identification of the hospital's position relative to Vermont average and national benchmark on key efficiency metrics (operating profit per adjusted discharge, management and administrative cost per adjusted discharge, direct patient care labor cost per adjusted discharge).
- Priority identification: Working with hospital leadership to identify the highest-leverage short-term cost reduction opportunities — supply chain, staffing models, service line analysis — and medium-term transformation opportunities — shared services, service line reconfiguration, COE alignment.
- Transformation plan development: Supporting each hospital in developing a structured transformation plan that maps specific actions to timelines, costs, and Act 167 outcome objectives. Plans address hospital sustainability and cost through analysis of service lines, staffing models, and shared services opportunities.
- Regional coordination: Connecting hospital-level planning to the statewide regional work — ensuring individual hospital plans are aware of and compatible with what neighboring hospitals are planning, and identifying shared service opportunities that require cooperation across organizational boundaries.

The RHRC contract concluded in October 2025 — a planned conclusion, not a termination. AHS designed the engagement as a time-limited capacity-building investment, not an ongoing dependency. The explicit goal was for hospitals to develop the internal planning capability and the collaborative relationships needed to continue transformation work independently, with AHS providing ongoing oversight, grant funding, and analytical support rather than direct technical assistance.

This design philosophy reflects a lesson from failed state transformation initiatives: external consultant-driven processes that do not build internal organizational capacity produce plans that expire when the consultant leaves. Vermont's approach — using RHRC to build the planning process and the inter-hospital relationships, then transitioning to AHS-led coordination with hospital-level implementation — is more likely to produce durable change.

The Analytics Gap and Its Resolution

The November 2025 AHS transformation report identified one critical operational gap in the current system: the absence of an analytics platform capable of evaluating hospital transformation proposals quantitatively, modeling the cost and quality implications of different service configurations, and assessing alignment with statewide goals. This gap means AHS currently cannot answer with confidence the questions that the transformation planning process is supposed to produce answers to: if Springfield Hospital converts to a Rural Emergency Hospital, what is the net financial impact on the system? If orthopedic surgery is concentrated at Southwestern Vermont and Copley, what travel burden does that create for patients in the affected communities?

AHS responded to this gap by procuring an analytics vendor in partnership with GMCB — a joint procurement that reflects the appropriate division of labor: GMCB brings regulatory data and rate-setting expertise; AHS brings population health and SDOH context. The analytics platform, once operational, will enable the state to evaluate hospital proposals in terms of quality, cost, and access; assess alignment with regional and statewide goals; and quantify potential cost savings using evidence-based methods. This platform is also the technological foundation for monitoring the Statewide Strategic Plan's implementation metrics after December 2028\.

The Vermont Hospital Performance Dashboard — Where Each Hospital Stands

AHS's November 2025 transformation report established Vermont's official performance measurement framework — the set of outcome measures against which progress toward Act 167's five goals will be tracked. The framework covers three domains: provider efficiency and sustainability, payer affordability, and patient health outcomes. The following table presents the current performance landscape, with Vermont's actual values compared to benchmarks set at the 75th percentile of national peers.

Measure	VT Current	Benchmark (75th pctile)	Gap	Act 167 Goal
30-Day all-cause readmission rate	14.8%	14.5%	\+0.3% (within range)	Improve health outcomes
Potentially avoidable ED visits as % of all ED visits	32.3%	No benchmark set yet	Significant opportunity	Reduce inefficiencies; access
Operating profit per adjusted discharge — PPS hospitals	$5,345	$4,162	\+$1,183 above benchmark	Lower costs
Operating profit per adjusted discharge — CAH hospitals	$938	$2,784	\-$1,846 BELOW benchmark	Lower costs / sustainability
Mgmt. and admin cost per adjusted discharge — PPS	$2,730	$1,427	\+$1,303 above benchmark	Reduce inefficiencies
Vermonters with a personal health care provider (PCP)	91%	87%	\+4% above benchmark	Access / equity
Primary care practices accepting new Medicaid patients	59%	Not set	Access gap for Medicaid	Access / equity
Blueprint PCMH patients with hypertension controlled	77%	Not set	Quality management	Improve health outcomes
ED visits for suicide ideation/self-harm per 10K ED visits	245.5	Not set	Behavioral health access gap	Access / equity
ED visits for opioid overdose per 10K ED visits	16.1	Not set	SUD treatment access gap	Access / equity

Figure 7.3 — Vermont health system transformation outcome measures: current performance vs. benchmarks. Source: AHS Health Care System Transformation Report, November 2025\.

Several findings from this dashboard deserve specific attention from healthcare leaders and policymakers:

The CAH operating profit gap is the most urgent financial signal. Critical Access Hospitals are running at $938 operating profit per adjusted discharge, against a benchmark of $2,784 — a $1,846 per-discharge gap. This means Vermont's smallest, most rural hospitals are the furthest from financial sustainability. The RBP and global budget reforms will affect these hospitals differently from PPS hospitals, and Act 68 explicitly requires GMCB to consider CAH-specific adjustments. The RHT Program's explicit inclusion of CAH stabilization as a priority use of funds reflects this gap.

The administrative cost premium is massive. PPS hospitals are spending $2,730 per adjusted discharge on management and administrative costs, against a benchmark of $1,427 — a 91% premium over what efficient peer hospitals spend. This $1,303 per-discharge gap is exactly the administrative cost reduction that Oliver Wyman identified as one of the three primary sources of direct savings from transformation. If Vermont's PPS hospitals can reach the benchmark administrative cost level, the system savings are substantial — and Oliver Wyman estimated administrative reduction as contributing over $100 million of the $400M+ transformation savings target.

The behavioral health gap is measurable and alarming. With 245.5 ED visits per 10,000 for suicide ideation and self-harm — and rates statistically higher in Rutland and Windham counties, and among Vermonters aged 15-44 — the crisis presentation data confirms what Oliver Wyman's community meetings documented: Vermont's behavioral health access problem is driving a significant share of avoidable hospital utilization. This is not a clinical quality problem; it is an access problem. The CCBHC expansion (5 new entities by July 2026\) and the behavioral health investments funded through AHEAD's EAST Fund are the operational response.

Management and administrative cost per adjusted discharge: Vermont PPS hospitals vs. benchmark

Vermont PPS hospitals (actual)	**$2730**
National 75th percentile benchmark	**$1427**
Gap (excess admin cost)	**$1303**

Figure 7.4 — Administrative cost gap: Vermont PPS hospitals vs. national benchmark. This $1,303 per-discharge gap represents the primary operational inefficiency target for Vermont's transformation. Source: AHS Transformation Report, November 2025 (NASHP via CMS).

Regionalization — The Operational Logic of Right-Sizing a 14-Hospital System

Regionalization is the operational translation of Oliver Wyman's third imperative: move all care possible out of hospitals, and concentrate what must stay in hospitals at sites with sufficient volume to maintain expertise. The concept is straightforward; the execution is complex. This section develops the operational methodology of regionalization — how you actually move from a system of 14 hospitals each attempting full-spectrum care to a differentiated regional network where each facility has a clearly defined role, adequate patient volume, and sustainable financial structure.

The Regional Network Design Framework

Oliver Wyman's regionalization blueprint organized Vermont's hospital network around three facility types, each with a distinct role in the transformed system:

Tier 1 — Regional Specialty Centers (RSCs)

RSCs are the hospitals with sufficient population base, financial position, and existing expertise to sustain inpatient beds and serve as Centers of Excellence in multiple specialty areas. Under Oliver Wyman's blueprint, UVMMC in Burlington is the state's primary RSC with 16 COE designations. Southwestern Vermont Medical Center, Brattleboro Memorial Hospital, Rutland Regional Medical Center, and Central Vermont Medical Center function as secondary RSCs, each with 5-9 COE designations serving their regional populations.

The RSC model requires a fundamental change in how these hospitals think about their role: from serving their immediate geographic catchment area to accepting referrals from the entire region for their designated specialties. This means building the transfer infrastructure, the outreach relationships with smaller hospitals, and the care management capacity to serve a larger and more complex patient population.

Tier 2 — Community Hospitals with Focused Scope

Community hospitals with focused scope are inpatient facilities that maintain specific service lines where they have adequate volume and expertise — their designated COE specialties — while transferring other complex cases to RSCs. Northwestern Medical Center (St. Albans), Northeastern Vermont Regional Hospital (St. Johnsbury), Springfield Hospital, and Copley Hospital (Morrisville) fall into this category, with 2-4 COE designations each.

The operational challenge for this tier is managing the transition: reducing reliance on service lines being regionalized elsewhere while building capacity in designated specialties and managing the patient volume shift without destabilizing the hospital's financial position. This requires precisely the kind of short-term, medium-term, and long-term planning that RHRC's transformation plan methodology was designed to support.

Tier 3 — Transformed Facilities (REH, CACC, or Home-Based Models)

Tier 3 facilities are hospitals that cannot sustain inpatient beds in the long term — either because their population base is too small, their financial position is too distressed, or both. Rather than closing unexpectedly and leaving communities without any healthcare access, Oliver Wyman's blueprint proposes orderly conversion to one of three alternative models: Rural Emergency Hospital (REH — 24/7 emergency and observation without inpatient beds), Community Ambulatory Care Center (CACC — outpatient and primary care services), or a Care at Home support hub. Grace Cottage, Gifford Medical Center, North Country Hospital, and Porter Medical Center are among the facilities whose futures require further discussion as part of Vermont's regionalization planning.

The REH designation is particularly significant for Vermont because it preserves the emergency access function that communities depend on while eliminating the expensive inpatient infrastructure that these hospitals cannot financially sustain. A community with an REH and reliable EMS transport to an RSC 45 minutes away is better served than one with a financially distressed inpatient hospital that cannot staff its surgical suite or maintain specialist coverage.

Hospital (HSA)	Current COEs	Tier	Key transformation considerations
UVM Medical Center (Burlington)	16	Tier 1 — Primary RSC	Administrative overhead reduction is critical — $3,826 admin cost/adj. discharge vs. $1,427 benchmark. Examine non-patient-care provider activities (OW specific finding).
SW VT Medical Center (Bennington)	9	Tier 1 — Secondary RSC	Strong COE position. Key southern VT anchor. Psych adult/adolescent COE important given behavioral health gaps.
Brattleboro Memorial Hospital	6	Tier 1 — Secondary RSC	Acute general surgery COE is regionally critical. Windham County has higher-than-average suicide/SH and opioid rates — behavioral health access priority.
Rutland Regional Medical Center	5	Tier 1 — Secondary RSC	Central Vermont anchor; neurology and psych-adult COEs important. Rutland County has higher opioid overdose rates.
Central VT Medical Center (Barre)	5	Tier 1 — Secondary RSC	Radiation therapy and neurology COEs. Washington County (Barre/Montpelier) is second most populous HSA.
NW Medical Center (St. Albans)	4	Tier 2 — Focused scope	Northern VT anchor. Cancer surgery and radiation COEs. Franklin County workforce hub.
NE VT Regional Hospital (St. Johnsbury)	4	Tier 2 — Focused scope	Northeast Kingdom anchor — Essex/Caledonia/Orleans have highest uninsurance rates (6-8%) and most rural populations.
Springfield Hospital	2	Tier 2 — Focused scope	Memory care and psych-adult COEs. High SUD rates in Windsor/Windham corridor.
Copley Hospital (Morrisville)	2	Tier 2 — Focused scope	Orthopedics and rheumatology COEs. Lamoille County access point. Rural workforce housing challenge.
Mt. Ascutney (White River Jct.)	1	Tier 2 — Focused scope	Rehabilitation COE. Dartmouth Health affiliation creates cross-border care opportunities.
Grace Cottage, Gifford, North Country, Porter	TBD	Tier 3 — Requires discussion	Further assessment needed. REH conversion, CACC, and home-based care models all under consideration. RHT Program funds available for transition support.

Figure 7.5 — Vermont 14-hospital regionalization framework: COE designations, tiers, and key operational considerations. Sources: Oliver Wyman Act 167 Report (2024); AHS Transformation Reports (2025).

The Operational Mechanics of Regionalization

Regionalization is not a policy decision that happens once — it is an operational process that must be managed over years. The transition from a hospital system where every facility attempts full-spectrum care to one organized around regional specialization requires changes across five operational domains:

Transfer infrastructure is the most visible immediate need. Oliver Wyman documented that Vermont currently has 31 separate EMS agencies that a provider may need to contact to arrange a patient transfer — a fragmentation that is simultaneously a patient safety risk and an operational cost. Vermont's RHT Program application includes EMS transformation as a priority investment: professionalization and regionalization of EMS, improved inter-facility transfer coordination, and expanded telehealth to reduce unnecessary transfers. The November 2025 AHS report identified inter-facility transfer coordination as one of the primary statewide regionalization themes emerging from regional hospital convenings.

Clinical network agreements formalize the referral relationships between RSCs and Tier 2/3 facilities. Oliver Wyman recommended that RSCs establish formal agreements to receive referrals from emergency departments at smaller hospitals for their designated COE specialties — ensuring that when a patient arrives at Springfield Hospital needing complex general surgery, the transfer pathway to Brattleboro Memorial Hospital (the acute general surgery COE for that region) is established, protocolized, and reliable. These agreements do not currently exist in a systematic form across Vermont's hospital network; the Act 68 transformation planning process is the vehicle for creating them.

Administrative shared services offer some of the largest near-term cost savings in the transformation agenda. Oliver Wyman identified three shared service opportunity areas as high-priority: supply chain (group purchasing), administrative functions (billing, coding, human resources, credentialing), and IT infrastructure. Vermont hospitals have begun early-stage discussions on group purchasing during the regional convenings. The administrative cost gap — $1,303 per discharge above benchmark — is where shared services can make the most immediate impact on the $400M+ savings target.

Workforce redistribution is the most sensitive operational challenge. When a hospital transitions from full inpatient to REH or CACC status, the clinical staff whose roles were tied to inpatient services must either transition to new roles, relocate to RSCs, or leave the healthcare workforce. Oliver Wyman explicitly recommended that healthcare workforce affected by system changes could be redistributed or retrained to perform services needed by the community — recognizing that the workforce challenge is not just about attracting new providers to Vermont but about optimally deploying those already there. Vermont's RHT Program workforce investments — tuition assistance, recruitment incentives with 5-year service obligations, scope-of-practice expansion for nurses and APRNs — are the supply-side response; workforce redistribution planning is the demand-side response.

Community engagement is the political prerequisite for all operational change. Oliver Wyman's process demonstrated that communities are willing to accept difficult changes — service reductions, facility reconfiguration, changes in travel distances — when they have been genuinely engaged in the decision-making process and when they understand the alternative (unexpected closures) is worse. AHS has committed to continuing community engagement as transformation plans develop, expanding beyond hospital leadership to include community providers and other health and human services stakeholders as priorities are identified.

The Workforce Crisis — Vermont's Most Binding Operational Constraint

Of all the operational challenges Vermont faces, workforce is the one most likely to constrain the pace and ambition of transformation. Vermont's healthcare workforce problem is not a single issue — it is a cluster of interconnected problems that Oliver Wyman's systems map showed cascading across the entire healthcare system. Understanding the workforce problem in its full complexity is prerequisite to designing interventions that actually work.

The Dimensions of Vermont's Workforce Crisis

Primary care physician shortfall by 2030 370 FTEs 112 family medicine, 190 other primary care (RHT application)	**Hospitals with physician shortages cited as primary operational challenge All 14 Physician shortage cited in Oliver Wyman operational challenges for every HSA**
RHT Program workforce investment $195M *Over 5 years; tuition assistance, recruitment, 5-yr service obligations*	**Vermont unemployment rate (August 2025\) 2.5%** *Below 4.3% national rate; extremely tight labor market limiting healthcare recruitment*

Figure 7.6 — Vermont workforce crisis key metrics. Sources: Vermont RHT Program Application (November 2025); Oliver Wyman Act 167 Report; Vermont Department of Labor.

Vermont's workforce problem has five distinct but interconnected dimensions, each requiring a different intervention approach:

Primary care shortage is both a supply problem and a productivity problem. Oliver Wyman made a counterintuitive finding that Vermont's primary care shortage is partly a productivity issue, not purely a headcount problem: HRSA recognizes no Health Profession Shortage Areas in Vermont, and if primary care providers were supported to see three patients per hour, the state would have adequate supply. The implication is that scope-of-practice expansion for APRNs and physician assistants, team-based care models that use clinical staff at the top of their licensure, and administrative burden reduction (prior authorization reform, documentation support) can meaningfully increase primary care capacity without adding headcount.

Specialist shortage drives regionalization urgency. Vermont cannot maintain specialist coverage at 14 hospitals across a state of 647,000 people. The population density and economics do not support it. The Oliver Wyman COE framework is, in part, a response to the specialist shortage: rather than trying to recruit and retain a cardiologist or neurosurgeon for every Vermont community, concentrate those specialists at facilities with sufficient volume to justify their positions and deliver the clinical excellence their presence enables.

Nursing shortage is the most acute staffing crisis at individual hospitals. All Vermont hospitals cited physician shortages and difficulty recruiting staff since COVID-19 as primary operational challenges in Oliver Wyman's assessment. Travel nursing — the expensive emergency staffing solution that many hospitals have relied on since 2020 — has significantly worsened hospital financial positions. Travel nurse costs are typically two to three times the cost of a permanent hire. Vermont hospitals that remain heavily dependent on travel nursing are in a financial hole that deepens with every shift filled by a travel agency.

Housing is the hidden workforce constraint. Oliver Wyman's systems map traced the connection from housing shortage to workforce shortage to healthcare system fragility explicitly. Vermont's rental vacancy rate of 3% — well below the 5% rate of a healthy market — means healthcare workers recruited to Vermont communities cannot find housing. Half of Vermont renters are cost-burdened. This is not a healthcare problem that healthcare reform can solve; it requires housing policy responses. But AHS's transformation mandate, properly understood, extends to advocating for and enabling the housing investments that make healthcare workforce recruitment possible. This is one of the reasons Oliver Wyman's first imperative — build housing and fix transportation — comes before payment reform in the three-imperative sequence.

Scope-of-practice restrictions limit Vermont's ability to use the workforce it has effectively. Vermont has taken some steps toward scope-of-practice expansion — joining the Social Work Licensure Compact, allowing pharmacist-extended prescriptions, approving mental health professionals without master's degrees for certain psychotherapy roles — but Oliver Wyman's action list called for further expansion of professional licensure and practice scope for nurses, EMTs, and pharmacists. The RHT Program's workforce investments include scope-of-practice-enabling training, and Act 68's workforce goal — attracting and retaining high-quality health care professionals — implicitly requires the scope-of-practice environment to make Vermont competitive with surrounding states.

Vermont's Workforce Strategy: The RHT Investment Framework

Vermont's Rural Health Transformation Program application dedicated a substantial portion of its $195M award to workforce infrastructure — treating workforce as capital investment rather than operating expense. The distinction matters: using one-time capital funds to build durable workforce pipelines (tuition assistance that creates committed local providers, training infrastructure that enables scope expansion, housing investments that make recruitment viable) is fundamentally different from using them to fund ongoing salaries.

RHT Workforce Investment Priorities

Vermont's RHT Program application included five workforce investment areas, each designed to produce durable capacity rather than temporary staffing:

- Tuition assistance and loan forgiveness for nurses, physicians, APRNs, LNAs, home health aides, and other providers, with 5-year service obligations in Vermont — building a locally-rooted workforce pipeline
- On-site clinical training infrastructure — supporting teaching programs that keep medical students and residents in Vermont communities during training, increasing the probability they establish practices there
- Scope-of-practice training and support for APRNs, physician assistants, pharmacists, and EMTs — enabling Vermont to use its existing workforce at maximum capacity
- Community health worker training and deployment — building the SDOH navigation and care coordination workforce that population health management requires, drawing from the communities being served
- Interstate Medical Licensure Compact (IMLC) participation — enabling out-of-state providers to practice via telehealth in Vermont, reducing specialist access gaps without requiring physical relocation

The Operations Metrics Framework — What AHS Must Track

Effective operations management requires clear, measurable indicators of progress. AHS's current transformation outcome framework (Figure 7.3) establishes a baseline measurement architecture, but it has acknowledged limitations: most data reflects 2023 or earlier performance, the impact of 2025 transformation activities will not appear in measures until 2026 or later, and some critical domains — mental health, SDOH, post-acute care — lack the consistent data infrastructure to support real-time monitoring.

For the Statewide Strategic Plan that AHS must deliver by December 2028, a more complete operations metrics framework is required. Drawing on Oliver Wyman's five Act 167 goals and Act 68's accountability metrics requirements, the following framework organizes the key operational indicators by domain:

Domain	Key metrics	Data source	Target direction
Hospital financial sustainability	Operating profit per adjusted discharge by hospital type; management/admin cost per adjusted discharge; days cash on hand; bond ratings; travel nurse cost as % of total labor	GMCB hospital budget data; CMS cost reports; NASHP Hospital Cost Tool	CAH: toward $2,784 benchmark. PPS admin cost: toward $1,427 benchmark. Travel nursing: reduce toward 5-10% of total labor
Care access	% primary care practices accepting new Medicaid patients; wait times for new patient appointments; potentially avoidable ED visit rate; 30-day follow-up after ED visit for mental illness and SUD	VCCI; GMCB; VDH; Blueprint for Health	Primary care acceptance: increase from 59%. Avoidable ED visits: reduce from 32.3%. 30-day MH follow-up: increase from 76%
Population health	Hypertension control rate in PCMH patients; diabetes HbA1c not in control; all-cause readmission rate; ED visits for suicide ideation/self-harm; ED visits for opioid overdose	Blueprint for Health; VDH; CMS	Hypertension control: maintain 77%; diabetes: reduce 22% not-controlled; readmission: maintain at/below 14.8%

Domain	Key metrics	Data source	Target direction
Cost and affordability	Per-member-per-month total cost of care trend; commercial premium growth rate; individual market affordability for medium-high utilization family; medical loss ratio for marketplace plans	GMCB; insurance rate filings; CMS; VHCURES	PMPM cost: below AHEAD total cost of care targets. Premium growth: reduce toward CPI. MLR: maintain 89%+
Equity	Geographic variation in primary care access by county; health outcomes variation by income and race/ethnicity; uninsured rate by county; Medicaid acceptance rates by region	VDH BRFSS; GMCB; VHCURES	Northeast Kingdom uninsurance gap: reduce from 6-8% toward statewide 3%. Medicaid acceptance: increase across all regions
Operations efficiency	Hospital-to-hospital transfer completion rate and time; EMS response time by county; shared service adoption rate; administrative cost trend; COE referral volume by specialty	EMS records; GMCB; hospital transformation plans	Transfer completion: establish baseline and reduce time. Admin cost trend: negative year-over-year. COE referral volume: increase as regionalization advances

Figure 7.7 — Vermont health system transformation: comprehensive operations metrics framework. Sources: Act 68 of 2025; AHS Transformation Report (November 2025); Oliver Wyman Act 167 Report.

One structural observation about this metrics framework: many of the most important indicators will not show measurable improvement for 18-24 months after the interventions that should drive them are implemented. Healthcare system change is slow, and the data infrastructure to measure it is even slower. AHS's acknowledgment in its November 2025 report that most available measures reflect 2023 or earlier performance is not an evasion — it is an accurate description of the measurement reality. Policymakers and oversight bodies need to understand this lag and resist the temptation to declare failure when early data does not yet show improvement.

AHS as System Operator — Organizational Design for the 2028 Mandate

The operations chapter concludes with the organizational design question that underpins everything else: is AHS currently structured to execute the transformation it has been mandated to lead? The honest answer, reflected in Oliver Wyman's findings and AHS's own public reports, is: not yet, but the organization is restructuring.

The Current Organizational Gaps

Oliver Wyman identified four specific organizational gaps in AHS that impede transformation execution. These are not criticisms of individuals — they reflect an organizational structure designed for a different era that has not yet adapted to the system operator role Act 68 requires.

- **Misalignment between AHS sub-unit organization and Hospital Service Areas. Vermont's AHS is organized around program types (mental health, substance use, aging services, Medicaid, etc.) rather than geographic service areas. This means that when a hospital in the Northeast Kingdom is trying to address the housing, transportation, behavioral health, and primary care gaps that drive its ED volume, it must work with multiple separate AHS programs — each with its own administrative requirements, reporting cycles, and performance metrics. Oliver Wyman's recommendation to align AHS sub-units with Hospital Service Areas and meet regularly with hospitals and providers in each HSA is a structural reorganization, not just a coordination improvement.**

- **Absence of a Planning and Effectiveness function. AHS currently lacks a dedicated analytical unit that can calculate the financial and access impacts of service site changes, monitor access and affordability across community providers, and track the quality, access, and equity metrics that the Statewide Strategic Plan will require. The analytics vendor procurement in partnership with GMCB addresses part of this gap, but the internal capacity to interpret, apply, and act on analytical outputs is a separate organizational requirement.**

- **Under-resourcing for the transformation management function. Act 68 requires AHS to simultaneously negotiate with CMS on AHEAD implementation, manage 14 hospital transformation planning processes, develop the Statewide Strategic Plan, oversee RHT Program implementation, coordinate with GMCB on RBP and global budget design, and report monthly to the legislature. The Health Care Reform Office, recently reorganized and supplemented by contractor support, is managing a workload that probably requires 50-70% more staff than currently exist. The RHT Program award explicitly includes funds for hiring staff to oversee its implementation.**

- **Limited SDOH integration across AHS programs. The Oliver Wyman analysis demonstrated that housing, transportation, food security, and behavioral health — all areas where AHS has programmatic authority — are primary drivers of healthcare utilization. But AHS's current organizational structure treats these as separate program areas rather than as integrated components of a population health strategy. The Statewide Strategic Plan mandate creates the opportunity to redesign this integration explicitly.**

The Emerging Solution: A Population Health Management Operating Model

The organizational model that AHS's Strategic Plan should articulate — and that the AHS restructuring chapter later in this book develops in full — is a population health management operating model organized around Vermont's Hospital Service Areas, with clear roles for each level of the system.

At the statewide level, AHS functions as the system architect: setting the strategic direction, establishing the payment reform framework (in partnership with GMCB), allocating capital (RHT Program, AHEAD EAST Fund, Act 68 grants), and monitoring system performance against the five Act 167 goals.

At the HSA level, regional care networks — organized around the Tier 1 RSC for each region — function as the operational unit: coordinating transfer pathways, managing shared service arrangements, aligning primary care and behavioral health investment with hospital transformation plans, and connecting healthcare planning with housing, transportation, and SDOH infrastructure planning.

At the provider level, individual hospitals, primary care practices, designated agencies, FQHCs, and community-based organizations function as service delivery nodes: executing transformation plans, participating in regional coordination, and reporting performance against standardized metrics that allow AHS to monitor progress and identify where additional support is needed.

This three-level operating model is not new to health system design — it is the architecture of every effective regional health system in the country. What is new in Vermont is the explicit statutory mandate (Act 68's Strategic Plan requirement), the financial resources to build the infrastructure (RHT Program, AHEAD EAST Fund), and the political will — demonstrated through the extraordinary legislative sequence of Acts 167, 51, 55, and 68 — to execute the structural change rather than continuing to manage the decline.

Key Concepts Introduced in This Chapter

Term	Definition
Regional Specialty Center (RSC)	A hospital designated to concentrate volume and clinical expertise in multiple specialty areas, serving as the referral destination for those specialties across a multi-HSA region. Vermont's Tier 1 hospitals under the Oliver Wyman regionalization blueprint.
Rural Emergency Hospital (REH)	A federal CMS designation allowing a hospital to provide 24/7 emergency and observation services without inpatient beds. A financially sustainable alternative for Vermont hospitals that cannot sustain full inpatient capacity

Term	Definition
	but must maintain emergency access for their communities.
Community Ambulatory Care Center (CACC)	An outpatient and primary care facility model proposed by Oliver Wyman as a potential conversion option for hospitals unable to sustain inpatient operations.
Potentially Avoidable ED Visit	An emergency department visit that could have been prevented through timely primary care, care coordination, or SDOH intervention. Vermont's rate of 32.3% of all ED visits represents a major quality and cost improvement opportunity.
Rural Health Redesign Center (RHRC)	AHS's primary technical assistance contractor for hospital transformation planning in 2025, providing financial assessment, change management support, and transformation plan development across all 14 Vermont hospitals. Contract concluded October 2025\.
Hospital Service Area (HSA)	The geographic area defined around each Vermont hospital and the population it primarily serves. Vermont has 14 HSAs. Oliver Wyman recommended organizing AHS sub-units around HSAs to improve coordination between AHS programs and healthcare providers.
Travel nursing cost	The cost of temporary nurses hired through staffing agencies to fill vacancies, typically 2-3x the cost of permanent hires. Heavy travel nursing dependence is a primary driver of hospital financial distress across Vermont's system.
Administrative shared services	Centralized provision of non-clinical administrative functions (billing, coding, HR, credentialing, IT, group purchasing) across multiple hospitals, reducing the per-hospital administrative cost that is currently 91% above benchmark for Vermont PPS hospitals.
Population health management operating model	An organizational structure that manages healthcare resources at the population level — tracking health outcomes, utilization patterns, and SDOH factors across a defined geography — rather than managing individual patient encounters. The organizational model appropriate for a global budget environment.

Sources: Oliver Wyman Healthcare and Life Sciences Group, Act 167 Community Engagement: Recommendations (August 2024, revised October 2024); Vermont Agency of Human Services, Health Care System Transformation Report (November 1, 2025); Vermont Agency of Human Services, Health Care System Transformation Report (August 2025); Vermont Agency of Human Services / Brendan Krause, Vermont's Health Care Reform Efforts, House Health Care Committee (January 31, 2025); Act 68 of 2025 (Vermont); Vermont Rural Health Transformation Program Application (November 3, 2025); Green Mountain Care Board, 2024 Annual Report (January 15, 2025); Vermont Agency of Human Services, Health Care Transformation website (healthcarereform.vermont.gov), accessed April 2026\.

Chapter 8: The Operations Pillar in Practice — Revenue Cycle, HCC Coding, Credentialing, and Workforce in a Value-Based Environment

The Operations Pillar in Practice — Revenue Cycle, HCC Coding, Credentialing, and Workforce in a Value-Based Environment

The Operational Infrastructure That Makes or Breaks VBC Contracts: Clinical Documentation Excellence, Risk Adjustment Strategy, Credentialing Efficiency, and the Workforce Operations Framework for Transformed Health Systems

Operations Where Money Actually Lives

Healthcare transformation is ultimately executed through the operational infrastructure that most health system leaders think of as administrative — revenue cycle management, clinical documentation, credentialing, workforce management. These functions are not adjacent to clinical transformation; they are the substrate on which it runs. An organization that redesigns its care model for value-based care but fails to adapt its revenue cycle, clinical documentation, and workforce operations will achieve neither the quality outcomes nor the financial results the new model promises.

This chapter develops the operational mechanics of healthcare transformation — the specific processes, metrics, and management disciplines that determine whether value-based care contracts produce the results they are designed for. It is organized around four operational domains: revenue cycle transformation for VBC, HCC coding and risk adjustment excellence, credentialing efficiency, and the workforce operations framework.

Revenue Cycle Transformation — From FFS Optimization to VBC Management

Revenue cycle management under fee-for-service is a known discipline: charge capture, coding accuracy, claim submission, denial management, underpayment identification. Hospitals and physician practices have invested heavily in FFS RCM over the past two decades, and the industry has developed sophisticated tools and processes for maximizing FFS revenue. None of this investment is wasted under VBC — but all of it must be extended.

VBC revenue cycle adds three functions that FFS RCM does not require: attribution management (ensuring that patients are correctly attributed to the organization for shared savings calculation), total cost of care monitoring (tracking what attributed patients cost across all providers and settings, not just at your organization), and VBC contract reconciliation (understanding and disputing the financial calculations that determine shared savings or loss settlements).

Attribution Management — The VBC-Specific Revenue Function

Under FFS, revenue is determined by what care the organization delivers. Under VBC, revenue is determined partly by which patients are attributed to the organization — and attribution errors can silently reduce shared savings or inflate shared losses by millions of dollars. Every organization in a significant VBC arrangement needs an attribution management function.

Attribution error type	How it happens	Financial impact	Remediation
False attributions	Patients who receive care elsewhere are attributed to your organization due to a primary care visit that triggered attribution before the patient transferred care	Your organization bears cost of care you did not deliver and cannot manage	Regular attribution list review; proactive patient outreach to confirm active care relationship; request attribution corrections from payer using patient-level evidence
Missing attributions	Your highest-cost, highest-need patients are not attributed because they lack a primary care visit that would trigger attribution	You lose the opportunity to earn shared savings for care management programs that benefit this population	Proactive outreach to unattributed high-risk patients; care management programs that include primary care visits to establish attribution; PCMH recognition to improve attribution algorithms
Incorrect benchmark attribution	Attribution list errors inflate or deflate the risk adjustment factor applied to your benchmark	Incorrect RAF produces a benchmark that doesn't reflect your population's actual cost burden	Compare attribution list to actual patient panel; use clinical data to validate risk adjustment; submit corrected diagnoses codes through formal channels
Retroactive attribution changes	Payer changes attribution methodology mid-contract, shifting patients in or out of your attributed population	Shared savings calculations change unexpectedly after performance year ends	Negotiate prospective attribution methodology in contract; require prior notice of methodology

Attribution error type	How it happens	Financial impact	Remediation
			changes; maintain your own attribution model to detect changes

Figure 8.1 — Attribution error taxonomy and remediation. Source: the framework Operations pillar framework; ACO REACH and MSSP operational guidance.

Total Cost of Care Monitoring — Seeing What You Cannot Bill

The most difficult revenue cycle adaptation for FFS-trained teams is monitoring costs they do not receive. Under VBC, the organization's financial performance is determined by the total cost of care for its attributed population — including care delivered by specialists, hospitals, skilled nursing facilities, home health agencies, and any other provider outside the organization. A primary care group in an ACO may be held accountable for whether their attributed patients receive unnecessary specialist imaging ordered by a cardiologist who has no financial relationship with the ACO.

Vermont's VHCURES all-payer claims database is the analytical foundation for total cost of care monitoring — providing claims data across all payers and provider types for attributed Vermont beneficiaries. The GMCB analytics vendor procurement, and individual hospital analytics investments through the RHT Program, are building the capability to use VHCURES data for real-time population health management rather than retrospective reporting.

HCC Coding Excellence — The Risk Adjustment Foundation

Hierarchical Condition Category (HCC) coding is the mechanism through which Medicare risk adjustment translates patient diagnoses into the Risk Adjustment Factor (RAF) that scales a beneficiary's expected cost. In VBC arrangements, accurate HCC coding is not just a compliance requirement — it is the foundation of financial sustainability. An organization that systematically under-codes its patients' diagnoses will have a benchmark set too low for its population's actual cost burden, and will appear to be underperforming even when its care management is excellent.

The HCC Model — How It Works

Medicare's HCC model assigns each patient a RAF based on their demographic characteristics (age, sex, Medicaid status, disability) and their active chronic conditions as documented in claims. The RAF multiplies the base capitation or benchmark rate — a patient with a RAF of 1.5 has a benchmark 50% higher than the average Medicare beneficiary. Accurate diagnosis coding is essential because the RAF is calculated from diagnosis codes on claims; diagnoses that are not coded do not affect the RAF.

Vermont's rural hospital population has two characteristics that make HCC coding excellence especially important: high prevalence of complex chronic conditions (diabetes, COPD, CHF, CKD) that generate large HCC score differences based on coding specificity, and significant dually eligible (Medicare-Medicaid) population that has additional risk adjustment mechanisms. Vermont hospitals that do not code to the highest level of specificity are systematically under-representing their population's complexity and receiving benchmarks that do not reflect actual cost burden.

The HCC Gap Closure Process

The HCC gap closure process — identifying diagnoses that should be coded based on clinical evidence but are not currently documented in claims — is one of the highest-ROI operational investments available to organizations in VBC arrangements. The process has four steps:

- **RAF analysis: Compare the organization's average RAF against national and regional benchmarks for comparable populations. A RAF significantly below benchmark for a population with high chronic disease prevalence signals a coding gap.**
- **Retrospective chart review: Review clinical documentation for attributed patients to identify diagnoses that are clinically evident (in problem lists, clinical notes, medication lists) but not documented in claims codes. Each uncoded HCC condition represents a missed RAF point.**
- **Prospective coding education: Educate clinical staff on HCC coding specificity requirements — the difference between coding 'diabetes' (HCC 19, lower RAF) versus 'diabetes with diabetic CKD' (HCC 18 \+ HCC 136, higher RAF) when clinical evidence supports the more specific code.**
- **Annual wellness visits: AWVs are the primary vehicle for capturing annual HCC updates — CMS requires that chronic conditions be documented at least annually to be included in RAF calculations. Systematic AWV completion for the attributed population is both a quality metric and an HCC gap closure mechanism.**

Vermont in Practice: HCC Coding at Vermont Community Hospitals

Oliver Wyman's Act 167 assessment of Vermont's hospital system did not specifically examine HCC coding accuracy, but the management and administrative cost data tells a related story: Vermont PPS hospitals spend $2,730 per adjusted discharge on management and administrative costs against a benchmark of $1,427. Part of this gap reflects genuine administrative inefficiency — systems and processes that could be streamlined. But part of it also reflects underinvestment in the clinical documentation and coding infrastructure that makes revenue cycle management accurate under both FFS and VBC.

Vermont hospitals entering the AHEAD global budget model in 2027 will have their budgets set based on historical Medicare spending adjusted for their population's risk complexity. An organization that has under-coded its population's HCC conditions will receive a lower global budget than its actual cost burden warrants — and will need to manage within that lower budget for the full performance year before the error can be corrected. Getting HCC coding right before AHEAD launches is not optional — it is foundational financial preparation.

Recommended action: Every Vermont hospital should conduct a retrospective HCC gap analysis for their AHEAD-attributed Medicare population before January 2027\. The analysis should compare clinical documentation to coded diagnoses, identify the largest gaps by HCC category, and develop a provider education and AWV completion program to close the gaps prospectively.

Credentialing Efficiency — The Hidden Operations Bottleneck

Provider credentialing — the process of verifying a clinician's education, training, licenses, and competency before granting hospital privileges or health plan participation — is one of the most operationally consequential and consistently underinvested functions in American healthcare administration. The average credentialing cycle takes 90-120 days for a new provider. In a state with a 370 FTE primary care shortage by 2030, a 90-day credentialing delay represents 90 days of foregone care — and foregone revenue — for every provider recruited.

Credentialing as a Transformation Bottleneck

Vermont's transformation agenda accelerates the credentialing importance for two specific reasons. First, regionalization — moving to a tiered hospital network where patients follow clinical protocols to Centers of Excellence — requires providers to have privileges at multiple facilities, not just their home institution. A surgeon designated as a COE orthopedic provider must be credentialed at the regional COE facility to practice there, and the credentialing process must occur before the regionalization takes effect. Vermont hospitals that do not begin cross-facility credentialing before COE designations are finalized will face delays in implementing the regionalization blueprint.

Second, scope-of-practice expansion — enabling APRNs, PAs, and other non-physician clinicians to practice at the top of their licensure — requires updated credentialing standards and privilege delineations at every facility. Oliver Wyman's workforce recommendation to expand professional licensure and practice scope is not a paperwork exercise; it requires systematic credentialing policy updates at 14 hospitals, coordinated through the Vermont CIN's shared services infrastructure.

The Credentialing Efficiency Framework

Three operational improvements reduce credentialing cycle time from 90-120 days to 45-60 days without reducing verification rigor:

- **Primary source verification automation: Replace manual verification of medical school diplomas, residency completions, and license verifications with automated primary source connections to NPDB, state licensing boards, and medical schools. Several credentialing platforms (VerityStream, Symplr, symplr MD-Staff) provide automated PSV that reduces verification time from weeks to hours.**

- **Continuous monitoring replacing point-in-time verification: Implement ongoing license and sanction monitoring that alerts the credentialing office when a provider's license status changes, rather than discovering changes at 2-year re-credentialing intervals. Continuous monitoring reduces risk while eliminating the administrative burden of complete re-verification at each cycle.**

- **Unified credentialing for the Vermont CIN: Vermont's 14 hospitals sharing a common credentialing database — so that a provider credentialed at one CIN member facility has their verified credentials available to all member facilities — reduces the per-facility credentialing burden for providers practicing at multiple sites. This is a direct shared service application that Oliver Wyman's CIN design makes possible.**

Workforce Operations — Managing the Care Team in a Transformed System

The Operations pillar's workforce domain encompasses three distinct management challenges that often get conflated: workforce supply (recruiting enough providers), workforce deployment (organizing providers in care team models that maximize effective capacity), and workforce retention (reducing attrition that compounds the supply problem). Vermont faces challenges in all three domains, and the solutions are different.

The Workforce Supply Challenge — What Can Actually Be Done

Vermont projects a 370 FTE primary care shortage by 2030\. Physician recruitment pipelines take 7-10 years from medical school enrollment to practice — making it impossible to recruit Vermont out of a 2030 shortage with current policies. The supply interventions that can produce meaningful results within the relevant timeframe are:

- Scope-of-practice expansion for APRNs and PAs: Oliver Wyman specifically recommended this. Vermont APRNs operating under full practice authority (without physician supervision requirements) can see three patients per hour in PCMH settings, effectively adding primary care capacity equivalent to additional physicians without waiting for new physicians to train.

- International Medical Graduate (IMG) recruitment and J-1 visa waiver programs: Vermont has J-1 visa waiver slots through the Conrad 30 program, enabling internationally trained physicians to practice in underserved areas in exchange for 3-year service commitments. Active management of this program — filling all available slots, targeting Northeast Kingdom communities — can meaningfully supplement domestic recruitment.

- Loan repayment and service obligation programs: Vermont's RHT Program workforce investments include tuition assistance and loan repayment with 5-year service obligations. The evidence on loan repayment is consistent: it works for early-career providers with high debt burdens. Vermont's program should be structured with the largest awards for the communities with the greatest access gaps.

- Regional medical education investment: The evidence on training location and practice location is strong — physicians trained in rural settings are more likely to practice in rural settings. Vermont's partnership with University of Vermont College of Medicine, and potential expansion of clinical training sites in Northeast Kingdom communities, is a 7-10 year investment with compound returns.

Workforce Deployment — Team-Based Care at Scale

Oliver Wyman's finding that Vermont does not have an absolute primary care shortage but may have a care delivery model problem is operationally actionable. A primary care team in which physicians see 3 patients per hour — supported by medical assistants, care coordinators, CHTs, and behavioral health care managers who handle pre-visit preparation, care gap follow-up, and care coordination — has twice the effective capacity of a physician practicing in a traditional solo model.

Vermont's Blueprint PCMH model embeds this team-based care logic in its recognition standards. The operational challenge is that many Vermont practices — particularly smaller rural practices — lack the staffing depth to implement true team-based care. A rural practice with one physician and one MA cannot implement a full care team model regardless of how well the physician is trained. The RHT workforce investments, specifically the APRN and CHW training programs, are designed to build the care team depth that team-based care requires.

Workforce Retention — The Math of Turnover

Physician and clinician turnover in rural healthcare is both a symptom of system stress and a cause of it. Every physician departure costs approximately $500,000-$1 million to replace (recruitment costs, onboarding time, lost productivity, temporary coverage costs). In a system where 14 hospitals are competing for a limited rural provider pool, high turnover is financially devastating — and the signal it sends to communities about healthcare stability intensifies the system fragility it creates.

The operational drivers of rural provider retention are well-documented: burnout driven by documentation burden (addressed by AI scribe technology), isolation from professional peers (addressed by telemedicine networks and CIN peer connectivity), inadequate support staff (addressed by team-based care investment), housing unavailability (addressed by Oliver Wyman's first imperative), and work-life balance limitations in single-provider practices (addressed by coverage sharing agreements within the CIN network). Vermont's transformation agenda addresses all of these — the question is whether the investments are made before the next wave of provider departures.

Administrative Simplification — The Vermont Agenda

Oliver Wyman's community engagement found that providers across Vermont cited administrative complexity — prior authorization, CON processes, billing burden, duplicative reporting requirements — as major constraints on care delivery efficiency. Administrative simplification is not a soft nicety; it is an Operations pillar requirement that directly affects the effective capacity of Vermont's clinical workforce.

The Vermont Administrative Simplification Priorities

AHS and GMCB have identified three administrative simplification priorities that must be addressed in the Statewide Strategic Plan:

- **Prior authorization reform: As developed in Chapter 5, Vermont's Act 167 and Act 68 both address prior authorization complexity. The specific operational target is moving to gold-carding arrangements — automatic approval for providers with demonstrated compliance histories — and eliminating PA requirements for evidence-based, guideline-concordant care in high-volume service categories.**

- **CON process streamlining: Vermont's Certificate of Need process for hospital capital investments was cited as a barrier to efficient regionalization — providers cannot build the facilities and purchase the equipment that transformation requires if CON approval takes 12-18 months and involves litigation risk. Act 167 directed GMCB to evaluate CON process improvements; Vermont's transformation requires faster turnaround for transformation-aligned capital investments.**

- **Standardized quality reporting: Vermont's quality reporting landscape requires providers to submit data to multiple programs (Blueprint, GMCB, AHEAD, commercial payers) with overlapping but non-identical measures. The Blueprint Quality Measure Crosswalk begins to address this; the Statewide Strategic Plan should commit to further standardization that eliminates duplicative data submission for commonly required measures.**

Key Concepts

Term	Definition
Attribution management	The operational function of ensuring that patients are correctly attributed to an ACO or VBC organization for shared savings calculations, including regular attribution list review, correction request processes, and proactive outreach to confirm care relationships.
HCC (Hierarchical Condition Category)	The Medicare risk adjustment model that assigns RAF scores based on patient diagnoses. Accurate and complete HCC coding is essential for VBC contract financial management — under-coded diagnoses produce benchmarks that understate the population's expected cost.
RAF (Risk Adjustment Factor)	The multiplier applied to a benchmark or capitation rate based on a patient's HCC profile. A RAF of 1.5 means the patient is expected to cost 50% more than the average Medicare beneficiary.
AWV (Annual Wellness Visit)	A Medicare-covered annual visit focused on health risk assessment, preventive care planning, and chronic condition documentation. The primary mechanism for capturing annual HCC updates; systematic AWV completion is both a quality metric and an HCC gap closure tool.
HCC gap closure	The process of identifying diagnoses that are clinically evident but not currently documented in claims, through retrospective chart review and prospective coding education. A high-ROI operational investment for organizations entering VBC arrangements.
Gold carding	A prior authorization arrangement in which providers with demonstrated compliance histories receive automatic approval without requiring case-by-case review. Vermont's administrative simplification agenda includes gold carding as a priority PA reform.
Primary source verification (PSV)	The credentialing process of directly confirming provider credentials with the issuing source (medical school, residency program, state licensing board) rather than accepting secondary documentation. Automation of PSV reduces credentialing cycle time from weeks to hours.
Conrad 30 program	A federal J-1 visa waiver program enabling internationally trained physicians to practice in medically underserved areas in exchange for 3-year service commitments. Vermont has 30 annual waiver slots; active management of this program is a near-term primary care supply intervention.

Sources: the framework Operations pillar framework; Oliver Wyman Act 167 Report (2024); Act 68 of 2025; Vermont RHT Program Application (November 2025); CMS ACO REACH and MSSP operational guidance; CMS Medicare Risk Adjustment documentation; AHS Transformation Reports (2025); Vermont Blueprint for Health Annual Report (2024).

Chapter 9: The Clinical Pillar — Redesigning Care Delivery for a Transformed System

The Clinical Pillar — Redesigning Care Delivery for a Transformed System

The Blueprint for Health, Behavioral Health Integration, CCBHC Expansion, Primary Care Transformation, and Vermont's Clinical Quality Architecture

Sources: Vermont Blueprint for Health Annual Report 2024; Vermont DMH CCBHC Program; AHS Mental Health Integration Pilot Evaluation 2025; Oliver Wyman Act 167 Recommendations; AHS Transformation Reports (2025); Act 68 of 2025; Vermont RHT Program Application; SAMHSA CoCM resources; Population Health Management journal (Blueprint ROI study).

The Clinical Pillar: What It Is and Why It Is Not the Same as Clinical Quality

A common confusion in healthcare transformation discussions conflates the clinical pillar with clinical quality measurement — HEDIS scores, Star ratings, readmission rates. Clinical quality measurement is important, and this chapter addresses it, but the clinical pillar is a larger concept. It encompasses the design of care delivery itself: how care is organized, who provides it, where it is delivered, and how the various elements of the healthcare system are coordinated around the patient's actual health needs rather than around administrative or financial convenience.

In the context of Vermont's transformation, the clinical pillar question is specific and urgent: what does primary care look like in a system no longer organized around hospitals as the default site of care? What does behavioral health integration look like in a state with 245 ED visits per 10,000 for suicide ideation and self-harm, and inadequate community mental health capacity? What does care coordination look like for Vermont's rapidly growing elderly population, with the highest-cost, most complex needs? And how do these clinical design questions connect to the payment reform and operational transformation happening simultaneously?

Vermont has more developed answers to these clinical design questions than most states, because Vermont has been running the Blueprint for Health — the most sophisticated all-payer primary care transformation program in the country — for more than fifteen years. The Blueprint is not a pilot or a demonstration. It is operating infrastructure, funded by all payers, embedded in primary care practices across all 14 Hospital Service Areas, and producing measurable results. Understanding the Blueprint is the foundation for understanding Vermont's clinical pillar strategy.

The Vermont Blueprint for Health — Fifteen Years of Primary Care Transformation

The Blueprint for Health is Vermont's all-payer primary care transformation program, established in 2006 and reaching statewide implementation by 2013\. It is, in the view of most independent analysts, the most successful sustained primary care transformation initiative in American health policy. Its longevity, its all-payer architecture, its integration with Vermont's health data infrastructure, and its documented results make it the clinical foundation on which Vermont's broader healthcare transformation depends.

The Blueprint Architecture

The Blueprint operates through three integrated components that together constitute a population health management system at the primary care level:

Component	What it is	How it is funded	What it does
Patient-Centered Medical Home (PCMH)	Primary care practices recognized by NCQA as meeting advanced standards for care coordination, quality	Per-member-per-month (PMPM) payments from all payers — Medicare, Medicaid, and commercial insurers —	Transforms primary care practices from reactive, visit-based care to proactive, population-based care; coordinates care

Component	What it is	How it is funded	What it does
	improvement, population management, and patient engagement	with a base payment plus a performance payment tied to quality outcomes	across providers; manages chronic conditions; screens for behavioral health and SDOH needs
Community Health Team (CHT)	Multi-disciplinary teams of social workers, nurses, community health workers, and behavioral health professionals embedded in or closely connected to primary care practices	Blueprint program funding through DVHA; supplemented by OneCare (now transitioning), Medicaid, and grant funding	Provides care coordination, SDOH navigation, behavioral health support, and connection to community services for patients with complex needs; bridges the medical-nonmedical divide
Health Information Technology	VITL-based health information exchange linking practices to the statewide clinical data registry and VHCURES all-payer claims database	VITL funding; Blueprint program funding for data analytics support	Enables population-level HEDIS measurement; provides practices with care gap reports and patient registries; supports quality improvement with data-driven insights

Figure 9.1 — Vermont Blueprint for Health: three integrated components. Sources: Blueprint for Health website; Vermont Blueprint Overview (GMCB, 2022); Blueprint Annual Report 2024\.

The Evidence: What the Blueprint Has Produced

Vermont's Blueprint is one of the most extensively studied primary care transformation programs in the country. The evidence base spans more than a decade of operation and multiple independent evaluations.

The most rigorous evaluation — a six-year longitudinal study using VHCURES all-payer claims data published in Population Health Management — compared Blueprint participants to non-Blueprint primary care populations and found consistent advantages across expenditure, utilization, and quality outcomes. The critical financial finding: medical expenditures decreased by approximately $5.8 million for every $1 million spent on the Blueprint initiative. This is a 5.8:1 return on investment, driven primarily by lower hospitalization rates and reduced outpatient facility use — precisely the kinds of avoidable utilization that Oliver Wyman identified as the largest source of indirect savings from Vermont's transformation.

Medicare specifically found statistically significant healthcare savings for its members in Vermont's Blueprint program. The Blueprint's own annual ROI analyses, comparing the Blueprint period to the pre-Blueprint baseline and to non-participating practices, consistently showed a positive all-payer return on investment. The finding that Blueprint patients had better outcomes across payer types — including Medicaid beneficiaries, who typically face the most barriers to effective primary care — is particularly significant for Vermont's equity goals.

Blueprint ROI: medical expenditure reduction per $1M invested $5.8M Lower hospitalization and outpatient facility use (Population Health Management study)	**Vermont residents with a personal health care provider 91% vs. 87% national benchmark — 4 points above (AHS Nov 2025 report)**
Blueprint PCMH patients with hypertension controlled 77% *Blueprint quality data 2023 — maintained above national average*	**Blueprint PCMH patients with diabetes HbA1c NOT in control 22%** *Ongoing opportunity — population health management target*

Figure 9.2 — Blueprint for Health clinical outcomes and ROI. Sources: Population Health Management journal; AHS November 2025 Transformation Report; Blueprint for Health 2024 data.

The Blueprint and AHEAD: A Critical Transition

Vermont's Blueprint program has operated since 2010 under a CMS Multi-Payer Advanced Primary Care Practice Demonstration that allowed Medicare to participate in PCMH and CHT payments. That agreement is scheduled to end with the conclusion of the All-Payer ACO Model. The AHEAD State Agreement resolves this transition risk: Primary Care AHEAD provides a voluntary program for primary care practices to receive enhanced prospective, risk-adjusted Medicare payments for coordinated care, with four payment pathway options including fully prospective models.

The transition from the current Medicare Blueprint payment to Primary Care AHEAD is operationally significant. Practices that currently receive Blueprint PMPM payments for Medicare patients will need to evaluate whether to participate in PC AHEAD, understand the new payment pathway options, and potentially redesign their care model to meet AHEAD's enhanced primary care requirements. AHS and DVHA have committed to supporting practices through this transition, recognizing that disrupting the Blueprint's Medicare payment stream would undermine one of Vermont's most effective clinical programs at precisely the moment transformation demands more of it.

Vermont's Blueprint enrollment target under AHEAD is ambitious: the program aims to substantially increase the percentage of Vermont's population — and particularly its Medicare population — attributed to Blueprint-participating practices. This expansion requires both new practice recruitment and support for existing practices to deepen their care management capabilities.

Vermont in Practice: The Blueprint Quality Measure Crosswalk

Vermont's primary care quality landscape is unusually complex because practices must simultaneously meet the quality requirements of multiple programs: Blueprint PCMH standards, AHEAD quality metrics, HEDIS measures used by commercial payers, and GMCB's hospital quality framework. Oliver Wyman documented the fragmentation this creates — multiple measurement systems with overlapping but not identical measures, creating administrative burden for practices trying to optimize across all of them.

AHS and Blueprint have developed a Quality Measure Crosswalk — an interactive tool that maps measures across programs, identifies overlaps, and helps practices understand which quality investments serve multiple accountability requirements simultaneously. This is operationally important: a primary care practice that installs a hypertension management protocol does not need to report separately to Blueprint, AHEAD, and commercial payers if those programs are aligned around the same clinical measure.

The crosswalk also reveals the gaps — places where programs require incompatible measures, creating impossible choices for practices with limited data infrastructure. These gaps are a specific target for the AHEAD implementation process and the Statewide Strategic Plan's quality alignment provisions.

The Behavioral Health Crisis — Vermont's Most Urgent Clinical Challenge

Vermont's behavioral health data, documented in the November 2025 AHS transformation report, describes a system in crisis. With 245.5 ED visits per 10,000 for suicide ideation and self-harm — statistically higher in Rutland and Windham counties, and among Vermonters aged 15-44 — and with 30-day follow-up after mental health ED visits reaching only 76%, Vermont's behavioral health system is failing patients at the point of highest acuity. The crisis presentation data is not just a quality indicator; it is evidence of system-level failure to provide adequate access to behavioral health services before patients reach the emergency department.

Oliver Wyman's community meetings documented the texture of this failure in qualitative terms: patients with limited access to mental health services; care that is discriminatory for people who don't speak English as a first language; an observed weight bias and fatphobia making it difficult for patients to get appropriate care. The quantitative and qualitative data together describe a behavioral health system that is overwhelmed, under-resourced, and inequitably distributed — concentrated near urban centers and inaccessible in the rural communities where need is often greatest.

The Three-Layer Behavioral Health Architecture Vermont Is Building

Vermont is developing a behavioral health system with three complementary layers that together can serve the full spectrum of mental health and substance use disorder needs — from universal screening in primary care to intensive community treatment to acute crisis response.

Layer 1	**Mental Health Integration in Primary Care (CoCM and MHI) The Collaborative Care Model (CoCM) is the only integrated behavioral health model with designated Medicare billing codes and a 90+ randomized controlled trial evidence base. CoCM embeds a behavioral health care manager (BHCM) and consulting psychiatrist within a primary care practice, enabling screening, brief intervention, and treatment for low-to-moderate acuity mental health and SUD conditions without referral. Vermont's Mental Health Integration pilot (MHI) through the Blueprint embedded mental health clinicians and community health workers in primary care settings starting in 2023, with nearly 80% of administrative entities reporting increased CHT staffing. The 2025 evaluation found improved access and engagement but sustainability concerns when temporary funding ends — a direct argument for making MHI permanent through AHEAD Primary Care funding.**
Layer 2	**Certified Community-Based integrated Health Centers (CCBHCs) Vermont became an official CCBHC Demonstration State in 2024, receiving enhanced Medicaid funding to expand community behavioral health services. As of 2025, Vermont has two active CCBHC demonstration sites. Vermont's RHT Program application plans to certify five additional CCBHC entities by July 2026 — Health Care and Rehabilitation Services (Springfield), Howard Center (Burlington), Northwest Counseling and Support Services (St. Albans), Northeast Kingdom Human Services (Newport), and Washington County Mental Health Services (Barre). CCBHCs provide comprehensive behavioral health services including crisis intervention, substance use treatment, and care coordination — welcoming all patients regardless of age, diagnosis, or insurance status.**
Layer 3	**Crisis System and Acute Behavioral Health Infrastructure Vermont has developed mobile crisis response (operational since January 2024), centralized dispatch from the 988 Suicide and Crisis Lifeline, and new psychiatric residential treatment facilities for youth and forensic populations. Vermont's six 'Designated Hospitals' have inpatient psychiatric units; the Brattleboro Retreat serves as the state's primary psychiatric hospital. AHS has added hospital-level youth mental health beds, psychiatric residential treatment beds, and enhanced transport to the Brattleboro Retreat as part of its Act 167-mandated stabilization actions. The 30-day follow-up rate of 76% after mental health ED visits — while above national averages for some measures — indicates that crisis presentations are not consistently connected to ongoing care.**

The Collaborative Care Model: Clinical Standard of Practice for Primary Care Integration

The Collaborative Care Model merits extended treatment because it is the clinical standard of practice for behavioral health integration in primary care — not just a promising approach, but the only integrated care model with a clear evidence base across 90+ randomized controlled trials, and the only model with dedicated Medicare billing codes (CPT codes 99492, 99493, 99494). Understanding CoCM's structure is important for any primary care or health system leader contemplating behavioral health integration.

CoCM component	Role	What they do	Vermont application
Primary Care Provider (PCP)	Treating and billing provider	Identifies patients with behavioral health needs through validated screening (PHQ-9, GAD-7); refers to BHCM; provides warm handoffs; bills CoCM codes monthly	Blueprint PCMH practices are the natural CoCM deployment sites — they already have the population management infrastructure and quality reporting that CoCM requires
Behavioral Health Care Manager (BHCM)	Day-to-day care coordinator	Maintains patient registry; provides brief interventions; monitors treatment response with validated rating scales; escalates to psychiatric consultant as needed; coordinates with community resources	MHI pilot found 80% of administrative entities increased CHT staffing — CHT staff are the natural BHCM pool; sustainability challenge when pilot funding ends
Psychiatric Consultant	Clinical supervision	Regular case review with BHCM; consultation on treatment recommendations; direct care for highest-acuity cases requiring psychiatric input; not the primary provider for most CoCM patients	Vermont's psychiatric shortage makes the CoCM model especially valuable — one psychiatrist can support an entire primary care practice's behavioral health population through consultation rather than direct care

Figure 9.3 — Collaborative Care Model components and Vermont application. Sources: SAMHSA CoCM program; CMS Behavioral Health Integration Services (January 2026); Vermont MHI Pilot Evaluation (2025).

The financial sustainability of CoCM in Vermont's environment depends on Medicare and Medicaid coverage of the billing codes, which Act 68 and AHEAD together support. Vermont Medicaid's coverage of CoCM codes is a prerequisite for practices serving high-Medicaid rural populations to implement the model — and DVHA's coordination with the Blueprint and AHEAD implementation is the mechanism for ensuring that coverage is in place. The AHEAD EAST Fund's explicit investment priority in mental health and substance use disorder services is the vehicle for funding the BHCM positions that primary care practices need to implement CoCM at scale.

"*The MHI initiative allowed primary care teams to serve more patients and provide more consistent follow-up. Flexible and team-based care models helped patients engage in services, particularly those facing complex medical and social challenges.*"

— Vermont Blueprint Mental Health Integration Pilot Qualitative Outcomes Evaluation, 2025

The SUD Continuum: Vermont's Hub and Spoke Model

Vermont's substance use disorder treatment system is organized around a Hub and Spoke model that has been nationally recognized as a model for rural SUD treatment. Hubs provide intensive addiction treatment services; Spokes — office-based opioid treatment (OBOT) practices — provide medication-assisted treatment in primary care settings. The Blueprint's practice finder maps active Spoke locations across Vermont's primary care network. AHS actions consistent with Act 167 have expanded this system: rate increases for residential SUD treatment, creation of 'hublets' in treatment deserts, support for co-occurring mental health and SUD treatment at hubs, contingency management for stimulant use disorder, and completion of gaps analyses to identify communities without adequate access.

The 30-day follow-up rate after ED visits for alcohol use — 68% in the November 2025 data — indicates that the care coordination infrastructure between crisis and community-based SUD treatment is not fully functional. Vermont is making specific investments to close this gap through the CCBHC expansion and the MHI permanent embedding of behavioral health in primary care, but the transition from crisis to community care remains the most operationally challenging step in the SUD care continuum.

Primary Care Redesign — Vermont's Strategy for the Front Door

Oliver Wyman's prescription for Vermont's primary care system was both clear and counterintuitive: Vermont does not have a primary care physician shortage in the traditional sense. HRSA recognizes no Health Profession Shortage Areas in Vermont. The problem is that primary care is not operating at the productivity and care model levels needed to serve Vermont's population. If primary care providers were supported to see three patients per hour, operating in team-based care models where clinical staff work at the top of their licensure and administrative burden is minimized, Vermont would have sufficient primary care supply for well into the future.

This finding has profound implications for Vermont's primary care strategy. Rather than focusing exclusively on recruiting more physicians — an expensive, slow, and uncertain strategy — Vermont can meaningfully increase primary care capacity by transforming how existing primary care operates. The Blueprint's PCMH model is the vehicle for this transformation; AHEAD's enhanced primary care payments are the financial fuel.

The Vermont Comprehensive Primary Health Care Initiative

Act 68 established the Vermont Steering Committee for Comprehensive Primary Health Care — a body whose recommendations must be incorporated into the Health Care Delivery Advisory Committee's work and AHS's Statewide Strategic Plan. This committee represents a policy commitment to treating primary care not as one service line among many but as the foundational layer of Vermont's reformed delivery system — the front door through which most Vermonters access the healthcare system and the setting where most disease management, behavioral health integration, and SDOH screening should occur.

Vermont's comprehensive primary health care vision, as embedded in Act 68 and the AHEAD State Agreement, has five operational components:

Component	What it means in practice	Vermont investment vehicle
Team-based care	Every PCMH practice operates with a full care team — physician or APRN, medical assistant, care coordinator, behavioral health care manager, and CHT connection — with each team member working at the top of their licensure. The physician's time is reserved for the clinical decisions only a physician can make.	Blueprint PCMH payments; AHEAD enhanced primary care payments; RHT Program workforce investment in APRN training and scope expansion
Population health management	Every PCMH practice maintains a panel registry, tracks care gaps proactively, and reaches out to patients who need preventive care, chronic disease management, or behavioral health support before they present in crisis.	Blueprint data infrastructure; VHCURES linkage; GMCB analytics vendor procurement
SDOH screening and navigation	Universal screening for health-related social needs (housing, food security, transportation, financial insecurity) at every primary care visit, with warm handoffs to CHT staff for navigation and community resource connection.	Blueprint HRSN screening expansion; AHEAD EAST Fund SDOH investments; RHT Program community health worker deployment
Behavioral health integration	CoCM or equivalent behavioral health integration at every PCMH practice, providing screening, brief intervention, and care management for mental health and SUD conditions without requiring a separate specialty referral.	MHI pilot expansion through AHEAD and EAST Fund; CCBHC partnerships with primary care
Care coordination for complex patients	Intensive care management for Vermont's highest-cost, highest-complexity patients — the elderly with multiple chronic conditions, dual-eligible Medicare/Medicaid beneficiaries, individuals with serious mental illness — linking primary care to specialist care, post-acute care, and community support.	CHT staff; dual eligible care planning; AHEAD population health requirements; Blueprint Chronic Care Initiative

Figure 9.4 — Vermont comprehensive primary health care: five operational components and investment vehicles. Sources: Act 68 of 2025; AHEAD State Agreement (January 2025); AHS Transformation Plans 2025\.

Primary Care's Role in Reducing Hospital Utilization

The connection between primary care investment and hospital utilization reduction is one of the best-established relationships in health services research, and Vermont's own Blueprint data confirm it. The 2013 evaluation finding — lower hospitalization rates and reduced outpatient facility use for Blueprint participants — is the mechanism through which primary care investment produces hospital savings under a global budget.

Vermont's November 2025 data shows 32.3% of all ED visits are potentially avoidable — meaning they represent primary care access failures, not genuine emergency needs. At Vermont's scale (approximately 14 hospitals, roughly 200,000+ ED visits annually across the system), reducing potentially avoidable ED visits from 32.3% to a benchmark level of 25% would eliminate approximately 14,600 ED visits per year. At average ED visit costs in the $800-1,200 range, this reduction represents $12-18 million in annual savings — and that is before accounting for the downstream hospital admissions from avoidable ED visits that could have been prevented by earlier primary care intervention.

This is why Act 68's explicit goal of increased investment in primary care is not a soft preference — it is a financial necessity. Under global budgets, every avoidable hospitalization or ED visit is a direct cost without offsetting revenue. Hospitals operating under global budgets have a powerful financial incentive to invest in the primary care infrastructure that keeps their attributed population out of the hospital. Vermont's AHEAD EAST Fund, with its explicit investment priority in primary care, is designed to accelerate this investment.

Vermont's Clinical Quality Architecture — Measuring What Matters

Clinical quality measurement in Vermont operates through multiple overlapping frameworks that were not designed to be coherent with each other. HEDIS measures used by commercial payers, Blueprint quality measures for PCMH payments, GMCB hospital quality reporting, AHEAD accountability metrics, and Act 167/Act 68 outcome measures all exist simultaneously, creating a measurement burden that Oliver Wyman specifically cited as a driver of administrative complexity and provider frustration.

The Statewide Strategic Plan that AHS must deliver by December 2028 presents the opportunity — and creates the obligation — to align Vermont's clinical quality measurement framework around a coherent set of indicators that serve multiple accountability purposes simultaneously. The following framework represents the clinical quality architecture that Vermont's reform requires:

Domain	Priority measures	Why this measure	Current VT status
Preventive and chronic care	Hypertension control rate; diabetes HbA1c management; cervical cancer screening; colorectal cancer screening; childhood immunization	Chronic disease management is the highest-volume primary care activity; these measures predict long-term utilization and cost	Hypertension 77% controlled; diabetes 22% not controlled. Both reflect Blueprint PCMH performance; non-Blueprint practices likely worse
Behavioral health access and quality	30-day follow-up after MH ED visit; 30-day follow-up after SUD ED visit; depression screening rate; SUD treatment initiation and engagement	ED visits for MH/SUD are the clearest evidence of access failure; follow-up rates measure whether crisis intervention connects to ongoing care	76% 30-day follow-up after MH ED visit; 68% after SUD ED visit — both indicate coordination gaps
Primary care access	% practices accepting new Medicaid patients; wait time for new patient appointment; % population with attributed primary care provider	Access to primary care is the front door to the system; Medicaid acceptance is the equity indicator	59% of primary care practices accepting new Medicaid patients — significant equity gap; 91% of Vermonters have a personal health provider
Hospital quality and efficiency	30-day all-cause readmission rate; potentially avoidable ED visit rate; operating profit per adjusted discharge; admin cost per adjusted discharge	Hospital metrics measure both clinical quality and financial sustainability	Readmission: 14.8% (within range). Avoidable ED: 32.3% (major opportunity). Admin cost: 91% above benchmark
Equity and SDOH	Health outcome variation by county, income, race/ethnicity; insurance coverage rate by geography; SDOH screening completion rate; community resource referral completion	Equity measures are required by Act 68 and Act 167; geographic variation within Vermont is large and not adequately measured	Northeast Kingdom uninsured 6-8% vs. 3% statewide; rural mental health access gaps; SDOH screening completeness not consistently tracked

Figure 9.5 — Vermont clinical quality measurement framework: priority measures, rationale, and current status. Sources: AHS Transformation Report (November 2025); Blueprint for Health data; Oliver Wyman Act 167 Report.

The HEDIS Improvement Methodology — Vermont Context

The HEDIS improvement methodology — five-step framework from performance baseline through root cause analysis to sustainable systems design — applies directly to Vermont primary care practices and hospitals participating in Blueprint and AHEAD. The Vermont-specific context adds three considerations:

- All-payer measurement is both an advantage and a challenge. Vermont's VHCURES all-payer claims database, linked to the Blueprint clinical data registry, enables HEDIS measurement across the full population — not just the commercial population a health plan would report on. This means Vermont can identify disparities that would be invisible in single-payer reporting. The challenge is that the database requires VITL connectivity that remains voluntary and incomplete, particularly among community-based providers.
- Blueprint field staff are the quality improvement infrastructure. Unlike states where practices must manage their own quality improvement in isolation, Vermont's Blueprint field staff — one Quality Improvement Facilitator per HSA region — provide ongoing support, data analysis, and evidence-based practice coaching. This infrastructure is a competitive advantage for Vermont's transformation; it means quality improvement capacity is not limited by the resources of individual practices.
- Performance payment alignment creates a direct financial incentive for quality improvement. Blueprint's PMPM payments include a performance payment component tied to quality outcomes and utilization measures. AHEAD's enhanced primary care payments include quality-based adjustments. This means Vermont primary care practices have a direct financial incentive for HEDIS improvement — not just a regulatory compliance requirement.

Care for Vermont's Aging Population — The Clinical Imperative

Vermont's demographic trajectory — 65+ population growing 57% by 2040, exceeding 30% of total population — creates a clinical imperative that is simultaneously a financial imperative. The aging population drives demand for the most expensive and most complex services: cancer treatment, cardiac surgery, dementia care, frailty-related hospitalization, long-term care, end-of-life care. Designing the clinical system to serve this population well — which means keeping elderly Vermonters healthy, connected to their communities, and out of hospitals and nursing homes to the extent clinically appropriate — is both the right thing to do and the financial requirement of the global budget model.

The PACE Model and Vermont's Long-Term Care Gap

The Program of All-Inclusive Care for the Elderly (PACE) integrates medical, social, and long-term care services for frail elderly patients who would otherwise require nursing home placement, enabling them to remain in their communities. PACE is one of the most effective and cost-efficient models for managing the population segment that drives the highest healthcare costs in any state system. Vermont has no PACE programs. This is a significant gap that Vermont's transformation planning must address.

Oliver Wyman's community meetings documented the dimensions of Vermont's long-term care capacity crisis: lack of staffed social care facilities leads to hospital crowding when patients cannot be discharged; nursing home occupancy is high and new capacity is constrained; home and community-based services (HCBS) — the preferred alternative to institutional care — are limited by workforce shortages. Vermont's RHT Program application specifically addresses long-term care gaps: funding for dialysis units in nursing homes so dually eligible residents can receive long-term care without institutional transfer, and a second nursing home ventilator unit to double capacity for high-acuity nursing home care.

The clinical model for Vermont's aging population requires three integrated components: strong primary care for chronic disease management and prevention; robust HCBS and PACE to support aging in place; and smooth hospital-to-community transitions when hospitalization is unavoidable. The first component is the Blueprint's domain. The second requires the SDOH investment in housing and transportation that Oliver Wyman made Imperative 1\. The third requires the care coordination infrastructure that the CHT model provides.

Dementia Care: The Coming Clinical Challenge

Dementia affects an estimated 14 million Americans nationally by 2060, and Vermont's older demographic profile means its per-capita rate of dementia will reach the national projection earlier. Vermont currently lacks dementia-specialized primary care at scale, adequate memory care capacity in its nursing home sector, and family caregiver support infrastructure proportionate to the approaching need. Oliver Wyman's COE framework designates memory care as a COE specialty — recognizing that dementia care at adequate quality requires specialized expertise and cannot be replicated at every community hospital.

The clinical pillar implications are clear: Vermont's transformation plan must include explicit investment in dementia care infrastructure — the training of primary care providers in cognitive assessment and early dementia management, the development of memory care COE capacity at designated facilities, the expansion of adult day programs and respite care for family caregivers, and the integration of dementia care into the community-based care model that Vermont's global budget environment requires.

Clinical Pillar Implications for AHS's Statewide Strategic Plan

The Statewide Health Care Delivery Strategic Plan that AHS must deliver by December 2028 must address the clinical pillar explicitly. Based on Vermont's current clinical landscape and the transformation agenda established by Acts 167 and 68, the plan should contain five clinical design commitments:

Commitment	What it requires	Timeline	Primary vehicle
Universal Blueprint PCMH coverage	Every primary care practice in Vermont participates in Blueprint PCMH recognition and receives all-payer PMPM payments; no Vermont resident is more than 30 minutes from a PCMH practice	2025-2028	Blueprint expansion; AHEAD PC primary care recruitment; RHT workforce investment
Behavioral health integration at every PCMH	Every PCMH practice implements CoCM or equivalent behavioral health integration, with BHCM staff and psychiatric consultation available to all attributed patients	2025-2028	MHI permanent funding through AHEAD EAST Fund; CCBHC partnerships; BHCM workforce training through RHT Program
CCBHC network covering all HSAs	Certified Community-Based integrated Health Centers established in every Hospital Service Area, providing comprehensive community behavioral health services including crisis response, SUD treatment, and care coordination	2026 (5 new CCBHCs) through 2028	CCBHC Demonstration State enhanced Medicaid; RHT Program funding; Act 68 grants
PACE program development	At least one PACE program operational in Vermont, serving the highest-acuity elderly population that would otherwise require nursing home placement; feasibility assessment for additional programs	2026-2028 (feasibility and launch)	AHEAD EAST Fund; RHT long-term care infrastructure investment; Medicaid PACE program rules
Dementia care infrastructure	Memory care COE designation at 3-5 facilities; primary care dementia assessment protocols embedded in all PCMH practices; family caregiver support programs in all HSAs	2026-2030	RHT Program; COE designation process; Blueprint quality measures expansion

Figure 9.6 — Clinical pillar commitments for Vermont's Statewide Health Care Delivery Strategic Plan. Sources: Act 68 of 2025; Oliver Wyman Act 167 Report; AHS Transformation Plans; Vermont RHT Program Application.

Key Concepts Introduced or Expanded in This Chapter

Term	Definition
Blueprint for Health	Vermont's all-payer primary care transformation program, operating since 2006\. Provides per-member-per-month payments to NCQA-recognized Patient-Centered Medical Home practices and funds Community Health Teams to provide wraparound care coordination services. The foundational layer of Vermont's clinical pillar.
Patient-Centered Medical Home (PCMH)	A primary care practice recognized by NCQA as meeting advanced standards for care coordination, population management, quality improvement, and patient engagement. The organizational model for Vermont's primary care system under Blueprint and AHEAD.
Community Health Team (CHT)	Multi-disciplinary team of social workers, nurses, community health workers, and behavioral health professionals supporting PCMH practices with care coordination, SDOH navigation, and connection to community services.
Collaborative Care Model (CoCM)	The integrated behavioral health care model with the strongest evidence base — 90+ RCTs — embedding a behavioral health care manager and psychiatric consultant in primary care to address mental health and SUD needs without requiring specialty referral. The only integrated care model with designated Medicare billing codes.
Certified Community-Based integrated Health Center (CCBHC)	Vermont's term for what CMS calls Certified Community Behavioral Health Clinics. Comprehensive community-based behavioral health facilities receiving enhanced Medicaid funding through the federal CCBHC Demonstration program.
Mental Health Integration (MHI)	Vermont Blueprint's pilot program embedding mental health clinicians and community health workers in primary care settings; evaluated in 2025 as highly effective but facing sustainability challenges when pilot funding ends.
PACE (Program of All-Inclusive Care for the Elderly)	An integrated medical, social, and long-term care model enabling frail elderly individuals to remain in their communities rather than entering nursing homes. Vermont currently has no PACE programs — a significant gap given its aging demographics.
Hub and Spoke	Vermont's SUD treatment model: Hubs provide intensive addiction treatment; Spokes are office-based opioid treatment practices in primary care settings providing medication-assisted treatment.
HRSN (Health-Related Social Needs)	The social needs that affect health outcomes — housing, food security, transportation, financial insecurity, social isolation — now universally screened for in Vermont's Blueprint primary care practices.

Sources: Vermont Blueprint for Health Annual Report 2024; Vermont DMH CCBHC Program website (mentalhealth.vermont.gov); Vermont Blueprint Mental Health Integration Pilot Qualitative Outcomes Evaluation (2025); Oliver Wyman Act 167 Community Engagement: Recommendations (2024); Vermont Agency of Human Services Health Care System Transformation Report (November 2025); Act 68 of 2025; Vermont AHEAD State Agreement (January 2025); Vermont Rural Health Transformation Program Application (November 2025); Population Health Management (Blueprint ROI study, 2016); SAMHSA CoCM Program; CMS Behavioral Health Integration Services (January 2026).

Chapter 10: The Equity Pillar — Closing Gaps, Not Just Averaging Them

The Equity Pillar — Closing Gaps, Not Just Averaging Them

Rural Health Disparities, Racial and Demographic Equity, Social Determinants of Health, the GLP-1 Access Crisis, and Vermont's Equity Architecture

Sources: Vermont Department of Health Health Equity Data Report (January 2025); Oliver Wyman Act 167 Recommendations (2024); Vermont RHT Program Application (November 2025); KFF Medicaid GLP-1 Coverage Analysis (January 2026); CMS BALANCE Model announcement (December 2025); Health Affairs rural health equity research (June 2024); AHS Transformation Reports (2025); Act 68 of 2025\.

The Equity Pillar: Why Average Outcomes Are the Wrong Target

The most common failure mode in healthcare quality measurement is optimizing for the average. A system that improves its average readmission rate, average primary care access, or average chronic disease control while allowing — or widening — the gap between its best-served and worst-served populations has not achieved quality improvement. It has achieved statistical improvement while perpetuating or deepening the inequity that produces the worst outcomes.

The Equity pillar of the six-pillar framework is founded on a different target: every population segment should have access to the conditions that support health, and disparities in access and outcomes that are attributable to race, income, geography, disability, language, sexual orientation, or gender identity should be eliminated, not averaged away. This is not a soft aspiration. It is an analytically precise goal with measurable indicators, specific intervention categories, and accountability structures that Vermont's Act 167 and Act 68 have embedded in law.

Vermont's equity challenge is distinctive enough from the typical urban healthcare equity narrative that it requires explicit treatment before the analytical framework can be applied effectively. Vermont is approximately 94% white — one of the least racially diverse states in the country. Its 4.5% foreign-born population is concentrated in Burlington and a handful of other communities. The equity disparities that drive headlines in Boston, Chicago, or Los Angeles — stark racial gaps in maternal mortality, cardiovascular disease mortality, diabetes management, and access to specialty care — are present in Vermont but at smaller scale and with different geographic distribution. Vermont's dominant equity challenge is geographic: the health and healthcare access gap between Chittenden County (Burlington, the state's economic and medical center) and the rural counties — particularly the Northeast Kingdom — where poverty is higher, providers are scarcer, transportation is unreliable, and broadband is limited.

This does not mean racial equity is unimportant in Vermont. The Vermont Department of Health's 2024 health equity data shows clear disparities: Black adults in Vermont have an 82% health insurance coverage rate compared to 94% statewide. Adults who are Asian, Native Hawaiian or Pacific Islander, Black, or another race are significantly less likely to have a primary care provider than white adults. LGBTQ+ Vermonters, Vermonters with disabilities, and lower-income Vermonters delay care at higher rates due to cost. And Vermont's BIPOC communities face the same structural racism in healthcare settings that affects every state — weight bias, cultural incompetence, language barriers, and documented differential treatment quality.

Vermont's equity agenda must address both the geographic and the demographic dimensions simultaneously — and the analytical framework for doing so is the same in both cases: identify the disparity, trace it to its root cause, and design interventions that address the cause rather than merely measuring the symptom.

Vermont's Equity Landscape — What the Data Shows

Geographic Equity: The Rural-Urban Divide

Vermont's most pervasive and consequential health equity disparity is geographic. The Northeast Kingdom — Caledonia, Essex, and Orleans counties — concentrates the state's most severe access, affordability, and outcome disparities:

Uninsured rate, Essex County (NE Kingdom) 8% vs. 3% statewide — nearly 3x the state average despite Vermont's near-universal coverage	**Counties with above-average suicide/self-harm ED rates Rutland \+ Windham Statistically higher than state average per 10,000 ED visits (AHS Nov 2025 data)**
Counties with above-average opioid overdose ED rates Chittenden \+ Bennington \+ Windham *Geographic concentration of SUD burden that does not align with service concentration*	**Rural counties with highest poverty rates Essex \+ Windham \+ Orleans** *Higher disability rates, lower broadband access, limited transportation in same counties*

Figure 10.1 — Vermont geographic equity disparities: key indicators. Sources: AHS November 2025 Transformation Report; Vermont RHT Program Application; Vermont Department of Health.

The Northeast Kingdom's geographic isolation creates a compound equity burden that is different in character from urban poverty. A low-income resident of Burlington faces affordability barriers but has access to UVMMC, an FQHC network, mental health services, and public transportation. A low-income resident of Essex County faces the same affordability barriers compounded by 45-minute drives to the nearest primary care provider, inadequate broadband that limits telehealth access, unreliable public transportation that makes scheduled appointments difficult to maintain, and a housing market where the few available rental units are concentrated in towns without healthcare services.

Oliver Wyman's community engagement found that this compound geographic burden produces a distinct pattern of care avoidance: community members described not going to care because the premiums and out-of-pocket costs are too high; long ambulance waits; long and difficult travel to care sites; and long waits in the ED when they do access care. These are not isolated experiences — they are the structural conditions of rural healthcare access in Vermont's most underserved areas.

Racial and Demographic Equity

Vermont's racial equity disparities, while smaller in absolute number than in more diverse states, are documented and consequential. The 2024 Vermont Department of Health equity report shows that statewide, 94% of Vermont adults have health insurance — but only 82% of Black adults. Primary care access follows a similar pattern: while 90% of Vermont adults overall have a personal healthcare provider, the rate for Asian or Pacific Islander adults (81%), Black adults (80%), and adults of another race (79%) is significantly lower. These gaps are not attributable to lack of coverage; they reflect structural barriers to access that persist even when insurance is present.

BIPOC Vermonters, LGBTQ+ Vermonters, Vermonters with disabilities, and lower-income Vermonters were all more likely to delay care due to cost in a 2024 state survey. The same populations were more likely to report their community was not safe for walking — an indicator of the broader social and built environment conditions that shape health. Eight percent of all Vermonters said they did not get medical care because of cost in the prior year; that rate is higher among people with less education, lower income, BIPOC adults, LGBTQ+ adults, and people with a disability.

Vermont's Indigenous population — primarily Abenaki communities — faces specific equity challenges that the state's data systems are not yet adequately equipped to measure. A June 2024 state health equity report documented disparities for the American Indian or Alaska Native population based on 2021-2022 survey data, but Vermont's small and geographically dispersed Indigenous population creates statistical challenges for survey-based measurement. Vermont Public reporting in 2025 found that several Indigenous community organizations had not been in contact with the Health Department, suggesting that outreach and engagement gaps compound the measurement gaps.

The SDOH Burden

Vermont's equity data from the RHT Program application quantifies the SDOH burden that Oliver Wyman identified as both a driver of healthcare utilization and a barrier to healthcare access:

SDOH domain	Vermont statistic	Equity implication
Housing	Rental vacancy rate 3% (healthy market \= 5%); half of Vermont renters cost-burdened; per capita homelessness rate second highest nationally	Housing instability is both a health risk factor and a barrier to care — unhoused individuals cannot reliably access scheduled appointments; housing-unstable families face stress that drives both physical and mental health deterioration
Broadband access	80% of rural Vermonters have broadband; 14% of all Vermonters lack access	Broadband gap concentrates in the Northeast Kingdom — the same counties with highest uninsurance and most limited primary care access. Telehealth cannot reach the communities that need it most without broadband.
Food insecurity	9% of Vermonters are food insecure	Food insecurity is a risk factor for diabetes, cardiovascular disease, and mental health conditions — the chronic diseases that drive Vermont's highest healthcare costs. Higher rates in rural counties compound the geographic equity burden.
Transportation	Public transportation limited or unreliable in most rural counties, particularly Northeast Kingdom and Rutland County	Transportation barrier to care is the single most frequently cited access problem in Vermont's community meetings. Without transportation, insurance coverage and provider availability are irrelevant — patients who cannot get to care cannot receive care.
Income	Rural counties (Essex, Windham, Orleans) report higher poverty rates; Northeast Kingdom has highest unemployment	Low income interacts with every other SDOH factor: cost-burdened renters have less money for food, transportation, and healthcare; lower-income workers have less job flexibility to take time off for appointments; financial stress directly affects mental and physical health.

Figure 10.2 — Vermont SDOH burden: key indicators and equity implications. Sources: Vermont RHT Program Application (November 2025); Oliver Wyman Act 167 Report; Vermont Department of Health.

The Disparity Root Cause Taxonomy — Why Vermont's Equity Gaps Exist

Identifying a health disparity is the beginning of an equity intervention, not the end. The same disparity in access or outcome can arise from fundamentally different causes — and an intervention that addresses the wrong cause will not close the gap. Vermont's equity strategy must be organized around the root causes of specific disparities, not around the disparities themselves.

Drawing from the Health Equity pillar framework, Vermont's equity gaps fall into five root cause categories. For each category, Vermont has a documented disparity, a traceable mechanism, and a set of interventions — some already underway, some required.

1 Geographic Access Barriers	**Mechanisms: Physical distance from providers; transportation infrastructure inadequacy; provider shortage in rural areas; broadband gaps limiting telehealth; limited hours of operation at available facilities. Vermont interventions: COE regionalization with reliable EMS transport; telehealth expansion through RHT Program; broadband infrastructure investment (Oliver Wyman Imperative 1); Blueprint PCMH expansion to all HSAs; community paramedicine.**
2 Social Needs Barriers	**Mechanisms: Housing instability that prevents sustained engagement with healthcare; food insecurity that drives chronic disease risk; transportation barriers to scheduled care; economic stress that forces delayed care; broadband gaps that prevent telehealth access. Vermont interventions: AHEAD EAST Fund SDOH investment; Blueprint HRSN universal screening with warm handoffs to community resources; AHS HSA Coordinator model linking health and social services; RHT Program investment in community health workers; housing and transportation advocacy as healthcare policy.**

3 Clinical Practice Variation	**Mechanisms: Documented differential treatment quality by race, ethnicity, disability, and body weight. Oliver Wyman community meetings specifically documented weight bias and fatphobia affecting care access; racial disparities in pain management and referral patterns documented nationally and present in Vermont. Vermont interventions: Provider diversity recruitment and retention; implicit bias training as a condition of Blueprint and AHEAD participation; standardized clinical protocols that reduce provider discretion in ways that produce differential outcomes; patient advisory councils including underrepresented communities; cultural competency standards.**
4 Insurance and Coverage Barriers	**Mechanisms: Despite Vermont's 97% coverage rate, 8% of Essex County residents are uninsured; 18% of 25-34 year olds are uninsured; income groups between 251-400% of federal poverty line have the highest uninsurance rates despite being ineligible for Medicaid. Even with coverage, high deductibles and out-of-pocket maximums cause care avoidance. Vermont interventions: ACA enrollment outreach in rural communities; FQHC eligibility screening; Act 68 affordability benchmarks; RBP impact on premium reduction that makes insurance more affordable; AHEAD EAST Fund support for FQHCs serving uninsured and underinsured patients.**
5 Trust and Engagement Barriers	**Mechanisms: Historical and ongoing experiences of discrimination in healthcare settings — documented for LGBTQ+ Vermonters, BIPOC Vermonters, and Vermonters with disabilities. Community meeting feedback specifically cited lack of culturally competent care, stigma, and discrimination as barriers to engagement. Vermont interventions: Community health worker deployment from communities served; HEART program for rural LGBTQ+ Vermonters; BIPOC community partnership grants through VDH Health Equity Capacity Building Program; multilingual patient education materials; patient-centered care training with equity focus; CCBHCs as culturally accessible entry points.**

The five root cause categories are not mutually exclusive — a Black rural resident of Windham County may face geographic access barriers, SDOH barriers, and trust barriers simultaneously. Vermont's equity interventions are most effective when they are designed to address multiple root causes for the populations with compound equity burdens, rather than addressing each category in isolation.

Rural Equity — Vermont's Defining Challenge

Vermont's rural equity challenge is analytically important for a national audience because it represents a type of health disparity that is often underweighted in national health equity frameworks dominated by urban racial equity narratives. Rural health disparities are not less serious than urban racial disparities — they are different in character, different in mechanism, and require different interventions.

The Health Affairs research on rural health equity in northern New England — directly relevant to Vermont — identifies five principles for addressing rural health equity that align closely with Vermont's transformation agenda. Rural communities need investments that address the social and economic determinants of health, not just the clinical delivery system. Rural health equity requires provider diversity that reflects community demographics and culture. Rural telehealth must be accompanied by the broadband and device infrastructure that makes it genuinely accessible. Rural community organizations must be genuine partners in healthcare design, not passive recipients of state-designed programs. And rural health equity data must be disaggregated at the county and sub-county level — state averages mask the severity of rural disparities.

The Northeast Kingdom: Vermont's Equity Priority

The Northeast Kingdom — Caledonia, Essex, and Orleans counties — is Vermont's clearest and most severe equity priority. It is the most rural part of the state, the most economically distressed, the most underserved by healthcare providers, and the most distant from UVMMC and the Burlington healthcare hub. The three counties that make up the Northeast Kingdom have some of the lowest population densities in the eastern United States, unemployment rates above the state average, high rates of disability, limited broadband coverage, and the highest uninsurance rates in the state.

The healthcare facilities serving the Northeast Kingdom — North Country Hospital in Newport, Northeastern Vermont Regional Hospital in St. Johnsbury, and Copley Hospital in Morrisville for the Lamoille/Orleans border area — are among Vermont's most financially distressed. North Country and Northeastern Vermont Regional are candidates for Tier 3 designation under the Oliver Wyman regionalization blueprint — meaning they may not be able to sustain full inpatient operations in the long term. Any transition away from inpatient services at these facilities, without robust REH conversion and EMS/telehealth investment, would represent a catastrophic equity regression for the communities they serve.

Vermont's AHS must be explicit in the Statewide Strategic Plan that the Northeast Kingdom is an equity priority that constrains the pace and structure of regionalization. The financial logic of consolidating services at RSC hospitals in Burlington, Rutland, or Bennington cannot override the equity obligation to maintain emergency access in the most rural, most isolated communities in the state. The tension between system financial sustainability and rural access equity is genuine, and pretending it is not does not resolve it.

Vermont in Practice: The Northeast Kingdom Equity Constraint

Any Vermont hospital transformation plan that closes emergency services in the Northeast Kingdom without replacing them with services that can actually be accessed by Northeast Kingdom residents — not hypothetically accessible via 90-minute transport to Burlington, but genuinely accessible in the community — fails the equity test that Act 167 and Act 68 have embedded in Vermont law.

The equity constraint does not mean the Northeast Kingdom's hospitals cannot change. It means the sequence matters: investments in EMS capacity, telehealth infrastructure, and community paramedicine must precede or accompany any reduction in inpatient services. The Rural Emergency Hospital designation provides a viable path precisely because it maintains 24/7 emergency access while eliminating the unsustainable inpatient infrastructure. The RHT Program's priority investments in the Northeast Kingdom — broadband, EMS, RPM, telehealth — are equity investments as much as access investments.

The equity test for any transformation proposal affecting Northeast Kingdom hospitals should be specific: after this change, can a 70-year-old resident of Canaan, Vermont who has a heart attack at 2 a.m. receive timely emergency care? Can a person with serious mental illness in Newport access a CCBHC without a two-hour drive? Can a child in Lyndonville see a primary care provider within 14 days? These are not abstract equity metrics. They are the minimum conditions for health equity in rural Vermont.

The GLP-1 Access Crisis — The Most Important New Equity Issue

The emergence of GLP-1 receptor agonists — semaglutide (Wegovy, Ozempic) and tirzepatide (Zepbound, Mounjaro) — as transformatively effective treatments for obesity, type 2 diabetes, and cardiovascular disease risk has created what may be the most consequential new health equity issue in American healthcare since the introduction of statins. The drugs work. The evidence is unambiguous: they produce average weight loss of 15-20% of body weight, dramatically reduce cardiovascular events in high-risk populations, and improve glycemic control in type 2 diabetes. For patients at the intersection of obesity, diabetes, and cardiovascular disease — a population concentrated in lower-income and rural communities — GLP-1s could be transformative at the population level.

The equity problem is pricing and coverage. At list prices exceeding $1,000 per month before rebates, GLP-1s for obesity are financially inaccessible to the vast majority of patients who would benefit from them. As of January 2026, only 13 state Medicaid programs covered GLP-1s for obesity treatment — down from 16 in late 2025, as four states eliminated coverage in response to budget pressure from Medicaid cuts. Vermont Medicaid covers GLP-1s for medically accepted FDA-approved indications but expressly excludes coverage for weight loss under its State Plan. Vermont Medicaid covers GLP-1s for type 2 diabetes management but not for obesity as a primary indication.

The equity consequence is straightforward: GLP-1s are currently accessible primarily to patients with commercial insurance generous enough to cover them, and to patients with type 2 diabetes or cardiovascular disease (who qualify under Medicare and Medicaid for the diabetes and cardiovascular indications). Patients with obesity but not yet diagnosed with type 2 diabetes — many of them lower-income Vermont Medicaid patients in the communities with highest obesity prevalence — cannot access these drugs under current coverage unless they pay full price out of pocket. At $1,000+ per month, full price is not an option for Medicaid patients.

"*Limitations on coverage for obesity drugs in Medicare and Medicaid mean that millions of people who have obesity and might benefit from taking GLP-1s may be unable to access them unless they are able to pay the full cash price out of their own pockets, which would likely be prohibitive for people with Medicaid who must have low incomes to qualify for the program.*"

— KFF, What to Know About the BALANCE Model for GLP-1s in Medicare and Medicaid, 2026

The BALANCE Model: A Partial Solution

In December 2025, CMS announced the BALANCE (Better Approaches to Lifestyle and Nutrition for Comprehensive hEalth) Model — a voluntary program enabling state Medicaid agencies and Medicare Part D plans to cover GLP-1 medications for obesity by negotiating prices with manufacturers. The model launches in Medicaid as early as May 2026 and in Medicare Part D in January 2027, with a Medicare GLP-1 Bridge program providing $50/month co-pay access for Medicare beneficiaries beginning July 2026\.

BALANCE is a meaningful step toward resolving the GLP-1 equity crisis, but it has three significant limitations that Vermont and every state must navigate. First, manufacturer participation is voluntary — if drug manufacturers do not negotiate with CMS under BALANCE terms, the program produces no coverage expansion. Second, state Medicaid agency participation is voluntary — the four states that just eliminated GLP-1 obesity coverage are unlikely to opt into BALANCE given the budget pressures that drove the elimination. Third, the negotiated prices, while reduced from list prices, may still be expensive for state Medicaid budgets already under pressure from the 2025 reconciliation legislation's Medicaid cuts.

Vermont's DVHA must evaluate BALANCE participation as an equity policy decision, not purely a budget decision. The populations who would benefit most from GLP-1 obesity coverage under Vermont Medicaid — lower-income rural Vermonters with obesity, diabetes risk, and cardiovascular disease risk — are precisely the populations whose healthcare costs are most likely to be reduced by effective obesity treatment. The long-term fiscal argument for coverage is strong; the short-term budget pressure against it is real.

The GLP-1 Equity Implication for Vermont's Transformation

Vermont's transformation narrative must grapple with the GLP-1 equity question explicitly, for two reasons. First, obesity and its metabolic consequences — type 2 diabetes, cardiovascular disease, hypertension, sleep apnea — are major drivers of the high-cost utilization that Vermont's global budget system will be trying to reduce. GLP-1s have demonstrated efficacy in reducing all of these conditions. If GLP-1s become broadly accessible to Vermont's Medicaid population through BALANCE participation, they could accelerate the utilization reduction that global budgets are designed to incentivize — producing population health improvement and healthcare cost reduction simultaneously.

Second, the GLP-1 access gap is the most vivid current example of what the book's Part V identified as the equity reckoning in pharmaceuticals: potentially transformative drugs whose benefits disproportionately accrue to higher-income patients who can afford them, while lower-income patients — who have higher obesity rates, higher diabetes rates, and higher cardiovascular disease rates, partly because of the same SDOH conditions that drive Vermont's equity gaps — cannot access the treatment. Vermont's equity commitment requires active engagement with this issue, not passive monitoring.

Vermont's Equity Architecture — What Is Already Built and What Remains

Vermont has more equity infrastructure than most states of comparable size — a product of its near-universal insurance coverage, the Blueprint for Health's SDOH screening mandate, the Health Equity Advisory Commission, the Health Equity Capacity Building Program, and the explicit equity requirements in Act 167 and Act 68\. But Vermont also has documented, persistent equity gaps that this infrastructure has not yet closed. The Statewide Strategic Plan's equity section must be honest about both.

What Is Already Operational

- **Universal SDOH screening: Blueprint PCMH practices conduct universal screening for health-related social needs (HRSN) at every patient encounter, using validated tools and generating warm handoffs to Community Health Teams for navigation and resource connection. This is the most systematic SDOH screening infrastructure of any state primary care program.**
- **Health Equity Capacity Building Program: Vermont Department of Health grants to community-based organizations working on health disparities, funded through CDC. Organizations working on equity with LGBTQ+ populations, justice-involved individuals, rural communities, and Indigenous communities have received grants through this program.**
- **Health Equity Advisory Commission: A standing body advising state government on health equity policy. Act 68 requires the Health Care Delivery Advisory Committee to consult with the Commission, embedding equity oversight in the transformation governance structure.**
- **CCBHC access policy: Vermont's CCBHCs are explicitly required to serve all patients regardless of age, diagnosis, insurance status, or geographic location — the most equity-protective access policy for behavioral health services.**
- **Act 167 and Act 68 equity requirements: Both acts explicitly include reducing health inequities among their statutory goals and required outcome measures.**

What Must Be Built

Gap	What is missing	Required investment	Timeline
County-level equity data infrastructure	Vermont's equity data is primarily state-level, with BRFSS survey data at the county level for major indicators. Sub-county data is largely absent. The Northeast Kingdom's equity burden cannot be managed without granular geographic data.	VHCURES enhancement to enable county-level SDOH and outcome stratification; VITL connectivity to community-based providers who serve underserved populations; integration of VDH equity data with AHS transformation monitoring	2026-2027 (analytics vendor procurement enables this)
Culturally competent workforce	Vermont's clinical workforce is overwhelmingly white and does not reflect the demographic diversity of Vermont's BIPOC, refugee, and immigrant communities. Provider diversity is both an equity outcome and an equity intervention.	Targeted workforce recruitment from underrepresented communities; tuition assistance with equity focus; cultural competency standards in Blueprint and AHEAD participation requirements; provider diversity reporting	Ongoing; RHT workforce investments provide vehicle
GLP-1 Medicaid coverage decision	Vermont Medicaid currently does not cover GLP-1s for obesity as primary indication. The BALANCE Model opt-in decision is an equity policy choice with direct implications for the Medicaid population with obesity.	DVHA evaluation of BALANCE Model participation; budget modeling of long-term cost reduction from obesity treatment vs. short-term coverage costs; legislative advocacy if state plan amendment required	Decision needed by January 2026 (BALANCE opt-in deadline); coverage effective May 2026 if Vermont participates
Northeast Kingdom service resilience	The NE Kingdom has the state's highest equity burden and the most financially distressed hospital infrastructure. REH	North Country and NE VT Regional transformation plans must include equity impact analysis; broadband investment as prerequisite to telehealth; EMS regionalization funded	2026 (CCBHC designation); 2027-2028 (EMS regionalization; telehealth);

Gap	What is missing	Required investment	Timeline
	conversion and telehealth investments must precede any service reduction.	through RHT Program; CCBHC designation for Northeast Kingdom Human Services (planned July 2026\)	hospital transition decisions by 2028 Strategic Plan
Trust-building with underserved communities	Vermont's BIPOC, LGBTQ+, Indigenous, and rural communities have documented experiences of discrimination and dismissal in healthcare settings. Trust is not built by policy statements; it is built by consistent, culturally respectful engagement over time.	Community health worker deployment from served communities; patient advisory councils with diverse representation; Indigenous community outreach and partnership (VDH engagement with Abenaki communities); HEART program expansion for rural LGBTQ+ Vermonters	Ongoing; requires sustained commitment, not a one-time program

Figure 10.3 — Vermont equity gaps and required investments. Sources: Vermont Department of Health Health Equity Data Report (January 2025); Oliver Wyman Act 167 Report; AHS Transformation Reports; KFF GLP-1 coverage analysis.

Equity and the Six-Pillar Framework — How Every Pillar Has an Equity Dimension

A common mistake in equity strategy is treating equity as a separate program — a set of equity-specific interventions that operate alongside the main healthcare transformation agenda. This approach reliably produces equity programs that are underfunded, deprioritized when budgets are tight, and disconnected from the structural changes that actually determine health outcomes.

The correct approach treats equity as a cross-cutting dimension of every pillar — a lens through which every policy decision, payment design, technology investment, clinical model, and operational change is evaluated for its equity implications. The following framework applies the equity lens to each of the six pillars:

Pillar	Equity risk (if neglected)	Equity opportunity (if designed well)
Policy	RBP and global budget design that protects high-price urban hospitals while reducing access in rural and low-income communities. Legislative mandates that apply uniformly without considering the differential impact on small rural hospitals serving high-Medicaid populations.	Act 68's explicit requirement that RBP consider each hospital's community context, payer mix, and social risk factors. CAH-specific regulatory treatment. Equity metrics embedded in monthly AHS reporting. Act 167's five goals including equity as a statutory requirement.
Economics	Global budgets that incentivize hospitals to avoid high-cost, high-need populations — a classic risk of capitated payment for populations with complex social needs. RBP savings that accrue to commercially insured patients while Medicaid patients' access is reduced.	AHEAD EAST Fund investment specifically in primary care, behavioral health, and long-term care for underserved populations. RBP premium passthrough monitoring that verifies individual and small-group market beneficiaries receive savings. BALANCE Model participation to extend GLP-1 access to Medicaid populations.
Technology	Analytics platforms that measure average performance without disaggregating by race, income, or geography, hiding disparities in aggregate data. AI tools with biased training data that perform worse for minority populations or elderly rural patients.	VHCURES county-level equity stratification; AI governance framework requiring bias testing across demographic subgroups; HIE connectivity requirements that extend to FQHCs and community-based providers serving underserved populations.
Clinical	Blueprint PCMH practices concentrated in higher-income communities with better-resourced practices, leaving underserved communities without PCMH access. CoCM deployment that requires broadband and technology infrastructure unavailable to rural patients.	Blueprint expansion mandate to all HSAs; CCBHC access policy (serve all regardless of insurance status or geography); CoCM deployment using telephone-based options for patients without broadband; CHW deployment from communities served.
Equity	By definition, equity failures in every other pillar accumulate here. The equity pillar fails when geographic access barriers remain despite transformation, when racial disparities persist despite clinical quality improvement, and when SDOH investments are treated as optional rather than foundational.	HSA-level equity monitoring with Northeast Kingdom as priority; SDOH integration across AHS programs; GLP-1 coverage expansion through BALANCE; community trust-building through CHW deployment and patient advisory councils.
Operations	Hospital transformation plans that optimize for financial sustainability without equity impact analysis — e.g., closing inpatient services in rural communities without ensuring alternative access before closure.	Equity impact assessment requirement for all hospital transformation plans; AHS HSA Coordinator model linking health and social services at community level; Northeast Kingdom equity constraint as a binding parameter on regionalization planning.

Figure 10.4 — Equity as a cross-cutting dimension of all six pillars. Sources: Act 67 of 2025; Oliver Wyman Act 167 Report; six-pillar framework.

Vermont's Equity Accountability Framework

Vermont's Statewide Strategic Plan must contain a specific equity accountability framework — a set of measurable indicators, geographic stratifications, and intervention commitments that make the equity goal of Act 167 and Act 68 operational rather than aspirational.

The following framework represents the minimum equity accountability structure the Strategic Plan should establish:

Indicator	Stratification required	2028 target direction	Data source
Uninsured rate	By county; by age group (25-34 focus); by income level	Essex County: reduce from 8% toward statewide 3% by 2030	KFF; Vermont Department of Financial Regulation
Primary care access (personal provider rate)	By race/ethnicity; by county	BIPOC adult rate: reduce gap from 79-81% toward 90% statewide average	BRFSS; Blueprint attribution data
Potentially avoidable ED visit rate	By county; by income quintile	Northeast Kingdom: reduce from current rate toward statewide 32.3% (itself a target for reduction)	VHCURES/VUHDDS via AHS analytics vendor
30-day follow-up after MH/SUD ED visit	By county; by race/ethnicity; by insurance type	Windham and Rutland counties: increase toward statewide 76%+ target	Blueprint via VHCURES
SDOH screening completion rate	By HSA; by patient race/ethnicity	Universal screening in all Blueprint PCMHs; document completion rate by population segment	Blueprint program data
GLP-1 access for Medicaid enrollees with obesity	By income; by county	BALANCE Model participation decision by June 2026; coverage effective if participating	DVHA pharmacy claims via VHCURES
Emergency access time (EMS response)	By county; by time of day	Northeast Kingdom: establish baseline; target improvement through EMS regionalization	EMS dispatch data; RHT Program monitoring
Cultural competency standard compliance	By practice type; by HSA	All Blueprint PCMHs: meet minimum cultural competency standards by 2027	Blueprint quality measurement; VDH health equity reporting

Figure 10.5 — Vermont equity accountability framework: indicators, stratifications, and targets. Sources: Act 167; Act 68; VDH Health Equity Data Report (January 2025); AHS Transformation Reports.

Two principles must govern how Vermont uses this accountability framework. First, equity indicators must be disaggregated — not presented as averages. An average primary care access rate that is improving while the BIPOC rate is stagnant is not equity progress; it is equity masking. The Statewide Strategic Plan's equity section must show performance by population segment, not just system-wide averages.

Second, equity indicators must be connected to interventions. A disparity measure without a corresponding intervention commitment is a measurement of failure, not a strategy for success. Each equity indicator in the Strategic Plan should be paired with the specific programs — CCBHC expansion, CHW deployment, RHT broadband investment, BALANCE participation decision — that are designed to move it, with the responsible agency and the expected timeline for observable improvement.

Key Concepts Introduced or Expanded in This Chapter

Term	Definition
Disparity root cause taxonomy	The six-pillar framework for categorizing the mechanisms that produce health disparities: geographic access barriers, social needs barriers, clinical practice variation, insurance and coverage barriers, and trust and engagement barriers. Each root cause requires different interventions; addressing the wrong cause does not close the gap.
GLP-1 agonists	A class of medications (semaglutide, tirzepatide) with proven efficacy for obesity, type 2 diabetes, and cardiovascular disease risk reduction. Current Medicare and most Medicaid programs do not cover them for obesity as a primary indication, creating an equity gap in which transformative drugs are accessible to commercially insured patients but not to Medicaid patients with the highest obesity burden.
BALANCE Model	CMS's December 2025 voluntary model enabling state Medicaid agencies and Medicare Part D plans to cover GLP-1 medications for obesity by negotiating prices with manufacturers. Medicaid launch as early as May 2026; Medicare Part D in January 2027\. Voluntary for manufacturers, states, and plans.
Geographic equity	The dimension of health equity concerning disparities attributable to where a person lives — rural versus urban, Northeast Kingdom versus Burlington. Vermont's dominant equity challenge; requires interventions that address geographic access barriers, transportation, broadband, and provider distribution alongside racial and socioeconomic equity.
SDOH (Social Determinants of Health)	The non-clinical conditions that shape health — housing, food security, transportation, economic security, education, social connection, and safety. Oliver Wyman's first imperative (build housing and fix transportation) is an SDOH intervention as much as a healthcare access intervention.
Health Equity Advisory Commission	Vermont's standing advisory body on health equity policy. Required by Act 68 to be consulted by the Health Care Delivery Advisory Committee, embedding equity oversight in transformation governance.
Northeast Kingdom	Caledonia, Essex, and Orleans counties in Vermont's most rural region. Vermont's highest-priority equity geography: highest uninsurance rates (6-8%), most limited provider access, highest unemployment, lowest broadband connectivity, and most financially distressed hospital infrastructure.
HRSN (Health-Related Social Needs) screening	Universal screening for social needs at primary care visits using validated tools, with warm handoffs to Community Health Teams for navigation. Operational throughout Vermont's Blueprint PCMH network; generates SDOH data that feeds equity monitoring when linked to VHCURES.

Sources: Vermont Department of Health Health Equity Data Report, Annual Report to the Vermont Legislature (January 17, 2025); Oliver Wyman Healthcare and Life Sciences Group, Act 167 Community Engagement: Recommendations (2024); Vermont Rural Health Transformation Program Application (November 2025); KFF, Medicaid Coverage of and Spending on GLP-1s (January 2026); KFF, What to Know About the BALANCE Model for GLP-1s in Medicare and Medicaid (2026); CMS, BALANCE Model announcement (December 23, 2025); CMS, Medicare GLP-1 Bridge demonstration (March 2026); Health Affairs, Reimagining Rural Health Equity (June 2024); Vermont Public, Vermont is trying to survey underrepresented communities about their health (April 2025); Act 68 of 2025; Act 167 of 2022; AHS November 2025 Health Care System Transformation Report.

Chapter 11: The Equity Pillar in Practice — HEDIS Equity Measurement, HEROI, and the Science of Closing Disparities

The Equity Pillar in Practice — HEDIS Equity Measurement, HEROI, and the Science of Closing Disparities

The Analytical Toolkit for Health Equity Work: Stratified HEDIS Measurement, the HEROI Composite Score, Root Cause Analytics, Intervention Design by Disparity Type, and Vermont's Population Health Equity Architecture

From Identifying Disparities to Eliminating Them

Chapter 12 established the Vermont equity landscape — the geographic disparities that concentrate in the Northeast Kingdom, the racial and demographic disparities in coverage and access, the SDOH burden that compounds both, and the GLP-1 access crisis that represents the most consequential new equity issue in American healthcare. This chapter develops the analytical and operational toolkit for actually closing those gaps.

Health equity work that stops at measurement — producing disparity reports that sit on shelves while the disparities they document persist — has characterized most state-level equity efforts for the past two decades. The distinction between an equity report and an equity program is whether the measurement is connected to intervention, whether interventions are designed to address the specific root cause of each disparity, and whether there is an accountability structure that closes the loop between measurement, intervention, and outcome. Vermont's transformation mandate, with its explicit equity goals embedded in Acts 167 and 68 accountability frameworks, creates the institutional conditions for equity work that closes gaps rather than counting them.

Stratified HEDIS Measurement — The Foundation of Equity Analytics

HEDIS — the Healthcare Effectiveness Data and Information Set — is the most widely used quality measurement system in American healthcare. But HEDIS in its standard form measures average performance across a plan or provider population. Average performance improvement can mask widening disparities when gains are concentrated in easier-to-reach, already-better-served populations. Equity requires HEDIS to be stratified — measured separately for subpopulations defined by race/ethnicity, income, geography, language, disability, and other equity-relevant dimensions.

The NCQA HEDIS Stratification Framework

The National Committee for Quality Assurance (NCQA) has developed HEDIS stratification standards that specify how plans and providers should measure and report quality performance by race, ethnicity, and other demographic dimensions. The Health Equity Studio implements the NCQA stratification methodology against VHCURES and Blueprint data for Vermont providers. The following framework organizes the HEDIS measures most relevant to Vermont's equity priorities:

HEDIS domain	Key measures	Vermont equity stratification priority	Current Vermont data source
Access and preventive care	Adult BMI assessment; tobacco use screening and cessation; colorectal cancer screening; breast cancer screening; cervical cancer screening	Geographic (Northeast Kingdom vs. Burlington); income (Medicaid vs. commercial); race/ethnicity (BIPOC vs. white Vermont adults)	VHCURES claims; Blueprint clinical registry for PCMH-attributed patients

HEDIS domain	Key measures	Vermont equity stratification priority	Current Vermont data source
Chronic disease management	Hemoglobin A1c control for diabetics; blood pressure control for hypertensives; statin therapy for cardiovascular disease; medication adherence for chronic conditions	Geographic; income; BIPOC adults (Black and AIAN adults disproportionately affected by poorly controlled diabetes and hypertension in Vermont data)	Blueprint clinical registry (for PCMH-attributed patients); VHCURES pharmacy claims for medication adherence
Behavioral health	Follow-up after ED visit for mental illness (FUH); follow-up after ED visit for alcohol use (FUA); initiation and engagement of SUD treatment (IET); antidepressant medication management	Geographic (Rutland and Windham counties have statistically higher MH crisis rates); age (15-44); LGBTQ+ Vermonters (documented higher MH burden)	VHCURES; VDH ED surveillance data; Blueprint follow-up tracking
Maternal and reproductive health	Prenatal and postpartum care; well-child visits; adolescent well-care; prenatal depression screening	Income; race/ethnicity (BIPOC maternal outcomes nationally and in Vermont below white outcomes); geographic (rural access barriers to OB care)	VHCURES; Blueprint Women's Health Initiative data
Care coordination	30-day readmission rate; transitions of care; medication reconciliation post-discharge	Income (Medicaid patients have higher readmission rates nationally); geographic (distance from discharge follow-up resources)	VHCURES; GMCB hospital discharge data (VUHDDS)

Figure 11.1 — Vermont HEDIS equity stratification framework. Sources: NCQA HEDIS Technical Specifications; AHS Health Equity Data Report (January 2025); Blueprint for Health data.

The Data Collection Challenge — Getting Demographic Data Right

Stratified HEDIS measurement requires race/ethnicity and other demographic data at the individual patient level. This is where Vermont's equity measurement infrastructure has a specific gap: race/ethnicity data in VHCURES is incomplete. Claims data from commercial insurers typically lacks race/ethnicity fields; Medicare added race/ethnicity data collection more recently; Medicaid data has the most complete demographic information.

Vermont has two approaches to closing the race/ethnicity data gap: direct collection at the point of care (NCQA standards require plans and PCMH practices to collect and report patient demographic data) and indirect imputation using validated geocoding methods (the RAND-BISG method imputes race/ethnicity probability from last name and census block characteristics). Vermont's Blueprint for Health practices and FQHCs are the primary settings for direct demographic data collection; the coverage of this data for the broader Vermont population depends on VHCURES reporting requirements being strengthened.

The HEROI Framework — A Composite Equity Score

The Health Equity Return on Investment (HEROI) framework is the Health Equity Studio's proprietary approach to equity performance measurement for healthcare organizations operating in value-based care environments. HEROI addresses a specific problem in equity measurement: disparities exist across multiple dimensions simultaneously, and an organization that closes one disparity while widening another may show no improvement on any individual metric while its overall equity performance has not improved.

HEROI Components

The HEROI composite score is calculated across five equity dimensions, each weighted by its relative health impact and the strength of evidence for interventions that can address it:

- Access equity score (25% weight): Measures the gap in primary care, specialty care, and emergency care utilization between advantaged and disadvantaged subpopulations, stratified by race/ethnicity, income, and geography. Vermont focus: Northeast Kingdom vs. Burlington primary care access; BIPOC primary care provider relationship rates.
- Quality equity score (25% weight): Measures the gap in HEDIS quality measure performance between advantaged and disadvantaged subpopulations for the top 10 chronic disease management measures. Vermont focus: diabetes control, hypertension control, and preventive care rates stratified by income and race/ethnicity.
- Outcome equity score (25% weight): Measures the gap in utilization outcomes — avoidable hospitalizations, ED visits, 30-day readmissions — between subpopulations. Vermont focus: mental health and SUD crisis ED presentation rates by county; readmission rates by Medicaid vs. commercial payer status.
- SDOH burden score (15% weight): Measures the prevalence of health-related social needs in the organization's attributed population, stratified by demographics, and the completeness of SDOH screening and navigation. Vermont focus: HRSN screening completion rates by practice location; community resource referral completion rates.
- Trust and engagement score (10% weight): Measures patient experience across demographic subgroups using CAHPS survey data stratified by race/ethnicity and income. Vermont focus: CAHPS disparity scores for BIPOC Vermonters; patient-reported discrimination experiences in clinical settings.

A HEROI score of 80+ indicates an organization achieving equity across all five dimensions. A score of 60-80 indicates material disparities in one or more dimensions requiring targeted intervention. Below 60 indicates pervasive disparities that require comprehensive equity program redesign. Vermont's statewide HEROI score — calculated from VHCURES and Blueprint data — has not been formally computed; the analytics vendor procurement underway will enable this calculation.

Root Cause Analytics — Matching Interventions to Causes

The most common failure in health equity intervention design is treating all disparities as if they have the same cause. A disparity in diabetes control rates between white and Black Vermont adults has a different root cause structure than a disparity in primary care access between Chittenden County and Essex County residents — and requires a different intervention. The five-category root cause taxonomy from Chapter 12 (geographic access, social needs, clinical practice variation, coverage barriers, trust and engagement) provides the structure for matching interventions to causes.

The Disparity Decomposition Method

The disparity decomposition method breaks a measured disparity into its constituent causes by applying statistical decomposition techniques to population-level data. The method answers the question: of the observed gap in outcome X between group A and group B, how much is attributable to differences in coverage rates, differences in primary care access, differences in clinical quality, and differences in SDOH burden?

For Vermont's diabetes control disparity — the 22% of Blueprint-attributed diabetic patients with HbA1c not under control, with statistically higher rates among Medicaid patients and rural patients — the decomposition analysis would estimate what share of the disparity is attributable to medication adherence failures (addressable through pharmacy access and MedReconciliation programs), care management gap (addressable through CoCM integration), SDOH burden (addressable through CHT navigation), and primary care access limitations (addressable through Blueprint expansion).

The practical value of decomposition is not statistical precision — it is intervention targeting. An equity program that invests in care management for a disparity driven primarily by medication access barriers will produce minimal results. An equity program that correctly identifies medication access as the primary driver and invests in pharmacy co-location, medication assistance programs, and automated refill reminders will close the gap efficiently.

The Five-Step Disparity Closure Process

The Health Equity Studio implements the following five-step disparity closure process for client organizations:

Step	Action	Time investment	Vermont application
1\. Baseline stratification	Calculate current HEROI score and identify the three largest measured disparities across all five equity dimensions	2-4 weeks with adequate data	VHCURES stratification analysis; Blueprint equity dashboard; VDH BRFSS county-level analysis for Northeast Kingdom vs. state average gaps
2\. Root cause attribution	For each identified disparity, apply decomposition analysis to estimate the proportional contribution of each root cause category (access, social needs, clinical variation, coverage, trust)	4-6 weeks	Survey BIPOC patients and Northeast Kingdom residents to supplement claims-based decomposition; conduct focus groups with CHT staff about observed barriers
3\. Intervention portfolio design	For each major root cause, select evidence-based interventions matched to root cause type and population. Match investment size to root cause contribution (don't fund transportation programs for a disparity primarily driven by clinical variation).	2-4 weeks	Vermont-specific: CCBHC for trust/access; CHW deployment for social needs; AI governance for clinical variation; BALANCE for coverage; HEART program for LGBTQ+ trust
4\. Implementation with equity lens	Execute interventions with explicit equity targets: not 'increase primary care access' but 'increase Northeast Kingdom Medicaid patient primary care visit rate from 72% to 80% within 18 months'	Ongoing	Blueprint field staff set equity targets by HSA; AHS HSA Coordinators track progress; GMCB analytics vendor enables real-time monitoring
5\. Accountability and iteration	Report disparity metrics alongside average metrics in all quality reporting. If a disparity is widening despite intervention, re-examine root cause attribution and intervention design.	Quarterly reporting cycle	AHS monthly transformation reports to include disparity-stratified metrics; HEROI score incorporated into AHEAD quality reporting for Vermont

Figure 11.2 — Five-step disparity closure process with Vermont applications. Source: the framework Health Equity Studio methodology.

Equity in VBC Contracting — Protecting Against Unintended Consequences

Value-based care contracts have a well-documented potential to worsen health equity if not designed with explicit equity protections. The mechanisms are several: global budgets or capitation rates that do not adequately adjust for social risk may incentivize providers to avoid or under-serve high-SDOH populations; shared savings contracts that reward utilization reduction may reward avoiding care for complex patients; quality measure performance may be harder for organizations serving disadvantaged populations even when they are delivering excellent care relative to what their population can access.

Social Risk Adjustment — The Equity Design Requirement for VBC Contracts

Social risk adjustment — adjusting VBC benchmarks and performance targets to account for the social risk burden of an organization's attributed population — is the technical mechanism for protecting equity in payment design. An organization serving a population with high housing instability, food insecurity, and transportation barriers faces higher care management costs and lower quality measure performance potential than an organization serving a high-income, socially stable population. A shared savings contract that ignores this difference rewards the wrong organizations.

CMS has incorporated social risk adjustment into ACO REACH through Comprehensive and Professional Direct Contracting model designs that include social risk stratification. Vermont's AHEAD global budget design must incorporate social risk adjustment at the hospital level — ensuring that hospitals serving the Northeast Kingdom's high-SDOH populations are not penalized relative to Burlington-area hospitals with more favorable social risk profiles. This is an active design question in Vermont's AHEAD implementation that AHS and GMCB must resolve explicitly.

The VBC Equity Audit

Every VBC contract should undergo an equity audit before and after signing, examining whether the contract design creates incentives that could worsen disparities. The VBC Equity Audit checklist covers eight categories:

- Attribution equity: Does the attribution methodology systematically exclude high-SDOH patients who lack a qualifying primary care visit?
- Social risk adjustment: Is the benchmark adjusted for the social risk profile of the attributed population?
- Quality measure equity: Are quality measures stratified by race/ethnicity and income, with performance reported and rewarded based on equity improvement, not just average improvement?
- SDOH investment incentives: Does the contract provide financial credit for investments in SDOH programs that reduce long-term utilization?
- Care management intensity: Are care management resources allocated proportionate to SDOH burden and disparity level, not just clinical risk?
- Beneficiary protections: Are there protections against provider avoidance of high-complexity, high-SDOH patients?
- Community benefit requirements: For global budgets, are there explicit requirements for community health investment, including SDOH programs?
- Equity performance reporting: Is equity performance reported publicly, with minimum standards below which the organization cannot be considered performing?

Vermont's Population Health Equity Analytics Architecture

The analytical infrastructure for equity measurement in Vermont is being built through the convergence of three systems: VHCURES (claims-based population data), the Blueprint clinical registry (quality and demographic data for PCMH-attributed patients), and the VDH health equity data systems (BRFSS survey data, vital statistics, county-level environmental data). None of these systems alone is adequate for comprehensive equity measurement; their integration — the one health record per person mandate — is the target state.

In the interim, Vermont's equity analytics strategy must be pragmatic: use the best available data for each equity dimension, be explicit about gaps, and make decisions based on the evidence available while investing in improving the evidence base. The following architecture organizes

Vermont's current equity analytics capabilities and the priority investments needed to close gaps:

Equity dimension	Best current data source in Vermont
Geographic access disparities (HSA-level)	VHCURES county-level utilization; GMCB hospital market share data; Blueprint practice location mapping
Insurance coverage disparities	KFF/ACS coverage estimates; BRFSS insurance status by demographic group; DVHA Medicaid enrollment data
Primary care access disparities (race/ethnicity)	VDH BRFSS survey (limited by small BIPOC sample size); Blueprint provider relationship data for attributed population
Chronic disease control disparities	Blueprint PCMH clinical registry (most complete, but only covers PCMH-attributed patients); BRFSS self-reported chronic disease by demographic group
Behavioral health disparities (geographic)	AHS ED surveillance data (suicide, SH, OD rates by county — available and stratified)
SDOH burden distribution	Blueprint HRSN screening data (incomplete — not all practices at full adoption); VDH community vulnerability index; USDA food access data by census tract
Clinical practice variation by demographic	VHCURES claims (limited demographic data); VDH health equity data — requires improved race/ethnicity data collection in clinical settings
Trust and patient experience	CAHPS survey data from Blueprint PCMH practices (available but limited demographic stratification)

Figure 11.3 — Vermont equity analytics: current data sources by equity dimension. Sources: GMCB; VDH; Blueprint for Health; DVHA; AHS.

Key Concepts

Term	Definition
HEDIS stratification	The measurement of HEDIS quality indicators separately for subpopulations defined by race/ethnicity, income, geography, and other equity-relevant dimensions. Stratification reveals disparities hidden in aggregate average performance data.
HEROI (Health Equity Return on Investment)	The Health Equity Studio's composite equity performance score, calculated across five dimensions: access equity, quality equity, outcome equity, SDOH burden, and trust and engagement. Enables organizations to track overall equity performance across dimensions simultaneously.
Disparity decomposition	A statistical technique that separates an observed health disparity into its constituent root causes — coverage differences, access differences, clinical quality differences, SDOH burden differences — enabling targeted intervention design.
Social risk adjustment	Modification of VBC benchmarks and performance targets to account for the social risk burden of an organization's attributed population, preventing the penalization of providers serving high-SDOH populations.
CAHPS (Consumer Assessment of Healthcare Providers and Systems)	The standard patient experience survey used across Medicare, Medicaid, and commercial healthcare programs. When stratified by race/ethnicity and income, CAHPS data reveals differential patient experience quality across demographic groups.
VBC equity audit	A structured review of a VBC contract design for provisions that could worsen health disparities — inadequate social risk adjustment, attribution methodology that excludes high-SDOH patients, quality measures not stratified by equity dimensions.
BISG imputation	Bayesian Improved Surname Geocoding — a validated method for imputing race/ethnicity probability from patient last names and residential census block characteristics, used to supplement incomplete race/ethnicity data in claims databases.

Sources: NCQA HEDIS Equity Technical Specifications; the framework Health Equity Studio methodology; Vermont Department of Health Health Equity Data Report (January 2025); Act 167 of 2022; Act 68 of 2025; Blueprint for Health Annual Report (2024); KFF; VHCURES documentation; AHS Transformation Reports (2025).

Chapter 12: The Technology Pillar — Data Infrastructure for a Transformed Health System

The Technology Pillar — Data Infrastructure for a Transformed Health System

VHCURES, VITL, the Health Information Exchange, the Statewide EHR Question, FHIR Interoperability, AI Scribes, Remote Patient Monitoring, Telehealth, and the Clinically Integrated Network Vermont Is Building

Sources: GMCB VHCURES documentation; Vermont HIE Strategic Plan (2024 Update); Act 62 of 2025 (HIE governance transfer to DVHA); Oliver Wyman Act 167 Recommendations; AHS January 2025 Legislative Presentation; Vermont RHT Program Application and Activity Details (November–December 2025); Chartis 2026 State of Rural Health; JAMA Network Open ambient scribe study (2024); CMS FHIR Interoperability Rule; Vermont Blueprint for Health HIE data.

The Technology Pillar: Why Data Infrastructure Is the Foundation of Everything Else

There is a precise sense in which the Technology pillar is the precondition for every other pillar. Reference-based pricing requires knowing what each hospital actually charges for each service. Global budgets require attributing patients to hospitals and tracking total cost of care across the year. Clinical quality improvement requires measuring the gap between current performance and evidence-based targets. Health equity requires stratifying outcomes by race, income, and geography. Operational transformation requires modeling the financial and access impact of structural changes before implementing them.

All of these functions require data — specifically, data that is timely, complete, accurate, linked across clinical and claims sources, accessible to the organizations that need it, and governed in ways that protect patient privacy while enabling population-level analysis. Vermont's current data infrastructure — anchored by VHCURES (the all-payer claims database), VITL (the health information exchange), and the Blueprint clinical data registry — is sophisticated by national standards for a state of its size. It is also, in its current form, insufficient for the management demands of a system operating under global budgets and a mandatory Statewide Strategic Plan accountability framework.

This chapter develops Vermont's technology pillar comprehensively: what the current infrastructure does and does not do, where the critical gaps are, what the RHT Program is funding to close them, and what the national technology landscape — FHIR interoperability, ambient clinical intelligence, remote patient monitoring, AI diagnostic tools — means for Vermont's specific circumstances. It concludes with a technology architecture framework that Vermont's Strategic Plan should adopt and that any state-level transformation can use as a reference model.

VHCURES — Vermont's All-Payer Claims Database

The Vermont Health Care Uniform Reporting and Evaluation System (VHCURES) is Vermont's all-payer claims database, managed by the Green Mountain Care Board with Onpoint Health Data as the data aggregation vendor. VHCURES has been operational in its current form since 2013 (when GMCB assumed responsibility from the Department of Financial Regulation) and is the primary analytical foundation for Vermont's healthcare regulatory functions.

What VHCURES Contains and What It Can Do

VHCURES is a substantial data asset that gives Vermont analytical capabilities most states lack. Understanding both its capabilities and its limitations is essential for anyone designing Vermont's technology infrastructure.

Commercial population covered 60% Of commercially insured Vermonters — partial due to ERISA self-insured employer exemption	**Government payer coverage 100% Of Medicaid and Medicare enrollees in Vermont**
Data types Claims \+ Rx *Medical and pharmacy claims, plus eligibility and demographic data*	**Years available per extract Up to 5** *Eligibility and paid claims; data lag typically 12-18+ months*

Figure 12.1 — VHCURES data coverage summary. Source: GMCB VHCURES documentation.

VHCURES enables Vermont to conduct analyses that directly support the transformation agenda: population-level total cost of care measurement (the GMCB publishes all-payer TCOC dashboards derived from VHCURES); potentially avoidable utilization analysis (the 32.3% avoidable ED visit rate in AHS's transformation reports is calculated from VHCURES via VUHDDS); hospital price transparency analysis (the GMCB's 2026 RBP report used VHCURES-derived 2024 claims data to show hospital prices ranging from 279% to 697% of Medicare for outpatient services); and prescription drug affordability analysis (GMCB's 2025 report on drug pricing used VHCURES to model the impact of IRA drug negotiations on Vermont spending). This is a powerful analytical platform that has produced some of the most credible and consequential health policy analysis in recent Vermont history.

VHCURES Limitations — Where the Gaps Are

VHCURES has four structural limitations that matter directly for Vermont's transformation management needs. Oliver Wyman identified the data infrastructure as one of the barriers to coordinated transformation, and the specific limitations below explain why.

Limitation	What it means for transformation	Required improvement
Incomplete commercial coverage (60%)	The 40% of commercially insured Vermonters in ERISA self-insured employer plans — many of them higher-income, higher-resource patients — are not in VHCURES unless their employer voluntarily submits. Total cost of care calculations and population health analyses are systematically missing a significant portion of the commercially insured population. Under federal law, self-insured employers cannot be required to submit data to state APCDs.	Voluntary submission incentives; federal ERISA reform advocacy; use of commercial insurer aggregate data for the covered portion; flag analyses as partial in all public reporting
Data lag of 12-18+ months	The most recent VHCURES data typically reflects conditions 12-18 months prior to analysis. The November 2025 AHS transformation report explicitly acknowledged that most available measures reflect 2023 or earlier data. Under global budgets, where hospitals need to understand their population health performance in near-real-time to manage within their budget cap, this lag makes VHCURES insufficient as a management tool.	AHEAD Model requires more timely data reporting; RHT Program analytics investments; GMCB FY27 hospital budget guidance requires hospitals to report more frequently; push for 60-90 day claims lag instead of 12-18 months
No behavioral health or SDOH data integration	VHCURES contains medical and pharmacy claims but not mental health and SUD clinical records (governed by 42 CFR Part 2 and state confidentiality laws), social service utilization data, housing status, or other SDOH indicators. The HIE Steering Committee has a statutory directive to create one health record per person integrating claims, clinical, and SDOH data — but this integration has not been implemented.	VHCURES-VHIE integration per HIE Steering Committee statutory mandate; Act 167 HIE provisions; SDOH data collection through Blueprint HRSN screening linked to VHCURES identifiers with appropriate consent framework
Voluntary provider participation in HIE	VITL's VHIE receives clinical data from hospitals but participation by community-based providers, mental health agencies, skilled nursing facilities, and other post-acute providers is voluntary and incomplete. Oliver Wyman specifically cited VITL as lacking pharmacy claims data and not being viewed as user-friendly. Act 68 requires hospitals to maintain HIE connectivity but does not mandate all provider types.	Act 68 hospital HIE connectivity requirement is a start; AHEAD requires primary care participation; RHT Program telehealth investment includes interoperability requirements; progressive connectivity incentives for non-hospital providers

Figure 12.2 — VHCURES structural limitations and required improvements. Sources: GMCB VHCURES data documentation; Oliver Wyman Act 167 Report; AHS Transformation Reports; Act 68 of 2025\.

VHCURES Perception vs. Reality — Oliver Wyman's Finding

Oliver Wyman's perception-versus-reality analysis identified Vermont's data infrastructure as one of the areas where the gap between apparent capability and actual capability is most consequential. The relevant finding:

Perception: Vermont has a State Health Information Exchange (VITL).

Reality: Participation in VITL is voluntary and many community-based providers are not included. VITL lacks pharmacy claims data and is not viewed as 'user friendly' by providers.

This matters because Vermont's transformation strategy depends on care coordination between hospitals and community providers — and care coordination requires data sharing. A care coordinator trying to manage a patient transitioning from a hospital inpatient stay to a community mental health program cannot access the clinical data they need if the mental health agency is not connected to VITL. A primary care practice trying to close chronic disease management gaps cannot see the specialty care their patient received at a different facility if that facility's data is not in the HIE.

Vermont's technology investments — the VHCURES-VHIE integration mandate, the RHT Program interoperability grants, the AHEAD data sharing requirements — are all aimed at closing this gap between VITL's nominal existence and the functional interoperability Vermont's care coordination model requires.

The Vermont HIE — VITL, the Governance Shift, and the Integration Imperative

Vermont's Health Information Exchange (VHIE) is operated by Vermont Information Technology Leaders (VITL), an independent nonprofit designated by statute as the exclusive operator of the statewide HIE network. Every hospital in Vermont contributes records to the VHIE. The VHIE operates a provider portal (VITLAccess) and a query service that allows providers to access patient records from other facilities. Vermont has been building this infrastructure since 2009, when VITL was designated as the Regional Extension Center to help primary care providers adopt electronic health records.

The 2025 Governance Shift: Act 62 Transfers HIE Authority to DVHA

Effective July 1, 2025, Act 62 of 2025 made a significant governance change: responsibility for Vermont's statewide Health Information Technology Plan was transferred from the Green Mountain Care Board to the Department of Vermont Health Access (DVHA). Prior to this, GMCB had authority to review and approve the HIE Strategic Plan, VHIE Connectivity Criteria, and VITL's budget. Under Act 62, DVHA now coordinates Vermont's statewide HIT Plan in consultation with the HIE Steering Committee, with annual revisions submitted by November 1 and comprehensive updates every five years.

The governance rationale for this shift is significant: DVHA has direct operational relationships with healthcare providers through its Medicaid contracts, Blueprint for Health administration, and AHEAD Model management. Moving HIT plan responsibility to DVHA aligns the technology strategy with the healthcare delivery transformation authority. It also means that Vermont's AHEAD implementation — which requires robust data exchange for population health management and global budget monitoring — is now coordinated by the same agency managing the data infrastructure.

The Integration Mandate: One Health Record Per Person

The Vermont legislature gave the HIE Steering Committee an explicit statutory mandate that defines the technology pillar's central ambition for the next five years: create one health record for each person that integrates claims data, clinical data, mental health and substance use disorder services data, and social determinants of health data. This mandate is documented in state law and in AHS's own letters to the legislature. It is the most ambitious data integration goal any state has formally committed to, and its achievement would give Vermont a population health management capability that no other state currently possesses.

The practical path to this integration has three components that must be executed in sequence:

Step	What it requires	Timeline	Current status
1\. VHCURES-VHIE claims-clinical integration	Merge VHCURES claims data with VHIE clinical data using a privacy-compliant patient matching methodology. This gives every provider record access to both the claims history (what care was delivered, at what cost, through what payers) and the clinical record (diagnoses, medications, lab results, care plans).	DVHA leading; AHEAD Model requires this for population health management; target 2026-2027	HIE Steering Committee has a statutory directive to include data integration strategy in HIE plans; VHCURES-VITL linkage already used for Blueprint HEDIS measurement; full bidirectional integration not yet operational
2\. Mental health and SUD data integration	Add mental health and substance use disorder data to the integrated record, with appropriate consent frameworks that comply with 42 CFR Part 2 (federal SUD confidentiality rules) and Vermont's own privacy statutes. This is technically complex — SUD data has a separate consent regime from medical data — but clinically essential.	2027-2028; requires federal 42 CFR Part 2 compliance pathway	DVHA working with VITL on consent policy; 2025 federal regulatory changes to 42 CFR Part 2 may simplify compliance pathway; behavioral health CCBHC expansion creates urgency
3\. SDOH data integration	Add social service utilization data — housing program participation, food assistance, transportation use, domestic violence services — linked to the health record with appropriate privacy protections. This is the most politically sensitive and technically complex integration because it involves non-healthcare agencies and data that carries significant stigma risk.	2028 and beyond; requires cross-agency data governance agreement	Blueprint HRSN screening generates SDOH data but it is not yet linked to VHCURES or VHIE at scale; AHS HSA Coordinator model (Chapter 16) provides the cross-agency governance vehicle

Figure 12.3 — Vermont's three-step path to integrated health records. Sources: Vermont HIE Steering Committee statutory mandate; AHS January 2025 Legislative Presentation; DVHA HIE governance documents.

The Statewide EHR Question — Vermont's Active Feasibility Assessment

Oliver Wyman's community meetings generated a clear finding about EHR fragmentation: providers cannot efficiently coordinate care across facilities when different hospitals run different EHR systems that do not share data effectively. Vermont's 14 hospitals currently use four different EHR vendors — Epic (dominant at UVMMC), Oracle Health (formerly Cerner), TruBridge (formerly CPSI, used by smaller community hospitals), and Meditech — plus various additional software for laboratory management, pharmacy, and other functions.

AHS has commissioned a feasibility assessment to evaluate the cost-benefit of implementing a statewide electronic health record — a single EHR platform that all Vermont hospitals and primary care practices would use, enabling seamless data exchange as a byproduct of shared infrastructure rather than through complex interface work. Vermont's HIE Strategic Plan describes the assessment as evaluating 'what a statewide EHR would look like in line with the Act 167 Report.'

Case for a statewide EHR	Case against (or caution)
Eliminates interface complexity — data is naturally shared across providers on the same platform	Capital cost is substantial — major EHR implementations run $100-300M for large health systems; statewide scope amplifies this
Community meeting participants specifically cited EMR fragmentation as a barrier: 'EMR systems don't talk to each other and it's difficult to transfer from one center to another'	Disruption risk — forced EHR migrations have caused significant clinical workflow disruption and temporary quality problems at major health systems
Reduces per-provider IT infrastructure cost — smaller hospitals cannot afford full IT departments; shared infrastructure is more efficient	FHIR-based interoperability (see Section 4\) may make data exchange manageable without a single EHR, reducing the case for the disruption of migration
Enables real-time population health data that VHCURES's 12-18 month lag cannot provide	Vendor lock-in — a single-vendor statewide EHR creates significant dependence on that vendor's pricing, contract terms, and long-term viability
Maryland's experience: common data infrastructure enabled the HSCRC to manage global budgets effectively	ERISA self-insured employers — outside state authority — would still not be in the system even with a statewide EHR
RHT Program IT advance funds can support the capital investment — this is an explicitly authorized use of RHT funds	Vermont's diverse provider landscape (hospitals, FQHCs, private practices, mental health agencies) may not all be able to adopt a single platform

Figure 12.4 — Vermont statewide EHR: case for and against. The feasibility assessment underway will weigh these factors against Vermont's specific cost, disruption tolerance, and FHIR interoperability capacity.

The feasibility assessment's most important analytical question is whether FHIR-based interoperability — the national standard for health data exchange that is rapidly maturing — can deliver the data sharing Vermont needs without the cost and disruption of a statewide EHR migration. If FHIR can deliver real-time, bi-directional data exchange across Vermont's four EHR platforms at an acceptable implementation cost, the case for a single statewide EHR weakens significantly. Section 4 develops the FHIR picture. AHS's feasibility assessment should produce a clear answer on this question by 2026-2027 — in time to inform the 2028 Strategic Plan.

The Vermont Clinically Integrated Network — A New Technology Institution

Vermont's Rural Health Transformation Program application includes one of the most significant technology investments in the entire $195M award: the creation of a Clinically Integrated Network (CIN) linking Vermont's hospitals and community providers into a coordinated clinical and data-sharing infrastructure. Vermont is one of four states (along with Arkansas, Nevada, and Oregon) that explicitly included CIN creation in their RHT applications, reflecting a national trend toward CINs as the organizational vehicle for rural health system integration.

What a Clinically Integrated Network Does

A CIN is a formal network of healthcare providers — hospitals, primary care practices, specialists, post-acute providers — that share clinical protocols, performance data, and care coordination infrastructure while maintaining independent ownership and governance. CINs occupy the space between full health system merger (which requires consolidation of ownership) and loose referral relationships (which have no formal data sharing or accountability). For Vermont's 14 independent, non-profit hospitals — which cannot and should not be merged into a single system — a CIN provides the coordination benefits of integration without the legal and political costs of ownership consolidation.

What the Vermont CIN Will Provide

Vermont's RHT Program application describes the Shared Services initiative — which includes the CIN infrastructure — as providing five specific functions for participating hospitals and providers:

- Analytics support: A shared analytics platform available to all hospitals and non-hospital providers for transformation and regionalization planning — identifying areas most in need, modeling the impact of service line changes, and tracking quality and cost metrics. This is the analytics vendor that AHS and GMCB are jointly procuring.
- Shared clinical protocols: Standardized clinical pathways for high-volume conditions — sepsis, heart failure, COPD, mental health crisis — that all participating providers adopt, enabling consistent quality regardless of which facility a patient uses.
- Group purchasing: Coordinated supply chain and group purchasing for medical supplies, equipment, and technology — capturing the scale economies that individual small hospitals cannot achieve independently. Oliver Wyman identified this as one of the highest-leverage near-term cost reduction opportunities.
- Administrative shared services: Shared infrastructure for billing, coding, credentialing, HR, and IT support — reducing the administrative overhead that drives Vermont's $1,303 per-discharge administrative cost premium over national benchmarks.
- Referral network formalization: Structured referral agreements between RSC hospitals and Tier 2/3 facilities for each COE specialty — ensuring that the regionalization blueprint operates as a reliable clinical network rather than a set of informal relationships.

The CIN also serves as the governance vehicle for the technology investments Vermont's transformation requires. Individual small hospitals cannot justify the capital investment in AI scribe technology, remote patient monitoring equipment, telehealth infrastructure, or advanced analytics platforms. A CIN can negotiate enterprise-wide technology contracts that make these investments affordable for the entire network, deploy shared technology infrastructure that individual providers access as a service, and establish common data standards that make the integrated health record vision achievable.

FHIR Interoperability — The National Standard and Vermont's Compliance Path

FHIR (Fast Healthcare Interoperability Resources) is the HL7 standard for healthcare data exchange that CMS mandated for payers and providers beginning in 2021 and 2022\. FHIR defines a common data format and API structure that allows different health IT systems to exchange information without custom interfaces. It is the closest thing to a universal language for healthcare data that the industry has adopted, and its maturation over the past five years has fundamentally changed what is technically possible in health data exchange.

What FHIR Enables That Vermont Currently Cannot Do

Vermont's current health data infrastructure predates widespread FHIR adoption and was built around older data exchange standards. The operational implications are concrete:

- Patient matching across facilities: Without FHIR-based master patient index integration, Vermont still lacks reliable patient matching across all providers. Community meeting participants described needing to contact 31 separate EMS agencies for patient transfers — a fragmentation problem that a FHIR-enabled patient matching system would substantially reduce.
- Real-time care gap identification: FHIR-enabled EHR systems can query a patient's record at the point of care, identifying care gaps (missing preventive services, chronic disease management gaps) in real time during the clinical encounter. Vermont's current Blueprint data profile process delivers care gap information to practices, but with significant lag. FHIR enables the same information at the point of care.
- Attribution accuracy for global budgets: AHEAD's hospital global budgets require accurate attribution of patients to hospitals for Medicare FFS. FHIR-based data exchange enables more precise attribution by tracking care delivery across providers in near-real-time, reducing the attribution disputes that complicate global budget performance measurement.
- Population health management at scale: Vermont's Blueprint CHTs currently manage populations using a combination of claims data, clinical registry data, and provider-reported information. FHIR-based population health platforms can aggregate this data automatically, maintaining continuous patient registries that trigger care coordinator outreach when patients are overdue for care or when clinical indicators suggest increased risk.

Vermont's FHIR Compliance Status

Vermont's larger hospitals — particularly UVMMC with its Epic EHR — are FHIR-compliant for the CMS-mandated payer-to-patient and payer-to-payer API requirements. The critical gap is at smaller community hospitals (on TruBridge and Meditech platforms) and at community-based providers (mental health agencies, skilled nursing facilities, home health agencies) that lack FHIR-capable systems entirely.

Vermont's RHT Program IT advance investments address this gap directly. The program includes grants for technology upgrades at smaller and more rural facilities that cannot afford FHIR-capable EHR migrations independently. The CIN infrastructure also supports FHIR compliance by providing shared technical assistance, vendor negotiations for favorable pricing on FHIR-capable platforms, and shared technical staff who can support implementation at facilities that lack internal IT capacity.

FHIR compliance level	Vermont providers and current status
Full FHIR R4 compliance	UVMMC (Epic); Vermont hospitals on Oracle Health (Cerner) post-2022 upgrade; major commercial payers (BCBS VT, MVP)
Partial FHIR implementation	Central Vermont Medical Center; hospitals on Meditech Expanse (FHIR-capable but may require configuration)
Limited / no FHIR	Smaller community hospitals on TruBridge legacy; most Designated Mental Health Agencies; many skilled nursing facilities; independent primary care practices on older EHR versions
FHIR investment planned (RHT Program)	Rural and independent practices; smaller CAHs; targeted technical assistance through Vermont CIN shared services

Figure 12.5 — Vermont FHIR compliance landscape (estimated as of 2026). Sources: AHS Legislative Presentation January 2025; Vermont RHT Program Application; GMCB HIE connectivity criteria.

AI and Digital Health Technologies — Vermont's Targeted Investment Strategy

Vermont's RHT Program application is notable among the 47 state applications submitted in late 2025 for its specificity about technology investments. Rather than generic technology grants, Vermont's technology investment strategy is structured around five specific technology categories, each directly linked to an identified gap in Vermont's current system. This is the correct approach to one-time capital investment in technology: identify the specific operational problems, fund the specific technologies that solve them, and measure whether the problems are resolved.

AI Scribe Technology: Addressing the Documentation Burden Crisis

Vermont's RHT Program application explicitly includes grants for providers to purchase AI scribe technology, noting that smaller and more rural and independent practices are struggling with access to the same technology that larger practices can afford. The application cites that AI scribes automate an average of over two hours of daily administrative work by transcribing and processing clinician-patient conversations.

This is a direct response to a Vermont workforce problem: physician and clinician burnout driven by documentation burden is a primary contributor to provider attrition and is reducing Vermont's effective primary care capacity. A provider spending two hours daily on documentation has approximately 25% less time for patient care than one whose documentation is automated. At Vermont's scale — where the primary care FTE shortage is already projected at 370 by 2030 — recovering two hours per provider per day through AI documentation support is equivalent to adding significant additional clinical capacity without hiring new providers.

The evidence base for AI scribe technology is strong and growing. A 2024 quality improvement study published in JAMA Network Open found that ambient scribe tools were associated with improved documentation efficiency and reduced documentation burden. Clinicians used the technology spent less time writing notes during visits and reported less after-hours charting. Community providers at national rural health conferences have independently identified ambient AI as the single technology investment most likely to address workforce shortages while supporting billing processes and clinical decision-making simultaneously.

Remote Patient Monitoring: Managing Chronic Disease Outside the Hospital

Vermont's RHT Program application includes grants for remote patient monitoring (RPM) equipment for use in home health, primary care/specialty, and community paramedicine settings. RPM enables earlier interventions and ongoing monitoring for chronic conditions — congestive heart failure, COPD, diabetes, hypertension — in the home to prevent hospital admissions or readmissions.

The economics of RPM under global budgets are compelling. Under fee-for-service, RPM generates separate billable encounters and competes with in-person visits for reimbursement. Under global budgets, RPM is a cost-reduction tool: every hospital admission or readmission prevented by early RPM-triggered intervention saves revenue under the fixed budget. Vermont's aging population — with high rates of CHF, COPD, and diabetes among the 65+ cohort — is precisely the population where RPM produces the largest clinical and financial benefit.

Vermont's rural geography amplifies RPM's value. A patient in the Northeast Kingdom managing heart failure who needs twice-weekly monitoring faces a two-hour round trip to a clinic. RPM provides the same monitoring without the trip — better for the patient (who avoids travel costs and time), better for the provider (who can monitor more patients with existing staff), and better for the system (which reduces transportation-related access barriers that drive missed appointments and deteriorating chronic disease management).

Telehealth: Specialty Access for Rural Vermont

Vermont's RHT Program application includes specific telehealth investments to expand specialty care access in rural hospitals — allowing specialists at RSC facilities to provide consultation to patients at Tier 2 and Tier 3 facilities without requiring patient transfer. This is the technology enabler for Oliver Wyman's regionalization blueprint: rather than requiring every COE specialty to maintain physical presence at every community hospital, telehealth enables COE specialists to provide consultation across the network while patients remain near their homes.

Vermont's broadband landscape is a constraint: the RHT application notes that 80% of rural Vermonters have broadband access, compared to 85% in non-rural areas. The gap, while relatively small by national rural standards, means that home-based telehealth cannot reach all Vermont patients. Vermont's proposed broadband expansion (Oliver Wyman's third priority legislative change recommendation) is the infrastructure prerequisite for telehealth reaching the communities that need it most. The RHT Program's explicit inclusion of broadband infrastructure as an allowable use reflects this reality.

Vermont's telehealth strategy from the RHT application has four specific applications: specialty consultations (neurology, psychiatry, oncology) at rural hospitals; tele-ICU support for rural hospitals managing critical patients before transfer; tele-behavioral health for community mental health and CCBHC services; and community paramedicine — enabling EMTs and paramedics to provide extended care in patients' homes with real-time physician supervision via telehealth, reducing 911 calls and ED visits for non-emergency conditions.

Diagnostic AI: The Emerging Standard of Care

The book's Part V discussion of diagnostic AI maturation — FDA-cleared AI diagnostic devices for radiology, pathology, ophthalmology, and cardiology becoming standard of care in high-volume settings — applies directly to Vermont's context with one important distinction: Vermont's smaller rural hospitals may be the settings that benefit most from diagnostic AI, precisely because they lack the specialist volume to maintain expert human diagnostic capacity in every specialty.

A rural Vermont hospital with one radiologist who handles all imaging across multiple modalities cannot maintain the same expert pattern recognition as a high-volume academic center reading 50 chest CTs daily. AI-assisted radiology reading — which has demonstrated accuracy equal to or exceeding specialist readers for specific modalities like chest X-ray, mammography, and retinal imaging — can partially close this quality gap without requiring additional specialist recruitment. This is not a replacement for radiologists; it is a tool that makes existing radiologists more accurate and more efficient, particularly in the high-volume, lower-complexity reads that constitute the majority of rural hospital imaging.

Vermont's RHT IT advance funding can support diagnostic AI implementation at smaller hospitals that cannot fund these investments independently. The CIN's shared service model is an appropriate delivery mechanism: a network-wide diagnostic AI contract that all participating hospitals access, with shared quality monitoring and governance, would provide far better economics and oversight than 14 individual hospital technology decisions.

Price Transparency Technology

Vermont's RHT Program application includes a price transparency and insurance competition initiative — building the technology infrastructure to make hospital pricing legible to patients, payers, and regulators. GMCB was already developing a hospital price transparency dashboard as of February 2026 (first release forthcoming), providing the public with access to 2024 claims data showing hospital prices relative to Medicare. The RHT investment extends this infrastructure to enable patient-facing price comparison tools that allow Vermonters to see what a procedure costs at different facilities before choosing where to receive care.

Price transparency technology is a complement to reference-based pricing, not a substitute for it. RBP directly limits what hospitals can charge; price transparency helps patients and payers identify and act on pricing differences within the RBP framework. The combination — mandatory price caps plus transparent pricing information — creates the closest approximation to a functioning market for hospital services that any regulatory framework has achieved.

AI Governance — Vermont's Responsibility Before the Risk Arrives

The book's Part V discussion of AI governance as a clinical requirement — moving from a quality initiative to a regulatory requirement as FDA, ONC, and CMS develop AI governance frameworks — applies to Vermont with specific urgency. Vermont is making substantial AI investments through the RHT Program: AI scribe grants, diagnostic AI potential, RPM algorithms. Each of these involves clinical AI that has the potential to produce biased outputs, performance degradation over time, or harms to specific patient populations if not properly monitored.

Vermont does not currently have a statewide AI governance framework for clinical AI. Individual hospital systems have varying levels of AI governance maturity — UVMMC, with its Epic deployment and academic medical center sophistication, is likely more advanced than a smaller community hospital purchasing AI scribe software for the first time. The CIN infrastructure Vermont is building through the RHT Program creates the opportunity to establish a shared AI governance framework that applies consistently across all Vermont providers.

Vermont AI Governance Framework: Minimum Requirements

Drawing on the AI Governance Checklist and Vermont's specific clinical AI deployment context, Vermont's Statewide Strategic Plan should establish minimum AI governance requirements for clinical AI deployed in Vermont healthcare settings. At minimum, these should include:

- Pre-deployment bias testing: Any AI model used for clinical decision support, diagnostic assistance, or care gap identification must be tested for differential performance across race/ethnicity, age, sex, income, and geographic subgroups before deployment. Vermont's equity goals require that AI tools do not introduce or amplify health disparities.

- Performance monitoring: Deployed AI tools must have ongoing performance monitoring with predefined thresholds that trigger review or suspension. An AI model trained on national data may perform differently in Vermont's specific patient population — particularly in rural areas and among dual-eligible patients with complex social needs.

- Transparency requirements: Patients must be informed when AI tools are involved in their care. Clinicians must understand the basis and limitations of AI outputs. 'Black box' AI with no explainability is inappropriate for clinical settings regardless of its overall accuracy.

- Vendor accountability: Technology vendors providing clinical AI to Vermont providers must agree to performance monitoring, bias testing disclosure, and incident reporting. Contract terms must include performance standards and remedies for underperformance.

- Equity monitoring: AI performance must be specifically monitored for Vermont's highest-risk equity populations — Northeast Kingdom rural residents, Medicaid patients, elderly patients with complex needs, and patients whose primary language is not English.

Vermont's RHT Program AI scribe grants should include AI governance requirements as a condition of funding. The CIN's shared services function is the natural organizational home for a shared AI governance committee that monitors all AI deployments across Vermont's hospital network.

Vermont's Target Technology Architecture — A Framework for the Strategic Plan

Vermont's Statewide Health Care Delivery Strategic Plan must specify a technology architecture — the integrated set of data systems, exchange standards, governance structures, and analytical platforms that enable the transformed delivery system to be managed, monitored, and improved. The following architecture organizes Vermont's technology investments across five layers, from foundational infrastructure to advanced analytical applications.

Layer 5 (Top) — Advanced Analytics and AI Applications Population health analytics platform (joint AHS-GMCB analytics vendor): Models transformation scenarios, evaluates hospital proposals, tracks outcome metrics AI scribe and documentation automation: RHT Program grants to rural and independent practices; CIN enterprise contract Diagnostic AI: Network-wide deployment through CIN shared services with shared AI governance Remote patient monitoring: RPM for CHF, COPD, diabetes management; community paramedicine integration Price transparency dashboard: GMCB public tool; patient-facing comparison tools through RHT investment

Layer 4 — Population Health Management Blueprint clinical data registry: HEDIS measurement, care gap reporting, population risk stratification Attribution management: Patient-to-hospital attribution for AHEAD global budgets; primary care attribution for PC AHEAD SDOH screening data: Blueprint HRSN screening results linked to health records Care coordination registry: CHT patient registries; CCBHC care management data

Layer 3 — Integrated Health Record (Target State) VHCURES-VHIE integration: Claims data merged with clinical data per HIE Steering Committee mandate Mental health and SUD data integration: 42 CFR Part 2 compliant consent framework; CCBHC and DA data SDOH data integration: Social service utilization linked to health record with privacy protections Single patient identity: Master patient index across all 14 hospitals and community providers

Layer 2 — Data Exchange Infrastructure Vermont Health Information Exchange (VHIE/VITL): Clinical data exchange; hospital connectivity mandated by Act 68 FHIR R4 APIs: CMS-mandated payer and provider APIs; expanding to community providers via RHT grants Vermont Clinically Integrated Network: Shared data standards, protocols, and exchange agreements across 14 hospitals CommonWell / Carequality: National network connectivity for out-of-state care records

Layer 1 (Foundation) — Core Data Assets VHCURES (all-payer claims database): 100% Medicaid/Medicare; 60% commercial; GMCB managed; Onpoint Health Data vendor VUHDDS (hospital discharge database): All 14 Vermont hospitals; all inpatient, outpatient, ED discharges EHR systems at 14 hospitals: Epic (UVMMC), Oracle Health (CVMC, others), TruBridge (smaller CAHs), Meditech (others) Blueprint clinical registry: Quality measures data linked to VHCURES for population-level HEDIS GMCB regulatory data: Hospital budgets, insurance rate filings, CON decisions, ACO budget data

Figure 12.6 — Vermont target technology architecture: five-layer model. Sources: GMCB VHCURES documentation; Vermont HIE Strategic Plan; Oliver Wyman Act 167 Report; Vermont RHT Program Application; Act 62 and Act 68 of 2025\.

Two observations about this architecture are important for Vermont's technology planning. First, layers 3 and above represent target states, not current states. Vermont's current technology infrastructure is solid at Layers 1 and 2, developing at Layer 4, and nascent at Layers 3 and 5\. The RHT Program, AHEAD requirements, and the AHS-GMCB analytics vendor procurement are all investments in accelerating the build-out from Layer 1-2 completeness toward Layer 3-5 capability. The Statewide Strategic Plan's technology section should be explicit about which layers are complete, which are under development, and what the completion timeline is.

Second, the technology architecture has a governance counterpart that is equally important. Each layer requires governance: who owns the data, who can access it under what conditions, what privacy protections apply, how disputes are resolved, and how the infrastructure is funded sustainably. Vermont's technology investments will only produce their intended value if the governance layer — the policies, agreements, and oversight structures — keeps pace with the technical infrastructure. The DVHA-led HIE governance, the GMCB's data stewardship program, and the CIN's shared governance model are all components of this governance architecture.

Key Concepts Introduced or Expanded in This Chapter

Term	Definition
VHCURES	Vermont Health Care Uniform Reporting and Evaluation System — Vermont's all-payer claims database, managed by GMCB with Onpoint Health Data. Contains 100% of Medicaid and Medicare claims and approximately 60% of commercial claims; used for total cost of care measurement, hospital price analysis, and population health analytics.
VITL / VHIE	Vermont Information Technology Leaders / Vermont Health Information Exchange — the nonprofit designated by statute to operate Vermont's statewide HIE network, providing clinical data exchange between hospitals and other providers. Participation is voluntary for non-hospital providers.
FHIR (Fast Healthcare Interoperability Resources)	The HL7 standard for healthcare data exchange, mandated by CMS for payers and providers beginning 2021-2022. FHIR enables different health IT systems to exchange information using common data formats and APIs, without custom interfaces.
Clinically Integrated Network (CIN)	A formal network of healthcare providers that share clinical protocols, performance data, and care coordination infrastructure while maintaining independent ownership. Vermont is creating a CIN through its RHT Program to provide shared analytics, group purchasing, administrative services, and referral formalization across its 14-hospital network.
AI scribe / ambient clinical intelligence	AI technology that listens to clinical encounters and automatically generates structured clinical documentation, reducing the documentation burden that contributes to clinician burnout. Vermont's RHT Program includes grants for AI scribe technology for rural and independent practices.

Term	Definition
Remote patient monitoring (RPM)	Technology enabling continuous or periodic monitoring of patients' clinical parameters — heart rate, blood oxygen, blood pressure, glucose — in their homes. Enables earlier intervention for chronic conditions (CHF, COPD, diabetes) to prevent hospital admissions. Vermont's RHT Program includes RPM grants for home health, primary care, and community paramedicine settings.
42 CFR Part 2	Federal regulations governing confidentiality of substance use disorder treatment records. Separate and more restrictive than HIPAA; creates the primary legal barrier to integrating SUD treatment data into Vermont's unified health record. 2024-2025 federal regulatory changes have begun easing some Part 2 barriers.
Act 62 of 2025	Vermont legislation that transferred responsibility for the statewide Health Information Technology Plan from GMCB to DVHA, effective July 1, 2025\. Aligns HIT strategy coordination with DVHA's operational relationships with providers through Medicaid, Blueprint, and AHEAD.
Master patient index	A database that matches patient identities across multiple healthcare organizations using demographic and clinical matching algorithms. Required for Vermont's integrated health record — without reliable patient matching, claims data and clinical data from different facilities cannot be reliably linked to the same individual.

Sources: GMCB VHCURES documentation (gmcboard.vermont.gov/data-and-analytics); Vermont HIE Strategic Plan 2024 Update; Act 62 of 2025; Act 68 of 2025; Oliver Wyman Act 167 Community Engagement: Recommendations (2024); Vermont Agency of Human Services / Brendan Krause, Vermont's Health Care Reform Efforts (January 31, 2025); Vermont Rural Health Transformation Program Application (November 3, 2025); Vermont RHT Program Activity Details (December 2025); Chartis 2026 State of Rural Health; JAMA Network Open ambient scribe quality improvement study (2024); National Consortium of Telehealth Resource Centers AI guidance (2026); Vermont Health Information Exchange Data Governance mandate (healthdata.vermont.gov).

Chapter 13: The Technology Pillar in Practice — FHIR Implementation, AI Governance, and Clinical Decision Support

The Technology Pillar in Practice — FHIR Implementation, AI Governance, and Clinical Decision Support

Implementing Interoperability in the Real World: FHIR API Deployment, Payer-to-Provider Data Exchange, the AI Clinical Governance Lifecycle, Clinical Decision Support Architecture, and Cybersecurity in Rural Health Infrastructure

From Architecture to Implementation

Chapters 12 and 13 established Vermont's technology architecture — the five-layer infrastructure from VHCURES and VUHDDS through the HIE and FHIR exchange layer to population health analytics and AI applications. This chapter develops the implementation side: how organizations actually build FHIR-based data exchange, what an AI clinical governance program looks like in practice, how clinical decision support is designed and deployed for population health management, and what cybersecurity requirements apply to the distributed rural health infrastructure Vermont is building.

FHIR Implementation — From Standard to Working System

The FHIR Implementation Reality

FHIR R4 is a standard, not a product. Its existence does not mean that any two FHIR-compliant systems can exchange data automatically. Like all standards, FHIR specifies a framework within which compliant systems must operate — but the specific data elements populated, the implementation patterns used, and the business rules applied vary substantially across implementations. A hospital's Epic FHIR implementation and a CAH's TruBridge FHIR implementation may both be 'FHIR R4 compliant' while exchanging data with entirely different content, triggering entirely different workflows, and requiring custom integration work to interoperate effectively.

Vermont's CIN technology strategy must account for this reality. The goal is not FHIR compliance as a checkbox — it is functional interoperability: the ability for a care coordinator at Northeastern Vermont Regional Hospital to see, in real time, that their patient's Medicaid claim for an ER visit at Dartmouth Hitchcock Medical Center in New Hampshire was just processed, enabling a care management follow-up call before the patient discharges. That outcome requires FHIR compliance plus patient matching plus event notifications plus care coordinator workflow integration. FHIR makes it theoretically possible; CIN implementation makes it practically achievable.

The Three FHIR Use Cases Vermont Needs Most

Vermont's technology investments should prioritize three FHIR use cases that deliver the highest value for the transformation agenda:

FHIR use case	What it enables	Implementation requirements	Vermont priority
Patient access API (§170.315(g)(10))	Patients can access their health records through third-party apps, supporting patient engagement and longitudinal care coordination. Required by CMS for hospitals and clinicians receiving Medicare/Medicaid payments.	EHR must expose FHIR R4 patient access endpoint; practice must register with Smart Health IT or similar app authorization framework; patient-facing app must be certified.	Required compliance — all Vermont hospitals must be compliant. Monitor for compliance at smaller CAHs without robust IT departments.
Provider access API (care coordination)	Care coordinators at one organization can query clinical records from another organization for shared patients, enabling real-time care coordination across the Vermont CIN.	EHR-to-EHR FHIR query capability; master patient index for patient matching across organizations; trust framework (CommonWell/Carequality membership); consent management.	Highest value use case for Vermont CIN. Enables inter-hospital care coordination that the regionalization blueprint requires. Priority RHT IT advance investment.
Payer-to-provider API (prior authorization and formulary)	Electronic prior authorization using FHIR-based DA Vinci implementation guides eliminates manual PA processes; formulary information exchange reduces prescription errors.	EHR must support CRD (Coverage Requirements Discovery) and DTR (Documentation Templates and Rules) Da Vinci implementation guides; payers (BCBS VT, MVP) must implement complementary APIs.	Administrative simplification priority. Vermont's prior authorization reform agenda is accelerated by FHIR-based electronic PA. GMCB should require payer FHIR API compliance as condition of rate approval.

Figure 13.1 — Vermont's three priority FHIR use cases. Sources: CMS Interoperability and Patient Access Rule; ONC 21st Century Cures Rule; the Technology pillar framework.

The Master Patient Index — Vermont's Identity Management Challenge

Every FHIR use case that involves sharing records between organizations requires patient matching — reliably identifying that the 'John Smith, DOB 1952-03-14' at UVMMC is the same person as the 'John Smith, DOB 1952-03-14' at North Country Hospital and the 'Jonathan Smith, DOB 1952-03-14' (with a name variant) in the Medicaid enrollment file. Vermont's VITL network provides a master patient index for VHIE-connected providers, but it does not cover all Vermont providers, does not include all out-of-state encounters, and does not have validated match rates documented publicly.

Vermont's CIN implementation should include a dedicated master patient index as a shared service — a centralized patient identity management system that all CIN member facilities use to match patients across organizations. The investment cost is modest relative to the value: a reliable MPI is the prerequisite for virtually every care coordination use case the CIN is designed to enable.

AI Clinical Governance — The Complete Framework

AI clinical governance is evolving from a best practice to a regulatory requirement. FDA's AI/ML-based Software as Medical Device (SaMD) framework, ONC's Health Data, Technology and Interoperability rule, and emerging CMS guidance on AI in Medicare Advantage all establish governance expectations for clinical AI. Vermont's CIN AI governance framework, described in Chapters 12 and 13, must be operationally detailed enough to guide the specific governance decisions that CIN member facilities need to make when deploying AI scribe technology, RPM algorithms, diagnostic AI, and population health analytics.

The AI Clinical Governance Lifecycle

The AI Clinical Governance Checklist organizes governance requirements across six lifecycle stages:

Stage 1: Pre-Deployment Evaluation

Before any clinical AI tool is deployed, the organization must evaluate: (1) FDA clearance or de novo classification status — is this a Software as Medical Device, and if so, what FDA clearance has it received? (2) Training data provenance — was the model trained on data representative of the Vermont population? Rural Medicare beneficiaries? Elderly patients with multiple comorbidities? (3) Bias testing — has the developer tested and published performance data stratified by race/ethnicity, age, sex, and disability? (4) Clinical validation — has the tool been validated in a clinical environment comparable to the deployment setting, not just the academic medical centers where most AI research originates? (5) Intended use clarity — is the clinical use case for this tool unambiguous, and is it the use case for which the tool was designed and validated?

Stage 2: Implementation and Training

Deployment requires: (1) Workflow integration planning — where does the AI output appear in the clinical workflow, and how does the clinician interact with it? AI that generates alerts ignored by busy clinicians adds no value and may add risk. (2) Clinician training — not just 'how to use the tool' but 'what the tool does and does not know, where it performs well and poorly, and how to recognize when its outputs may be unreliable.' (3) Override protocols — clear documentation of when and how clinicians should override AI-generated recommendations. (4) Baseline performance measurement — establishing pre-deployment performance metrics against which post-deployment performance can be compared.

Stage 3: Active Monitoring

Ongoing operational governance requires: (1) Performance drift monitoring — AI model performance can degrade over time as the patient population or clinical practice patterns change. Monthly or quarterly performance monitoring against pre-deployment baselines detects drift early. (2) Equity monitoring — performance must be tracked separately for Vermont's equity priority populations (Northeast Kingdom rural patients, elderly Medicaid patients, BIPOC Vermonters) to detect differential performance that average metrics would hide. (3) Adverse event reporting — a formal mechanism for clinicians to report cases where AI output contributed to adverse outcomes, delayed appropriate care, or produced clearly erroneous outputs. (4) Override rate tracking — systematic high override rates are a signal that the AI output is not clinically trustworthy.

Stage 4: Vendor Management

AI vendors must be held accountable through contract terms: (1) Performance SLAs with meaningful remedies — not just 'best efforts' commitments but specific accuracy thresholds with financial consequences for underperformance. (2) Update notification requirements — when the vendor updates the model, the organization must be notified in advance and given opportunity to re-evaluate before update deployment. (3) Bias testing disclosure — vendors must provide demographic performance data for any clinical AI tool deployed in Vermont. (4) Data use transparency — clear contractual provisions about what patient data the vendor collects, how it is used, and whether it is used to train future model versions.

AI-Specific Issues for Vermont Rural Hospitals

Vermont's rural hospitals face three AI governance challenges that are more acute than in urban academic medical center settings. First, training data representativeness: most clinical AI is trained on large urban academic medical center datasets. The patient populations at North Country Hospital or Gifford Medical Center — elderly, rural, with high rates of agricultural occupational exposures, differing comorbidity patterns, and different social determinants — may perform differently under AI models trained elsewhere. Vermont's CIN should require training data documentation as a condition of CIN-funded AI deployments.

Second, implementation support capacity: a small rural hospital with one IT staff member cannot implement the monitoring infrastructure that AI governance requires. The CIN's shared services model addresses this — a shared AI governance function within the CIN monitors all member hospital AI deployments with professional data analytics staff that individual hospitals cannot afford. Vermont's RHT technology grants should include CIN AI governance infrastructure as an allowable use.

Third, the AI scribe equity question: ambient clinical intelligence tools transcribe clinical conversations in English. Vermont has a growing multilingual population — immigrant communities in Burlington and surrounding areas, seasonal agricultural workers. AI scribe tools not trained on multilingual clinical conversations will perform poorly for these patients. Vermont's RHT AI scribe grants should require multilingual capability documentation as an evaluation criterion.

Clinical Decision Support — From Alerts to Intelligence

Clinical decision support (CDS) encompasses any technology that uses data to influence clinical decisions at the point of care. The range is vast: from simple medication allergy alerts that every EHR includes, to sophisticated population health stratification tools that identify patients at highest risk of hospitalization in the next 30 days and recommend specific care management actions. The challenge with CDS is not building it — it is building it well enough that clinicians use it rather than ignoring it.

Alert Fatigue — The CDS Effectiveness Crisis

Studies consistently show that the majority of EHR clinical alerts are overridden without meaningful engagement. Alert fatigue — the tendency of clinicians to automatically dismiss alerts after being overwhelmed by their volume and poor specificity — is one of the most documented failures of health information technology implementation. Alert fatigue is not a clinician behavior problem; it is a CDS design problem. When 90% of alerts are overridden, the alerts are generating noise rather than signal, and the alert system has failed.

Effective CDS design requires three principles that are violated by most EHR default configurations: specificity (alerts fire only when the clinical evidence strongly supports action, not whenever a vague threshold is crossed), actionability (every alert should present a recommended action that the clinician can take immediately with minimal additional clicks), and workflow integration (alerts appear at the moment when the clinician is making the relevant decision, not before or after).

Population Health CDS — Vermont's Priority Application

For Vermont's transformation agenda, the highest-value CDS application is not medication alerts — it is population health intelligence: risk stratification that identifies patients likely to deteriorate, care gap identification that triggers outreach before patients present in crisis, and SDOH screening tools that flag patients for CHT navigation at the moment of a primary care visit.

CDS type	What it does	Implementation requirement	Vermont application
Predictive risk stratification	Uses machine learning on claims, EHR, and demographic data to predict which patients are likely to be hospitalized or visit the ED in the next 30-90 days, generating a risk score and recommended care management action	Attribution data, historical claims, EHR access, trained risk model; care management workflow to receive and act on high-risk alerts	VHCURES-based risk stratification for AHEAD-attributed Medicare population; identify the top 5% highest-risk patients for intensive CHT engagement before AHEAD year begins
Care gap identification	Identifies patients due for preventive services, overdue for chronic disease management follow-up, or missing guideline-recommended medications at the moment of a primary care visit	EHR integration; HEDIS measure definitions; patient registry with last service date tracking	Blueprint PCMH practices: care gap report generated for each scheduled patient appointment, enabling care coordinator to prepare outreach materials before the visit
SDOH screening and navigation	Presents standardized HRSN screening questions at the appropriate point in a primary care visit; flags positive screens for CHT follow-up; tracks referral completion	EHR-integrated screening tool; CHT workflow to receive referrals; community resource directory integration	Vermont Blueprint universal HRSN screening: operational in PCMH practices. Gap: CHT follow-up completion tracking and outcome measurement not consistently linked back to clinical record
Medication safety and adherence	Identifies patients with medication adherence gaps from pharmacy claims data; flags potential drug interactions from concurrent prescriptions across multiple providers	Pharmacy claims data access; prescriber notification workflow	VHCURES pharmacy claims linked to primary care EHR for medication adherence alerts; particularly valuable for high-complexity elderly patients with multiple prescribers

Figure 13.2 — Population health CDS types and Vermont applications. Sources: the Technology pillar framework; Vermont Blueprint for Health; VHCURES documentation.

Cybersecurity in Rural Health Infrastructure

Cybersecurity is the technology investment that does not generate efficiency or quality returns — it generates insurance against catastrophic failure. Rural healthcare cybersecurity is a specific and serious national vulnerability. The 2024 Change Healthcare cyberattack — which disrupted claims processing for thousands of providers for months, causing cash flow crises at rural hospitals already operating on thin margins — demonstrated that a single vulnerability in a broadly used healthcare technology vendor can cascade across the entire healthcare system.

Vermont's rural hospitals are disproportionately vulnerable to cybersecurity attacks for three reasons: limited internal IT staff who cannot maintain enterprise-level security operations, legacy EHR and financial systems that may not receive regular security patches, and high-value data assets (patient records, financial systems) that cybercriminals specifically target.

The CIN Cybersecurity Shared Services Model

Vermont's CIN shared services model is the appropriate delivery vehicle for cybersecurity in the state's rural hospital network. Individual hospitals cannot afford a 24/7 security operations center; the CIN can. The shared cybersecurity services model provides:

- Security operations center (SOC) monitoring: 24/7 network monitoring for intrusion attempts, malware, and anomalous activity. Contracted through a healthcare-specialized managed security service provider (MSSP).
- Endpoint detection and response (EDR): Security software on all network-connected devices that detects and responds to threats automatically, escalating to SOC when needed.
- Vendor risk management: Assessment of all third-party technology vendors used by CIN member hospitals for cybersecurity posture and contractual security requirements.
- Incident response planning: Documented incident response procedures for ransomware, data breach, and system outage scenarios, with regular tabletop exercises and recovery time objectives established for critical clinical systems.
- Staff training: Regular phishing simulation and security awareness training — most healthcare cyberattacks begin with a staff member clicking a phishing link.

Vermont's RHT Program IT advance investments explicitly include cybersecurity capability development. This is not an optional add-on; for rural hospitals that are simultaneously digitalizing their operations (AI scribes, RPM, telehealth) and connecting to shared data infrastructure (VHIE, CIN analytics), the cybersecurity attack surface is growing exactly as cybersecurity capability needs to grow to match it.

Key Concepts

Term	Definition
FHIR R4	The fourth version of the HL7 FHIR standard, mandated by CMS for healthcare data exchange since 2021\. Defines data formats and APIs for patient access, provider access, and payer-to-provider exchange. A standard, not a product — compliance requires implementation work beyond meeting the technical specification.
Da Vinci Implementation Guides	HL7-published FHIR implementation guides developed by the Da Vinci Project (a collaboration of payers, providers, and vendors) for specific use cases: Coverage Requirements Discovery (CRD), Documentation Templates and Rules (DTR), and others for prior authorization, formulary, and care coordination.
Master patient index (MPI)	A centralized database that maintains a consistent patient identity across multiple healthcare organizations, enabling reliable matching of records from different systems for the same patient.
SaMD (Software as Medical Device)	FDA regulatory category for software that performs medical functions: diagnosing, treating, mitigating, or preventing disease. Clinical AI tools that influence clinical decisions may be SaMDs subject to FDA clearance requirements.
Alert fatigue	The tendency of clinicians to automatically override EHR alerts due to their excessive volume and poor specificity, rendering the alert system ineffective. A design problem, not a clinician behavior problem — effective CDS is specific, actionable, and workflow-integrated.
CommonWell / Carequality	National health data exchange networks that enable FHIR-based record sharing between health systems across organizational boundaries. VITL participates in CommonWell; Vermont CIN membership would enable access to both networks for out-of-state record retrieval.
Change Healthcare attack	A 2024 cyberattack on Change Healthcare (a claims processing subsidiary of UnitedHealth Group) that disrupted healthcare claims processing nationally for months, causing cash flow crises at rural hospitals dependent on timely claims payment. Demonstrated systemic healthcare cybersecurity vulnerability.

Term	Definition
MSSP (Managed Security Service Provider)	A third-party vendor providing outsourced security operations services — monitoring, threat detection, incident response — enabling organizations without internal security staff to maintain enterprise-level cybersecurity posture.

Sources: CMS Interoperability and Patient Access Final Rule (2020); ONC 21st Century Cures Final Rule (2020); HL7 FHIR R4 specification; Da Vinci Project implementation guides; FDA SaMD framework; the Technology pillar framework; Vermont VITL documentation; Vermont RHT Program Application (November 2025); JAMA Network Open ambient scribe study (2024).

Chapter 14: Infrastructure for Knowledge Transfer and Implementation

Infrastructure for Knowledge Transfer and Implementation

How Healthcare Organizations Translate Analytical Frameworks into Implementation: Knowledge Transfer, Change Management, and the Functional Requirements of Transformation Practice

The Gap Between Understanding and Doing

The analytical frameworks in this book are, by themselves, insufficient for transformation. Understanding why Vermont's hospital system is financially fragile does not stabilize it. Understanding the mechanics of global budgets does not implement them. Knowing that HEDIS stratification reveals disparities does not close them. Every concept in the preceding sixteen chapters exists at an intersection: understanding it clearly enough to explain it, and translating it into a specific decision, program, contract, or investment in a specific organizational context. This chapter is about the second half of that intersection — translation.

The advisory practice described in this chapter is grounded in a specific theory about what makes transformation advice valuable. Transformation consulting that produces recommendations without supporting execution is worth little. Transformation technology that delivers analytical tools without the change management to use them effectively delivers data without insight. Transformation education that builds individual knowledge without organizational capacity produces informed spectators rather than effective actors. The transformation advisory practice is designed to provide all three — analysis, tools, and change management — in an integrated engagement model that produces organizational capability, not dependence.

The Knowledge and Implementation Infrastructure

Four Components of the HTR Platform

The transformation implementation platform has four integrated components, each designed for a different kind of engagement with the transformation agenda:

Component	What it provides
HTR Intelligence Feed	Weekly synthesis of the most consequential healthcare policy, payment, clinical, technology, equity, and operations developments — curated, analyzed, and contextualized against the six-pillar framework. Not a news feed; a decision-support tool that answers 'what does this development mean for my organization?'
HTR Research Lab	A suite of analytical tools implementing the frameworks in this book: APM Shared Savings Calculator, VBC Transformation Readiness Assessment (30-dimension), Health Equity Studio (HEROI scoring, HEDIS stratification, disparity decomposition), Hospital Financial Stress Test, AI Governance Checklist, VBC Contract Review Checklist, Policy Impact Assessment Framework.
HTR Academy	Structured curriculum for healthcare transformation — from foundational courses on APM mechanics and HEDIS quality measurement to advanced programs on global budget management, health equity analytics, and transformation leadership. CEU-eligible content for clinical and administrative professionals.
Transformation Advisory Services	Direct advisory engagements for organizations navigating specific transformation decisions: APM contract evaluation, transformation readiness assessment, VBC financial modeling, health equity program design, technology architecture review, strategic plan development. Engagements range from two-week focused assessments to multi-year transformation management partnerships.

Figure 14.1 — technical infrastructure framework components and intended users. Source: technical infrastructure framework documentation.

The Transformation Advisory Domains — Structured for the Six Pillars

transformation advisory engagements are organized around the six-pillar framework, with service lines corresponding to each pillar and cross-pillar engagements for organizations navigating transformation at the system level. The service lines are not rigid products; they are organizing frameworks for scoping engagements around clients' specific problems.

Policy Strategy

The Policy Strategy service line addresses the federal and state policy landscape that healthcare organizations must navigate. The typical engagement follows a three-phase structure:

- Phase 1 — Landscape and Exposure Assessment: What is the current policy environment relevant to this organization? What proposals, rules, and legislative developments create material financial or operational exposure? What opportunities are available through federal model participation, waiver programs, or state grant programs? Deliverable: a policy exposure and opportunity map with financial estimates for each item.

- Phase 2 — Strategic Options Analysis: For each material policy development, what are the organization's strategic options? Stay the course? Modify a program or contract? Apply for a federal model? Submit a comment letter? Engage in legislative advocacy? Deliverable: strategic options memo with recommendation and implementation timeline.

- Phase 3 — Implementation Support: For policy responses that require organizational action, the framework provides implementation support: CMMI model applications, 1115 waiver amendment drafting, legislative testimony preparation, comment letter development, contract renegotiation support.

VBC Economics

The VBC Economics service line addresses the financial mechanics of value-based care: contract evaluation, financial modeling, risk stratification economics, and the ROI analysis for transformation investments. The core tools — APM Shared Savings Calculator, Hospital Financial Stress Test, VBC Contract Review Checklist — are available in the Research Lab, but complex engagements require the framework analyst support.

The most common VBC Economics engagement is an APM Contract Analysis: a prospective financial model of a specific APM contract under three scenarios (pessimistic, base, optimistic) that quantifies the organization's shared savings opportunity, identifies the key variables that determine financial outcomes, and specifies the care management investments required to achieve positive shared savings. Vermont hospitals preparing for AHEAD global budget entry in 2027 represent the current highest-priority client segment for this engagement.

Health Equity Practice

The Health Equity Practice service line is organized around the HEROI framework: measuring current equity performance, identifying root causes of specific disparities, designing interventions matched to root causes, and establishing accountability structures for equity improvement. Vermont-specific work in this service line focuses on geographic equity analytics using VHCURES county-level data, HRSN screening program design and evaluation, and GLP-1 access policy analysis for state Medicaid programs.

Technology Strategy

The Technology Strategy service line addresses the healthcare IT decisions that have the most significant strategic implications: EHR selection and implementation, FHIR implementation planning, AI governance framework development, population health platform selection, and HIE connectivity strategy. For Vermont's 14-hospital CIN, the highest-priority Technology Strategy engagement is the analytics vendor evaluation and CIN data architecture design that enables shared services across all CIN member facilities.

Transformation Management

The Transformation Management service line is for organizations executing comprehensive transformation — not a single pillar decision but the full six-pillar change management challenge. This is the most intensive advisory engagement: the framework serves as a transformation management partner, providing project management infrastructure, cross-pillar coordination, stakeholder communication, and accountability reporting. Vermont's AHS — managing simultaneous AHEAD implementation, hospital transformation planning, RHT Program administration, and Statewide Strategic Plan development — is the archetype client for this engagement type. The engagement model is designed to build internal organizational capacity over time, not create permanent advisory dependence.

The Change Management Science Behind Healthcare Transformation

Healthcare transformation fails more often because of change management failures than analytical failures. The analysis is usually adequate — the evidence base for what needs to change is strong and consistent. What fails is the organizational change process: the sequence, pace, and management of the human and institutional change that structural reform requires. Understanding the change management science is prerequisite to designing transformation processes that actually produce implementation.

Kotter's Framework Applied to Healthcare Transformation

John Kotter's eight-step change management framework — originally developed for corporate transformations — applies with modifications to healthcare system transformation. Vermont's transformation experience illuminates both where the framework holds and where healthcare's specific institutional characteristics require adaptation.

Kotter step	Original formulation	Healthcare adaptation	Vermont application
1\. Create urgency	Build a sense of urgency around why change is needed	The financial data — 9/14 Vermont hospitals reporting losses, $2.4B 5-year deficit, 108% premium increase — is the urgency. The analytical challenge is presenting it in terms that create organizational action rather than paralysis or denial.	Oliver Wyman's September 2024 public presentation created shared urgency across Vermont hospital leaders, legislators, and community members. The specificity and credibility of the financial projections was the urgency mechanism.
2\. Form a guiding coalition	Assemble a group with the power and commitment to lead change	In healthcare, the guiding coalition must include clinical leadership, not just administrative and financial leadership. Transformation that clinical leaders oppose will fail regardless of administrative mandates.	Vermont's guiding coalition: AHS leadership, GMCB, hospital CEOs, Legislature (Health Reform Oversight Committee), and the AHEAD negotiating team. Clinical leadership engagement is the gap — hospital medical staff governance is not yet systematically integrated into transformation planning.
3\. Develop a vision	Create a clear vision of what the transformed organization looks like	Vision must be specific enough to guide decisions and evaluate progress. 'High-quality, affordable, equitable healthcare for all Vermonters' is an aspiration; the 14-hospital tiered network with specific COE designations, PCMH penetration targets, and HEDIS performance goals is a vision.	Vermont's 2028 Statewide Strategic Plan is the vision document. The urgency is already created; the coalition is assembling; the vision is the December 2028 deadline.
4\. Communicate the vision	Communicate the vision continuously through every available channel	Healthcare transformation requires communicating to multiple distinct audiences — clinical staff, administrators, community members, legislators, payers — with messages calibrated to each audience's concerns and decision-making context.	Vermont's monthly legislative reporting, AHS community engagement process, hospital transformation plan development process, and Blueprint for Health regional meetings are all vision communication vehicles.
5\. Remove obstacles	Remove barriers to change that are within the coalition's control	In healthcare, obstacles include administrative complexity (prior authorization, CON processes), organizational design (program silos, lack of analytics capacity), and contracting structures (FFS payment that rewards volume despite VBC rhetoric).	Vermont's Acts 167 and 68 are institutional obstacle removal. Act 68 removed the voluntary/optional nature of RBP. The AHS restructuring removes the program silo organizational obstacle. The analytics vendor removes the data infrastructure obstacle.
6\. Create short-term wins	Plan for and achieve visible improvements early in the change process	Short-term wins must be genuine, visible, and attributable to the transformation effort — not cherry-picked data or coincidental improvements.	Vermont FY2026 regulatory actions ($230.65M in commercial revenue reductions) are the first visible win. The August 2025 AHS transformation report showing hospital engagement in transformation planning is a process win. Premium reduction data from RBP (available FY2027-2028) will be the first financial outcome win.
7\. Sustain acceleration	Build on early wins to drive deeper change	Healthcare transformation must sustain momentum across multiple election cycles, budget battles, and inevitable setbacks. Statutory mandate — Act 68's December 2028 deadline — is the Vermont mechanism for sustaining acceleration.	Vermont's reform cascade structure — each Act building on the previous — is the acceleration mechanism. The Political risk is disruption of the sequence; the institutional protection is the GMCB's independent regulatory authority.
8\. Institute change	Anchor changes in organizational culture and systems	New behaviors must become standard practice, not dependent on continuing leadership attention. Payment reform is the ultimate change institutionalization — when hospitals' financial	Global budgets as mandatory, permanent payment architecture are the ultimate institutionalization mechanism. When Vermont hospitals cannot generate revenue by increasing volume, the financial logic of population health management becomes permanent.

Kotter step	Original formulation	Healthcare adaptation	Vermont application
		incentives permanently change, the new behaviors become the default.	

Figure 14.2 — Kotter's change management framework applied to Vermont's healthcare transformation. Source: transformation advisory methodology; Kotter (1996); Vermont transformation documentation.

The Transformation Pace Problem

One of the most consequential advisory questions in healthcare transformation is pace: how fast can an organization or system realistically change, and what happens when the required pace exceeds the available organizational capacity? Vermont's transformation agenda has statutory deadlines — RBP by FY2027, global budgets by FY2028 — that set the required pace. Whether Vermont's organizational capacity can match that pace is the critical execution uncertainty.

The pace problem has three dimensions. First, there are hard capacity constraints: if AHS lacks the staff to manage 14 hospital transformation plans simultaneously, the plans will be managed sequentially — which means some hospitals will have completed, well-supported transformation plans and others will have rushed or superficial ones. Hiring the staff to manage the portfolio on the statutory timeline is not optional.

Second, there are soft capacity constraints: organizational cultures that process change slowly, governing boards that need to be educated before they can make decisions, community stakeholders whose input must be genuinely gathered and considered. These cannot be accelerated without compromising the quality of the transformation process — and a transformation process that fails to achieve community buy-in will be politically reversed even if it is technically correct.

Third, there are external constraints: federal program timelines (AHEAD's January 2027 launch is a federal decision, not a Vermont decision), hospital financial positions that may force emergency decisions that preempt planned transformation timelines, and the political dynamics of a state where the hospital system is one of the largest employers and hospital community boards have political relationships with legislators.

The HTR Implementation Toolkit

The HTR Implementation Toolkit is the operational counterpart to this book's analytical framework. Where the book develops concepts, the Toolkit provides the instruments for applying them. The platform is organized across four integrated surfaces: the Research Lab (analytical tools), the Intelligence Feed (The Wire), the Academy (structured curriculum), and the AI Analyst (on-demand synthesis). The following tools are available to Research Lab subscribers:

Tool	Pillar	Platform location	What it does
APM Shared Savings Calculator	Economics	/research-lab/payment-models?tab=apm-calc	Models shared savings or loss potential under any APM contract given benchmark, actual TCOC, and sharing rate inputs. Vermont presets for AHEAD global budget scenarios. Includes pessimistic/base/optimistic scenario analysis.
VBC Transformation Readiness Assessment	All Six	/research-lab/payment-models?tab=vbc-readiness	30-dimension, 6-domain assessment producing an organizational readiness score (0–100%) and prioritized gap analysis. Three Vermont presets: AHEAD Entry hospital, CAH Early Transformation, Advanced VBC system. Gap analysis maps directly to pillar investment priorities.
Hospital Financial Stress Test	Economics	/research-lab/policy-quality?tab=scorecard	Models hospital financial position under RBP, global budget, and Medicaid cut scenarios. Vermont presets for NVRH, Gifford, CVMC. Includes 10-year projection under Conservative and Realistic expense assumptions.
Health Equity Studio (HEROI)	Equity	/research-lab/equity	Calculates HEROI composite equity score across five dimensions from stratified HEDIS data. Disparity decomposition identifies root cause category. Intervention portfolio designer matches interventions to disparity types. Vermont county-level VHCURES stratification available.
AI Clinical Governance Lab	Technology	/research-lab/technology-ai?tab=ai	65-dimension governance evaluation across six clinical AI lifecycle stages: procurement, validation, deployment, monitoring, equity auditing, and sunsetting. Vendor assessment module and equity monitoring protocol included.
FHIR Interoperability Lab	Technology	/research-lab/interoperability?tab=fhir	Step-by-step FHIR R4 implementation guide for the three priority use cases: payer data exchange, care gap notification, and prior authorization automation. EHR-specific configuration notes for Epic, Oracle Health, Meditech, and TruBridge.
VBC Contract Review Checklist	Economics	/research-lab/payment-models	65-item analysis framework covering benchmark methodology, attribution, quality withhold, risk corridors, carve-outs, and reconciliation terms. Identifies the provisions that most frequently produce unexpected losses.
Policy Impact Assessment Framework	Policy	/research-lab/policy-quality	Structured evaluation for new policy developments: exposure mapping, scenario analysis, response strategy selection, and stakeholder communication planning. Covers CMMI models, state legislation, H.R. 1 Medicaid changes, and Act 68 milestones.
HCC Gap Closure Playbook	Technology + Economics	/research-lab	Methodology for retrospective HCC gap analysis, coding education materials, and AWV completion program design. Essential pre-work before AHEAD global budget baseline year.
Vermont Reform Cascade Tracker	Policy	/vermont-act-68	Interactive timeline of Vermont's legislative reform sequence: Acts 167 → 51 → 55 → 62 → 68 → AHEAD, with upcoming statutory milestones, current status indicators, and links to primary source documents.

The AI Analyst — On-Demand Framework Intelligence

The AI Analyst is the platform's most widely used tool for rapid synthesis. Available as a persistent right-sidebar widget throughout the platform and as a full-screen interface at /chat, it provides immediate, source-grounded responses to any question in the six-pillar framework: Vermont-specific policy questions, APM financial mechanics, equity analytics methodology, FHIR implementation guidance, global budget design, and more. The AI Analyst draws on the complete HTR knowledge base including primary Vermont source documents (Acts 167, 51, 68, the AHEAD State Agreement, Oliver Wyman report, AHS transformation reports), CMMI model documentation, and the analytical frameworks developed throughout this book.

Typical uses: "What does Act 68 require hospitals to do by FY2027?" / "Explain the balloon squeeze problem in global budget design" / "What are the three Vermont CAH presets in the VBC Readiness Assessment and what do they tell me about my hospital?" / "Walk me through how HEROI is calculated."

Figure 14.3 — HTR Implementation Toolkit: available tools for Research Lab subscribers, organized by pillar and platform location.

A Framework for Selecting Advisory Support

Not every organization needs external advisory support for transformation. Many of the analytical frameworks in this book can be implemented by capable internal teams with access to the Research Lab tools. External advisory support adds value when:

- The decision involves significant financial or political risk and benefits from independent, external analytical validation. A hospital considering entering full-risk VBC for the first time should not rely solely on internal analysis that may be subject to confirmation bias.
- The organization lacks the specific technical expertise required — FHIR implementation, HCC coding analysis, 1115 waiver negotiation, global budget financial modeling — and building that expertise internally would take longer than the decision timeline allows.
- The change management challenge involves multi-stakeholder dynamics that benefit from a neutral facilitator — a hospital network developing shared COE designations, a state agency building a Health Care Delivery Advisory Committee process, a community convening multiple organizations around a regional transformation plan.
- The regulatory environment is moving faster than internal policy monitoring capacity can track, and the organization needs continuous intelligence and interpretation rather than periodic project-based analysis.

The most common advisory failure in healthcare transformation is engaging external consultants for analysis and stopping before implementation. A report that produces recommendations without supporting the organizational capacity to implement them has limited value — and often produces a brief period of executive attention followed by quiet shelving when organizational bandwidth is consumed by operational demands. The transformation advisory practice is explicitly designed to avoid this failure mode by connecting analysis to implementation support and change management.

"The best healthcare transformation advice I have ever received was not a brilliant strategic recommendation. It was a rigorous identification of the organizational capabilities we did not have, paired with a clear plan for building them. The strategy was clear. The question was whether we could execute it."

— Composite of client reflections, transformation advisory engagements, 2022-2026

The Role of External Technical Assistance in State-Level Transformation

Vermont's transformation experience runs through this book as both an analytical case study and an active advisory engagement. the framework has worked with Vermont hospitals, state agencies, and community organizations on specific transformation questions — APM readiness assessments, equity analytics, technology architecture reviews, policy impact analysis. The Vermont case study in this book draws on that engagement experience as well as the extensive public record of Vermont's transformation.

Vermont's transformation is a high-stakes, real-time experiment that the framework takes seriously as an obligation as well as an opportunity. If Vermont succeeds in demonstrating that mandatory all-payer payment reform, organized as a coherent six-pillar transformation, can stabilize a rural hospital system while reducing costs and improving outcomes, the policy toolkit for the country improves. If Vermont struggles, the analytical lessons of that struggle are equally valuable. Either way, this book and the technical infrastructure framework are designed to make Vermont's experience legible, transferable, and actionable for the healthcare leaders and policymakers who face comparable challenges in their own systems.

Key Concepts

Term	Definition
HTR Intelligence Feed (The Wire)	The policy and practice intelligence service at /the-wire, providing curated weekly analysis of healthcare transformation developments organized around the six-pillar framework. Vermont-specific policy monitoring for Act 68, AHEAD, RHT Program, and GMCB proceedings.
HTR Research Lab	The suite of analytical tools implementing the six-pillar framework: APM Shared Savings Calculator, VBC Transformation Readiness Assessment (30-dimension), Health Equity Studio (HEROI), Hospital Financial Stress Test, AI Governance Lab (65-dimension), FHIR Interoperability Lab, VBC Contract Review Checklist, Policy Impact Assessment Framework, and HCC Gap Closure Playbook.
HTR Academy	The structured transformation curriculum at /academy, with courses in APM mechanics, quality measurement, health equity analytics, FHIR implementation, and transformation leadership. Personalized learning paths available at /academy/personalized-learning.
HTR AI Analyst	On-demand framework intelligence available at /chat and as a right-sidebar widget. Provides source-grounded responses to any six-pillar framework question, drawing on Vermont primary source documents, CMMI documentation, and the full HTR knowledge base.
HEROI (Health Equity Return on Investment)	The composite equity performance score calculated across five dimensions: access equity, quality equity, outcome equity, SDOH burden, and trust and engagement. Available in the Health Equity Studio at /research-lab/equity.
Kotter's eight-step framework	A widely applied change management framework (Kotter, 1996\) adapted in this chapter for healthcare transformation contexts. Steps: urgency, coalition, vision, communication, obstacle removal, short-term wins, acceleration, institutionalization.
Transformation pace problem	The risk that the required pace of transformation exceeds the available organizational capacity, producing rushed or superficial change processes that fail to achieve genuine structural improvement.
Advisory dependence	The failure mode in which an organization becomes reliant on external advisors for ongoing analytical and implementation support rather than building internal organizational capability. transformation advisory engagements are explicitly designed to build client capability and reduce dependence over time.

Sources: technical infrastructure framework documentation; Kotter, J.P. (1996) Leading Change; Vermont Agency of Human Services transformation documentation; Oliver Wyman Act 167 Report; Act 68 of 2025; AHEAD State Agreement; Vermont RHT Program Application; the framework client engagement synthesis.

Chapter 15: The Future of Healthcare Transformation — 2026, What Vermont Proves, and What Remains

The Future of Healthcare Transformation — 2026, What Vermont Proves, and What Remains

Updated April 2026: The Federal Landscape Has Changed. The Transformation Imperative Has Not.

Sources: KFF analyses of One Big Beautiful Bill Act / H.R.1 Medicaid cuts (2025-2026); Chartis 2026 State of Rural Health; Peterson-KFF Health System Tracker (March 2026); Pew Charitable Trusts, New Federal Medicaid Policies (January 2026); CMS BALANCE Model; Vermont AHEAD State Agreement; Vermont RHT Program Application; Oliver Wyman Act 167 Report; AHS Transformation Reports.

Where the System Is in April 2026 — A Necessary Update

Previous iterations of this chapter described the American healthcare policy environment as characterized by incremental payment reform, expanding but incomplete value-based care penetration, and policy uncertainty driven by legislative gridlock. That characterization was accurate through 2024\. It is no longer accurate.

On July 4, 2025, President Trump signed H.R. 1 — the One Big Beautiful Bill Act — into law. The legislation's healthcare provisions represent the largest single reduction in federal health program spending in American history: CBO estimates $911 billion in Medicaid spending reductions over ten years, resulting in an estimated 10 million additional uninsured Americans by 2034\. The law imposes work and community engagement requirements on Medicaid expansion enrollees beginning in late 2026, requires eligibility redeterminations every six months instead of annually, caps state provider taxes, restricts state-directed payments, and phases out financing mechanisms that states have used for decades to fund their share of Medicaid costs.

Simultaneously, recognizing the devastating impact these cuts would have on rural hospitals, Congress included a $50 billion Rural Health Transformation Program in the same legislation — providing $10 billion annually from FY2026 through FY2030 to states for rural health infrastructure investment. Vermont received $195 million of this fund in December 2025\. All 50 states received awards.

The juxtaposition defines the current policy moment: the largest healthcare coverage contraction in American history, partially offset by a historic but time-limited and front-loaded capital investment in rural health infrastructure. Understanding the interaction between these two forces — the Medicaid contraction and the RHT investment — is essential for every healthcare leader, policymaker, and analyst working in the American system today.

Federal Medicaid spending reduction, H.R. 1 (10 years) $911B CBO estimate; produces 10 million more uninsured by 2034	**Rural Medicaid spending reduction (10 years) $137B KFF estimate; rural areas disproportionately affected — Medicaid covers 1 in 4 rural adults**
Rural Health Transformation Program total (5 years) $50B *$10B/year FY2026-FY2030; temporary, front-loaded; does NOT offset Medicaid cuts*	**Rural hospitals currently operating at a loss \>40%** *Before Medicaid cuts; 52% in non-Medicaid-expansion states (Chartis 2026\)*

Figure 15.1 — The 2025 federal healthcare policy inflection point. Sources: KFF; CBO; Chartis 2026 State of Rural Health.

The fiscal arithmetic is unforgiving. KFF's analysis is explicit: the RHT Program is temporary and front-loaded, while nearly two-thirds of the Medicaid spending reductions are backloaded after FY2030. The rural health fund provides $50 billion over five years ending in 2030; the Medicaid cuts accelerate after 2030, continuing for the remaining five years of the 10-year budget window with no offsetting capital fund. Georgetown University's Center for Children and Families has documented that the RHT fund's federal guidance emphasizes investments in technology and new activities — not backfilling provider losses — and caps payments to providers at 15% of funds awarded to each state. It is an infrastructure fund, not a revenue replacement.

For Vermont, this federal landscape has direct operational consequences. Vermont's Medicaid program — which covers approximately 19% of the state's insured population and is a significant revenue source for all 14 hospitals — will face financial pressure from the Medicaid cuts beginning in 2026-2027. The work requirement implementation, beginning in late 2026, will add administrative burden on Vermont DVHA and risk some coverage losses among the expansion population. Vermont's 14 hospitals, already financially distressed, face additional revenue pressure from Medicaid reimbursement changes at the same time they are trying to execute transformation plans.

Vermont's response to this federal pressure is the correct one: AHS Secretary Samuelson's framing — the RHT funding is a tool for executing reform, not a substitute for reform — is the analytical frame every state should adopt. Vermont is using the $195 million for infrastructure investments that reduce ongoing structural costs: AI scribes that expand effective provider capacity, RPM that prevents hospitalizations, broadband and EMS that enable the regionalization Vermont needs. If executed well, these investments position Vermont to absorb the Medicaid revenue reductions through efficiency gains rather than service cuts. If executed poorly — if the capital is used to sustain unsustainable operations rather than transform them — Vermont will face the Medicaid cuts in 2030-2031 without the structural improvements needed to survive them.

Five Forces Shaping the Next Decade — Updated for 2026

Force 1: The Medicaid-RHT Tension — The Defining Policy Dynamic

The One Big Beautiful Bill Act creates a structural tension that will define American rural healthcare for the next decade: significant, permanent reductions in Medicaid revenue arriving after the expiration of a temporary, one-time capital fund. Every state that received RHT funds faces the same fundamental question: can we use this capital to build a system efficient enough to survive the Medicaid revenue reductions that arrive after the fund expires?

For most rural hospital systems, the honest answer is: possibly, if the capital is invested wisely, the structural changes are executed on the available timeline, and state-level policy responses are adequate. The states most likely to succeed are those that — like Vermont — had structural reform plans in place before the RHT Program was created, are using RHT funds as capital for pre-planned transformation rather than as operating support, and have the regulatory and governance infrastructure (Vermont's GMCB and AHS) to execute transformation systematically rather than hospital-by-hospital.

The states most at risk are those that use RHT funds for revenue replacement rather than structural change, that lack the regulatory framework to manage hospital network transformation, and that face the 2030 Medicaid revenue cliff without having made the structural changes that would allow them to manage it. Chartis estimates that 417 rural hospitals are currently vulnerable to closure nationally — and the Medicaid cuts that take effect after 2027, combined with the expiration of the RHT fund after 2030, will intensify pressure on the most vulnerable facilities.

Force 2: The AI Clinical Transformation — Accelerating Faster Than Governance

Artificial intelligence is moving from healthcare's technology periphery toward its clinical center faster than any governance framework has been able to track. In 2026, ambient clinical intelligence — AI systems that listen to clinical encounters and generate structured documentation automatically — has moved from early adopter to mainstream consideration. Physicians at Hattiesburg Clinic and similar practices nationally have reported significant reductions in documentation burden from AI scribe tools. Vermont's RHT Program explicitly funds AI scribe grants for rural and independent practices.

The next clinical AI wave — predictive population health at the individual level, diagnostic AI becoming standard in high-volume imaging and pathology, AI-enabled triage reducing unnecessary ED utilization — is 18-36 months behind ambient documentation. The convergence of FHIR-based data access, large language models, and all-payer claims databases will enable risk stratification that predicts not just which patients are high-risk but which specific clinical events — hospitalization, medication non-adherence, crisis escalation — care management programs should prevent.

AI governance is the critical gap. The FDA, ONC, and CMS are developing AI governance frameworks, but deployment of clinical AI tools is outpacing governance development at every level. Vermont's RHT AI investments — without explicit AI governance requirements as conditions of grant funding — risk deploying tools that have not been tested for differential performance across Vermont's equity populations. The Northeast Kingdom's elderly, rural Medicaid patients may perform differently in AI risk stratification models trained on national commercial data. Vermont's CIN-based AI governance framework, developed in Chapter 14, is the organizational response to this challenge.

Force 3: The Demographic Reckoning — No Longer a Future Problem

Vermont's Oliver Wyman analysis projected 65+ population exceeding 30% of the state's total by 2040\. As of 2026, Vermont is 14 years from that milestone — but the healthcare system consequences are not 14 years away. They are arriving now, in the form of rising Medicare enrollment, deteriorating hospital payer mix, increasing long-term care demand that existing capacity cannot meet, and a growing population of dual-eligible Medicare/Medicaid patients whose complex needs are the highest cost and most poorly coordinated care challenge in the system.

Vermont is the national canary for demographic healthcare stress. But by 2035, the challenge Vermont faces today will be present in every Northeastern state, most Midwestern states, and increasingly nationally. The Baby Boom generation that is fully in Medicare's coverage by 2030 will be in its 80s by 2040 — the frailty, dementia, and multi-morbidity phase of the demographic curve that drives the highest costs and the most inadequate care models. Vermont's efforts to develop dementia care infrastructure, PACE programs, and HCBS capacity are investments in solving a problem that will be national by 2040\.

Force 4: The Drug Pricing Reckoning — Complexity Compounding

The GLP-1 agonist spending wave represents the most consequential near-term pharmaceutical policy challenge for state healthcare systems. At full obesity indication coverage for Medicaid and Medicare, GLP-1s would add hundreds of billions of dollars annually to program spending — simultaneously offering potentially transformative reductions in the chronic disease burden that drives the highest healthcare utilization. The BALANCE Model (December 2025\) is the federal attempt to thread this needle: negotiating lower GLP-1 prices with manufacturers to enable coverage expansion without fiscal catastrophe.

Vermont's DVHA must make a BALANCE participation decision in 2026 that balances short-term budget pressure against long-term population health investment. The long-term argument for coverage is strong: Vermont's Medicaid population has high obesity rates, high diabetes prevalence, and high cardiovascular disease risk — exactly the population where GLP-1 efficacy is most demonstrated. Reducing obesity-driven chronic disease in this population would reduce the Medicaid spending on hospitalizations, specialist care, and long-term care that is far more expensive than the drugs themselves. But Vermont's near-term Medicaid budget is already under pressure from the federal cuts, making any new high-cost coverage decision politically and fiscally difficult.

The IRA drug price negotiation — semaglutide (Ozempic, Wegovy, Rybelsus) selected for 2025 negotiations with negotiated prices effective in 2027 at $274 for a 30-day supply, down from over $1,000 — will meaningfully reduce the fiscal cost of GLP-1 coverage expansion if states and plans participate in BALANCE. Vermont should evaluate BALANCE participation explicitly as an equity and population health policy decision, not purely as a budget decision.

Force 5: The Value-Based Care Tipping Point — Still Approaching

Despite 15 years of payment reform effort, the majority of U.S. healthcare spending remains in fee-for-service arrangements. VBC penetration is real — approximately 58% of commercially insured Americans are in some form of value-based arrangement — but has not yet achieved the tipping point at which FFS becomes the minority mode of payment. Vermont's mandatory RBP and global budget approach is a direct legislative response to the failure of voluntary VBC to achieve this tipping point.

The AHEAD Model — now operating in six states (Maryland, Connecticut, Hawaii, Vermont, Rhode Island, and New York) — represents the most serious federal commitment to all-payer total cost of care accountability since CMMI's founding. If Vermont's Cohort 2 participation (beginning January 2027\) produces documented savings and quality improvement by 2030, the political and policy case for expanding AHEAD to additional states strengthens significantly. Vermont's performance under AHEAD is not just a Vermont policy question — it is national evidence for or against the most ambitious federal payment reform currently operating.

The Six-Pillar Forecast: 2026-2035 Updated

Pillar	2026 starting point	Primary headwinds	Primary tailwinds	2035 the framework assessment
Policy	Vermont: Acts 167/68/AHEAD creating most complete state-level reform mandate in the country. National: H.R. 1 Medicaid cuts creating coverage contraction; RHT Program front-loaded.	Medicaid work requirements (late 2026\) adding administrative burden and coverage risk. Federal healthcare policy instability. State budget pressure from provider tax restrictions.	AHEAD Model expanding (6 states). Vermont's mandatory RBP/global budget as policy template. Price transparency momentum. BALANCE Model GLP-1 access expansion.	Moderate progress nationally; Vermont-specific progress high if execution holds. Mandatory APM participation remains the critical unresolved policy question for national transformation acceleration.
Economics	Vermont: RBP being implemented (FY2027); global budgets mandated (FY2028). National: VBC at \~58% commercial penetration; global budgets outside Vermont/Maryland very limited.	H.R. 1 Medicaid revenue reductions affecting hospital financial positions. GLP-1 spending wave threatening to overwhelm pharmaceutical savings from IRA negotiations. Commercial premium pressure continuing.	Vermont RBP producing documented savings already ($230M in FY2026 regulatory actions). IRA GLP-1 price negotiation (effective 2027). Maryland AHEAD results validating global budget savings. ACO REACH maturing.	Vermont: strong economics progress if global budgets implemented on schedule. National: VBC reaches 70-75% commercial by 2030; global budgets beyond Vermont/Maryland remain limited without mandatory policy.
Technology	Vermont: VHCURES solid; VITL/HIE voluntary and incomplete; analytics vendor being procured; CIN under development. National: FHIR mandate improving interoperability; AI deployment outpacing governance.	Two-year data lag in VHCURES limits real-time management. FHIR compliance incomplete at smaller community hospitals. AI governance frameworks absent in most states.	Vermont CIN providing shared analytics and AI governance infrastructure. RHT IT advance grants funding FHIR-capable upgrades at smaller facilities. Ambient AI reducing documentation burden nationally. BALANCE Model enabling GLP-1 utilization data collection.	Technology: fastest-improving pillar nationally. FHIR interoperability becoming operationally complete for major EHR systems by 2028\. AI deployment rapid; governance lagging — patient safety and equity risk if governance does not catch up.
Clinical	Vermont: Blueprint strong; CoCM/MHI pilot effective but not permanently funded; CCBHC expanding; PACE gap; primary care productivity constraint. National: Behavioral health access crisis worsening nationally.	MHI behavioral health integration sustainability uncertain without AHEAD EAST Fund commitment. Primary care workforce shortage intensifying (370 FTE gap by 2030 in Vermont alone). Medicaid cuts threatening safety-net primary care providers.	Blueprint PCMH infrastructure among strongest in the country. CCBHC expansion adding 5 new entities by July 2026\. AHEAD EAST Fund designed specifically to invest in primary care and behavioral health. CoCM Medicare billing codes enabling sustainable financing.	Clinical: steady improvement nationally in primary care quality; behavioral health integration accelerating. Vermont-specific: Blueprint \+ AHEAD primary care investment strongest in country; behavioral health integration is the critical clinical quality variable.
Equity	Vermont: geographic disparities documented; BIPOC disparities smaller but real; SDOH screening operational; GLP-1 Medicaid coverage gap; Northeast Kingdom access vulnerability. National: racial disparities not narrowing despite policy attention.	H.R. 1 Medicaid cuts disproportionately affecting low-income and rural populations — amplifying existing equity gaps. Federal funding reductions for health equity programs (VDH reported $7M retracted). GLP-1 access gap acutely affecting Medicaid populations.	Vermont BALANCE Model participation could close GLP-1 equity gap for Medicaid enrollees. CCBHC access policy (serve all regardless of insurance). Blueprint HRSN universal screening building SDOH data infrastructure. Act 68 equity requirements embedded in accountability framework.	Equity: highest-risk pillar nationally and in Vermont. Without explicit GLP-1 coverage expansion, SDOH investment, and Northeast Kingdom access protection, Vermont's equity gaps will widen as Medicaid cuts reduce coverage for the populations with greatest need.

Pillar	2026 starting point	Primary headwinds	Primary tailwinds	2035 the framework assessment
Operations	Vermont: RHRC engagement completed; hospital transformation plans in development; AHS restructuring underway; PMO being established; CIN under development. National: Administrative complexity not declining despite FHIR and prior authorization reform.	14 hospital transformation plans at varying stages; analytics vendor not yet operational. AHS restructuring requiring staff and organizational capacity that must be built. RHT Program implementation requiring project management infrastructure Vermont is actively hiring.	$195M RHT Program providing capital for transformation infrastructure. All 14 hospitals engaged in transformation planning. GMCB capacity added under Act 68\. Joint AHS-GMCB analytics vendor procurement underway. Vermont CIN providing shared services infrastructure.	Operations: transformation underway in Vermont — the question is whether execution pace matches statutory deadlines. National: administrative simplification progressing slowly; meaningful reduction in administrative burden requires federal prior authorization reform that has not yet passed.

Figure 15.2 — Six-pillar forecast 2026-2035: Vermont and national. Sources: the framework analysis; sources cited throughout this chapter.

Vermont's 10-Year Trajectory: Two Scenarios

Vermont's healthcare transformation trajectory from 2026 to 2035 is genuinely uncertain in ways that earlier versions of this chapter could not fully acknowledge — because the federal policy environment has changed materially since 2024\. The following two scenarios represent the range of plausible Vermont outcomes, not extreme predictions.

The Transformation Success Scenario

In the success scenario, Vermont executes its transformation agenda on or near the statutory timelines, uses its capital wisely, and demonstrates by 2030 that a small rural state can achieve genuine all-payer system transformation.

- FY2027: RBP implemented; hospital commercial rates declining toward benchmark; premium growth slows for the first time in a decade. First concrete evidence that mandatory price regulation can reduce insurance premiums without hospital closures.
- FY2028: Non-CAH hospital global budgets operational; AHEAD Cohort 2 in full operation; Vermont's hospital sector begins demonstrating the population health management improvements — reduced avoidable hospitalizations, improved chronic disease management — that global budgets are designed to incentivize.
- 2028: AHS delivers Statewide Strategic Plan with specific, quantified commitments for each pillar through 2031; Vermont's hospital network has been rationalized to a sustainable tier structure; Northeast Kingdom access protected through REH conversions paired with RHT-funded EMS and telehealth.
- 2030: RHT Program capital fully deployed; Vermont hospital sector stabilized with 10-12 sustainable facilities and 2-4 REH or CACC conversions; Medicaid revenue reductions from H.R. 1 absorbed through efficiency gains from transformation; per-capita healthcare cost growth below GDP growth for first time.
- 2031-2035: Vermont serves as national template; AHEAD model expands to additional states using Vermont's documented results as evidence base; Vermont's Blueprint \+ AHEAD primary care investment produces measurable population health improvements in chronic disease management; AHS restructuring model studied by other states facing comparable transformation mandates.

The Partial Transformation Scenario

In the partial transformation scenario, Vermont makes meaningful progress but faces execution gaps that prevent full realization of the transformation agenda by the statutory deadlines.

- FY2027: RBP implemented but contested — UVMMC and other large systems seek legislative relief from specific rate provisions; GMCB maintains implementation but at a modified glide path that delays full impact.
- FY2028: Global budget methodology not finalized for non-CAH hospitals due to GMCB resource constraints and AHEAD implementation complexity; deadline extended; transformation planning delayed.
- 2028: AHS delivers Statewide Strategic Plan but it is descriptive rather than prescriptive — identifying the right questions without making the specific structural commitments that accountability requires. Northeast Kingdom hospital transition planning delayed pending equity impact analysis.
- 2030: RHT Program capital deployed but partially misdirected toward operating support rather than structural transformation; Medicaid revenue reductions begin arriving; several CAH hospitals facing renewed financial distress without the structural transformation that would have enabled them to absorb the cuts.
- 2031-2035: Vermont's transformation is real but incomplete; the system is more sustainable than the status quo trajectory would have produced, but costs remain above what full transformation would achieve; a second reform wave required to address the gaps left by the first.

Neither scenario is inevitable. The success scenario requires execution discipline, political commitment, and the organizational capacity that Vermont is actively building but has not yet fully developed. The partial transformation scenario requires execution failures that are preventable with adequate investment in implementation infrastructure. The analytical framework this book provides — and the specific organizational design the AHS Restructuring chapter prescribes — is designed to make the success scenario more likely.

"The RHT funding is a tool that lets us put affordability plans into action. This is not our health care reform plan. This is an opportunity that we'll use to support all of the health care reform work that we're already doing in the state."

— Jenney Samuelson, Vermont AHS Secretary, January 2026

Vermont as National Template — The Decisions That Will Determine Outcomes

Vermont's transformation will produce a set of documented answers to questions that every state will eventually face. The national significance of Vermont's experiment lies not in Vermont's size but in its head start — and in the quality of the analytical record it is creating. The following decisions, made in Vermont over the next five years, will produce the evidence base that shapes the national policy toolkit for the decade after.

Decision	Vermont timeline	National implication if Vermont succeeds	National implication if Vermont struggles
Will mandatory all-payer RBP reduce hospital prices without reducing access?	FY2027 implementation; first evidence FY2028	Confirms that mandatory hospital price regulation can close the price gap without the hospital closures opponents predict; accelerates state adoption nationally	Provides ammunition for hospital system opponents of RBP; may delay national adoption by 5-10 years
Can hospital global budgets change hospital behavior	FY2028 global budgets; AHEAD data from 2027-2030	Demonstrates that global budgets produce the population health improvements Maryland demonstrated	Suggests that global budgets without adequate primary care and SDOH infrastructure do not produce population

Decision	Vermont timeline	National implication if Vermont succeeds	National implication if Vermont struggles
toward population health management?		but that skeptics attribute to Maryland-specific conditions	health improvement; strengthens case for community investment as prerequisite
Can AHS execute a comprehensive Statewide Strategic Plan by December 2028?	December 2028 statutory deadline	Proves that a state agency can successfully restructure itself as a population health system operator; provides organizational design template for other states	Reveals organizational capacity constraints that other states will need to address before attempting comparable mandates; valuable learning regardless of outcome
Can the RHT Program capital be used for structural transformation rather than operating support?	FY2026-FY2030 spending window	Demonstrates that one-time capital investment can produce durable structural change in rural healthcare — the fundamental question about whether the $50B was well spent	Suggests that rural healthcare structural challenges require ongoing revenue support, not just capital investment; major policy implication for post-2030 federal rural health policy
Will Vermont's AHEAD participation demonstrate savings and quality improvement?	AHEAD performance data from 2027 onward; CMS evaluation through 2035	Builds the national evidence base for state-level total cost of care accountability; may accelerate AHEAD expansion to additional cohorts	May lead CMS to modify or restructure AHEAD if Vermont's experience reveals design flaws; still valuable as evidence

Figure 15.3 — Vermont's transformation decisions and their national policy implications. Sources: the framework analysis; Vermont AHEAD State Agreement; Act 68 of 2025\.

A Closing Note: What This Book Is Betting On

Every analytical framework contains implicit bets about what matters and what does not. It is worth making this book's bets explicit.

This book bets that structural change is possible — that the combination of strong enough financial pressure, adequate capital investment, political will sufficient to maintain mandatory policy over hospital system resistance, and organizational capacity built over years of preparation can produce a genuinely different Vermont healthcare system by 2035\. This is not a certainty. It is the most plausible outcome given Vermont's specific combination of factors: a small, transparent, high-capacity state government; a hospital sector that is financially distressed enough to be willing to transform rather than resist; a regulatory body (GMCB) with genuine authority and demonstrated willingness to use it; a reform-minded legislature that has sustained a coherent policy agenda through multiple administrations; and now capital — $195 million in federal investment aligned to the pre-existing reform plan.

This book also bets that the transformations described here are transferable — that the analytical framework, the payment mechanisms, the clinical models, and the organizational designs are not Vermont-specific solutions to Vermont-specific problems, but general-purpose tools for the structural healthcare challenges that every American state will eventually face. Vermont's specificity — its small size, its transparency, its policy history — makes it an unusually good teaching case. But the lesson is generalizable.

The deepest bet this book makes is about the relationship between analysis and action. Healthcare transformation fails not primarily because of analytical errors — the evidence base for what needs to change is strong and consistent — but because analysis does not automatically produce the institutional capacity, political will, and organizational execution required to implement what the evidence recommends. This book provides the analytical framework. The institutional capacity, political will, and organizational execution are built by the people who read it and do the work.

Vermont's transformation is happening because specific decision-makers — legislators, regulators, hospital executives, clinicians, community health workers, and patients — are making specific decisions at specific moments. The analytical framework this book provides is in service of better decisions. What those decisions produce — a transformed system or a partial transformation, a national template or a cautionary tale — belongs to the people who make them.

Sources: KFF, A Closer Look at the $50 Billion Rural Health Fund in the New Reconciliation Law (September 2025); KFF, How Might Federal Medicaid Cuts in the Enacted Reconciliation Package Affect Rural Areas?; KFF, What to Know About the BALANCE Model for GLP-1s in Medicare and Medicaid (2026); Chartis, 2026 State of Rural Health (February 2026); Peterson-KFF Health System Tracker, Eight Trends Shaping 2026 Healthcare Costs (March 2026); Pew Charitable Trusts, New Federal Medicaid Policies Compound State Budget Pressures (January 2026); Georgetown Center for Children and Families, States Beginning to Grapple with Federal Medicaid Cuts Impact on Rural Health Care (March 2026); CMS BALANCE Model announcement (December 23, 2025); Vermont AHS Secretary Samuelson, remarks January 29, 2026; Oliver Wyman Act 167 Community Engagement: Recommendations (2024); Vermont AHEAD State Agreement (January 2025); Vermont RHT Program Application (November 2025).

CONCLUSION

What Vermont Proves, and What Remains to Be Proven

The Argument, Restated

This book opened with a room full of people in Vermont learning, with unusual specificity and unusual honesty, exactly how close their healthcare system was to a financial cliff. Oliver Wyman's September 2024 presentation named the numbers — nine hospitals losing money, thirteen of fourteen projecting losses by 2028, $2.4 billion in cumulative five-year deficit under realistic assumptions — and drew the causal chain from commercial cross-subsidy to demographic shift to premium inflation to the household economics that make healthcare the dominant financial anxiety of working Vermont families. The room was full because people already knew something was wrong. The presentation gave that feeling a precise shape.

The book's central argument has been this: Vermont's crisis is structurally produced, and structural crises require structural responses. The sixteen chapters between that opening room and this conclusion have developed what structural response means across six pillars — Policy, Economics, Operations, Clinical, Equity, and Technology — and have shown, through Vermont's specific legislative and administrative record, what structural response looks like when it is actually attempted rather than perpetually deferred.

The argument is not that Vermont has solved its healthcare crisis. It has not. As of April 2026, reference-based pricing has not yet taken effect. Global budgets are still being designed. The Statewide Strategic Plan has not been written. The analytics vendor has not been selected. The CCBHC network is still being certified. The Northeast Kingdom hospitals are still financially fragile. Vermont is mid-transformation — past the point of no return on the statutory timeline but nowhere near the destination. The argument is that Vermont has committed to structural change with sufficient specificity, sufficient statutory authority, and sufficient capital investment that the transformation has a credible path to completion. That is more than any comparable American state has done.

What the Six Pillars Prove

The six-pillar framework is this book's primary analytical contribution, and its validity deserves one final statement. The framework is not a consulting taxonomy or a strategic planning template. It is a description of the architecture of healthcare transformation — the six domains that must all function for transformation to produce its promised results, and the specific dependencies between them that explain why single-pillar interventions consistently fail.

Vermont's experience across each pillar confirms the framework's structure. The Policy pillar — Acts 167, 51, 68, and the AHEAD agreement — created the mandatory architecture that voluntary reform could not. Without mandatory RBP and mandatory global budgets, Vermont's hospital pricing would continue rising, premiums would continue their decade-long trajectory, and the financial crisis would deepen on the timeline Oliver Wyman projected. Voluntary reform produced OneCare Vermont, which produced modest results among hospitals that chose to participate and no results among those that did not. Mandatory reform changes the calculus for everyone simultaneously.

The Economics pillar confirms that payment architecture shapes behavior more reliably than education, exhortation, or aspiration. When Vermont hospitals operate under global budgets, they will have a direct financial incentive to prevent the hospitalizations that cost money without generating offsetting revenue. When Vermont insurance premiums fall — as RBP and global budget savings flow through to premium rates under Act 68's passthrough requirement — the affordability crisis that has made healthcare the defining economic anxiety of Vermont families will ease. Not immediately. Not completely. But measurably, on a documented timeline, with accountability metrics that allow the legislature to verify progress or demand correction.

The Technology pillar confirms that data infrastructure is not a technical support function — it is the management substrate on which population health management depends. Vermont cannot manage what it cannot measure. VHCURES, for all its limitations, already enables analyses that most states cannot conduct. The CIN analytics platform, the VHCURES-VHIE integration mandate, and the FHIR compliance investments are not investments in technology for its own sake; they are investments in the management capability that global budgets require.

The Clinical pillar confirms what Vermont's own Blueprint data has shown for fifteen years: when primary care is organized as a population health function — team-based, data-driven, SDOH-aware, behaviorally integrated — it produces the utilization reduction and quality improvement that every payment reform is designed to incentivize. The Blueprint's 5.8:1 return on investment is not a projection; it is a documented result from Vermont's own experience. The transformation agenda is, in its clinical dimension, a bet that what works for Blueprint-attributed patients will work for all Vermont patients when primary care investment becomes a system-level commitment rather than a program-level initiative.

The Equity pillar confirms the analytical requirement that transformation cannot be evaluated by averages. Vermont's statewide primary care access rate of 91% — four points above the national benchmark — is a genuine policy achievement that obscures an 11-point gap between white Vermont adults and BIPOC Vermont adults. Vermont's statewide uninsurance rate of 3% obscures an 8% uninsurance rate in Essex County. The transformation that closes the average gap while widening the disparity gap has failed by the standards that Acts 167 and 68 establish. The equity pillar is not a constraint on transformation; it is a definition of what transformation means.

The Operations pillar confirms the gap between strategy and execution that characterizes every healthcare reform effort that has produced documents rather than change. Vermont's transformation will succeed or fail in the operations domain — in whether AHS has the organizational capacity to manage 14 hospital transformation plans simultaneously, in whether the GMCB can design and implement global budget methodology on statutory timelines, in whether the Vermont CIN can build the shared services infrastructure that individual hospitals cannot build alone. The analytical frameworks in this book are necessary but not sufficient. The organizational capacity to execute them is the binding constraint.

"The deepest bet this book makes is about the relationship between analysis and action. Healthcare transformation fails not primarily because of analytical errors — the evidence base for what needs to change is strong and consistent — but because analysis does not automatically produce the institutional capacity, political will, and organizational execution required to implement what the evidence recommends."

— Chapter 3, this volume

Vermont as National Template — The Case for Optimism

The national significance of Vermont's transformation is not self-evident. Vermont is small. Its politics are idiosyncratic. Its hospital system is unusual — 14 non-profit community hospitals, no investor-owned chains, a regulatory body (the GMCB) with authority that most states have never granted any agency. The specific legislative pathway that produced Acts 167 and 68 — the years of community engagement, the Oliver Wyman analysis, the reform cascade structure that built political legitimacy step by step — is not easily replicated in states with more complex political economies, more powerful hospital lobbies, and less institutional trust in state government.

But the structural forces driving Vermont's crisis are universal. The demographic math of an aging population overwhelming a healthcare system built on working-age commercial insurance cross-subsidy applies in Maine and New Hampshire and Vermont and, on a 10-20 year lag, in Ohio and Michigan and Pennsylvania and eventually everywhere. The failure of voluntary payment reform to achieve the tipping point at which fee-for-service becomes the minority mode of payment is a national finding, not a Vermont finding. The administrative complexity that burns out physicians and consumes resources that should go to patient care is a national problem. The equity gaps that persist despite decades of health equity programming are a national pattern.

Vermont's value as a national template is not that other states should copy Vermont's legislation. They should not — the specific provisions of Act 68 reflect Vermont's specific hospital market structure, regulatory history, and political context. The value is that Vermont is generating, in public, with detailed documentation, the evidence base for a set of questions that every state will eventually need to answer: Can mandatory all-payer price regulation reduce hospital prices without hospital closures and access reductions? Can hospital global budgets change hospital behavior toward population health management? Can a state agency restructure itself as a population health system operator? Can a rural state use federal capital investment to build healthcare infrastructure that survives the expiration of the capital investment?

By 2030, Vermont will have answers to these questions. They will not be perfectly clean answers — healthcare systems are too complex for controlled experiments — but they will be the best answers available, grounded in the most comprehensive state-level transformation attempt in American health policy history. Those answers are worth more than any individual policy recommendation, because they are evidence rather than argument, and evidence is what the national conversation desperately needs.

The People This Book Cannot Reach

Every book has an audience it cannot reach, and this one is no different. The people most affected by Vermont's healthcare transformation — the Vermonter in Canaan who cannot afford her insulin, the primary care physician in Newport who is burning out under the weight of documentation, the hospital worker in St. Johnsbury who does not know whether his hospital will still be open in five years, the family in Rutland deciding whether to go to the emergency room or wait until morning because the copay is too high — are not the audience for a six-pillar analytical framework.

This book is written for the people who make the decisions that affect those Vermonters' lives: the legislators who vote on payment reform bills, the regulators who set hospital rates, the hospital executives who decide how to respond to financial pressure, the primary care physicians who choose whether to participate in Blueprint, the AHS staff who design and manage transformation programs. The book's ambition is to give those decision-makers better analytical frameworks, better evidence, and better language for the choices they face.

But the ultimate accountability of healthcare transformation is not to the analytical frameworks that describe it or the advisory practices that support it. It is to the Vermonter in Canaan who should be able to afford insulin, the physician in Newport who should be able to spend her time on patients rather than documentation, the hospital worker in St. Johnsbury who should be able to plan for a career in his community, and the family in Rutland whose healthcare decisions should not be shaped by fear of financial ruin. Those are the people transformation is for. Those are the people who will know whether it worked.

The Work That Remains

Vermont's transformation is incomplete in ways that are analytically important to acknowledge. The AHS restructuring is underway but not finished. The analytics vendor is being procured but has not yet produced the tools that strategic planning requires. The CCBHC network is expanding but has not yet covered all Hospital Service Areas. The GLP-1 BALANCE Model decision has not been made. The Northeast Kingdom equity constraints have been named but not fully addressed in the transformation planning process. The December 2028 Statewide Strategic Plan has not been written.

These are not failures. They are the normal condition of a system in mid-transformation — closer to the destination than to the starting point, but not there yet. The work that remains requires the same combination of analytical rigor, political commitment, and operational discipline that has produced the progress to date. It also requires something that no analytical framework can guarantee: the sustained attention of the people in positions of decision-making authority. Healthcare transformation is slow, and it is easy to let the urgency created by a financial crisis dissipate as the crisis is partially managed and other priorities compete for leadership attention.

Vermont's statutory timeline — the December 2028 Strategic Plan deadline, the mandatory milestones in Act 68, the AHEAD Model's nine-year performance period — is the institutional mechanism for sustaining attention. It converts transformation from a priority that can be deferred to a deadline that cannot. Vermont's legislature, in creating those deadlines, made the most important commitment of the entire reform agenda: the commitment to be held accountable, by specific dates, for specific outcomes. Whether that accountability is actually enforced — whether a Strategic Plan that describes the right questions without committing to specific answers is accepted, or whether the legislature demands the specificity that accountability requires — will determine whether Vermont's transformation produces the system change it promises.

The room in September 2024 was full because people knew something was wrong. This book has tried to give that knowledge a framework, evidence, and language for action. The action belongs to the people in the room.

the analytical framework — April 2026

Chapter 16: The AHS Restructuring Roadmap — A Six-Pillar Framework for System Architects

The AHS Restructuring Roadmap — A Six-Pillar Framework for System Architects

Redesigning Vermont's Agency of Human Services for the 2028 Strategic Plan Mandate: A Six-Pillar Framework for System Architects

Sources: Act 68 of 2025 (Vermont); Oliver Wyman Act 167 Community Engagement: Recommendations (2024); AHS Health Care System Transformation Reports (2025); Health Care Delivery Advisory Committee mandate; AHEAD State Agreement (January 2025); Vermont Rural Health Transformation Program Application (November 2025); AHS January 2025 Legislative Presentation; GMCB Act 68 Update (February 2026).

This chapter is written for a specific audience: the AHS staff, Health Care Delivery Advisory Committee members, Vermont legislators, GMCB officers, and hospital and community provider leaders who must translate Vermont's reform legislation into an operational reality. It is simultaneously academic — grounding every recommendation in evidence and analytical framework — and operational — providing the specific organizational design, governance structure, accountability metrics, and implementation timeline that Vermont's transformation requires. The six-pillar framework that organizes this entire book maps directly onto the Statewide Health Care Delivery Strategic Plan that AHS must deliver to the Vermont Legislature by December 2028\.

The Statutory Mandate — What Act 68 Actually Requires AHS to Do

The most important thing to understand about Act 68's mandate to AHS is that it is not a mandate to write a plan. It is a mandate to restructure a system. The Statewide Health Care Delivery Strategic Plan is the vehicle, not the destination. The destination is a Vermont healthcare system that achieves the five goals of Act 167: reduced inefficiencies, lower costs, improved health outcomes, reduced health inequities, and increased access to essential services. The plan is the document that makes AHS accountable for moving toward those goals.

Act 68 is specific about what the Strategic Plan must contain. Drawing from the statute and the Health Care Delivery Advisory Committee mandate, the plan must address:

Statutory requirement	What it means in practice	Deadline
Statewide Health Care Delivery Strategic Plan	A comprehensive plan for Vermont's healthcare delivery system covering all six dimensions: payment reform trajectory, primary care investment targets, hospital network configuration, behavioral health integration, equity goals, and administrative simplification. Updated every three years.	First delivery: December 1, 2028
Affordability benchmarks	Specific, measurable standards for health care affordability — not aspirational language. Must define what 'affordable' means for Vermont families and how progress toward that standard will be measured.	Established by Health Care Delivery Advisory Committee; incorporated in Strategic Plan
Total cost of care targets	Quantified targets for per-capita healthcare cost growth, aligned with AHEAD Model commitments and Act 68's hospital spending reduction requirements.	Act 68 requires 2.5% hospital spending reduction for FY2026; AHEAD TCOC targets from 2027
Standardized accountability metrics	A defined set of outcome measures — tied to the five Act 167 goals — against which AHS will report progress to the legislature monthly and the Strategic Plan will be evaluated.	AHS already reporting monthly; full metrics framework needed by Strategic Plan delivery
Primary care investment plan	A plan for increasing primary care investment as a percentage of total healthcare spending — an explicit AHEAD requirement — including Blueprint expansion, CCBHC development, and CoCM deployment.	AHEAD requires primary care investment targets from 2027; Strategic Plan formalizes the plan
Health equity strategy	A specific plan for reducing disparities in access and outcomes resulting from demographic factors or health status — required explicitly by Act 68 Goal 1\.	Embedded in Strategic Plan; equity measures in monthly AHS reporting
Workforce strategy	A plan for attracting and retaining high-quality healthcare professionals in Vermont — Act 68 Goal 5 — including the use of RHT Program workforce investments.	RHT Program plan covers 2026–2030; must be integrated into Strategic Plan

Figure 16.1 — Act 68 statutory requirements for the Statewide Health Care Delivery Strategic Plan. Sources: Act 68 of 2025; Health Care Delivery Advisory Committee mandate (healthcarereform.vermont.gov).

The Strategic Plan is not AHS's alone to write. Act 68 created the Health Care Delivery Advisory Committee — 18 members representing AHS, GMCB, the Health Care Advocate, hospitals, health centers, insurers, health professions, small businesses, and long-term care facilities — to develop and maintain the plan in collaboration with AHS. The committee also consults with the Health Equity Advisory Commission and the Vermont Steering Committee for Comprehensive Primary Health Care. This governance structure means the Strategic Plan must reflect multi-stakeholder input and consensus, not just AHS's internal analysis — a significant operational challenge and an important political constraint.

"Review existing AHS agency structure and program list to identify overlaps and opportunities for efficiency. Align Agency of Human Services agencies with Health Service Areas and meet regularly with hospitals and providers. Fully computerize and integrate Agency of Human Services subunits."

— Oliver Wyman Act 167 Community Engagement: Recommendations, September 2024 — Priority actions for AHS in 2025

The Organizational Diagnosis — Why AHS Must Restructure, Not Just Plan

Oliver Wyman's finding about AHS was specific and unambiguous. After 230 meetings with hospital leaders, community organizations, state agency staff, and Vermonters across all 14 Hospital Service Areas, the consultants identified AHS's organizational structure as one of the primary barriers to effective healthcare transformation — not because AHS lacks capable people or good intentions, but because the organization was designed for a different era and a different role.

The current AHS organizational model is what management theorists call a program-silo structure: it is organized around funding streams and program types (Medicaid, mental health, substance use, aging services, developmental services, Blueprint, health care reform) rather than around the populations it serves or the geographic communities those populations live in. This structure made sense when AHS's primary function was administering categorical benefit programs. It does not make sense for an organization whose primary function has become managing the transformation of an interconnected healthcare delivery system.

The Four Structural Gaps

Oliver Wyman identified four specific structural gaps that AHS's transformation must address. These are not recommendations for improvement — they are diagnoses of organizational failures that are currently impeding transformation.

Gap	What the gap looks like in practice	Consequence for transformation	Required structural change
Program-silo structure misaligned with Hospital Service Areas	A hospital in the Northeast Kingdom that needs to address housing, transportation, behavioral health, and primary care gaps simultaneously must work with separate AHS programs: DVHA for Medicaid, DMH for mental health, ADS for aging services, DAIL for developmental services, Office of Health Care Reform for transformation planning. Each has different administrative requirements, reporting cycles, and funding authorities.	Hospitals and community providers experience AHS as a collection of disconnected programs rather than a coordinated partner. Population health management — which requires crossing program boundaries — is administratively complex to the point of being effectively impossible.	Designate HSA-level coordinators within AHS who convene all relevant AHS programs around each hospital's service area; establish HSA-level governance structures with regular inter-program coordination
Absence of a Planning and Effectiveness function	AHS currently has no dedicated analytical unit that can model the financial and access impact of proposed service site changes, calculate the system-wide cost implications of different regionalization scenarios, or monitor whether transformation investments are producing the intended outcomes.	AHS cannot answer the questions the transformation planning process is supposed to generate: if Springfield Hospital converts to REH status, what is the net impact on healthcare access and system cost in Windsor County? If primary care investment increases by $X, what is the projected reduction in avoidable ED visits? Without this analytical capacity, strategic planning is opinion-based, not evidence-based.	Establish a Division of Planning, Analytics, and Effectiveness within AHS; partner with GMCB on a joint analytics platform; integrate VHCURES access for AHS planning functions
Under-resourcing for the transformation management function	Act 68 requires AHS to simultaneously: negotiate AHEAD implementation with CMS; manage 14 hospital transformation planning processes; develop the Statewide Strategic Plan; oversee $195M RHT Program implementation; coordinate with GMCB on RBP and global budget design; and report monthly to the legislature. The Health Care Reform Office has neither the staff nor the organizational authority to manage this portfolio effectively.	Transformation activities are sequenced by capacity rather than by strategic priority. The RHT Program contract management alone requires a project manager and contract oversight capacity that does not currently exist. The absence of adequate project management infrastructure is the single most likely cause of transformation plan failure.	Hire dedicated transformation staff using RHT Program implementation funds (explicitly authorized); establish a Project Management Office within the Health Care Reform Office; clarify reporting lines and authority between Health Care Reform and program divisions
Fragmented SDOH integration	AHS has programmatic authority over housing supports, food assistance, transportation programs, mental health, substance use, and developmental services — all of the social determinants that Oliver Wyman's systems map showed driving healthcare utilization. But these programs are administered separately, reported separately, and evaluated separately from healthcare outcomes.	Vermont cannot achieve the SDOH-driven utilization reduction that global budgets require if the programs that address SDOH needs are organizationally disconnected from the healthcare delivery system. Every hospital transformation plan that identifies housing or transportation as a utilization driver requires AHS to coordinate across at least three separate program areas to address it.	Establish cross-program SDOH integration protocols; create a shared data architecture that links social service utilization to healthcare outcomes in VHCURES; include SDOH program directors in HSA-level coordination structures

Figure 16.2 — AHS organizational gaps and required structural changes. Sources: Oliver Wyman Act 167 Recommendations (2024); AHS Transformation Reports (2025); Act 68 of 2025\.

The Redesigned AHS Operating Model — A Population Health System Operator

The organizational model that AHS must become — and that the Statewide Strategic Plan must articulate — is a Population Health System Operator: an agency that manages Vermont's healthcare and social services ecosystem as an integrated system oriented around population health outcomes, organized geographically around Hospital Service Areas, and accountable for measurable progress toward the five Act 167 goals.

This is a fundamentally different organizational identity than AHS has historically held. The shift is not cosmetic — it requires changes to organizational structure, staff roles, governance processes, data systems, external relationships, and the agency's own theory of how it creates value. The following sections develop this redesigned model across the three levels at which AHS must operate: statewide system architect, regional network convener, and program delivery operator.

Level 1 — Statewide System Architect

At the statewide level, AHS functions as the architect of Vermont's healthcare delivery system: setting the strategic direction, establishing the payment reform trajectory in partnership with GMCB, allocating capital investments (RHT Program, AHEAD EAST Fund, Act 68 grants), developing the accountability framework, and monitoring system performance against the five Act 167 goals.

The statewide architecture function requires four specific organizational capacities that AHS is currently building but has not yet fully developed:

- **Strategic planning capacity: The ability to develop, maintain, and update the Statewide Health Care Delivery Strategic Plan on a three-year cycle, incorporating input from the Health Care Delivery Advisory Committee, the Steering Committee for Comprehensive Primary Health Care, the Health Equity Advisory Commission, and other required stakeholders. This is a substantial ongoing function, not a one-time project.**

- **Analytics and modeling capacity: The ability to model the cost, quality, and access implications of proposed system changes before they are implemented — answering questions like 'what happens to access in Windsor County if Springfield Hospital converts to REH status?' and 'what is the projected PMPM cost reduction from the primary care investment plan?' This requires the analytics vendor being procured jointly with GMCB, plus internal staff who can interpret and apply the outputs.**

- **Federal relationship management: Active management of Vermont's relationships with CMS (AHEAD Model), HRSA (RHT Program), CMMI (transformation grants), and other federal partners. These relationships are high-stakes, technically complex, and require dedicated staff who understand both Vermont's specific circumstances and the federal program requirements. The AHEAD State Agreement alone requires ongoing negotiation of methodology, monitoring, and potential withdrawal conditions.**

- **Legislative accountability: The production and delivery of the required monthly transformation reports, the December 2028 Strategic Plan, and the three-year updates. Vermont's legislature has demonstrated active engagement with healthcare transformation — the monthly reporting requirement is a real accountability mechanism, not a formality. AHS must staff this function appropriately.**

The Health Care Delivery Advisory Committee: AHS's Governance Partner

Act 68 established an 18-member Health Care Delivery Advisory Committee to develop and maintain the Statewide Strategic Plan in collaboration with AHS. Understanding this committee's composition and mandate is essential for AHS's operational planning, because the committee is not an advisory body in the traditional sense — it has statutory authority to set affordability benchmarks and advise the legislature directly.

Committee composition (18 members):

- AHS Secretary (or designee)
- GMCB Chair (or designee)
- Office of the Health Care Advocate
- Representatives from hospitals, health centers (FQHCs), health insurers
- Representatives from nursing, pharmacy, primary care, and other health professions
- Small business representative
- Long-term care facility representative
- Additional members as specified

Primary responsibilities:

- Setting affordability benchmarks — defining measurable standards for what 'affordable healthcare' means for Vermont families
- Monitoring Vermont's healthcare system performance and its impact on public health
- Collaborating with AHS to develop and maintain the Statewide Strategic Plan
- Incorporating recommendations from the Steering Committee for Comprehensive Primary Health Care
- Advising GMCB on creating a continuous evaluation process for the delivery system
- Providing coordinated recommendations to the General Assembly on healthcare delivery

Operational implication for AHS: The committee requires dedicated staff support — meeting facilitation, research preparation, deliberation management, and recommendation documentation. AHS should treat committee support as a dedicated function, not an add-on to existing Health Care Reform Office responsibilities.

Level 2 — Regional Network Convener (HSA-Level)

At the Hospital Service Area level, AHS must function as a regional network convener — bringing together the hospitals, primary care practices, designated mental health agencies, FQHCs, community organizations, and social service providers within each HSA to develop and execute a coordinated regional transformation strategy.

This is where Oliver Wyman's most important organizational recommendation applies: align AHS sub-units with Hospital Service Areas and meet regularly with hospitals and providers. The mechanism for doing this is the designation of HSA-level AHS coordinators — staff whose primary function is to be the face of AHS within a defined geographic area, convening the relevant AHS programs and community partners around the hospital service area's specific transformation priorities.

Vermont has 14 Hospital Service Areas and a geographically distributed health system. The Blueprint for Health already deploys field staff in each HSA — its Quality Improvement Facilitators, Community Health Team coordinators, and field service staff are embedded in local communities. The Blueprint's HSA infrastructure is the organizational model AHS should replicate and build on for broader transformation coordination.

HSA Region	Key hospitals	Population and demographic profile	Priority transformation challenges
Burlington	UVM Medical Center	\~230K, growing, most diverse; Chittenden County non-rural by RHT definition	Administrative overhead reduction at UVMMC; primary care access for growing immigrant population; behavioral health integration at scale
Barre/Montpelier	Central VT Medical Center	\~65K, aging, state government center	Radiation therapy and neurology COE development; state employee health plan RBP transition; rural access in Washington County
Rutland	Rutland Regional Medical Center	\~62K, aging, lower income	COE development (surgery, neurology, psych); high opioid overdose rates; workforce housing shortage
Burlington-south (White River Jct.)	Mt. Ascutney Hospital	\~59K, aging, Dartmouth Health connection	REH or focused scope transition; Dartmouth Health cross-border coordination; rehabilitation COE
St. Albans	NW Medical Center	\~53K, rural, Franklin/Grand Isle counties	Cancer surgery and radiation COEs; workforce recruitment; Northern border community needs
Bennington	SW VT Medical Center	\~48K, aging, most rural	9 COEs — secondary RSC role; psych services critical; Berkshire/Taconic regional coordination
Middlebury	Porter Medical Center	\~46K, aging, Addison County	TBD tier status; Porter affiliated with UVM network; shared service opportunities with UVMMC
Brattleboro	Brattleboro Memorial Hospital	\~42K, aging, high SUD/MH burden	Acute general surgery COE; Windham County high suicide/SH rates; Brattleboro Retreat psychiatric coordination
Newport	North Country Hospital	\~41K, very rural, Northeast Kingdom	One of most financially vulnerable; REH candidacy assessment; Essex/Orleans county high uninsurance
St. Johnsbury	NE VT Regional Hospital	\~39K, very rural, Northeast Kingdom	Cancer surgery and infusion COEs; Caledonia/Essex highest poverty; broadband and telehealth priority
Springfield	Springfield Hospital	\~31K, rural, Windsor/Windham border	Memory care and psych-adult COEs; high SUD corridor; transformation plan most complex given financial position

HSA Region	Key hospitals	Population and demographic profile	Priority transformation challenges
Morrisville	Copley Hospital	\~31K, rural, Lamoille/Orleans	Orthopedics and rheumatology COEs; Stowe/ski area seasonal population; workforce housing critical
Randolph	Gifford Medical Center	\~28K, very rural, Orange County	TBD tier — among most financially vulnerable; comprehensive assessment needed; AHEAD CAH protections important
Grace Cottage	Grace Cottage Hospital	\~14K, very rural, Windham County; 19-bed Vermont's smallest hospital	REH conversion most likely path; coordination with Brattleboro Memorial essential; palliative care specialty worth preserving

Figure 16.3 — Vermont 14-Hospital Service Areas: regional profile and priority transformation challenges for AHS HSA-level coordination. Sources: Oliver Wyman Act 167 Report; Vermont RHT Program Application; AHS Transformation Reports 2025\.

Level 3 — Program Delivery Operator

At the program level, AHS continues to administer Vermont's healthcare and social service programs — Medicaid (through DVHA), mental health (through DMH), Blueprint for Health, developmental services, aging services, and others. The structural change is not the elimination of program-level operations but the addition of cross-program coordination mechanisms and the explicit alignment of program operations with the transformation goals and geographic structures established at the statewide and regional levels.

The critical program-level change is the integration of social service programs into the healthcare transformation framework. Oliver Wyman's three imperatives — build housing and fix transportation, pay with RBP and move to global budgets, move care out of hospitals — cannot be achieved by the healthcare programs alone. DVHA, Blueprint, and the Health Care Reform Office can transform payment and care delivery; they cannot build housing or run transportation programs. But the AHS programs that fund housing supports (LIHEAP, general assistance), transportation (Medicaid transportation, EMS), food assistance (SNAP administration), and developmental services are sitting in the same agency — they are simply not coordinated with the healthcare transformation agenda.

The specific structural mechanism for this integration is the HSA Coordinator role: a designated AHS staff member for each HSA region who convenes the relevant program staff from DVHA, DMH, Blueprint, ADS, DAIL, and the Health Care Reform Office around the hospital transformation plan for their region. The HSA Coordinator does not control all program resources — that would require a reorganization far beyond what Act 68 mandates — but they create the coordination layer that currently does not exist.

The Six-Pillar Framework Applied to AHS's Restructuring Mandate

The six-pillar framework — Policy, Economics, Operations, Clinical, Equity, and Technology — provides the analytical architecture for AHS's Statewide Strategic Plan. Each pillar corresponds to a domain of AHS's transformation mandate, a set of organizational capabilities AHS must develop, and a set of measurable outcomes AHS must track. The following section maps each pillar explicitly onto AHS's restructuring requirements.

Pillar 1 Policy AHS function: Legislative compliance, federal relationship management, regulatory strategy

AHS's policy pillar function is to ensure that Vermont's transformation agenda is legally grounded, federally funded, and legislatively supported. The specific organizational requirements are:

- AHEAD Model management: Active management of Vermont's AHEAD State Agreement, including ongoing CMS negotiation on global budget methodology, monitoring compliance with TCOC targets, and managing the conditions under which Vermont can withdraw if the model is not working. This requires dedicated staff with both healthcare finance expertise and federal program management experience.

  * Act 68 compliance tracking: Systematic monitoring of Act 68's statutory requirements — RBP by FY2027, global budgets for non-CAH hospitals by FY2028, Statewide Strategic Plan by December 2028 — and proactive management of the legislative reporting calendar.

  * RHT Program administration: Oversight of Vermont's $195M five-year award, including grant management, contractor oversight, annual reporting to CMS, and alignment of RHT activities with Act 167 and Act 68 goals. This is a major federal grant program requiring professional grant management infrastructure.

  * Federal policy monitoring: Tracking changes to Medicare, Medicaid, and federal healthcare policy that affect Vermont's transformation — including Medicaid reconciliation legislation, CMMI model changes, and CMS regulatory guidance — and updating Vermont's strategy in response.

Pillar 2 Economics AHS function: Payment reform sequencing, financial sustainability monitoring, ROI analysis

AHS's economics pillar function is to manage Vermont's payment reform transition — coordinating with GMCB on RBP and global budget design, monitoring hospital financial sustainability, and ensuring that payment reform produces the premium reduction and affordability improvement that Act 68 requires.

- Payment reform coordination: Regular coordination with GMCB on RBP methodology development, global budget design, and the premium passthrough monitoring that Act 68 requires. AHS brings the Medicaid perspective and the population health data; GMCB brings the regulatory authority. Neither can execute the economics transformation without the other.

  * Hospital financial monitoring: Tracking Vermont hospitals' financial positions against the Oliver Wyman projections — particularly the CAH operating profit gap ($938 vs. $2,784 benchmark) and the PPS administrative cost premium ($2,730 vs. $1,427 benchmark) — and intervening with technical assistance or grant support before hospitals reach crisis.

  * AHEAD EAST Fund management: Directing the up to $150M annually in additional Medicare funds toward the primary care, behavioral health, home health, and long-term care investments that make global budgets produce population health improvement rather than just cost reduction.

  * Transformation ROI tracking: Measuring whether Vermont's transformation investments — RHT Program, Act 68 grants, AHEAD EAST Fund — are producing the $400M+ in savings that Oliver Wyman projected. This requires the analytics capability described in the organizational gap analysis.

Pillar 3 Technology AHS function: Data infrastructure, HIE governance, analytics platform, EHR strategy

AHS's technology pillar function is to ensure Vermont has the data infrastructure needed to manage a transformed healthcare system. The current data environment — VHCURES as the all-payer claims database, VITL as the health information exchange, the Blueprint clinical data registry, and fragmented program-level data systems — was adequate for monitoring discrete programs. It is not adequate for managing population health at the system level.

- VHCURES expansion and real-time access: AHS and GMCB must expand VHCURES to include more timely claims data, enhance provider connectivity (currently voluntary and incomplete), and make VHCURES data accessible for transformation planning analytics. The November 2025 AHS report acknowledged that most available measures reflect 2023 or earlier data — a two-year lag that makes real-time transformation management impossible.

* HIE strategic development: The Act 167-mandated HIE feasibility assessment — currently evaluating the cost-benefit of a statewide electronic health record — is a technology pillar decision with major implications for the Strategic Plan. A statewide EHR would resolve the fragmented data environment that Oliver Wyman cited as a barrier to coordinated care; the question is whether the investment is justified and whether the governance model is viable.

* Analytics vendor implementation: The joint AHS-GMCB analytics vendor procurement underway must produce a platform capable of modeling transformation scenarios, evaluating hospital proposals, and generating the outcome metrics the Strategic Plan requires. This is the most urgent near-term technology investment.

* Cross-program data integration: AHS's SDOH programs (housing, transportation, food assistance) must be connected to healthcare outcome data so that the relationship between social service investment and healthcare utilization can be measured. Without this integration, AHS cannot demonstrate to the legislature that its SDOH investments are producing healthcare savings.

Pillar 4 Clinical AHS function: Primary care expansion, behavioral health integration, care model design

AHS's clinical pillar function is to design and support the care delivery models that Vermont's transformed payment system requires. Payment reform creates the incentive for clinical redesign; AHS must create the programmatic infrastructure that makes clinical redesign possible.

- Blueprint for Health expansion and AHEAD transition: Managing the transition of Blueprint's Medicare payments from the current demonstration to Primary Care AHEAD, expanding PCMH enrollment, and ensuring that the MHI pilot's behavioral health integration is made permanent through AHEAD EAST Fund investment.

  * CCBHC network development: Executing the plan to certify five additional CCBHCs by July 2026, covering all major HSA regions, and integrating the CCBHC network with primary care, hospital emergency departments, and the 988 crisis system.

  * Care model technical assistance: Supporting hospitals and primary care practices in redesigning their care models for the global budget environment — shifting from volume maximization to population health management, building care coordination capacity, and implementing the team-based care models that primary care transformation requires.

  * Long-term care capacity development: Addressing the PACE gap, supporting dementia care COE development, and working with DVHA to expand HCBS capacity so that Vermont's growing elderly population can age in their communities rather than consuming hospital capacity.

Pillar 5 Equity AHS function: Disparity elimination, geographic access, SDOH programs, culturally competent care

AHS's equity pillar function is to ensure that Vermont's transformation reduces rather than exacerbates health disparities — particularly the geographic disparities between Burlington's Chittenden County and the Northeast Kingdom, and the access disparities affecting Vermont's Medicaid population, rural residents, and linguistic minorities.

- Geographic equity monitoring: Tracking health outcomes, primary care access, and insurance coverage rates by county — with specific attention to the Northeast Kingdom (6-8% uninsurance vs. 3% statewide) and rural counties with higher rates of behavioral health crisis presentations, opioid overdose, and diabetes.

  * Medicaid access protection: Monitoring whether RBP and global budget implementation affects Medicaid patients' access to care — specifically whether the reduction in commercial prices affects hospitals' willingness to maintain Medicaid patient volumes. Act 68 explicitly requires consideration of access protection in the RBP design.

  * Culturally competent care investment: Addressing the equity gaps Oliver Wyman documented — lack of language access, cultural mismatch in clinical care, inadequate LGBTQ+ competency, weight bias — through workforce investment (provider diversity recruitment), training requirements, and community partnership programs.

  * SDOH program coordination: Ensuring that AHS's SDOH programs — housing supports, transportation, food assistance — are explicitly directed toward the populations and communities where SDOH gaps are most driving healthcare utilization and health disparities.

Pillar 6 Operations AHS function: Transformation plan management, RHRC successor, administrative simplification

AHS's operations pillar function is to manage the execution of Vermont's transformation — the 14 hospital transformation plans, the regional coordination processes, the technical assistance programs, and the administrative simplification agenda that Oliver Wyman identified as essential for reducing provider burden.

- Hospital transformation plan oversight: Managing the pipeline from Draft Care Transformation Strategy Outlines through Draft Plans to Final Plans for all 14 hospitals, ensuring plans are coordinated regionally and nationally, and tracking implementation against Act 167 outcome objectives.

  * Administrative simplification: Acting on Oliver Wyman's recommendation to further simplify prior authorization, the CON process, and Medicaid billing — reducing the administrative burden that drives physician frustration and consumes provider resources that should go to patient care.

  * Project Management Office: Establishing the PMO infrastructure needed to manage Vermont's transformation portfolio — RHT Program, AHEAD implementation, hospital transformation plans, Strategic Plan development — with professional project management, risk tracking, and escalation processes.

  * EMS and transportation transformation: Coordinating the EMS professionalization and regionalization that Oliver Wyman prioritized — addressing the 31-agency fragmentation that makes inter-facility transfers complex and expensive, and building the transportation infrastructure that population health management requires.

The AHS Restructuring Implementation Timeline

The AHS restructuring is not a future event — it is happening now, driven by statutory deadlines that are already creating organizational pressure. The following timeline organizes the key milestones that AHS must meet, grouped by the organizational capability they require.

Deadline	Milestone	Primary owner	Status
FY2026 (now)	Hospital transformation grant awards; all 14 hospitals engaged in transformation planning	AHS Health Care Reform Office	In progress — all eligible hospitals submitted applications
FY2026 (now)	Analytics vendor procurement (joint AHS-GMCB)	AHS \+ GMCB	Procurement underway as of November 2025
FY2026 (now)	CCBHC certification for 5 new entities (July 2026 target)	AHS Department of Mental Health	On track per RHT application plan
FY2026 (now)	AHEAD Medicaid global budget operational (January 2026\)	DVHA / CMS	Operational — Medicaid global budget in effect
FY2026 (now)	RHT Program implementation planning and staff hiring	AHS Health Care Reform Office	$195M award received December 2025; implementation planning underway
FY2026 Q2-Q4	Final hospital transformation plans (early 2026\)	AHS \+ 14 hospitals \+ RHRC successor	Plans expected early 2026; AHS integration work follows

Deadline	Milestone	Primary owner	Status
FY2026 Q2-Q4	HSA Coordinator roles established	AHS Secretary	Organizational change requiring Secretary-level decision
FY2026 Q2-Q4	Division of Planning and Effectiveness established	AHS Secretary \+ GMCB Chair	Recommended by OW; critical for Strategic Plan development
FY2027 (Oct 2026\)	RBP maximum rates in effect for commercial payers	GMCB (AHS coordination)	Statutory deadline; GMCB leads; AHS coordinates on Medicaid alignment
FY2027 (2027)	AHEAD Model Cohort 2 begins — Medicare FFS global budgets	AHS \+ DVHA \+ GMCB \+ CMS	January 1, 2027; nine-year model; requires ongoing federal coordination
FY2027 (2027)	Primary Care AHEAD operational — enhanced primary care payments begin	DVHA \+ Blueprint for Health	Medicare primary care payments through AHEAD replaces expiring demo
FY2028 (Oct 2027\)	Global hospital budgets for non-CAH hospitals	GMCB (AHS coordination)	Statutory deadline; requires completed RBP methodology and budget design
December 2028	Statewide Health Care Delivery Strategic Plan delivered to legislature	AHS Secretary \+ Health Care Delivery Advisory Committee	Statutory deadline; updated every three years thereafter
FY2030 (Oct 2029\)	Global hospital budgets for all Vermont hospitals including CAHs	GMCB (AHS coordination)	Statutory deadline; completes all-hospital global budget coverage

Figure 16.4 — AHS restructuring and Vermont transformation implementation timeline. Sources: Act 68 of 2025; AHEAD State Agreement (January 2025); AHS Transformation Reports; Vermont RHT Program Application.

A Framework for the 2028 Statewide Health Care Delivery Strategic Plan

The Statewide Health Care Delivery Strategic Plan is the most consequential document AHS will produce in the 2020s. It will define Vermont's healthcare system for the decade following its delivery. The following framework provides the structural architecture for the plan, organized around the six pillars and Act 68's statutory requirements.

Proposed Plan Structure

Section	Content	Length/depth
Executive Summary	Vermont's healthcare crisis in 2028: what changed since 2022; what the system looks like today; the five Act 167 goals and progress toward each; the plan's central commitments for 2028-2031	5-8 pages; accessible to general public and legislators
Section 1: System Context and Diagnosis	Updated version of the Oliver Wyman analysis: Vermont's demographic trajectory, hospital financial positions, payer landscape, workforce status, and SDOH burden as of 2028 baseline. What has improved; what remains urgent.	15-20 pages; includes data tables and visualizations
Section 2: Payment Reform Architecture (Economics Pillar)	Status of RBP implementation; global budget parameters for each hospital; AHEAD TCOC targets and performance to date; total cost of care trajectory 2028-2031; affordability benchmarks and progress	20-25 pages; includes hospital-by-hospital financial data
Section 3: Primary Care and Behavioral Health (Clinical Pillar)	Blueprint enrollment and performance; CCBHC network coverage; CoCM deployment status; Primary Care AHEAD performance; behavioral health access indicators and improvement targets 2028-2031	15-20 pages
Section 4: Hospital Network and Regionalization (Operations Pillar)	Confirmed tier designations for all 14 hospitals; COE network status; transfer infrastructure; shared services implementation; REH/CACC conversions completed or planned; workforce distribution	20-25 pages; includes hospital network map
Section 5: Data and Technology Infrastructure (Technology Pillar)	VHCURES enhancement status; HIE connectivity; statewide EHR decision; analytics platform capabilities; population health data architecture; interoperability with federal systems	10-15 pages
Section 6: Health Equity Strategy (Equity Pillar)	Geographic equity indicators by HSA; disparity measures by income and demographics; SDOH program coordination framework; culturally competent care investments; equity targets 2028-2031	15-20 pages
Section 7: Workforce Strategy	Current workforce supply and demand by profession and geography; RHT Program workforce investments and outcomes; scope-of-practice changes; pipeline programs; 2030 workforce projections	15-20 pages
Section 8: AHS Organizational Architecture (Policy and Operations Pillars)	AHS organizational structure as redesigned; HSA Coordinator model; Division of Planning and Effectiveness; cross-program SDOH integration; governance relationships with GMCB, DVHA, DMH, DFR	10-15 pages
Section 9: Accountability Framework	Complete metrics dashboard (all 17+ Act 167/Act 68 outcome measures); data sources and update frequencies; performance targets for 2028-2031; reporting calendar; conditions for plan revision	10-15 pages; appendix with full metrics table
Section 10: Financial Plan	Capital allocation plan for RHT Program (remaining 3-year balance); AHEAD EAST Fund allocation; Act 68 grant commitments; projected system savings from transformation; state budget implications	15-20 pages

Figure 16.5 — Proposed structure for Vermont's 2028 Statewide Health Care Delivery Strategic Plan. Sources: Act 68 of 2025; Health Care Delivery Advisory Committee mandate; six-pillar framework.

The Central Commitment the Plan Must Make

Beyond its analytical content, the Strategic Plan must make one central commitment that is currently absent from Vermont's transformation discourse: a specific, quantified trajectory for what Vermont's healthcare system will look like in 2031, 2035, and 2040, with the financial modeling to show it is achievable.

Oliver Wyman provided the diagnostic scenario: without transformation, 13 of 14 hospitals report losses by 2028, premiums reach unsustainable levels, and the healthcare system contracts through unplanned closures rather than orderly redesign. The Strategic Plan must provide the alternative scenario with equal specificity: if Vermont executes the transformation agenda on the timelines described in this chapter, what does the system look like in 2031? How many hospitals maintain inpatient beds? What is the primary care investment level? What is the premium growth rate? What is the uninsurance rate in the Northeast Kingdom?

This is the difference between a strategic plan and a strategic aspiration. Vermont's legislators, hospital leaders, community organizations, and Vermonters deserve to know not just what AHS is doing but what the state will look like if AHS succeeds. The six-pillar framework provides the analytical architecture for making that commitment with intellectual honesty about what is achievable and what remains uncertain.

Statutory deadline: Statewide Strategic Plan delivered to legislature Dec 2028 Act 68 of 2025 — updated every three years thereafter	**Health Care Delivery Advisory Committee: members 18 AHS, GMCB, HCA, hospitals, health centers, insurers, professions, small business, LTC**
RHT Program capital available over 5 years for transformation $195M *Vermont's December 2025 award; aligned with Act 167 and Act 68 goals*	**Oliver Wyman projected direct savings from transformation (5 years) \>$400M** *Independent of AHEAD and RBP payment reform savings*

Figure 16.6 — Key parameters for AHS's Strategic Plan development. Sources: Act 68; VTDigger; Oliver Wyman Act 167 Report; Vermont RHT Program Application.

Vermont as National Template — Why This Restructuring Matters Beyond Vermont

Vermont's AHS restructuring is not a local administrative exercise. It is the first attempt by a state agency to systematically redesign itself as a population health system operator in response to a statutory mandate for healthcare transformation. Every state that faces Vermont's demographic and fiscal trajectory — which is to say, every state in the Northeast, most of the Midwest, and eventually every state — will need to answer the same organizational design questions Vermont is answering now.

The questions are not Vermont-specific. How do you align a state human services agency around geographic service areas rather than program silos? How do you build the analytics capacity to model the impact of structural change before you implement it? How do you integrate social service programs with healthcare delivery reform so that SDOH investments actually reduce healthcare utilization? How do you manage a multi-stakeholder strategic planning process that includes hospitals, insurers, providers, and community organizations without producing a lowest-common-denominator plan that satisfies everyone and commits to nothing?

Vermont will have answers to these questions by December 2028 — answers grounded in real experience with a real system facing real pressures. Those answers will be the most valuable policy contribution Vermont has made to American healthcare in a generation. The national significance of Vermont's transformation is not that Vermont is large or economically powerful. It is that Vermont is early — early enough to produce a tested model before the rest of the country reaches the same crisis point, and early enough for the lessons to still be actionable.

This is why the book you are reading exists: to document Vermont's experience in a form that is analytically rigorous, practically useful, and transferable to the healthcare leaders and policymakers who will face Vermont's challenges in their own states and systems. The AHS restructuring chapter is not the end of the story — it is the moment when the story becomes fully operational.

Key Concepts Introduced in This Chapter

Term	Definition
Population health system operator	The organizational identity AHS must adopt — managing Vermont's healthcare and social services ecosystem as an integrated system oriented around population health outcomes, organized geographically, and accountable for measurable progress toward the five Act 167 goals.
HSA Coordinator	A proposed AHS organizational role: a designated staff member for each Hospital Service Area who convenes the relevant AHS programs and community partners around the hospital's transformation plan, creating the cross-program coordination layer that currently does not exist.
Division of Planning and Effectiveness	A proposed new organizational unit within AHS (also recommended as an addition to GMCB in Oliver Wyman's report) to provide the analytical capacity for modeling transformation scenarios, evaluating hospital proposals, and monitoring outcome metrics.
Statewide Health Care Delivery Strategic Plan	The plan AHS is required by Act 68 to deliver to the Vermont legislature by December 1, 2028, and update every three years. The plan must address Vermont's healthcare delivery system across all dimensions: payment reform, primary care, hospital network, behavioral health, equity, workforce, and AHS organizational architecture.
Health Care Delivery Advisory Committee	The 18-member committee established by Act 68 to develop and maintain the Statewide Strategic Plan in collaboration with AHS. Has statutory authority to set affordability benchmarks and advise the legislature directly on healthcare delivery.
Program-silo structure	AHS's current organizational model, organized around funding streams and program types rather than geographic service areas or population segments. Oliver Wyman identified this as a primary barrier to coordinated transformation.
Cross-program SDOH integration	The organizational mechanism for connecting AHS's social service programs — housing, transportation, food assistance — with healthcare transformation goals, so that SDOH investments can be explicitly directed toward the populations and communities where SDOH gaps are most driving healthcare utilization.
Reform cascade	The legislative sequence — Act 167 (2022) → Act 51 (2023) → Act 68 (2025) — in which each intervention builds on the evidence and institutional capacity created by its predecessor. Vermont's AHS restructuring is the organizational expression of this legislative cascade.

Sources: Act 68 of 2025 (Vermont); Oliver Wyman Healthcare and Life Sciences Group, Act 167 Community Engagement: Recommendations (August 2024, revised October 2024); Vermont Agency of Human Services Health Care Delivery Advisory Committee page (healthcarereform.vermont.gov); AHS Health Care System Transformation Reports (August 2025, November 2025); Vermont Agency of Human Services / Brendan Krause, Vermont's Health Care Reform Efforts, House Health Care Committee (January 31, 2025); AHEAD State Agreement (January 17, 2025); Vermont Rural Health Transformation Program Application (November 3, 2025); GMCB Act 68 Update on RBP and Global Budgets (February 17, 2026); All prior chapters of this book.

Appendix A: Vermont System Portrait {#appendix-a:-vermont-system-portrait}

Demographics, Hospitals, Insurance, Workforce, and System in Motion

How to use this appendix ***This appendix consolidates the Vermont-specific factual foundation for the book's national argument. Chapter 2 embeds only the fiscal crisis data needed to understand why sequencing matters in Vermont; this appendix provides the complete system portrait that Chapter 2 and subsequent chapters draw on. References throughout the book to Vermont's demographics, hospital system, regulatory architecture, and workforce data point here. Each section is self-contained and can be read independently. Former Chapter 2 content (Vermont System Portrait) has been fully integrated here, supplemented by the January 2026 hospital transformation plan data from AHS tracking records and the GMCB regulatory architecture from ch2_additions.***

A.1 Vermont: The State

Vermont is the second-smallest state by population in the United States. Its 647,000 residents live across 14 counties and 255 towns and cities in a predominantly rural landscape — 9,616 square miles of mountains, valleys, farms, and forest. The population density is 70 people per square mile, well below the national average of 94. Nearly two-thirds of Vermont residents live in areas classified as rural by the federal definition used for healthcare program eligibility.

Vermont's geography is not merely a demographic fact. It is a healthcare infrastructure constraint. A patient in Essex County — one of the most rural and least populated counties in the Northeast Kingdom, with an uninsurance rate of 8% compared to Vermont's statewide 3% — may be 45 minutes from the nearest hospital emergency department, 90 minutes from a specialist, and hours from a tertiary care center. Public transportation is limited or unreliable in most rural counties. Broadband penetration reaches 80% of rural areas, leaving 20% without the connectivity that telehealth and remote patient monitoring require.

Vermont's economy is diverse but small. Healthcare is one of the most common industries in the state — employing approximately 14% of the workforce and representing a major economic anchor in rural communities where the hospital is often the largest employer. The University of Vermont Medical Center in Burlington, with approximately 50% of the state's hospital market share, is the economic center of gravity of Vermont's healthcare system. Every reform that affects UVMMC affects the Vermont economy in ways that hospitals in larger, more economically diverse states do not.

A.2 Vermont Demographics and Population Projections

Vermont's demographic trajectory is the foundational fact of its healthcare transformation challenge. Oliver Wyman's population projections, derived from MPR Vermont population data, tell the story with precision.

Indicator	Value	Significance
Vermont population aged 65+ by 2040	30%+	Up from 21.7% in 2020 — a 57% increase in the elderly population
Working-age population change by 2040	–13%	From 367,000 to approximately 318,000 — the commercial insurance base shrinks
Total Vermont population 2020–2040	Flat	Approximately 612,000–624,000 throughout — aging, not growing
Vermont residents with health insurance (2025)	97%	One of the highest coverage rates in the country — ACA and Medicaid expansion impact
Primary care provider shortage projected by 2030	370 FTE	112 in family medicine, 190 in other primary care specialties (RHT Program application)
Rental vacancy rate	3%	Below healthy 5% threshold — housing as healthcare workforce constraint

Figure A.1 — Vermont demographic transformation: key projections and their healthcare implications. *Source: Oliver Wyman Act 167 Report (2024); Vermont RHT Program Application; KFF.*

The implications of these demographic trends are not complicated — they are simply severe. An aging population means a shift in the payer mix toward Medicare and Medicaid, both of which reimburse hospitals below cost. A shrinking working-age population means a shrinking commercially insured population, reducing the cross-subsidy that currently sustains hospital operations. More complex care needs among the elderly means higher costs per patient. And an aging healthcare workforce — Vermont has the same age distribution among its clinicians as among its population generally — means accelerating retirements of experienced providers in exactly the years when demand is rising.

Oliver Wyman's analysis named this dynamic directly: the cross-subsidy model that has sustained Vermont's rural hospital system is becoming structurally impossible to maintain as demographics shift. The commercial premiums that make it possible for hospitals to accept below-cost Medicare and Medicaid payments depend on a sufficient pool of commercially insured patients. As that pool shrinks relative to the Medicare and Medicaid population, the math stops working — not in a distant future scenario, but on the trajectory Vermont is already on.

A.3 Vermont's Regulatory Architecture: The Green Mountain Care Board

Vermont's healthcare transformation would be impossible without an institutional actor that most other states do not have. The Green Mountain Care Board — a five-member independent state body created in 2011 — is the regulatory engine that converts Vermont's legislative mandates into binding operational obligations for its hospital system. Understanding the GMCB is prerequisite to understanding why Vermont can attempt what other states cannot.

Vermont's hospital budgets have been subject to state review since 1983, but GMCB's assumption of regulatory authority beginning in fiscal year 2013 qualitatively changed the nature of that oversight. The GMCB does not merely review hospital budgets — it establishes them. Under Vermont statute, each hospital must operate within the budget the Board sets. When hospitals exceed those budgets, the GMCB can enforce by reducing future commercial rate approvals. When hospitals face financial distress, the GMCB Chair meets directly with hospital leadership. When the Legislature mandates transformation planning under Acts 167 and 68, GMCB is the institutional receptor for the resulting analysis and the body responsible for translating policy architecture into hospital-level operational requirements.

The practical significance of this regulatory architecture extends beyond Vermont's 14 hospitals. GMCB's annual budget review process, conducted entirely in public meetings under Vermont's Open Meeting Law with mandatory participation from the Office of the Health Care Advocate, has produced a documentary record of Vermont's hospital system that is unmatched in American health policy. For FY25, GMCB approved $3.7 billion in system-wide hospital spending — a 4.1% increase after hospitals collectively requested 8%. The Board also enforced FY23 budget overages against UVMMC ($80.3M) and RRMC ($11.1M), reducing future commercial rate approvals as a consequence.

GMCB function	What it does and why it matters for transformation
Hospital budget review	Annually establishes Net Patient Revenue (NPR) growth limits for all 14 hospitals. For FY25, GMCB approved $3.7 billion in hospital spending — a 4.1% increase after hospitals requested 8%. This regulatory compression drives hospital attention to efficiency and alternative revenue models.
Budget enforcement	When hospitals exceed approved budgets, GMCB can enforce by reducing future commercial rate approvals. In FY23, UVMMC exceeded its budget by $80.3M and RRMC by $11.1M — GMCB enforced both. This enforcement authority distinguishes Vermont's regulatory model from purely advisory state oversight.
Monthly performance monitoring	All 14 hospitals submit monthly financial performance reports throughout the year. This continuous monitoring capability is the operational substrate for Vermont's transformation accountability.
Reference-based pricing (RBP)	Act 68 mandates RBP beginning FY2027, with GMCB setting the methodology. RBP will cap commercial hospital rates as a percentage of Medicare. GMCB's existing rate review infrastructure makes mandatory RBP implementable.
Global budget design	GMCB will set prospective global hospital budgets beginning FY2028–2030, in coordination with the AHEAD Model. The analytics vendor procurement (McKinsey, 2025–2026) is GMCB building the technical infrastructure for global budget methodology.
Hospital sustainability planning	Under Act 167, GMCB commissioned the Oliver Wyman analysis and is the institutional receptor for its recommendations. GMCB's public meeting record is the documentary foundation for Vermont's transformation case.

Figure A.2 — GMCB regulatory functions and their significance for Vermont's transformation agenda. *Source: GMCB Hospital Budget Review Guide; Acts 167 and 68; GMCB Annual Report 2024.*

GMCB's institutional design explains a specific feature of Vermont's transformation that is analytically important: mandatory authority precedes payment reform deployment. The voluntary ACO era produced modest results because the highest-cost providers could decline to participate. GMCB's mandatory budget authority — its ability to set rates rather than merely recommend them — is the structural condition that makes Vermont's mandatory reference-based pricing and global budgeting achievable. Other states attempting payment reform without a comparable regulatory body are, in the framework's terms, attempting Economics pillar transformation without the Policy pillar architecture that gives payment reform enforcement teeth.

A.4 Vermont's Hospital System

Vermont has 14 general acute care hospitals, all of which are non-profit. This fact — no for-profit hospitals — is significant for the reform agenda: Vermont's hospital system does not have the shareholder value obligations that complicate transformation discussions in states with investor-owned hospital chains. Vermont's hospitals are community assets, governed by community boards, with legal missions oriented toward community health. This creates better conditions for the kind of collaborative transformation Vermont is attempting — but it does not eliminate the financial pressures, institutional interests, or organizational inertia that make transformation difficult everywhere.

Of Vermont's 14 hospitals, the University of Vermont Health Network comprises UVMMC, CVMC, and Porter Hospital as system hospitals. Dartmouth Health affiliates include SVMC and Mt. Ascutney. The remaining hospitals — Gifford, Copley, NCH, NVRH, BMH, Springfield, RRMC, Grace Cottage, and NMC — operate as independent community institutions or loose collaborative members, and it is these independent hospitals that face the most acute financial fragility.

```
VERMONT HOSPITAL FINANCIAL POSITIONS — FY2023 vs. NATIONAL BENCHMARKS

  Operating Margin (%)                    Admin Cost per Adjusted Discharge
```

```
National benchmark:  +3.0%
Vermont average:    -2.1%

UVMMC:          +0.5% (loss adj)
RRMC:             -4.2%  ▼
BMH:              -6.1%  ▼▼
Springfield:      -8.9%  ▼▼▼
NMC:              -3.8%  ▼
NCH:              -2.9%  ▼

9 of 14 hospitals in loss FY2023
Projection: 13/14 by 2028
            (absent intervention)
```

```
National benchmark:    $1,427
UVMMC:                 $3,826  ▓▓▓
Vermont CAH avg:        $2,730  ▓
Vermont PPS avg:        $2,100  ▓

CAH Operating Profit/Adj Disch:
National benchmark:    $2,784
Vermont CAH average:     $938  ▼
Grace Cottage FY25:    ~$1,400
  (improving; was −$4M in FY24)

Vermont admin gap = $1,303/disch
System–wide ≈ $100M+ annual cost
```

```
FIVE–YEAR CUMULATIVE DEFICIT PROJECTIONS (Oliver Wyman, September 2024)

  Conservative scenario (3.5% revenue / 5% expense growth):
    $700M deficit vs. break–even · $1.4B vs. 3% operating margin
  Realistic scenario (7–8% expense growth):
    $2.4B deficit vs. break–even · $3.1B vs. 3% operating margin
  UVM Health Network alone (realistic): $1.5B cumulative deficit
  Per Vermont resident (realistic):  $3,700–$4,800 over five years
```

Figure A.3 — Vermont Hospital Financial Positions: FY2023 operating margins, administrative cost per adjusted discharge, and five-year cumulative deficit projections vs. national benchmarks. *Sources: AHS Health Care System Transformation Report (November 2025); Oliver Wyman Act 167 Report (September 2024); NASHP via CMS; GMCB hospital budget data.*

The distribution of financial distress across hospital categories matters for reform design. Critical Access Hospitals — the smallest, most rural hospitals serving the communities with the fewest alternatives — are the most financially distressed, with an average operating profit of $938 per adjusted discharge versus a $2,784 benchmark. They are also the hospitals where service reductions or closures would have the most severe community consequences, and the hospitals that the AHEAD Model's global budget design must protect most carefully. UVMMC's administrative cost per adjusted discharge was $3,826 against a national benchmark of $1,427 — structural cost problems that voluntary reform cannot address.

The Fourteen Hospitals: Transformation Status and Challenges (January 2026)

The January 2026 transformation plans submitted to AHS under Act 68 provide a uniquely granular view of where each hospital stands. These are operational documents produced in response to a statutory mandate, reviewed by AHS staff, and directly tied to Rural Health Transformation Program grant funding. The following table characterizes all 14 hospitals by type, transformation plan grant status, primary focus, and key challenges as documented in the January 2026 submissions.

Hospital	Type	Grant	Primary transformation focus	Key challenges / concerns
UVM Medical Center (UVMMC)	Academic hub	Not eligible	Technology solutions for acuity management and transfers; statewide reference hub	Administrative cost $3,826/adj. discharge vs. $1,427 benchmark; FY23 budget overage enforcement ($80.3M)
Central Vermont Medical Center (CVMC)	Regional PPS	Awarded	Technology solutions for acuity management and patient transfers	Part of UVMHN system; subject to system-level budget constraints
Porter Hospital	Regional PPS	Awarded	Technology solutions for acuity management and patient transfers (shared strategy with CVMC)	Part of UVMHN system; FY23 budget overage of $11M
Rutland Regional Medical Center (RRMC)	Regional PPS	Awarded	Hospice utilization improvement; dermatology in community; regional collaboration with SVMC	FY23 budget overage of $11.1M (partially enforced); exploring regional service lines with SVMC
SW Vermont Medical Center (SVMC)	Regional PPS	Awarded	RN Bridge to PCP program; $29M cancer center; DaVinci robot; Epic EHR migration ($30M); adolescent mental health unit	Locum provider costs; care coordination gaps; transport barriers for non-acute patients
Brattleboro Memorial Hospital (BMH)	Rural PPS	Awarded	Mobile Integrated Health (MIH) program; FQHC Look-Alike feasibility; birthing and oncology sustainability	GMCB FY26 deep concern about solvency; operating losses FY22/FY24/projected FY25; executive bonus scrutiny
Springfield Hospital	Rural PPS	Needs follow-up	Clinical services redesign feasibility study; relationship with Northstar Health	Oncology and cardiology sustainability concerns; plan not approved as of Jan 2026
Gifford Medical Center	CAH + FQHC	Awarded	Hospital-to-hospital transfer program with UVMMC/DHMC; NECHN GPO ($793K projected savings); shared radiology	Targeting avg. daily census of 20 for sustainability; tele-radiology vendor capacity constraints
Copley Hospital	CAH	Awarded	Consulting services; primary care access expansion	Plan quality concerns; GMCB required FY25 budget modifications; sustainability of changes under discussion
North Country Hospital (NCH)	CAH	Awarded	Regionalization planning with NVRH; shared project manager and legal support; pulmonology; tele-neonatology	Limited IT staff; dependent on Vermont CIN shared services model for analytics capability
Northeastern VT Regional Hospital (NVRH)	CAH	Awarded	Regionalization planning with NCH; cardiology; tele-ICU	FY23 budget overage (not enforced); GMCB monitoring; limited workforce pipeline in Northeast Kingdom
Mt. Ascutney Hospital (MAHHC)	CAH + FQHC	Awarded	Age-Friendly Program / geriatric COE; Collaborative Care Model via DHMC; shared CEO/CFO/COO with Valley Regional ($600K savings)	EMR transition (Oct 2025) disrupted data; subsidizing pharmacy ($50K/yr); residential care facility support ($950K in FY25)
Grace Cottage Hospital	CAH	Awarded	$24M primary care clinic capital campaign; same-day access clinic; swing bed program; AI ambient scribe (Coverys grant)	Denial reduction $76K→$1,274 (FY25 success); FY24 loss of $4M reduced to ~$1M in FY25; no DSH payments
Northwestern Medical Center (NMC)	CAH	Awarded	Innovative birthing services retention; shared cardiologist with Copley; oncology	Questions on value of marketing campaigns; schedule not yet confirmed as of Jan 2026

Figure A.4 — Vermont's 14 hospitals: typology, transformation grant status, primary focus, and key challenges. *Source: AHS Hospital Tracking data (January 2026); individual hospital transformation plans (January 15, 2026); GMCB FY26 Budget Orders.*

```
VERMONT'S 14–HOSPITAL NETWORK — THREE–TIER STRUCTURE (January 2026)

  TIER 1 — REGIONAL SPECIALTY CENTERS (High volume · full–spectrum COE capability)

  UVM Medical Center (Burlington)     16 COE designations
```

```
│   Cancer (complex) · Neurology · Women's health · Rehabilitation │
│   Most surgical specialties · System hub for transfers statewide │
│   Admin cost $3,826/adj disch (benchmark $1,427) — reform target │
│                                                                  │
│  SW Vermont Medical Center (Bennington)  9 COE designations      │
│   Cancer surgery · Geriatrics · Orthopedics · Psychiatry         │
│   Radiation therapy · Epic migration ($30M) underway FY2025      │
└──────────────────────────────────────────────────────────────────┘

TIER 2 — COMMUNITY HOSPITALS (Defined specialty COEs · regional collaboration)
┌──────────────────────────────────────────────────────────────────┐
│  Rutland Regional (RRMC)       5 COEs · exploring SVMC collab     │
│  Central VT Medical Center     5 COEs · UVMHN system member       │
│  Brattleboro Memorial (BMH)    6 COEs · GMCB deep solvency concern│
│  Northwestern Medical (NMC)    4 COEs · birthing services focus   │
│  NE VT Regional Hospital (NVRH) 4 COEs · NCH regionalization joint│
│  Porter Hospital               PPS · UVMHN system · acuity mgmt   │
│  Springfield Hospital          2 COEs · plan not approved Jan 2026│
│  Copley Hospital (Morrisville)  2 COEs · plan quality concerns    │
└──────────────────────────────────────────────────────────────────┘

TIER 3 — TRANSITION ASSESSMENT (CAH designation · repurposing evaluation)
┌──────────────────────────────────────────────────────────────────┐
│  Gifford Medical Center     CAH+FQHC · transfer protocol w/UVMMC  │
│  North Country Hospital     CAH · NCH–NVRH regionalization joint  │
│  Mt. Ascutney (MAHHC)       CAH+FQHC · shared C-suite w/Valley Reg│
│  Grace Cottage Hospital     CAH · 5-star quality · FY25 turnaround│
│                                                                  │
│  Repurposing pathways available:                                 │
│  • Rural Emergency Hospital (REH) — 24/7 emergency, no inpatient  │
│  • Community Ambulatory Care Center — outpatient + primary care   │
│  • Care at Home support program — community-based, no facility    │
└──────────────────────────────────────────────────────────────────┘

Vermont CIN (Clinically Integrated Network) provides shared analytics,
IT services, and population health infrastructure across all 14 hospitals —
the Technology-pillar solution to the Operations-pillar constraint that no
individual CAH can afford to build analytics capability independently.
```

Figure A.5 — Vermont's 14-Hospital Network: three-tier structure, COE designations, transformation grant status, and repurposing pathways. *Sources: Oliver Wyman Act 167 Report (September 2024); AHS Hospital Transformation Plans (January 2026); AHS November 2025 Transformation Report*.

Patterns in the Data

The administrative cost problem is concentrated at the top of the system. Oliver Wyman's finding that UVMMC's administrative cost per adjusted discharge was $3,826 against a national benchmark of $1,427 is not merely an efficiency observation. It is evidence that Vermont's largest and most financially powerful hospital has structural cost problems that voluntary reform cannot address. GMCB's enforcement of UVMMC's FY23 budget overage, and its decision to not approve NPR growth for BMH in FY26 while expressing deep concern about BMH's solvency, illustrate the regulatory tools available when transformation accountability is backed by statutory authority.

The regionalization logic is visible in the transformation plans but is not yet architecturally formed. NCH and NVRH are explicitly partnering to share a project manager, legal counsel, and subject matter expertise for regionalization planning. Gifford has built a functioning hospital-to-hospital transfer protocol with UVMMC and Dartmouth Health. Mt. Ascutney and Valley Regional Hospital have shared their CEO, CFO, and COO since late 2024, generating approximately $600,000 annually in administrative savings. SVMC and RRMC are exploring shared specialty and support service lines. These are the early formations of the regional hub structure that Acts 167 and 68 envision, but they are bottom-up and incomplete rather than top-down and comprehensive.

The analytics gap is an operational reality, not a theoretical concern. The AHS Hospital Transformation Analytical Support RFP, issued in late 2025 and awarded to McKinsey, acknowledged that Vermont does not yet have the analytics infrastructure to manage global budgets, measure population health outcomes at the hospital level, or track patient migration across its 14-institution system. The scope of that procurement — VHCURES integration, facility risk profiling, patient migration modeling, scenario modeling — reveals both the ambition of Vermont's transformation and the distance between current analytics capability and what AHEAD's launch in January 2027 will require.

A.5 The Transformation Plans: Vermont's System in Motion (January 2026)

The 148 individual transformation initiatives documented across Vermont's hospital plans in January 2026 are not policy proposals. They are operational commitments with named responsible parties, defined timelines, cost estimates, and measurement frameworks — produced under statutory obligation and reviewed by AHS staff who rate their quality and track their progress. This is the Operations pillar working in real time.

Initiative theme	Initiative count	Representative examples from hospital plans
Clinical service redesign	~55	SVMC RN Bridge to PCP; BMH MIH formalization; MAHHC Age-Friendly Program; Grace Cottage same-day access clinic; RRMC hospice utilization improvement
Regionalization / transfers	~20	NCH-NVRH shared regionalization planning; Gifford hospital-to-hospital transfers with UVMMC/DHMC; MAHHC shared senior leadership with Valley Regional; SVMC-RRMC regional service lines (allergy, ENT, pain, ERCP)
Workforce development	~25	MAHHC Nursing Pathways Program ($190K for two cohorts); SVMC 'grow our own' RN pipeline; Grace Cottage near-elimination of travel nurses; Mt. Ascutney SANE forensic capacity building
Technology and data	~20	SVMC Epic migration ($30M); CVMC/Porter acuity management technology; Grace Cottage EMR interoperability interfaces; Grace Cottage ambient AI scribe (Coverys grant)
Shared purchasing / operational efficiency	~15	NECHN GPO launch (Gifford projected $793K savings); captive insurance pool across NE Collaborative members; MAHHC-Valley Regional shared C-suite ($600K annual savings)
Value-based care / payment alignment	~13	SVMC Belong Health Medicare Shared Savings ACO (2026); SVMC Medicaid global budget participation; Grace Cottage ACO participation and VATICA integration; Gifford FQHC model expansion

Figure A.6 — Vermont hospital transformation initiative themes, January 2026. Based on 148 initiatives from AHS Transformation Plans Analysis. *Source: AHS Transformation_Plans_Analysis.xlsx (January 2026).*

Three case studies in transformation from primary sources ***Grace Cottage Hospital reduced insurance denials from $76,160 in FY2024 to $1,274 in FY2025 — a 98% reduction — through concurrent review for acute admissions, documentation improvement, and provider education. Its financial trajectory — from a $4 million pre-audit operating loss in FY24 to approximately $1 million in FY25 to projected break-even in FY26 — with no DSH payments and a 5-star National Rural Health Star Rating, makes it the book's most important counterexample to the narrative that Vermont's small CAHs are uniformly failing. Mt. Ascutney Hospital's Community Health Team data — 1,132 individuals served over ten months in 2025, 80% over age 64, 22% requiring six or more contacts — is the most concrete available illustration of Vermont's demographic aging problem at the clinical level. The CHT is not serving a theoretical future population; it is already serving a population whose overwhelming predominance is geriatric, complex, and high-utilizing. Mt. Ascutney's shared senior leadership arrangement with Valley Regional Hospital — shared CEO since December 2024, shared CFO since November 2025, shared COO since January 2026 — generating $600,000 annually in administrative savings, while simultaneously subsidizing Windsor's only pharmacy ($50K/yr) after the town's private pharmacy closed in 2025. This is the equity problem that rural hospital transformation must address: a small CAH bearing community infrastructure costs that its operating revenue cannot sustainably support.***

What the Plans Reveal About the System's Constraints

Interoperability remains a solved problem only on paper. Grace Cottage's plan documents the reality with precision: working with multiple regional partners who operate on different electronic medical records, "the process from order to completion is arduous and full of administrative waste" — relying on "antiquated and insecure technologies" because secure interoperability infrastructure does not yet connect Vermont's system. Vermont's HIE, VITL, exists but is not functionally integrated with the workflows of most community hospitals. This is not a technology problem awaiting a solution — the solutions exist. It is an incentive and governance problem: no single hospital can justify the cost of building interoperability infrastructure alone, but the collective benefit to the system is substantial. This is a textbook Technology-Operations dependency failure.

Community pharmacy access has emerged as a new access gap. Mt. Ascutney is subsidizing Windsor's only pharmacy after the town's private pharmacy closed in 2025. Grace Cottage's pharmacy fills over 6,000 scripts monthly as a community service, collecting patients from Brattleboro who have lost local pharmacy access. When a hospital becomes a community's pharmacy, its cost structure reflects that community function in ways that standard efficiency benchmarks do not capture.

The evidence that voluntary participation cannot achieve structural change runs through the plans like a thread. BMH's MIH program explicitly identifies sustainable funding as its primary barrier, noting that standard billing cannot cover the cost and requesting that AHS provide reimbursement through RHTP funding. NVRH and NCH's regionalization work explicitly identifies legal counsel and project management as shared costs they cannot individually afford. These are not capability failures — they are financing failures. The hospitals know what needs to be done. The current financial architecture makes it impossible to fund. This is precisely the condition that Acts 167 and 68 are designed to address.

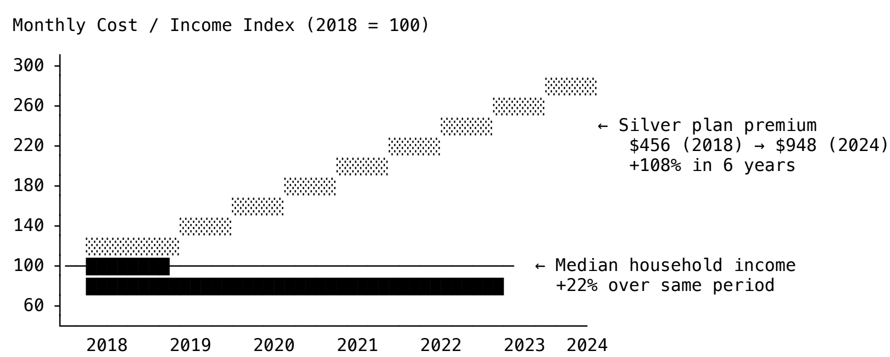
A.6 The Insurance and Premium Crisis

Vermont's health insurance market is dominated by Blue Cross Blue Shield of Vermont and MVP Health Care. Vermont was an early adopter of the ACA, expanding Medicaid in 2014 and establishing Vermont Health Connect, the state's insurance exchange. The result is a 97% coverage rate — one of the highest in the country — that is a genuine policy achievement.

The achievement of near-universal coverage has not, however, resolved Vermont's affordability crisis. Coverage is not the same as affordability. Vermont's individual market monthly premium rose from $456 in 2018 to $948 in 2024 — a 108% increase — while median household income grew 22%. Out-of-pocket maximums more than doubled. The insured Vermonter who delays necessary care because of the cost of their deductible or co-pay is not a healthcare access success story; they are an indictment of a system that has achieved coverage on paper while allowing costs to rise beyond reach.

The mechanism connecting hospital prices to insurance premiums is not complicated. Vermont hospitals charge commercial insurers 250–300% of Medicare rates on average for hospital services. In the Burlington area, UVMMC charged approximately 417% of Medicare for outpatient services in 2022. These prices — which bear no relationship to quality — translate directly into premiums. When a hospital charges 417% of Medicare for outpatient imaging, every insurer that covers that hospital's patients must charge premiums high enough to pay those prices and still maintain solvency. The result is the premium trajectory Vermont has experienced: driven primarily by hospital price increases that have no regulatory brake. This is what reference-based pricing is designed to address, and why Act 68's mandate to implement RBP represents the most consequential health policy legislation Vermont has passed since Act 48 created the GMCB in 2011.

VERMONT PREMIUM vs. INCOME GROWTH — THE AFFORDABILITY GAP 2018–2024



KEY DATA POINTS:

```
Silver plan premium:   $456/mo (2018)  →  $948/mo (2024)
Premium increase:      +108% (6 years)
Median HH income growth: +22% (2018–2022)
Out-of-pocket maximums: more than doubled (5 years)
Hospital approved charges: +38% vs. income +22%
BCBS VT per-member medical cost: $481/mo (2020)→$812 (2024)
                        +69% in 4 years

DRIVER: UVMMC outpatient charges ≈ 417% of Medicare (2022)
Hospital prices have no regulatory brake under FFS.
Act 68 RBP (FY2027) is the direct policy response.
```

Figure A.7 — Vermont premium vs. income growth 2018–2024: 108% premium increase vs. 22% income growth — the affordability gap that is the primary economic driver of Vermont's transformation mandate. *Sources: AHS January 2025 Legislative Presentation; GMCB rate filings; BCBS Vermont rate data; Oliver Wyman Act 167 Report (September 2024).*

A.7 Vermont Healthcare Workforce

Vermont's healthcare workforce faces a triple constraint that no single intervention can resolve: insufficient supply of primary care physicians, a specialist shortage that cannot be addressed by adding more specialists to each community, and a housing and economic environment that makes recruiting and retaining clinical staff increasingly difficult.

The primary care shortage is the most immediately actionable. Vermont's RHT Program application projects a shortage of 370 full-time equivalent primary care providers by 2030 — 112 in family medicine and 190 in other primary care specialties. Oliver Wyman's analysis offered a counterintuitive but important qualification: HRSA recognizes no Health Profession Shortage Areas in Vermont. The problem is not purely one of physician headcount — it is partly one of primary care productivity and care model. If primary care providers were supported to see three patients per hour in team-based care settings where clinical staff work at the top of their licensure and administrative burden is minimized, Vermont would have adequate primary care supply for well into the future.

The specialist shortage requires a different solution: regionalization. Vermont cannot maintain cardiologists, neurologists, radiation oncologists, and orthopedic surgeons at every one of its 14 hospitals. The patient volume at smaller community hospitals does not justify the positions, and the clinical volume needed to maintain specialist expertise cannot be generated at a hospital serving 30,000–50,000 people. Oliver Wyman's Center of Excellence framework — concentrating specialty services at facilities with sufficient volume — is the correct response. The key is ensuring that concentration does not mean inaccessibility: telehealth consultation, reliable EMS transport, and clear referral pathways are the operational complements to regionalization.

The housing constraint is the most intractable and the most important to name honestly. Vermont's rental vacancy rate of 3% is well below the 5% that characterizes a healthy rental market. Half of Vermont renters are cost-burdened. Vermont has the second-highest per-capita homelessness rate in the country. Healthcare providers recruited to rural Vermont communities face a housing market that may make it impossible to find an affordable place to live near the hospital where they would work. Oliver Wyman's first imperative — build housing and fix transportation — is a healthcare workforce policy as much as a social policy. The transformation plans submitted in January 2026 reflect this: SVMC's plans to reduce locum provider costs, Mt. Ascutney's Nursing Pathways Program funding two cohorts of nursing students at $190,000, and Grace Cottage's near-elimination of travel nurses through local retention strategies all reflect the same underlying constraint — rural Vermont cannot recruit and retain clinical staff at the volume and cost structure that fee-for-service hospital operations require.

A.8 Vermont Clinical Quality Indicators

Vermont's clinical quality performance reflects both the strengths of a high-coverage, primary care-oriented system and the equity gaps that demographic and geographic disadvantage produces. The Blueprint for Health, Vermont's statewide Patient-Centered Medical Home initiative, generated a 5.8 to 1 return on investment from its primary care investment — among the most robust ROI findings in the value-based care literature — and provides the clinical infrastructure on which the AHEAD Model's primary care redesign builds.

Vermont's 11-point primary care access gap between white and BIPOC residents, the Northeast Kingdom's elevated chronic disease burden, and the rural-urban disparities in behavioral health access are the equity targets that the transformation agenda must measure against. These indicators are tracked in GMCB's annual performance reports and provide the baseline against which AHEAD Model equity requirements will be assessed. See Chapter 10 (The Equity Pillar) for the full equity analytics framework.

Sources

- AHS Hospital Tracking data (January 2026)
- AHS Transformation_Plans_Analysis.xlsx (January 2026)
- Individual hospital transformation plans (January 15–31, 2026)
- GMCB Annual Report 2024
- GMCB FY25 and FY26 Budget Orders
- Oliver Wyman Act 167 Report (September 2024)
- AHS Health Care System Transformation Report (November 2025)
- Vermont RHT Program Application (2025)
- AHEAD State Agreement — Cohort 2 (2025)
- KFF State Health Facts — Vermont
- GMCB Act 68 RBP Update (February 2026)
- Mt. Ascutney Transformational Grant January 15 Report (2026)
- WCAX / Vermont Business Magazine — GMCB FY25 budget coverage
- RAND Hospital Transparency; GMCB FY2026 data

Appendix B: Act 68 Statutory Timeline and Milestones

Act 68 of 2025 (S.126) established a comprehensive sequence of healthcare transformation milestones. This appendix consolidates the complete timeline for reference.

Date / Deadline	Statutory requirement	Primary authority	Status (April 2026\)
January 1, 2026	AHEAD Medicaid global budgets begin (Cohort 2 cooperative agreement preparation; Medicaid alignment)	DVHA / CMS	Operational — Medicaid HGB in effect
FY2026 (ongoing)	Hospital budget orders implementing \-1% commercial rate benchmark	GMCB	In effect — $94.58M in FY2026 budget reductions ordered
FY2026 (ongoing)	$2M Act 68 transformation grants to hospitals (HIT fund)	AHS	Launched September 2025 — all eligible hospitals submitted applications
FY2026 (ongoing)	Act 55 drug pricing cap ($135M savings target)	GMCB / DFR	In effect — $104.3M in FY2026 drug cap savings
Early 2026	Final hospital Care Transformation Plans approved by AHS	AHS	In progress — hospitals in draft plan phase
May 2026	BALANCE Model GLP-1 Medicaid launch (voluntary state participation)	DVHA / CMS	DVHA participation decision pending
July 1, 2026	Five new CCBHC entities certified (RHT Program target)	AHS / DMH	On track per RHT application schedule
July 2026	Medicare GLP-1 Bridge demonstration launches ($50/month co-pay)	CMS	Federal program — Vermont Medicare beneficiaries eligible
FY2027 (October 1, 2026\)	Reference-based pricing (RBP) mandatory for commercial payers — maximum rates set by GMCB	GMCB	GMCB developing RBP methodology; vendor procurement underway
January 1, 2027	AHEAD Model Cohort 2 begins — Medicare FFS global budgets for Vermont	AHS / DVHA / GMCB / CMS	9-year model; primary care AHEAD and hospital global budgets launch
January 2027	BALANCE Model Medicare Part D coverage launches	CMS	Federal program
FY2028 (October 1, 2027\)	Global hospital budgets for non-CAH (PPS) Vermont hospitals	GMCB	Methodology under development; AHEAD alignment required
December 1, 2028	Statewide Health Care Delivery Strategic Plan delivered to Vermont Legislature	AHS \+ Health Care Delivery Advisory Committee	In planning — plan development begins 2026
FY2030 (October 1, 2029\)	Global hospital budgets for ALL Vermont hospitals including CAHs	GMCB	Final phase of global budget implementation

Date / Deadline	Statutory requirement	Primary authority	Status (April 2026\)
December 2031	BALANCE Model testing concludes	CMS	Federal program
October 1, 2032	All RHT Program funds must be spent	AHS / CMS	$195M award; spending window 2026-2032
Every 3 years from 2028	Statewide Strategic Plan updated	AHS \+ Advisory Committee	Ongoing statutory obligation

Source: Act 68 of 2025 (Vermont); AHEAD State Agreement (January 2025); Vermont RHT Program Application (November 2025); CMS BALANCE Model (December 2025).

Appendix C: The 14-Hospital Centers of Excellence Assignment Framework

Oliver Wyman's Act 167 Recommendations proposed a Centers of Excellence (COE) framework organizing Vermont's 14 hospitals by specialty service concentration. This appendix presents the proposed assignments.

Hospital (HSA)	COE count	Key COE specialties assigned	Tier designation	RHT rural status
University of Vermont Medical Center (Burlington)	16	All major specialties including cardiac surgery, neurosurgery, complex oncology, neonatal intensive care, transplant, trauma	Tier 1 — Primary RSC	Non-rural (Chittenden County)
Southwestern Vermont Medical Center (Bennington)	9	Acute general surgery, cardiology, orthopedics, psychiatry (adult and adolescent), rehabilitation, dermatology, endocrinology, GI, pain management	Tier 1 — Secondary RSC	Rural
Brattleboro Memorial Hospital (Brattleboro)	6	Acute general surgery, cardiology, obstetrics/high-risk OB, psychiatry (adult), oncology outpatient, GI	Tier 1 — Secondary RSC	Rural
Rutland Regional Medical Center (Rutland)	5	Neurology, psychiatry (adult), cancer surgery, radiation therapy, orthopedics	Tier 1 — Secondary RSC	Rural
Central Vermont Medical Center (Barre)	5	Radiation therapy, neurology, cardiac, general surgery, obstetrics	Tier 1 — Secondary RSC	Rural
Northwestern Medical Center (St. Albans)	4	Cancer surgery, radiation oncology, orthopedics, cardiology	Tier 2 — Focused scope	Rural
Northeastern Vermont Regional Hospital (St. Johnsbury)	4	Cancer surgery, infusion/chemotherapy, orthopedics, general surgery	Tier 2 — Focused scope	Rural
Springfield Hospital (Springfield)	2	Memory care/dementia, psychiatry (adult)	Tier 2 — Focused scope	Rural
Copley Hospital (Morrisville)	2	Orthopedics, rheumatology	Tier 2 — Focused scope	Rural
Mount Ascutney Hospital (White River Junction)	1	Rehabilitation	Tier 2 — Focused scope	Rural
Porter Medical Center (Middlebury)	TBD	Assessment ongoing — UVM network affiliation	Tier 2 — Focused scope	Rural
Grace Cottage Hospital (Townshend)	TBD	Palliative care specialty worth preserving; 19-bed Vermont's smallest hospital	Tier 3 — Transition assessment	Rural
Gifford Medical Center (Randolph)	TBD	Most financially vulnerable — comprehensive assessment needed	Tier 3 — Transition assessment	Rural
North Country Hospital (Newport)	TBD	Northeast Kingdom anchor; REH candidacy under assessment	Tier 3 — Transition assessment	Rural

Source: Oliver Wyman Act 167 Community Engagement: Recommendations (August 2024, revised October 2024 and January 2025). Note: COE designations are proposed frameworks from Oliver Wyman analysis; final designations subject to AHS and hospital transformation planning process.

Appendix D: AHEAD State Agreement — Key Terms and Provisions

Vermont's AHEAD Model State Agreement was signed January 17, 2025\. Vermont participates as a Cohort 2 state. This appendix summarizes the agreement's key terms for practitioner reference.

AHEAD provision	Vermont-specific terms
Cohort and start date	Cohort 2; model begins January 1, 2027; 9-year model through December 31, 2035
Total Cost of Care accountability	Vermont responsible for all-payer TCOC targets; Medicare FFS TCOC tracked annually; consequences for failure to meet targets after year 3
Hospital global budgets (Medicare FFS)	All Vermont hospitals receive prospective Medicare FFS global budgets beginning January 2027; methodology developed jointly with GMCB
Primary Care AHEAD (PC AHEAD)	Voluntary program for Vermont primary care practices; four payment pathway options including fully prospective models; replaces Blueprint Medicare PMPM payments
EAST Fund	Up to $150M per year in additional Medicare funds for Vermont to invest in primary care, behavioral health, home health, long-term care, and SDOH — the transformation capital fund
Behavioral health and substance use	EAST Fund explicitly directed toward MH and SUD services; CMMI-approved $1.2M in transformation grants from Cooperative Agreement Year 3 (2027)
Vermont GMCB regulatory independence	Agreement explicitly preserves GMCB's authority to set hospital rates; AHEAD does not supersede Vermont's state regulatory authority
Withdrawal rights	Vermont may withdraw before launch with 30-day notice; after launch with 6-month notice; unspent cooperative agreement funds must be returned to CMMI within 90 days of withdrawal
Cooperative agreement funds (pre-launch)	CMS provides cooperative agreement funds for pre-launch implementation activities; $1.2M approved for Cooperative Agreement Year 3 (2027)
Quality performance	Vermont must meet quality performance standards; quality measures aligned with Blueprint, HEDIS, and Act 167/68 reporting requirements

AHEAD provision	Vermont-specific terms
CMMI evaluation	CMS conducts independent evaluation of Vermont's AHEAD performance; results inform national AHEAD expansion decisions
Relationship to Act 68	AHEAD provides federal vehicle for implementing Vermont's statutory global budget mandate; Act 68 provides Vermont's independent statutory authority that does not depend on AHEAD continuation

Source: Vermont AHEAD State Agreement (January 17, 2025); CMMI AHEAD Model documentation; AHS January 2025 Legislative Presentation.

Appendix E: HTR Platform Tools and Methodology Overview

This appendix describes the Health Transformation Review platform tools referenced throughout the book. The platform is organized across four integrated surfaces: the Research Lab (analytical tools), The Wire (Intelligence Feed), the Academy (structured curriculum), and the AI Analyst (on-demand synthesis). Research Lab tools are available to Professional and Advisory subscribers at htrintelligence.com.

E.1 APM Shared Savings Calculator

Platform location: /research-lab/payment-models?tab=apm-calc

Models shared savings or loss potential under any APM contract. Inputs: benchmark PMPM, attributed member months, actual total cost of care, sharing rate, minimum savings rate, quality withhold percentage, and stop-loss threshold. Outputs: projected shared savings or loss under pessimistic/base/optimistic scenarios, sensitivity analysis by key variable, and break-even care management investment level.

Vermont presets pre-loaded for AHEAD global budget scenarios, MSSP Track 1, and ACO REACH Basic. Methodology follows the standard CMS shared savings calculation framework with Vermont-specific VHCURES benchmark data.

E.2 VBC Transformation Readiness Assessment

Platform location: /research-lab/payment-models?tab=vbc-readiness

A 30-dimension, 6-domain assessment scoring organizational readiness for APM entry on a 0–4 scale per dimension (maximum 120 points), expressed as an overall readiness percentage.

The six domains and their pillar alignment:

Domain	Pillar	Five dimensions assessed
Strategy & Leadership	Policy	Executive commitment, VBC roadmap, governance infrastructure, change management capability, policy monitoring system
Data & Analytics	Technology	Claims data access, patient attribution analytics, TCOC measurement, risk stratification, quality measure analytics
Clinical Operations	Clinical	Primary care transformation, care management program, specialist integration, post-acute management, behavioral health integration
Financial Readiness	Economics	Financial modeling capability, hospital financial sustainability, VBC revenue diversification, transformation investment capacity, shared savings distribution policy
Technology Infrastructure	Technology	EHR optimization, population health platform, FHIR interoperability, clinical AI governance, telehealth and RPM infrastructure
Health Equity	Equity	Stratified HEDIS measurement, SDOH screening, disparity analysis, equity-weighted program design, community partnerships

Scoring thresholds: 80%+ = Global Budget Ready; 60–79% = Advanced (12–18 months); 40–59% = In Progress (2–3 years); 20–39% = Early Stage; below 20% = Foundation Required.

Vermont presets: Vermont Hospital — AHEAD Entry (FY2027); Vermont CAH — Early Transformation; Integrated Health System — Advanced VBC. Available as self-assessment (Research Lab) or facilitated external assessment (Advisory).

E.3 Equity Analytics Framework (HEROI)

Platform location: /research-lab/equity

The Health Equity Studio calculates the HEROI composite score across five dimensions: Access Equity (25%), Quality Equity (25%), Outcome Equity (25%), SDOH Burden (15%), and Trust and Engagement (10%). Scores 0–100; 80+ = achieving equity across dimensions; 60–79 = material disparities in one or more dimensions; below 60 = pervasive disparities requiring comprehensive program redesign.

Integrates with VHCURES exports and Blueprint data for Vermont providers. Includes: disparity decomposition module separating observed disparities into root cause categories (geographic access, social needs, clinical variation, coverage barriers, trust barriers); intervention portfolio designer matching interventions to root causes; and Vermont county-level stratification using VHCURES HSA-level data with Northeast Kingdom focus.

E.4 Hospital Financial Stress Test Model

Platform location: /research-lab/policy-quality?tab=scorecard

Models hospital financial position under RBP, global budget, and Medicaid cut scenarios. Key inputs: current net patient revenue by payer, commercial price vs. Medicare ratio, operating cost structure, payer mix. Outputs: 10-year financial projections under three scenarios (status quo, partial transformation, full transformation) with sensitivity analysis.

Vermont presets for NVRH, Gifford Medical Center, and CVMC. Key Vermont scenarios: RBP at 200% vs. 250% of Medicare; global budget under Act 68 FY2028 mandate; H.R. 1 Medicaid revenue reductions 2027–2031; transformation investment ROI under AHEAD EAST Fund assumptions.

E.5 AI Clinical Governance Lab

Platform location: /research-lab/technology-ai?tab=ai

65-dimension governance evaluation across six clinical AI lifecycle stages: Pre-Deployment Evaluation, Implementation and Training, Active Monitoring, Adverse Event Response, Vendor Management, and Equity Monitoring. Includes FDA SaMD classification guidance, bias testing requirements by data type, override rate thresholds, vendor contract template provisions, and a structured equity audit protocol for each deployment. Vermont RHT-funded AI implementations (ambient scribes, acuity management tools, RPM) are primary use cases.

E.6 FHIR Interoperability Lab

Platform location: /research-lab/interoperability?tab=fhir

Step-by-step implementation guide for Vermont's three priority FHIR R4 use cases: Patient Access API (§170.315(g)(10) ONC compliance), Provider Access API for care coordination, and Payer-to-Provider API for prior authorization automation. EHR-specific configuration notes for Epic, Oracle Health/Cerner, Meditech Expanse, and TruBridge — the four EHR platforms across Vermont's 14 hospitals. Act 68 mandates VITL/VHIE connectivity; this lab is the implementation roadmap.

E.7 VBC Contract Review Checklist

Platform location: /research-lab/payment-models

65-item analysis framework in eight categories: Benchmark Methodology, Attribution, Quality Withhold, Risk Corridors and Stop-Loss, Carve-Outs, Reconciliation, Reporting Requirements, and Termination Provisions. Identifies the specific provisions that most commonly produce unexpected financial losses. Essential pre-work before signing any MSSP, ACO REACH, or AHEAD-aligned commercial contract.

E.8 Policy Impact Assessment Framework

Platform location: /research-lab/policy-quality

Structured evaluation for any new policy development across five steps: (1) Exposure mapping, (2) Financial impact estimation, (3) Scenario analysis, (4) Response strategy selection, and (5) Stakeholder communication plan. Pre-configured for Act 68 RBP methodology, AHEAD global budget design, H.R. 1 Medicaid changes, and CMMI model developments.

E.9 Risk Adjustment Accuracy Playbook (HCC Gap Closure)

Platform location: /research-lab

Methodology for retrospective HCC gap analysis. Covers: RAF comparison methodology vs. regional benchmark, chart review protocol for undocumented chronic conditions, HCC specificity coding education modules for the 20 highest-volume HCC categories, AWV completion program design, and ongoing gap closure monitoring. Vermont-specific: pre-loaded AHEAD Medicare attribution data and VHCURES-derived RAF benchmarks. Critical pre-work before AHEAD global budget baseline year — undercoded RAF scores produce benchmarks that eliminate shared savings opportunity.

E.10 HTR Intelligence Feed — The Wire

Platform location: /the-wire

Curated synthesis of consequential healthcare transformation developments organized around the six pillars. Coverage: Policy (federal rulemaking, CMMI developments, Act 68 and AHEAD milestones, H.R. 1 Medicaid changes), Economics (VBC market, hospital financial trends, payment model results), Technology (health IT, AI governance, FHIR compliance deadlines), Clinical (quality measurement, care model evidence, Blueprint updates), Equity (disparity data, SDOH policy, GLP-1 access), and Operations (workforce, EMS, administrative simplification). Vermont-specific commentary on every relevant development. Used by Vermont hospital CFOs, AHS staff, GMCB, and healthcare policy professionals.

E.11 HTR AI Analyst

Platform location: /chat (full interface) and right-sidebar widget (available throughout the platform)

On-demand framework intelligence grounded in the complete HTR knowledge base: Vermont primary source documents (Acts 167, 51, 55, 62, 68; AHEAD State Agreement; Oliver Wyman report; AHS transformation reports; GMCB budget data), CMMI model documentation, national VBC literature, and the analytical frameworks throughout this book. Supports instant synthesis, document retrieval, concept explanation, and scenario analysis on any six-pillar framework question.

For subscription information and platform access, visit htrintelligence.com.

BIBLIOGRAPHY AND SOURCE NOTES

Sources are organized by category and type. All URLs accessed between January and April 2026 unless otherwise noted. Vermont government documents are cited with their official legislative or agency identifiers. the framework analysis refers to original the analytical framework analytical work conducted in preparation of this book.

Bibliography and Source Notes

Vermont Primary Source Documents

Vermont Legislation

Act 167 of 2022 (Vermont). An Act Relating to Health Care Delivery System Transformation. Signed June 2022\. Established the Act 167 hospital diagnostic process and directed Oliver Wyman analysis. legislature.vermont.gov

Act 51 of 2023 (Vermont). Healthcare reform continuation. Directed hospital sustainability assessments and CON process review.

Act 49 of 2024 (Vermont). Relating to insurer solvency and the health insurance market. Green Mountain Care Board enforcement provisions.

Act 55 of 2025 (Vermont). Relating to prescription drug pricing caps and hospital 340B reporting requirements. Signed 2025\. $135M in annual drug savings target.

Act 62 of 2025 (Vermont). Relating to Health Information Technology Plan governance. Transferred HIE oversight from GMCB to DVHA effective July 1, 2025\.

Act 68 of 2025 (Vermont). An Act Relating to Health Care Payment and Delivery System Reform. Signed 2025\. Primary statutory vehicle for RBP, global budgets, and Statewide Strategic Plan. legislature.vermont.gov/Documents/2026/Docs/ACTS/ACT068/ACT068%20As%20Enacted.pdf

Vermont Government Reports and Documents

Agency of Human Services. Health Care System Transformation Report (August 1, 2025). Submitted to Health Reform Oversight Committee and Joint Fiscal Committee. Prepared by AHS Office of Health Care Reform, Sarah Rosenblum, Interim Director. legislature.vermont.gov

Agency of Human Services. Health Care System Transformation Report (November 1, 2025). Second quarterly report. Includes hospital-specific performance data from NASHP via CMS. legislature.vermont.gov

Agency of Human Services / Brendan Krause. Vermont's Health Care Reform Efforts. Presentation to Vermont House Health Care Committee, January 31, 2025\. Includes premium data, UVMMC operating data, AHEAD signing details.

Agency of Human Services. Vermont Rural Health Transformation (RHT) Program Application (November 3, 2025). Federal application to CMS for $195M award under H.R. 1\. ljfo.vermont.gov

Agency of Human Services. Vermont RHT Program Application Activity Details (December 2025). Describes 30 grant opportunities, 4 contractors, 5 key initiatives. healthcarereform.vermont.gov

Agency of Human Services. Health Care Delivery Advisory Committee mandate and membership. healthcarereform.vermont.gov/health-care-reform-delivery-advisory-committee

Agency of Human Services / Department of Vermont Health Access. AHEAD State Agreement (signed January 17, 2025). Vermont Cohort 2 participation in Achieving Healthcare Efficiency through Accountable Design Model. 9-year model beginning January 1, 2027\.

Green Mountain Care Board. Act 68 Update on Reference-Based Pricing and Global Budgets (February 17, 2026). Full report including hospital price data, 2024 claims analysis, phased implementation timeline. GMCB board meeting materials.

Green Mountain Care Board. 2024 Annual Report (January 15, 2025). Hospital budget reviews, insurance rate decisions, data and analytics overview.

Green Mountain Care Board. Vermont Health Care Uniform Reporting and Evaluation System (VHCURES) documentation. gmcboard.vermont.gov/data-and-analytics

Green Mountain Care Board. Health Information Technology Plan documentation, including Act 62 governance transfer notice. gmcboard.vermont.gov/hit

Department of Vermont Health Access. Vermont Health Information Exchange Strategic Plan (2024 Update). Submitted November 1, 2024\. healthdata.vermont.gov

Vermont Department of Health. Health Equity Data Annual Report to the Vermont Legislature (January 17, 2025). Prepared by Office of Health Equity. legislature.vermont.gov

Vermont Department of Mental Health. Certified Community-Based integrated Health Centers (CCBHC) Program. mentalhealth.vermont.gov/about-us/department-initiatives/ccbhc

Vermont Blueprint for Health. Annual Report 2024\. Submitted January 31, 2025\. legislature.vermont.gov

Vermont Blueprint for Health. Mental Health Integration (MHI) Program evaluation materials (2025). blueprintforhealth.vermont.gov

Vermont Legislature. Act 68 of 2025 Health Care Delivery Advisory Committee. healthcarereform.vermont.gov

Oliver Wyman Analysis

Oliver Wyman Healthcare and Life Sciences Group. Act 167 Community Engagement: Recommendations. August 2024\. Revised October 2024 and January 2025\. Primary source for three imperatives, hospital financial projections, COE framework, AHS restructuring recommendations, and community meeting findings. Commissioned by Vermont Agency of Human Services under Act 167 of 2022\. Available at legislature.vermont.gov

Federal Government Sources

Centers for Medicare and Medicaid Services

Centers for Medicare and Medicaid Services. AHEAD (Achieving Healthcare Efficiency through Accountable Design) Model documentation. innovation.cms.gov/innovation-models/ahead

Centers for Medicare and Medicaid Services. BALANCE (Better Approaches to Lifestyle and Nutrition for Comprehensive hEalth) Model announcement. December 23, 2025\. cms.gov/newsroom/press-releases

Centers for Medicare and Medicaid Services. Medicare GLP-1 Bridge demonstration. FAQs released March 2026\. $50/month co-pay for Medicare Part D beneficiaries July–December 2026\.

Centers for Medicare and Medicaid Services. Behavioral Health Integration Services. MLN909432. January 2026\. Includes CoCM billing codes CPT 99492, 99493, 99494\.

Centers for Medicare and Medicaid Services. Rural Health Transformation Program. Established under H.R. 1 (One Big Beautiful Bill Act, July 2025). $50B over 5 years. cms.gov

Centers for Medicare and Medicaid Services. Making Care Primary (MCP) Model documentation. 8-state initial cohort. innovation.cms.gov

Centers for Medicare and Medicaid Services. ACO REACH (Realizing Equity, Access, and Community Health) Model. innovation.cms.gov

Congressional Budget Office. Estimate of One Big Beautiful Bill Act (H.R. 1). July 2025\. $911B in Medicaid spending reductions over 10 years; 10 million additional uninsured by 2034\.

Other Federal Sources

Office of the National Coordinator for Health Information Technology (ONC). 21st Century Cures Final Rule. 2020\. FHIR R4 patient access and interoperability requirements.

Food and Drug Administration. Software as a Medical Device (SaMD) regulatory framework. fda.gov

SAMHSA. Promoting the Integration of Primary and Behavioral Health Care: Collaborative Care Model (PIPBHC-CoCM) program. samhsa.gov/grants

H.R. 1 (One Big Beautiful Bill Act). Signed by President Trump, July 4, 2025\. Medicaid spending reductions, work requirements, provider tax restrictions, RHT Program establishment.

National Research and Policy Organizations

KFF (Kaiser Family Foundation)

KFF. Medicaid Coverage of and Spending on GLP-1s. January 2026\. kff.org/medicaid/medicaid-coverage-of-and-spending-on-glp-1s

KFF. What to Know About the BALANCE Model for GLP-1s in Medicare and Medicaid. March 2026\.

KFF. How Might Federal Medicaid Cuts in the Enacted Reconciliation Package Affect Rural Areas? September 2025\.

KFF. A Closer Look at the $50 Billion Rural Health Fund in the New Reconciliation Law. September 25, 2025\.

Chartis

Chartis. 2026 State of Rural Health. February 10, 2026\. chartis.com/insights/2026-rural-health-state-state. Includes 417 vulnerable rural hospitals statistic; RHT Program analysis.

Other Policy and Research Organizations

Georgetown University Center for Children and Families. States are Beginning to Grapple with Federal Medicaid Cuts Impact on Rural Health Care. March 31, 2026\. ccf.georgetown.edu

Pew Charitable Trusts. New Federal Medicaid Policies Compound State Budget Pressures. January 14, 2026\. pew.org

Peterson-KFF Health System Tracker. Eight Trends Shaping 2026 Healthcare Costs. March 2026\. healthsystemtracker.org

National Academy for State Health Policy (NASHP). Hospital Cost Tool. Data used in Vermont AHS transformation reporting.

Onpoint Health Data. VHCURES data collection and analysis vendor for GMCB. onpointhealthdata.org

Health Affairs. Reimagining Rural Health Equity: Understanding Disparities and Orienting Policy, Practice, and Research in Rural America. June 2024\. doi: 10.1377/hlthaff.2024.00036

Population Health Management. Vermont's Community-Oriented All-Payer Medical Home Model Reduces Expenditures and Utilization While Delivering High-Quality Care. 2016\. Blueprint for Health ROI study — $5.8M reduction in medical expenditure per $1M invested.

JAMA Network Open. Quality improvement study on ambient scribe tools and documentation efficiency. 2024\.

Vermont Health Data Sources

Vermont Health Care Uniform Reporting and Evaluation System (VHCURES). Vermont's all-payer claims database, managed by GMCB. gmcboard.vermont.gov/data-and-analytics/data-collection/vhcures

Vermont Uniform Hospital Discharge Data System (VUHDDS). Hospital discharge database, all 14 Vermont hospitals. Managed by GMCB.

Vermont Health Information Exchange (VHIE). Operated by Vermont Information Technology Leaders (VITL). vitl.net

Vermont Behavioral Risk Factor Surveillance System (BRFSS). Annual survey data. Vermont Department of Health.

Vermont Department of Health. healthvermont.gov/stats/surveillance-reporting-topic/health-equity-data

Vermont Futures Project. Racial Diversity and Equity data compilation. vtfuturesproject.org

Selected Academic and Clinical Sources

Kotter, J.P. (1996). Leading Change. Harvard Business Review Press. Change management framework applied in Chapter 16\.

NCQA (National Committee for Quality Assurance). HEDIS Technical Specifications. Annual publication. Foundation for quality measurement framework in Chapters 13 and 10\.

HL7 International. FHIR R4 Specification. hl7.org/fhir/R4. Foundation for technology interoperability framework in Chapters 14 and 15\.

Archer J, Bower P, Gilbody S, et al. Collaborative care for depression and anxiety problems. Cochrane Library. October 2012\. Evidence base for CoCM model — 90+ RCTs.

RAND Corporation. Hospital Price Transparency Study. 2022 data. Vermont hospital prices vs. Medicare benchmark — UVMMC outpatient 417% of Medicare.

Maryland Health Services Cost Review Commission (HSCRC). Total Cost of Care Model performance data. $1.4B in Medicare savings; 7% hospital admission reduction. hscrc.state.md.us

National Consortium of Telehealth Resource Centers. AI-Enabled Without the Hype: Practical Guidance for Rural and Community Providers. April 2026\. telehealthresourcecenter.org

Note on Sources and Currency

This book was written between January and April 2026\. Vermont's healthcare transformation is ongoing, and specific data — hospital financial positions, GMCB regulatory actions, AHEAD implementation status, RHT Program grant awards — will continue to evolve after this writing. Readers should consult the HTR Intelligence Feed (relevant policy monitoring resources) for current developments. Vermont legislative documents are available at legislature.vermont.gov; GMCB publications at gmcboard.vermont.gov; AHS Health Care Reform materials at healthcarereform.vermont.gov.

All Vermont government documents cited in this book are publicly available. The Oliver Wyman Act 167 analysis is available at the Vermont Legislature's website. AHEAD State Agreement terms are available through CMS's public AHEAD Model documentation.

URLs were verified as active as of April 2026\. Vermont government websites are stable; federal CMS program pages may change with administrative transitions.

Index

Key terms, organizations, legislation, and concepts referenced in this book.

APPENDICES